



Complementary
Medicine

Ear Acupuncture

A Precise Pocket Atlas
Based on the Works of Nogier/Bahr

Beate Strittmatter, MD

2nd edition



Thieme

Ear Acupuncture

A Precise Pocket Atlas

Based on the Works of Nogier/Bahr

Beate Strittmatter, MD

Chief Instructor

German Academy of Acupuncture

Physician in Private Practice

Saarbrücken, Germany

2nd edition

267 illustrations

Thieme

Stuttgart · New York

*Library of Congress Cataloging-in-Publication
Data*

Strittmatter, Beate, 1956-

[Taschenatlas Ohrakupunktur nach Nogier/
Bahr. English]

Ear acupuncture: a precise pocket atlas
based on the works of Nogier/Bahr/Beate
Strittmatter; [translator, Ursula Vielkind;
illustrator, Beate Strittmatter]. — 2nd ed.
p.; cm.

This book is a revised new edition based
on the 4th German edition of Taschenatlas
Ohrakupunktur nach Nogier/Bahr, pub-
lished and copyrighted 2008 by Georg
Thieme Verlag, Stuttgart, Germany.
Includes indexes.

ISBN 978-3-13-131962-3 (alk. paper)

1. Ear—Acupuncture—Atlases. I. Title.

[DNLM: 1. Acupuncture, Ear--Atlases. WB
17]

RM184.S77613 2010615.8'92—dc22

2010033135

Translator: Ursula Vielkind, PhD,
Dundas, Ontario, Canada

Illustrator: Dr. Beate Strittmatter

© 2011 Georg Thieme Verlag,
Rüdigerstraße 14, 70469 Stuttgart,
Germany
<http://www.thieme.de>
Thieme New York, 333 Seventh Avenue,
New York, NY 10001, USA
<http://www.thieme.com>

Cover design: Thieme Publishing Group
Typesetting by Primustype Hurler,
Notzingen, Germany
Printed in India by Gopsons Paper Limited,
New Delhi
ISBN 978-3-13-131962-3

Important note: Medicine is an ever-changing science undergoing continual development. Research and clinical experience are continually expanding our knowledge, in particular our knowledge of proper treatment and drug therapy. Insofar as this book mentions any dosage or application, readers may rest assured that the authors, editors, and publishers have made every effort to ensure that such references are in accordance with **the state of knowledge at the time of production of the book.**

Nevertheless, this does not involve, imply, or express any guarantee or responsibility on the part of the publishers in respect to any dosage instructions and forms of applications stated in the book. **Every user is requested to examine carefully** the manufacturers' leaflets accompanying each drug and to check, if necessary in consultation with a physician or specialist, whether the dosage schedules mentioned therein or the contraindications stated by the manufacturers differ from the statements made in the present book. Such examination is particularly important with drugs that are either rarely used or have been newly released on the market. Every dosage schedule or every form of application used is entirely at the user's own risk and responsibility. The authors and publishers request every user to report to the publishers any discrepancies or inaccuracies noticed. If errors in this work are found after publication, errata will be posted at www.thieme.com on the product description page.

Some of the product names, patents, and registered designs referred to in this book are in fact registered trademarks or proprietary names even though specific reference to this fact is not always made in the text. Therefore, the appearance of a name without designation as proprietary is not to be construed as a representation by the publisher that it is in the public domain.

This book, including all parts thereof, is legally protected by copyright. Any use, exploitation, or commercialization outside the narrow limits set by copyright legislation, without the publisher's consent, is illegal and liable to prosecution. This applies in particular to photostat reproduction, copying, mimeographing, preparation of microfilms, and electronic data processing and storage.

Preface to the Second English Edition

The scientific foundations of body acupuncture have been established during the last 15 years. By contrast, ear acupuncture (auricular acupuncture, auriculotherapy) is still viewed by many as a therapeutic side issue, despite its immediate and long-lasting effect.

In a recent study representative of many scientific investigations, David Alimi at the University of Paris has demonstrated by functional MRI that auricular reflex zones are directly connected to the corresponding brain areas [1]. Needling the auricular point for the right thumb reached exactly the same brain area, in the precentral gyrus, as did direct stimulation of the right thumb (performed and measured separately prior to auricular acupuncture). The fact that Alimi was able to show by functional MRI such a precise and direct connection between an auricular reflex zone and the corresponding area in the brain is an impressive proof of the neurobiological effect of ear acupuncture. Furthermore, the fact that needling of the right ear stimulated the thumb area on the left half of the brain made it clear that the ear point representing a specific organ (or body part) must be needled on the same side of the body where the affected organ is.

Finally, ear acupuncture is becoming accepted throughout the world.

By initiating and conducting educational seminars and tutorials in the United States, Canada, and the United Arab Emirates and by giving presentations at international congresses, I had the privilege to plant the seeds for a wider distribution of this method. The activities of the Auriculotherapy Certification Institute (ACI) of Dr. Terry Oleson in Los Angeles, USA (www.auriculotherapy.com), and also of the Canadian school of Dr. Muriel Agnes, who teaches the German version of auricular acupuncture (www.vitalprincipal.ca), have aroused the interest of many physicians, nurses, and acupuncturists. As a result, a considerable number of qualified auriculotherapists are now practicing in both the United States and Canada—much to the benefit of patients who do not get help from evidence-based conventional medicine. For several years now, interest in this special form of treatment has been steadily increasing among medical students and TCM instructors, even in China (Frank R. Bahr, MD, and coworkers).

The same picture is emerging everywhere around the world: those who get to know ear acupuncture are quickly filled with the same excitement that gripped me more than 25 years ago when I had the privilege of learning auricular acupuncture from Dr. Bahr when it was still in its infancy. Thanks to the

unflagging research efforts of Dr. Bahr and coworkers, this system has since matured into a concept for specific, highly effective treatment of many diseases, particularly in patients resistant to conventional treatment.

After all, science is based on discovery, observation, and critical analysis, and thus science creates knowledge. In view of this, the scientific community can no longer ignore the fascinating discoveries in auricular acupuncture and is currently working on a model that may explain and monitor its effects.

When the first German edition of this pocket atlas was published in 2001, we did not expect it to meet with such success. By the time the fourth German edition appeared in 2007, it became obvious from the numerous positive responses coming from my colleagues that the basic concept of the book has been highly successful, both in didactical and practical terms. All other books showed maps with several reflex points on the same ear. In the medical practice, however, patients present with an “empty” ear—without any points for reference. In addition to overview maps, therefore, it seemed important to provide ear maps showing only one reflex point and accurately describe its localization. But such an approach required sufficient space, and I wish to thank the publisher once again for generously supporting such a paper-intensive presentation.

With this in mind, I wish you an interesting and informative reading. As always, I am grateful for any comments or criticism that will lead to reader-friendly improvements.

Beate Strittmatter

[1] Alimi D, Geissmann A, Gardeur D. Auricular Acupuncture Stimulation Measured on Functional Magnetic Resonance Imaging. *Medical Acupuncture* 2002;13(2):18-21.

Preface to the First English Edition

Acupuncture is a healing method, the value of which has been established through successful application over thousands of years. Unlike in the United States, in Europe acupuncture is widely used by physicians and is being taught in medical school and researched at universities. In Germany, Austria, and Switzerland alone, there are more than 20 000 physicians who apply a special form of acupuncture, namely, ear acupuncture (auriculotherapy).

The French physician, Paul Nogier, discovered this form of acupuncture 50 years ago and established its foundations. During the last 20 years, Frank R. Bahr, MD, and his coworkers consistently pursued the continuing development of ear acupuncture with respect to basic knowledge, application, and indication. In addition to the classical reflex points on the ear, other points (so-called Functional Points) have been found that are important in a physician's practice: points for pain and addictions and those with psychotropic and/or druglike effects. The reflex zones of neurological structures, described only partially by Nogier, have been refined and mapped on the ear.

Another important development of practical implication is the opportunity to use ear acupuncture for identifying focal processes in the body, the diagnosis and therapy of which are often the key to success in difficult cases. Ear points indicating focal disturbances that can influence the disease process from far away (so-called Indicator Points) play a key role in ear acupuncture, thus setting it apart from other forms of acupuncture.

Ear acupuncture with its many applications is now well established as an efficient, inexpensive, and quick procedure that has very few side effects. The main indications are pain (migraine, musculoskeletal pain), allergic disorders, and all functional diseases, such as inflammation, proneness to infection, gastrointestinal disorders, and gynecological and medical disorders.

The idea for this book came up at the International Consensus Conference on Acupuncture, Auriculotherapy, and Auricular Medicine (ICCAAAM) in Las Vegas in 1999, when participants asked me for a book in English. Our host at that time was Terry Oleson, PhD, LA, chairman of the conference and also chairman of the Auriculotherapy Certification Institute (ACI). He, too, is actively involved in research and promotion of acupuncture in the United States. My work has been enriched in many ways through intense dialogue with him, also in connection with my educational activities at the ACI, and through his continuous and vivid interest in new developments and the integration of these methods into conventional medicine.

I was also encouraged by the workshops held subsequently (and now regularly) at the University of Miami at the invitation of Janet Konefal, PhD, Chief of Complementary Medicine, Department of Psychiatry and Behavioral Sciences. Dr. Konefal made it her task to promote ear acupuncture as a part of complementary medicine at university level, thus contributing to its spread all over the world.

The present *Pocket Atlas of Ear Acupuncture* is equally well suited as a concise textbook for beginners and as a compact reference book for more experienced practitioners. It was important for me to apply a didactic principle that makes it easy for the reader to learn and memorize: in addition to general maps of the ear that are easy to remember, each reflex zone or point is illustrated individually on an otherwise empty ear. This allows for quick and systematic orientation in daily practice.

My special thanks go to my teacher, Dr. Frank R. Bahr. Not only did I learn the method of ear acupuncture from him, but we both entered a mutually respectful collaboration in the continuous efforts to enrich ear acupuncture through new discoveries and therapeutic concepts. Dr. Bahr contributed considerably to the fact that ear acupuncture is now used throughout Europe. He helped to establish the method at universities and he now makes sure, through his persistent research activity, that this steadily growing healing method is developing further.

The translator of this book, Ursula Vielkind, PhD, Ontario, Canada, helped to improve the book through her scientific background and meticulous, discerning, and enthusiastic work. As an author, I have learned a lot during this collaboration.

My special thanks go to the publisher, especially to Liane Platt-Rohloff, PhD, for her support of this demanding project.

As so often before, I wish to thank my husband and children who supported me with endless patience and tolerance while I was working on this book, even during family vacations.

Fall 2002

Beate Strittmatter

Contents

1 Introduction	3	Projection of the Pelvis.....	28
Zones of Projection	6	Pelvis—Overview	28
2 Projection of the Locomotor System	8	Pubic Bone.....	28
Auricular Innervation,		Sacroiliac Joint	30
Projection of Germ Layers	8	Perineum	30
The Three Germ Layers:		Projection of the Lower Limb .	32
Ectoderm, Endoderm,		Entire Lower Limb.....	32
Mesoderm	8	Hip Joint.....	32
Projection of the Locomotor System.....	10	Knee Joint.....	34
Locomotor System—		Ankle Joint.....	34
Overview	10	Buttock.....	36
Projection of the Vertebral Column/Thorax.....	12	Thigh.....	36
Entire Vertebral Column	12	Lower Leg.....	38
Structures of the Vertebral Column.....	12	Foot	38
Cervical Spine.....	14	Heel.....	40
Thoracic Spine	14	Achilles Tendon	40
Lumbar Spine	16	Toes	42
C0/C1 (Atlanto-occipital Joint).....	16	Projection of the Upper Limb .	44
C7/T1.....	18	Upper Limb—Overview	44
T12/L1.....	18	Large Joints—Overview	46
L5/S1		Shoulder Joint.....	46
(Lumbosacral Transition)....	20	Acromioclavicular Joint	48
Coccyx.....	20	Elbow Joint	48
Sacral Spine.....	22	Wrist	50
Thorax—Overview.....	22	Upper Arm.....	50
Ribs	24	Forearm	52
Sternum	24	Finger	52
Clavicle	26	Metacarpophalangeal Joint of Thumb	54
Trunk Muscles.....	26	Carpometacarpal Joint of Thumb	54

3 Projection of the Internal

Organs	56
Internal Organs—Overview. .	56
Projection of the	
Cardiovascular System	58
Heart	58
Blood Vessels	58
Lymph Vessels	58
Projection of the Respiratory	
System	60
Respiratory System—	
Overview	60
Lung	60
Bronchi	62
Trachea	62
Throat (Pharynx)	64
Projection of the	
Gastrointestinal System	66
Gastrointestinal System—	
Overview	66
Esophagus	66
Stomach	68
Duodenum	68
Jejunum/Ileum	
(Small Intestine)	70
Colon (Large Intestine)	70
Appendix	72
Rectum	72
Anus	74
Liver	74
Gall Bladder	76
Pancreas	76
Projection of the Immune	
System	78
Spleen	78
Thymus Gland	78
Tonsils	80
Lymph Nodes	80

Projection of the Urogenital

System	82
Urogenital System—Overview	82
Kidney	82
Ureter	84
Urinary Bladder	84
Urethra	86
Uterus	86
Ovary/Testis	88
Vagina	88
Prostate/Fallopian Tube	90
Penis/Clitoris	90
Projection of the Endocrine	
System	92
Endocrine System—	
Overview	92
Thyroid Gland	92
Parathyroid Gland	94
Thymus Gland	94
Pancreas	96
Adrenal Gland (Cortisone	
Point)	96

4 Projection of the Head 98

Head—Overview	98
Bony Skull	98
Projection of the Temporo-	
mandibular Joint	100
Temporomandibular Joint ...	100
Projection of the Jaws and	
Teeth	102
Upper and Lower Jaws, Teeth	102
Projection of the Lips and	
Tongue	104
Lips	104
Oral Cavity, Tongue	104

Projection of the Tonsils and Throat	106	Cerebellum	140
Tonsils	106	Brain Stem	142
Throat (Pharynx)	106	Vomiting Point	142
Projection of the Eyes and Nose	108	Reticular Formation	144
Eye	108	Spinal Cord	146
Nose	108	Projection of the Cranial Nerves	148
Projection of the Paranasal Sinuses and Ear	110	Cranial Nerves—Overview ..	148
Paranasal Sinuses	110	Olfactory Nerve (CN I)	150
Ear	110	Optic Nerve (CN II)	150
Projection of the Facial Muscles and Parotid Gland	112	Oculomotor Nerve (CN III) ..	152
Facial Muscles	112	Trochlear Nerve (CN IV)	152
Parotid Gland	112	Trigeminal Nerve (CN V)	154
Projection of the Occipital Trigger Points	114	Abducens Nerve (CN VI)	154
Occipital Trigger Points	114	Facial Nerve (CN VII)	156
5 Projection of the Nervous System	117	Vestibulocochlear Nerve (CN VIII)	156
Projection of the Central Nervous System	118	Glossopharyngeal Nerve (CN IX)	158
Central Nervous System—Overview	118	Vagus Nerve (CN X)	160
Cerebral Lobes—Overview ..	120	Accessory Nerve (CN XI)	162
Cortex	120	Hypoglossal Nerve (CN XII) ..	162
Frontal Lobe	122	Projection of the Autonomic Nervous System	164
Parietal Lobe	124	Autonomic Nervous System—Overview	164
Temporal Lobe	126	Sympathetic Nervous System—Overview	166
Auditory Line	126	Parasympathetic Nervous System—Overview	168
Occipital Lobe	128	Sympathetic Nuclei of Origin (Preganglionic Sympathetic Nerves)	170
Corpus Callosum	128	Sympathetic Trunk (Paravertebral Chain of Sympathetic Ganglia, Postganglionic Sympathetic Nerves)	172
Thalamus	130	Superior Cervical Ganglion ..	174
Hypothalamus	132		
Pituitary Gland	134		
Pituitary Gland Points	136		
Limbic System	138		

Middle Cervical Ganglion . . .	174	Prostaglandin E1 Point (PGE1 Point)	206
Inferior Cervical Ganglion (Stellate Ganglion)	176	Prostaglandin E2 Point (PGE2 Point)	208
Nervous Organ Points	178	Interferon Point	208
Parasympathetic Nuclei of Origin	180	Renin/Angiotensin Point	210
Prevertebral Ganglions— Overview	180	Beta-1-Receptor Point (Beta-Blocker Point According to Bahr)	210
Bronchopulmonary Plexus . .	182	Beta-2-Receptor Point (Beta- Mimetic Point According to Bahr)	212
Cardiac Plexus	182	Medication Analogue Points . .	214
Celiac Plexus (Solar Plexus) .	184	Diazepam Analogue Point (Valium Point According to Bahr)	214
Superior Mesenteric Plexus .	186	Barbiturate Analogue Point . .	214
Inferior Mesenteric Plexus . .	186	Caffeine Analogue Point	216
Hypogastric Plexus	188	Nervous Organ Points	218
Peripheral Nervous System . .	190	Nervous Organ Points— Overview	218
6 Functional Points	192	Anger Point Syn. Nervous Liver Point	218
Functional Points—Overview	192	Psychotropic Points	220
Hormone and Metabolite Points	194	Psychotropic Points— Overview	220
Hormone and Metabolic Points—Overview	194	Master Omega Point (Broma- zepam Analogue Point)	220
Estrogen Point	194	Omega Point I	222
Progesterone Point	196	Omega Point II	222
Gonadotropin Point	196	Depression Point	224
ACTH Point	198	Anxiety Point	224
TSH Point	198	Worry Point	226
Prolactin Point	200	Anger Point	226
Endocrine Parathyroid Gland Point	200	Nicotine Analogue Point According to Bahr	228
Thyroid Gland Point	202	Aggression Point	230
Thymus Gland Point	202		
Endocrine Pancreas Point (Insulin Point)	204		
Adrenal Gland Point (Cortisone Point)	204		
Histamine Point (Allergy Point 1)	206		

Bridging Point According to Bahr	230	Retro-Celiac Plexus Point (Retro-Solar Plexus Point, Retro-Point Zero)—Corresponds to Cardinal Point SI-3 of Body Acupuncture	248
Frustration Point	232	Pineal Gland Point—Corresponds to Cardinal Point BL-62 of Body Acupuncture	250
Psychotherapy Point According to Bourdiol.....	232	Energy Points	252
Analgesic Points	234	Master Point of Oscillation According to Bahr	252
Thalamus Point.....	234	Master Point of Regulation According to Bahr	252
Prostaglandin E1 Point (PGE1 Point)	234	Super Omega Point According to Bahr	254
Laterality Point According to Bahr	236	Master Point of Qi Flow According to Bahr	254
<i>Shen Men</i> Point.....	238	Point Zero.....	256
Pain Memory Points According to Bahr	240	Point GV-4.....	256
Cardinal Points	242	Focus Indicator Points.....	258
Cardinal Points—Overview ..	242	Focus Indicator Points According to Bahr—Overview.....	258
Prostaglandin E1 Point (PGE1 Point)—Corresponds to Cardinal Point GB-41 of Body Acupuncture	242	Histamine Point (Allergy Point 1).....	258
Thymus Gland Point—Corresponds to Cardinal Point TB-5 of Body Acupuncture ..	244	Cyclophosphamide Analogue Point (Endoxan Point According to Bahr, Allergy Point 2).....	260
Lung Zone—Corresponds to Cardinal Point LU-7 of Body Acupuncture	244	Prostaglandin E1 Point (PGE1 Point)	260
Diazepam Analogue Point (Valium Point According to Bahr)—Corresponds to Cardinal Point KI-6 of Body Acupuncture...	246	Vitamin C Point	262
Stellate Ganglion Point—Corresponds to Cardinal Point PC-6 of Body Acupuncture	246	Laterality Point According to Bahr	262
Interferon Point—Corresponds to Cardinal Point SP-4 of Body Acupuncture	248	Tissue Layer Control Points...	264
		Tissue Layer Control Points—Overview	264

Indicator Points for Vitamins and Trace Elements	266	Auxiliary Lines through Point Zero	290
Indicator Points for Vitamins According to Bahr—Overview	266	Hormone Points	290
Indicator Points for Trace Elements According to Bahr—Overview	266	Cortisone Point, ACTH Point ..	290
Important Silver Points	268	Thymus Gland Point, Interferon Point, Laterality Point	292
Important Silver Points on the Dominant Ear	268	Spleen Point, Diazepam Analogue Point	292
Zone-Dominating Points	272	Depression Point, Tonsil Point, TMJ Point	294
Zone-Dominating Points—Overview	272	Trigeminal Nerve	294
Points Dominating Zone A (ZA Points)	276	First Rib Point	296
Points Dominating Zone B (ZB Points)	276	First Rib Point, Stellate Ganglion Point	296
Points Dominating Zone C (ZC Points)	278	Beta-1-Receptor Point	298
Points Dominating Zone D (ZD Points)	278	Endocrine Thyroid Gland Point	298
Points Dominating Zone E (ZE Points)	280	Auxiliary Lines near the Tragus	300
Points Dominating Zone F (ZF Points)	280	Diazepam Analogue Point ...	300
Points Dominating Zone G (ZG Points)	282	Phosphate Analogue Point ...	300
7 Auxiliary Lines of Ear Acupuncture	285	Nicotine Analogue Point, Pineal Gland Point	302
Barbiturate Point / Caffeine Point	287	Frustration Point	302
Barbiturate Analogue Point, Caffeine Analogue Point	286	Interferon Point	304
Auxiliary Line through Omega Points	288	Nose Point	304
Omega Points	288	Aggression Point	306
Aggression Point	288	ACTH Point	306
		Points of the Pituitary Gland Zone	308
		Gonadotropin Point	308
		Auxiliary Lines for Points of the Limbs	310
		Points of the Hip Joint, Knee Joint, Ankle Joint	310
		Wrist Point, Knee Joint Point ..	310
		Points of the Shoulder Joint, Elbow Joint, Wrist	312
		Kidney Point	312

Auxiliary Lines for Focus Indicator Points	314	9 Indications	335
Laterality Point.	314	Indications—Overview	336
Cyclophosphamide Analogue Point (Allergy Point 2)	314	Disorders of the Locomotor System.	338
Histamine Point (Allergy Point 1), Vitamin C Point	316	Headaches and Facial Pain.	364
8 Meridians on the Ear	318	Allergic Disorders	373
LU/LI	320	Respiratory Disorders	378
Lung Meridian	320	Gastrointestinal Disorders.	381
Large Intestine Meridian	320	Inflammatory and Functional Intestinal Disorders.	383
ST/SP	322	Cardiovascular Diseases.	386
Stomach Meridian.	322	Disorders of the Urogenital System.	391
Spleen Meridian.	322	Hormonal Disorders	394
HT/SI	324	Autonomic Symptoms	398
Heart Meridian.	324	Mental Health Disorders	402
Small Intestine Meridian.	324	Addictions	405
BL/KI	326	Patient Information Leaflet: Permanent Needles.	410
Bladder Meridian.	326	Appendix	412
Kidney Meridian	326	Ear Topography.	412
PC/TB.	328	Projection Zones, Points, and Meridians on the Ear.	415
Pericardium Meridian	328	Indications.	421
Triple Burner Meridian.	328		
GB/LR.	330		
Gall Bladder Meridian.	330		
Liver Meridian	330		
CV/GV	332		
Conception Vessel (<i>Ren Mai</i>).	332		
Governor Vessel (<i>Du Mai</i>)	332		

Explanation of Symbols

- Silver needle
- Gold needle
- △ Permanent needle
- ⊙ Hidden point locations

Note: If steel needles are used instead of gold and silver needles, always needle the Gold Point because this is the point that needs to be stimulated. For example, in case of the Diazepam Analogue Point, the Gold Point would be on the left ear in a right-handed person but on the right ear in a left-handed person.

Exemption: Anxiety Point, Worry Point, Aggression Point, Nicotine Analogue Point, Nervous Liver Point—here use Silver Point for steel needles (see also p. 271).

1 Introduction

2 Projection of the Locomotor System

3 Projection of the Internal Organs

4 Projection of the Head

5 Projection of the Nervous System

6 Functional Points

7 Auxiliary Lines of Ear Acupuncture

8 Meridians on the Ear

9 Indications

Appendix

Introduction

Ear acupuncture is an effective method for treating acute and chronic diseases without producing side effects. It therefore represents one of the most important complementary additions to conventional medicine today. Its main indication is certainly the treatment of pain, but a number of functional, organic, and mental disorders may be treated as well. Fortunately, this applies also to a number of diseases for which conventional medicine still has no cures to offer, for example, migraine and hay fever.

The origins of ear acupuncture can be traced back to the 4th century BC when Hippocrates tried to cure impotence by bloodletting at the ear. It is also known that pain has been treated in ancient Egypt by means of ear points. Throughout the centuries we find notes on similar treatments. The best-known document in the European region is certainly the painting “The Garden of Lust” by Hieronymus Bosch (17th century) which shows the Sciatica Point being pierced by a needle. A second needle held by the Satan pricks the two points known as External Genitals Point and Libido Point.

Ear acupuncture enjoyed a certain popularity also in China, but it fell into oblivion during the last centuries. Approximately 20 anterior and posterior ear points were known during the Tang dynasty (618–907 AD). The procedure then probably spread to Persia, Africa, India, and the Mediterranean area.

However, there is no evidence that a comprehensive reflex system on the auricle, complete with representations of the entire body, existed. We owe it to the French physician Paul Nogier that ear acupuncture has been rediscovered and that this happened in a way that provided astonishing opportunities for both diagnosis and therapy.

Nogier has been able to demonstrate that all organs of the body are represented as reflex zones on the outer ear (auricula or auricle).

The history of this rediscovery is as exciting as the entire therapeutic method itself. Around 1950, Nogier discovered in some of his patients certain scars at a specific site on the ear. The patients told him that they had been “treated” for back pain by applying a red-hot needle to this site of the ear, with the result that the pain disappeared. We owe it to Nogier’s unreserved objectivity that he followed up on this phenomenon and studied it. He was able to reproduce the phenomenon successfully and interpreted the site on the ear as reflex localization L5/S1.

Shortly thereafter he realized that, apart from this body area, all other sites or organs of the body must be projected on the ear. He also discovered how to demonstrate these sites: they showed an increased sensitivity to pressure if the corresponding region of the body was diseased. Subsequently, he recognized that active ear acupuncture points—those reflex points that indicate a pathological change in the body or are produced on the ear by such a pathology—have changed electrically (reduced resistance and increased conductance of the skin). This allows for objective measurements that are independent of the patient (electrical point finders with a double-ring electrode).

Step by step, Nogier established the auricular map used today. If the patient had a pathology (inflammation, pain, etc.), he only had to search for the corresponding electrically altered point on the ear. For example, to locate the reflex sites of large joints or skin areas, he would simply apply a pain stimulus in the respective body region; an active acupuncture point would immediately be created at the corresponding site on the ear and, hence, could be detected because of its sensitivity to pressure or its change in conductivity. (Even today, this is still an excellent method for practicing on the ear or identifying a point that corresponds to a distinct structure of the body.)

There are two basic lines of thought in ear acupuncture, the French school and the Chinese school. They differ in the specifications of point localizations and in their diagnostic and therapeutic approaches.

The present atlas describes the French school, the intellectual father and pioneer of which was Paul Nogier. His famous students include, among others, the French psychiatrist Bourdiol and the German physician Frank Bahr. The latter founded his own school in Germany and refined essential aspects of ear acupuncture (e.g., obstacles to therapy, focus diagnosis in patients resistant to therapy, projection of body meridians onto the ear, as well as various test procedures for medications, trace elements, vitamins, and homeopathic substances). The approach of the French school is rather pragmatic and corresponds to the pathophysiological knowledge of conventional medicine. The point localizations have distinct, anatomically oriented assignments to pathologies of the body. In addition, the unambiguous relationship of the ear points to the corresponding parts of the body can be clearly demonstrated by electrical changes (caused either by existing pathologies or by iatrogenic pain stimuli on the body, as described earlier).

On the one hand, these active points clearly indicate disorders of the body; on the other hand, they are also—via needle, laser, pressure massage—the therapeutic gateways to numerous diseases. This way, therapy can be applied in a

controlled manner; that is, treatment is or should be preceded by the diagnosis of active points in order to provide information on the origin and type of the disease in question.

The present atlas describes the classic ear points, which are well-known and anatomically well-defined, as well as so-called Functional Points that influence the entire system (Nogier/Bahr). The book is not intended to replace hands-on courses and bedside teaching; it rather serves to deepen the knowledge and to provide a quick reference for the practice. For more information on ear acupuncture techniques in diagnosis and treatment, see B. Strittmatter, *Overcoming Blockages to Healing—A Manual for Health Care Professionals*, Thieme Publishers, Stuttgart–New York, 2004.

 In a right-handed person, the right ear is the dominant one, and the left ear is non-dominant.

 In a left-handed person, the left ear is dominant, and the right ear is non-dominant.

 Our readers are already familiar with the topographic anatomy of the external ear (auricle). For quick reference, the auricular topography is nevertheless included at the end of the book: pp. 412/413.

 In this book, the most important points of each ear meridian are described together with their multiple functions. They carry the same names as the corresponding body acupuncture points. Points ending with “-1” are located close to the original point (e.g. TB-1-1 lies close to TB-1).

Zones of Projection

The reflex zones and points on the auricle are arranged in such a way that they produce the inverted projection of an embryo. This image serves as a helpful memory tool when learning the points assigned to specific organs of the body.

The Inverted Embryo

The head of the embryo occupies the entire earlobe (lobule). This area is indeed the projection zone of the most important parts of the head (sensory organs, brain, temporomandibular joint, teeth, maxillary sinus, facial muscles). The embryo lies with its back (spinal cord) against the posterior margin of the auricle (descending helix)—here are the reflex zones of the sensory and motor portions of the spinal cord. The embryo covers the concha with its abdomen—here are the reflex zones of almost all intestinal organs, again in the upside-down arrangement. The embryo crosses its legs over the triangular fossa—this is the reflex zone of the lower limb.

As a rule, the **sensory portions** are found on the front of the ear (lateral surface of auricle), while the **motor portions** are found on the back of the ear (medial surface of auricle).

Ear Terraces

When memorizing the different groups of points, it is of great help to inspect or palpate the different planes of the auricle (ear terraces):

- The lower terrace (concha) contains the reflex zones of (almost) all internal organs.
- The terrace above (scapha, antihelix, triangular fossa) is occupied by the reflex zones of the entire locomotor system.
- The uppermost terrace, which is formed by the anterior, superior, and posterior margins of the auricle (helix), contains the reflex zones of the spinal cord.

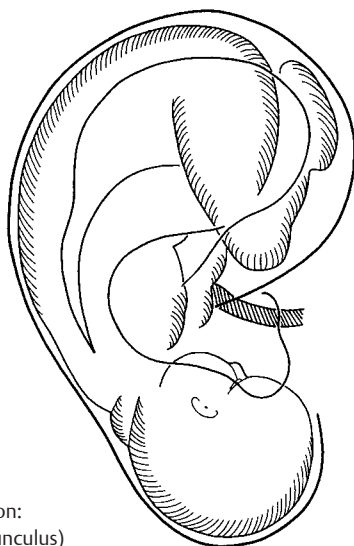


Fig. 1.1 Zones of projection:
the inverted embryo (homunculus)

The Three Germ Layers: Ectoderm, Endoderm, Mesoderm

The auricle is innervated by three nerves:

- **The great auricular nerve (originating from the cervical plexus).** It supplies the descending helix and the lobule. Here are the reflex zones of organs derived from the ectoderm (skin, nervous system).
- **The auriculotemporal nerve (a branch of the trigeminal nerve).** It supplies the ascending helix and superior helix as far as Darwin's tubercle, the triangular fossa, the scapha, the antitragus, and the antihelix including the antihelical wall. Organs derived from the mesoderm are projected in this region of the ear (muscles, bones, ligaments, heart, kidney).
- **The auricular branch of the vagus nerve.** It supplies the concha—the projection zone of organs derived from the endoderm (internal organs, with the exception of kidney and heart). This zone extends to the projection zone of the sympathetic trunk where the concha merges into the antihelical wall.

This **pattern of auricular innervation** is easy to memorize and will be of great help when learning the reflex zones of the ear. Each of the three areas of innervation corresponds to one of the three germ layers (ectoderm, mesoderm, and endoderm) and is thus assigned to a clearly defined group of organs.

Remarkably, the representation of organs on the auricle follows this pattern in almost all cases—for example, the Kidney Point does **not** lie in the zone of the endoderm like the reflex points of other internal organs; it is found beneath the ascending helix in the zone of the mesoderm from which the kidney is derived.

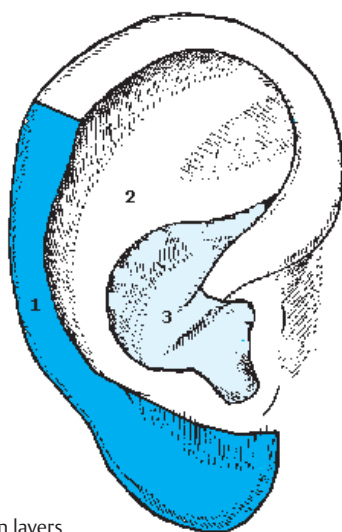


Fig. 2.1 The three germ layers

- 1 Ectoderm
- 2 Endoderm
- 3 Mesoderm

Locomotor System—Overview

The locomotor system projects onto the upper terrace of the ear (scapha, triangular fossa). In principle, the **sensory** points lie on the front of the ear (lateral surface), while the **motor** points are on the back of the ear (medial surface). The **forceps method according to Bahr** is based on this arrangement:

A gold needle in the sensory point on the lateral surface, and a silver needle in the corresponding motor point on the medial surface.

Body parts, organs, or anatomical structures belonging to the same region project very close to each other onto the ear. For example: knee joint, popliteal artery, peroneal nerve.

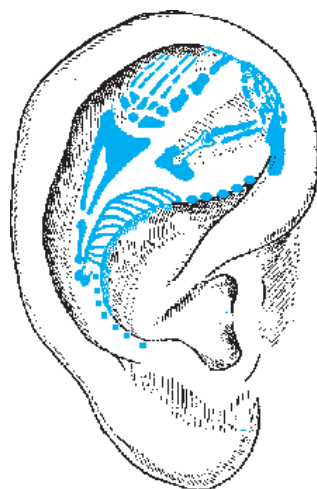


Fig. 2.2 Projection of the locomotor system

Entire Vertebral Column

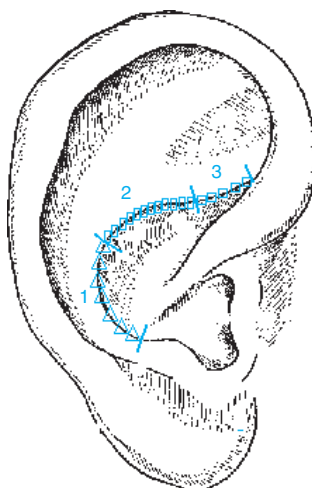
Location:

- The vertebral column, or spine, projects upside-down onto the entire antihelix.
- **Start:** Postantitragal fossa (Atlanto-occipital Joint Point, Point C0/C1).
- **End:** Coccyx Point at the end of the antihelix, underneath the ascending helix (only visible with the helix unfolded; very close to the Internal Anus Point). The outer aspect of the antihelical fold, or reflection (scapha-antihelical wall), is variably expressed, which helps in subdividing the zone of the vertebral column into Cervical Spine Zone, Thoracic Spine Zone, and Lumbar Spine Zone.

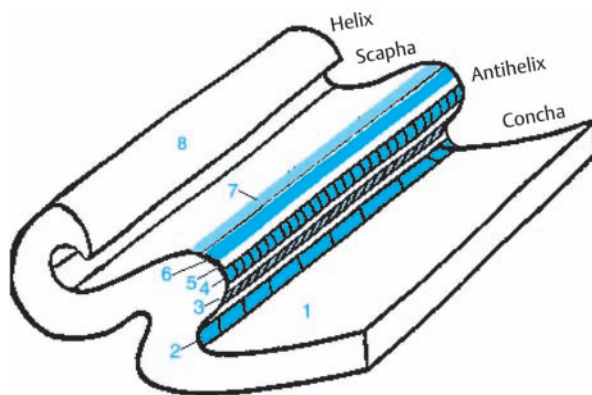
Structures of the Vertebral Column

Location:

- Like the various structures of a vertebra, the corresponding points lie close together on the edge of the antihelix:
- The small vertebral joints project slightly lateral to the edge of the antihelix (which is important for treating the very common “blockages” of the spine). The Zone of Paravertebral Muscles and Ligaments lies lateral to this Zone of Small Vertebral Joints.
- **Points nearby:** At the level of the transition from the upper third to the lower two thirds of the antihelical wall lies the Zone of Nervous Control Points of Endocrine Glands, at the foot of the antihelical wall lies the Zone of Sympathetic Trunk (Zone of Paravertebral Chain of Sympathetic Ganglia) with the reflex points for the nervous control of organs (e.g., Nervous Liver Point) and for the cervical ganglia (e.g., Stellate Ganglion Point). See Nervous System, page 173.

**Fig. 2.3**

- 1 Cervical spine
- 2 Thoracic spine
- 3 Lumbar spine

**Fig. 2.4**

- 1 Zone of Sympathetic Trunk, Neural Organ Points
- 2 Zone of Nervous Control Points of Endocrine Glands
- 3 Zone of Intervertebral Disks
- 4 Zone of Vertebrae
- 5 Zone of Small Vertebral Joints
- 6 Zone of Paravertebral Muscles and Ligaments
- 7 Zone of Spinal Cord
- 8 Zone of Parachordal Tissue

Cervical Spine

Location:

- In the inferior portion of the antihelix (Zone of Cervical Spine), the antihelical fold is sharply defined toward the conchal wall, like a tube with a small diameter.
- At the level of Point C7/T1, there is a distinct change: the sharp edge widens and becomes gently rounded (Zone of Thoracic Spine). The curvature of the antihelix also changes.

Thoracic Spine

Location:

- The zone between Points C7 and L1, with the antihelix being gently rounded (a sharp edge in the Cervical Spine Zone, a steep cartilage overhang in the Lumbar Spine Zone; see above and next page).
- Relative to the total length of the thoracic spine in the body, the segment of its representation on the ear is short.

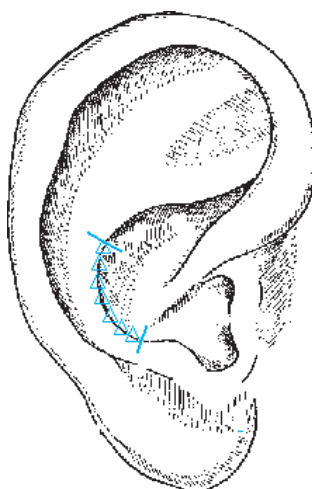


Fig. 2.5 Cervical spine

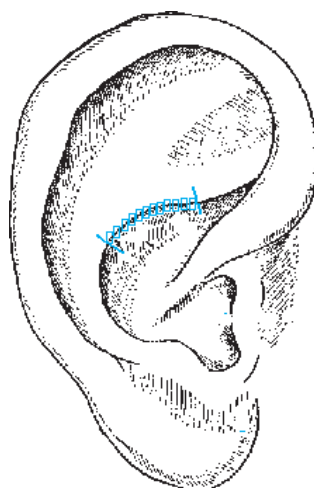


Fig. 2.6 Thoracic spine

Lumbar Spine

Location:

- **Start:** About 1 cm before the antihelix intersects the ascending helix. The soft rounding of the Thoracic Spine Zone abruptly changes into a steep or lamella-shaped cartilage overhang. The latter represents the Lumbar Spine Zone.
- **End:** Point L5/S1 at the intersection of antihelix and helix. From here on, underneath the ascending helix, the sacral vertebrae are projected.

C0/C1 (Atlanto-occipital Joint)

Location:

- Postantitragal fossa, easily detectable with the stirrup-shaped ear probe (see also Cervical Spine, p. 15).

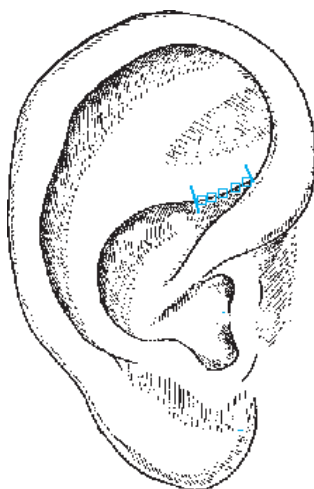


Fig. 2.7 Lumbar spine

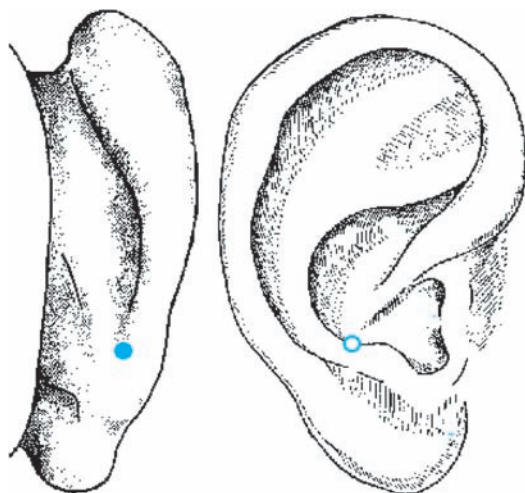


Fig. 2.8 C0/C1 (atlanto-occipital joint)

C7/T1

Location:

- Where the sharp edge of the Cervical Spine Zone turns into a gentle rounding. Easy to find with the stirrup-shaped ear probe (see also Cervical Spine and Thoracic Spine, p. 15).

T12/L1

Location:

- Where the gentle rounding of the Thoracic Spine Zone turns into a sharp, overhanging edge. Easy to find with the stirrup-shaped ear probe (see also Thoracic Spine and Lumbar Spine, pp. 15 and 17).

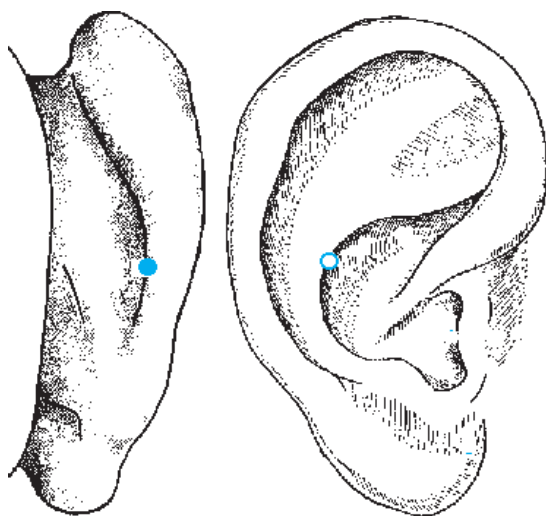


Fig. 2.9 C7/T1

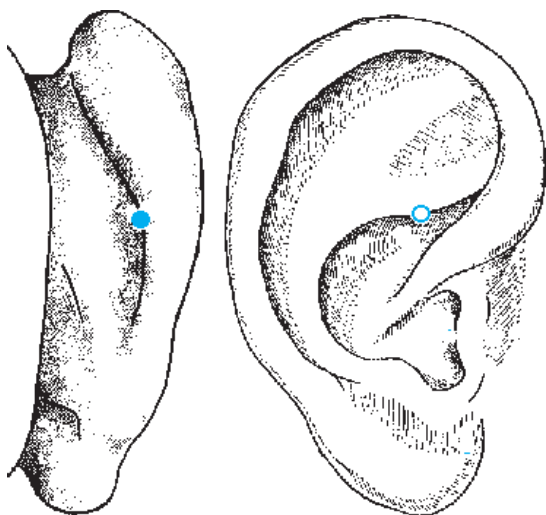


Fig. 2.10 T12/L1

L5/S1 (Lumbosacral Transition)

Location:

- On the antihelix, at the intersection with the ascending helix (see also Lumbar Spine, p. 17).

Coccyx

Location:

- At the end point of the antihelix, underneath the ascending helix. Easily palpable with the finger. All points of the sacrum lie on the antihelix hidden beneath the ascending helix.

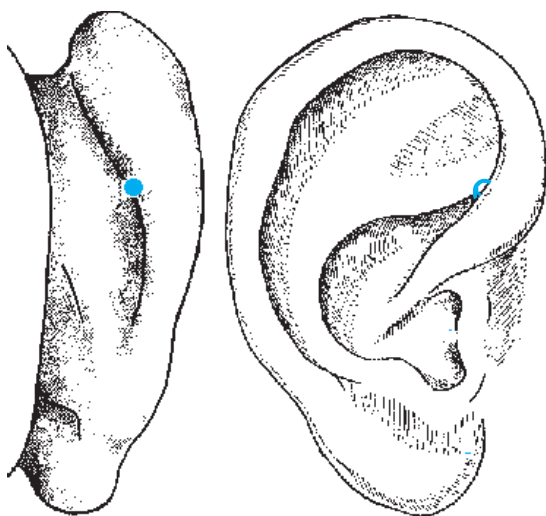


Fig. 2.11 L5/S1 (lumbosacral transition)

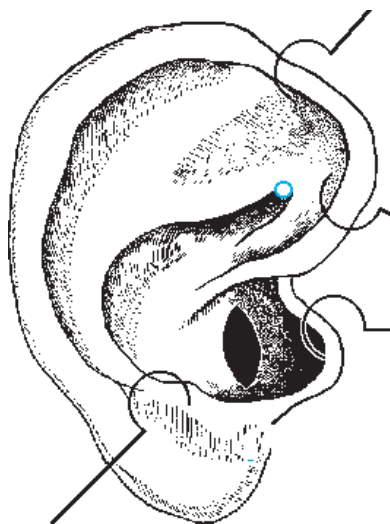


Fig. 2.12 Coccyx

Sacral Spine

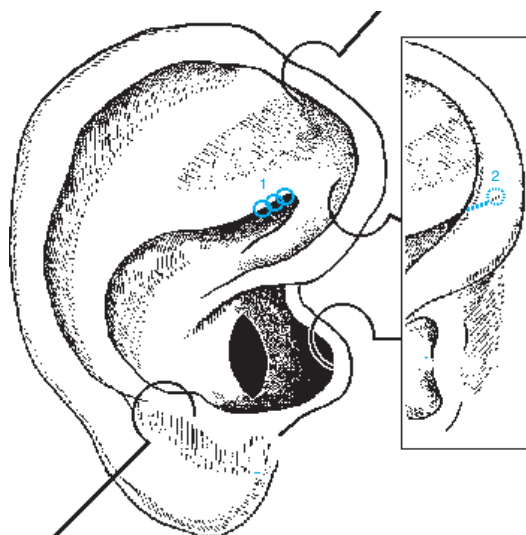
Location:

- The sacrum projects onto the part of the antihelix that runs beneath the ascending helix.
- **Start:** intersection of antihelix and ascending helix.
- **End:** Coccyx Point at the end of the antihelix (point easily palpable with the finger).

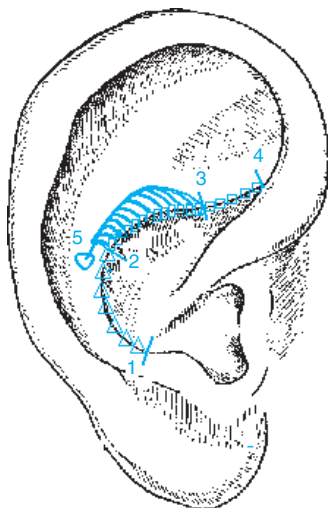
Thorax—Overview

Location:

- The ribs are represented on the scapha between the projections of the sternum and the corresponding vertebrae.
- All points of adjacent structures (acromioclavicular joint, head of the 1st rib, sternocostal joints, etc.) are also found in this region of the ear.

**Fig. 2.13**

- 1 Sacral spine
- 2 Thorax

**Fig. 2.14**

- 1 C0/C1
- 2 C7/T1
- 3 T12/L1
- 4 L5/S1
- 5 Scapula

Ribs

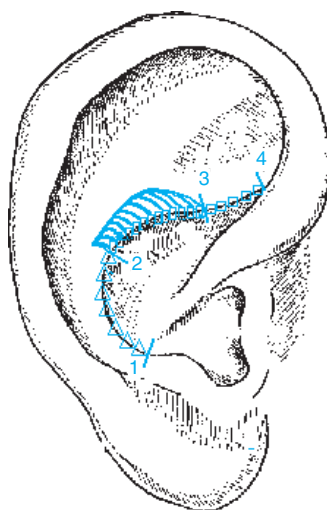
Location:

- On the scapha, at the level of the Thoracic Vertebrae Zone (uppermost ear terrace).

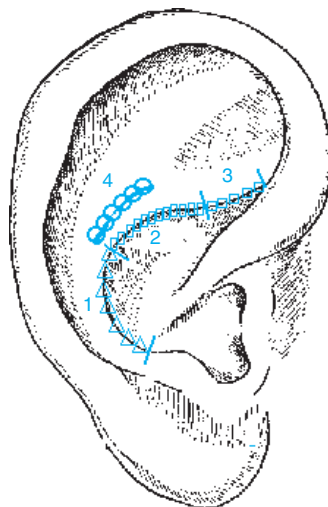
Sternum

Location:

- On the scapha, at the level of the Thoracic Vertebrae Zone (uppermost ear terrace), but slightly more lateral than the Rib Zone.

**Fig. 2.15**

- 1 C0/C1
- 2 C7/T1
- 3 T12/L1
- 4 L5/S1

**Fig. 2.16**

- 1 Cervical spine
- 2 Thoracic spine
- 3 Lumbar spine
- 4 Sternum

Clavicle

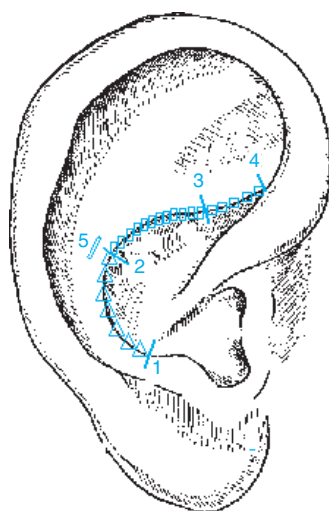
Location:

- On the scapha, at the level of the 1st rib (uppermost ear terrace), near the Shoulder Point.

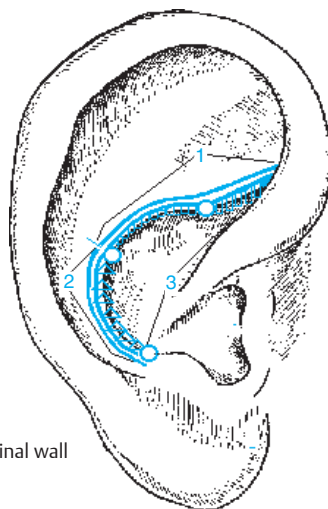
Trunk Muscles

Location:

- All muscles project basically onto the zones of the adjacent bone structures.
- As always, the respective points need to be searched for by using a point-finding device or by vascular autonomic signal (VAS) diagnosis.
- The back muscles project onto the scapha near the Zone of Vertebrae. The anterior muscle portions (abdominal and thoracic muscles) project **lateral** to the back muscles toward the rim of the helix.

**Fig. 2.17**

- 1 C0/C1
- 2 C7/T1
- 3 T12/L1
- 4 L5/S1
- 5 Clavicle

**Fig. 2.18**

- 1 Thoracic muscles/abdominal wall
- 2 Ventral cervical muscles
- 3 Erector spinae muscle

Pelvis—Overview

Location:

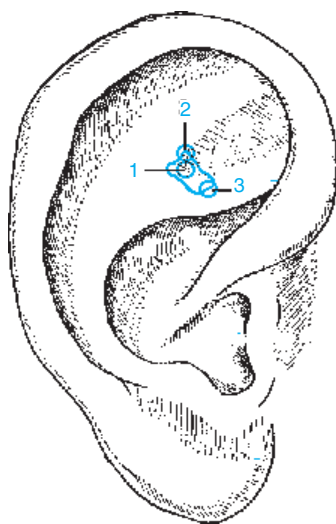
- The pelvis projects into the acute angle of the triangular fossa where the superior and inferior antihelical crura meet.
- Here are the reflex points of the hip joint, the pubic bone (important, for example, for complaints during pregnancy), the sacroiliac joint, and the points of the iliac crest (e.g., for treating tendopathy of the erector spinae muscle at the iliac crest).

Pubic Bone

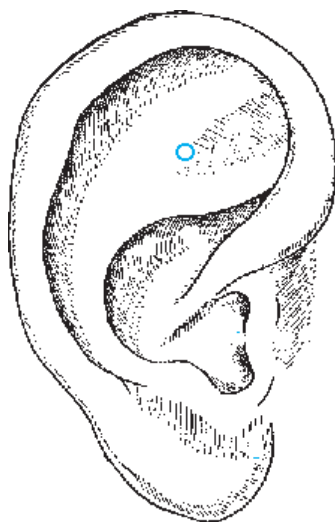
Location:

- Lateral to the Hip Joint Point.

Important location for the hormone-induced, painful loosening of the symphysis during pregnancy.

**Fig. 2.19**

- 1 Hip joint
- 2 Pubic bone, symphysis
- 3 Sacroiliac joint

**Fig. 2.20** Pubic bone

Sacroiliac Joint

Location:

- Near the Lumbar Spine Zone, approximately at the level of Point L2.

A gold needle accurately inserted at this point can abolish blockage in the sacroiliac joint—without manual therapy.

Be aware of counterblockages along the vertebral column; needle the corresponding points, if appropriate.

The ideal treatment also during pregnancy.

Perineum

Location:

- At the anterior edge of the palpable cartilage of the ascending helix, at the level of the Uterus Point.

Indications:

- Pain after episiotomy; episiotomy scars that have developed into a focal disturbance.

Note: The projection of the region between anus and genitals (anterior perineum) is an important reflex zone on the ear because scars from an episiotomy frequently develop into a focal disturbance, and this may cause resistance to therapy. (Use a gold needle in such cases.)

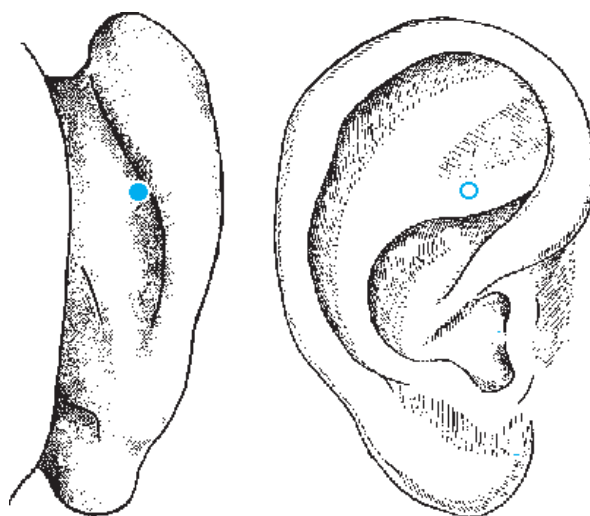


Fig. 2.21 Sacroiliac joint

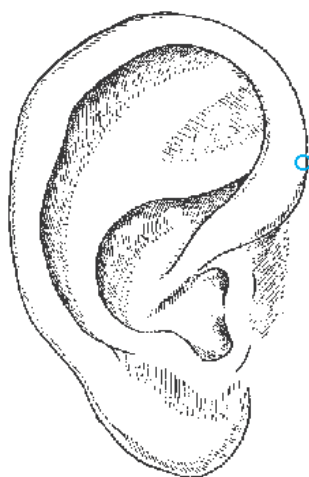


Fig. 2.22 Perineum

Entire Lower Limb

Location:

- The projection of the lower limb takes up all of the relatively small space of the triangular fossa.
- The points for the hip, knee, and ankle joints form an axis. The foot is represented perpendicularly to this axis on the uppermost margin of the scapha.

In principle, all structures that belong to the same body region project also within the reflex zone of this body part on the ear: for example, the tendons, ligaments, and capsule of the ankle joint.

The points are **not** identical but lie close together.

Hip Joint

Location:

- The hip joint projects onto the very tip of the triangular fossa where the two antihelical crura meet.

Also projected onto this area are all structures near the hip, such as the **inguinal region** (inguinal hernia, inguinal strain), the **greater trochanter**, the inferior portion of the **sciatic nerve**, and the bones of the bony pelvis with the **sacroiliac symphysis**.

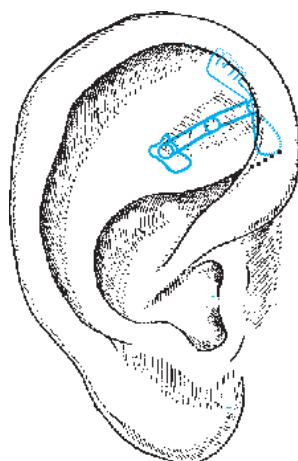


Fig. 2.23 Lower limb

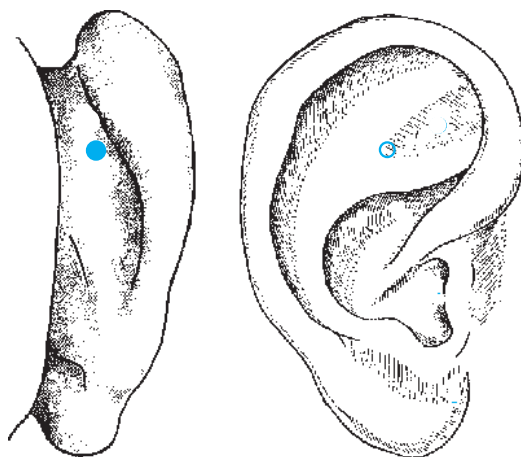


Fig. 2.24 Hip joint

Knee Joint

Location:

- In the deepest point of the triangular fossa; it is therefore easy to find.

The Knee Joint Point may be used for guidance in finding the Thigh and Leg Zones.

Ankle Joint

Location:

- On a line drawn through Hip Joint Point and Knee Joint Point, in superior direction.
- Partly hidden by the rim of the ascending helix.

To find this point on the rubber ear, insert the needle at the superior end of the lower limb axis, close to the rim of the ascending helix and **perpendicularly** to the scapha.

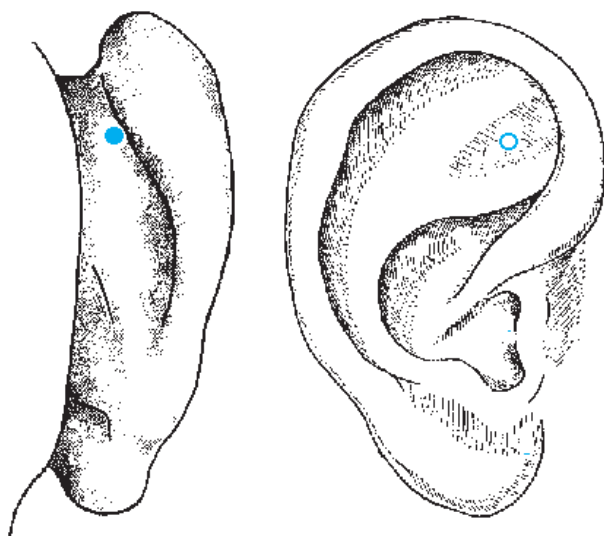


Fig. 2.25 Knee joint

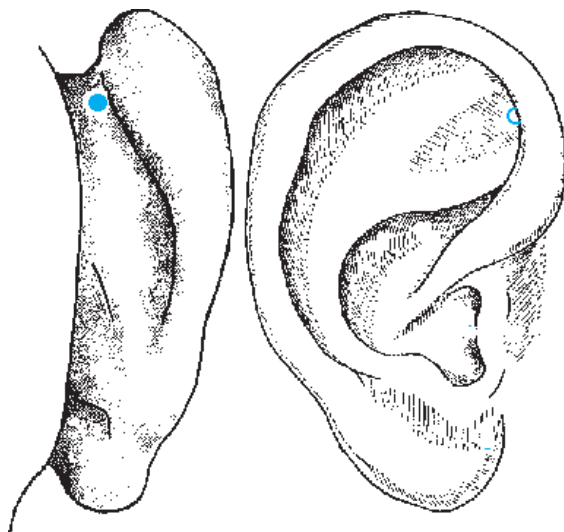


Fig. 2.26 Ankle joint

Buttock

Location:

- Similar to Sacroiliac Joint Point but occupying a larger area.

Thigh

Location:

- The area between Hip Joint Point and Knee Joint Point.
- Lateroanterior portions of the thigh (e.g., quadriceps muscle, blood vessels) are represented on the ear in lateroinferior direction, while medial portions (e.g., adductor muscles) project in medial direction toward the ascending helix.

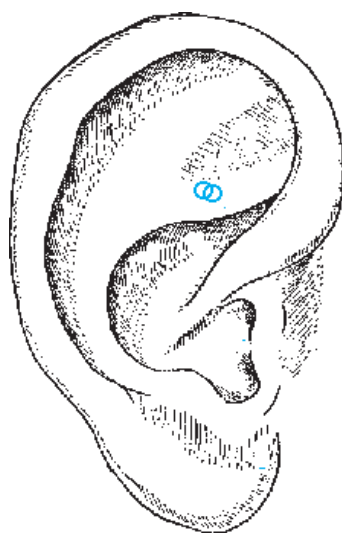


Fig. 2.27 Buttock

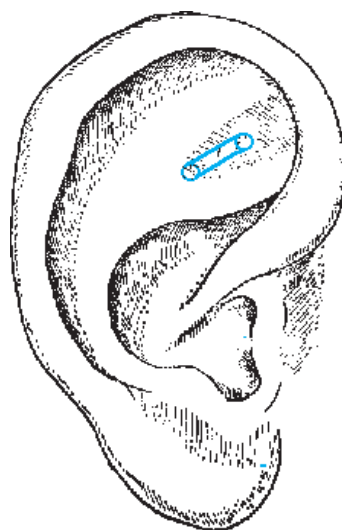


Fig. 2.28 Thigh

Lower Leg

Location:

- Area between Knee Joint Point and Ankle Joint Point.
- As with the thigh, the lateroanterior portions of the leg (e.g., peroneus muscle) are represented on the ear in lateroinferior direction, while the medial portions project in medial direction toward the ascending helix.

Foot

Location:

- The foot projects perpendicularly to the axis formed by the representations of the hip, knee, and ankle joints (as in the body).
- The **Heel Point** lies near the Coccyx Point and is **hidden** under the ascending helix.
- The area of the **Toe Points** reaches underneath the ascending helix and is partly covered by the helical rim.
- The **Big Toe Point** meets the Thumb Zone in the medial third of the helical rim.

(The dotted red areas represent hidden locations.)

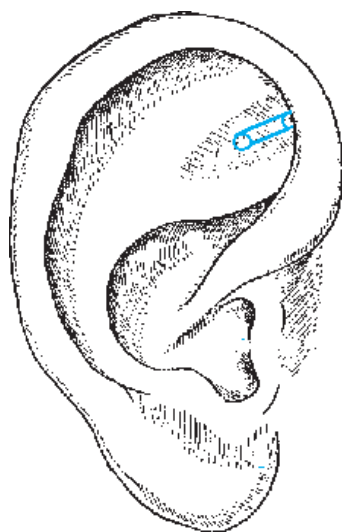


Fig. 2.29 Leg

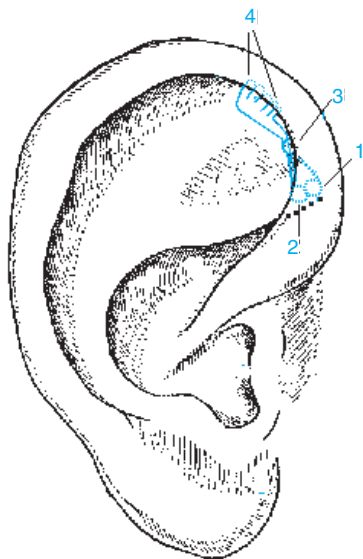


Fig. 2.30

- 1 Heel
- 2 Achilles tendon
- 3 Ankle joint
- 4 Toes

Heel

Location:

- Near the Coccyx Point and, therefore, **hidden** by the ascending helix.
- The location of the Heel Point is a favorite question in the ear acupuncture exam.

(The dotted red areas represent hidden locations; the broken black line shows the continuation of the antihelix toward the Coccyx Point beneath the ascending helix.)

Achilles Tendon

Location:

- Close and slightly inferior to the Heel Point.

(The dotted red areas represent hidden locations.)

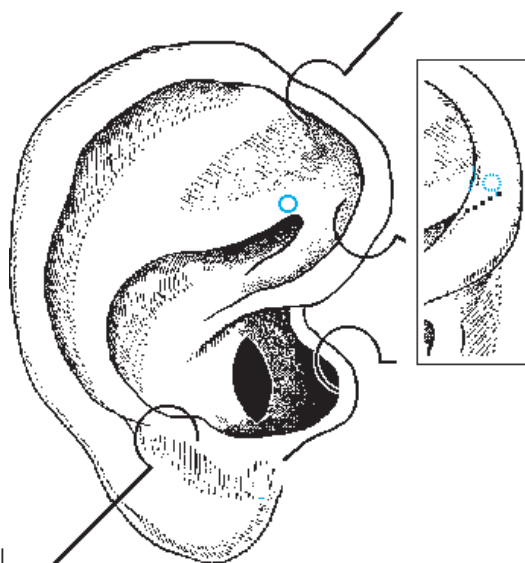
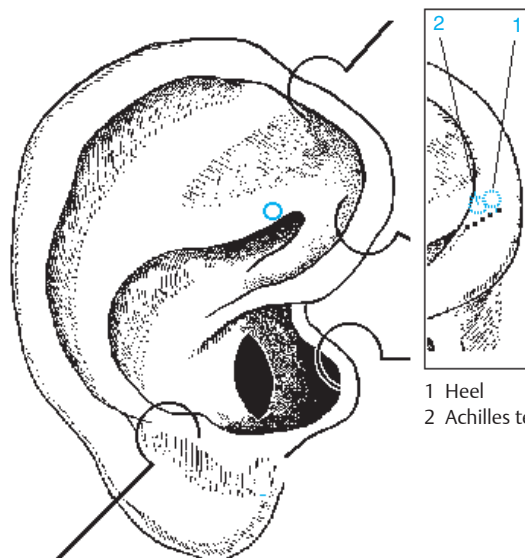


Fig. 2.31 Heel



- 1 Heel
- 2 Achilles tendon

Fig. 2.32
Achilles tendon

Toes

Location:

- Underneath the ascending helix and partly covered by the reflection.
- The Big Toe Point meets the Thumb Zone in the medial third of the helical rim.

(The dotted red areas represent hidden locations.)

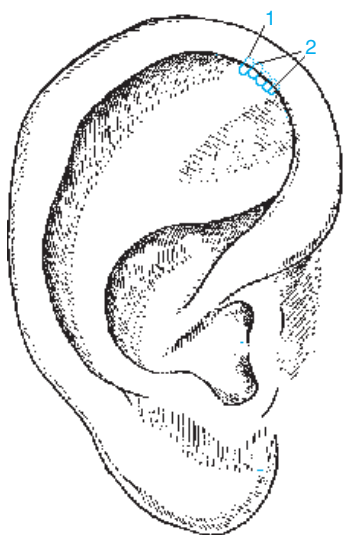


Fig. 2.33

1 Big toe

2 Other toes

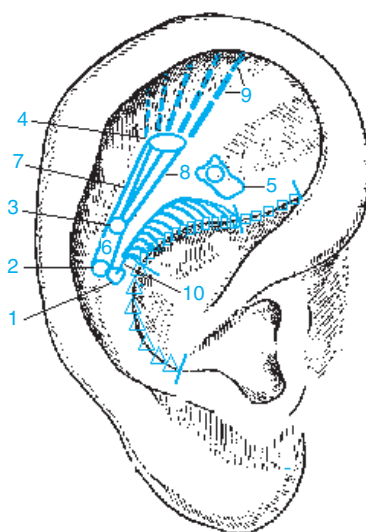
Upper Limb—Overview

Location:

- **Shoulder Joint Point:** at the level of Point C7.
- **Elbow Joint Point:** between the lower third and the middle third of the line connecting Shoulder Joint Point and Wrist Zone (see Auxiliary Lines, p. 313).
- **Wrist Zone:** a larger area at the level of the Knee Joint Point (see also the descriptions of individual joint points).

The location of a joint point or, in general, of a topographic structure in the locomotor system includes **all** of the parts belonging to this structure (that is, the points for the respective tendons, ligaments, muscles, peripheral nerves, blood vessels are close together).

In case of disease, the active point for the affected structure is searched for in the known reflex zone (point finder, VAS).

**Fig. 2.34**

- 1 Scapula
- 2 Shoulder joint
- 3 Elbow joint
- 4 Wrist
- 5 Pelvis with hip joint
- 6 Upper arm
- 7 Ulna
- 8 Radius
- 9 Thumb
- 10 Clavicle

Large Joints—Overview

Location:

- The upper limb projects onto the scapha.
- **Shoulder Joint Point:** at the level of Point C7.
- **Elbow Joint Point:** lower third between Shoulder Joint Point and Wrist Zone (see Auxiliary Lines, p. 309).
- **Wrist Zone:** a larger reflex area at the level of the Knee Joint Point (see also the descriptions of individual joint points).

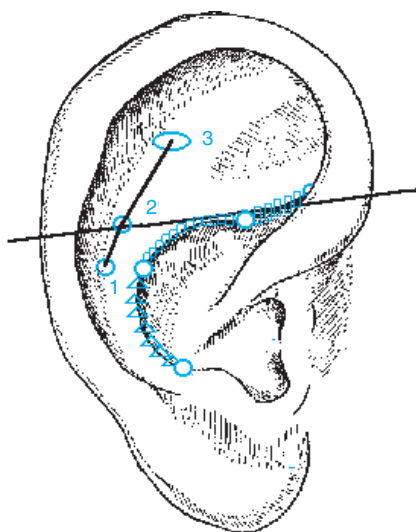
Shoulder Joint

Location:

- The shoulder joint and its adjacent structures (clavicle, scapula) project, as may be expected, approximately at the level of Point C7 onto the scapha.

Beginners often search for the Shoulder Joint Point too far down in the direction of the lobule.

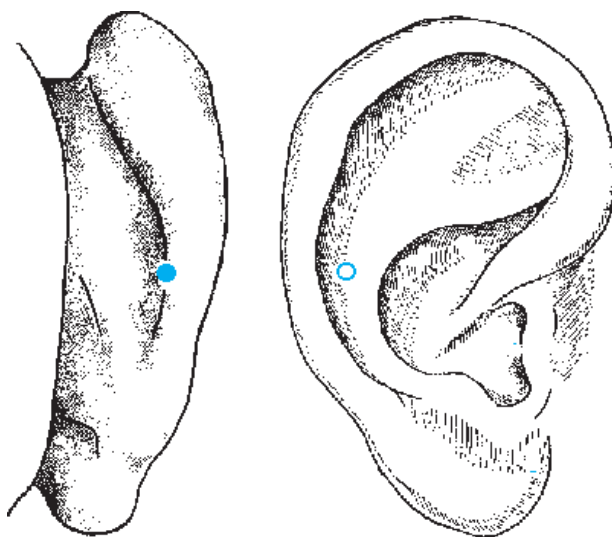
The active points for all pathologically altered structures of the shoulder region should be searched for by starting from this point (e.g., irritation of the rotator cuff, isolated endopathy, irritation of the bicipital tendon, frozen shoulder, inflammation of the acromioclavicular joint).

**Fig. 2.35**

1 Shoulder joint

2 Elbow joint

3 Wrist

**Fig. 2.36** Shoulder joint

Acromioclavicular Joint

Location:

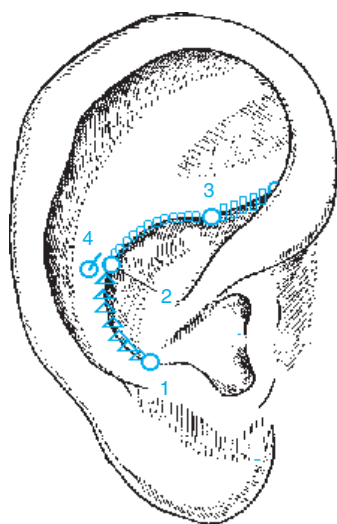
- At the level of Point C7 near the Shoulder Joint Point.

The joint is commonly affected by disorders of the shoulder.

Elbow Joint

Location:

- On the line formed by connecting the Shoulder Joint Point with the Wrist Zone while following the gentle curvature of the scapha in superior direction.
- On the intersection of this line with the line tangential to the antihelix in the region of the Lumbar Spine Zone (see Auxiliary Lines, p. 313).

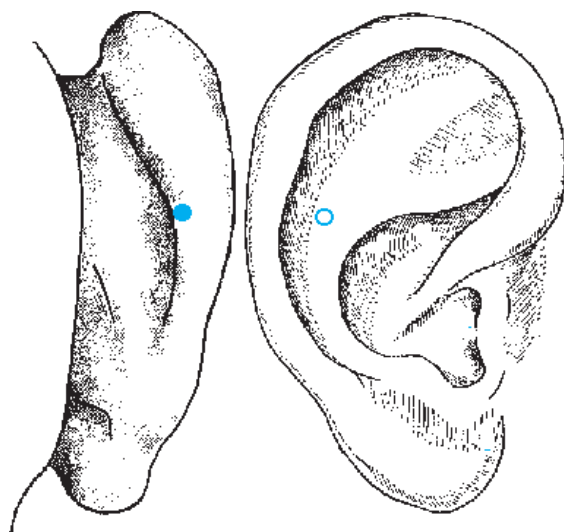
**Fig. 2.37**

1 C0/C1

2 C7/T1

3 T12/L1

4 Acromioclavicular joint

**Fig. 2.38** Elbow joint

Wrist

Location:

- The wrist projects onto a relatively large area, lateral to the Knee Joint Point, midway between the descending helix and the superior antihelical crus.
- A horizontal line through the Knee Joint Point leads to the Wrist Zone (see Auxiliary Lines, p. 311).
- The **metacarpal bones** and the **fingers** project onto the remaining area toward the edge of the ear (body of helix).

Upper Arm

Location:

- The area between Shoulder Joint Point and Elbow Joint Point.

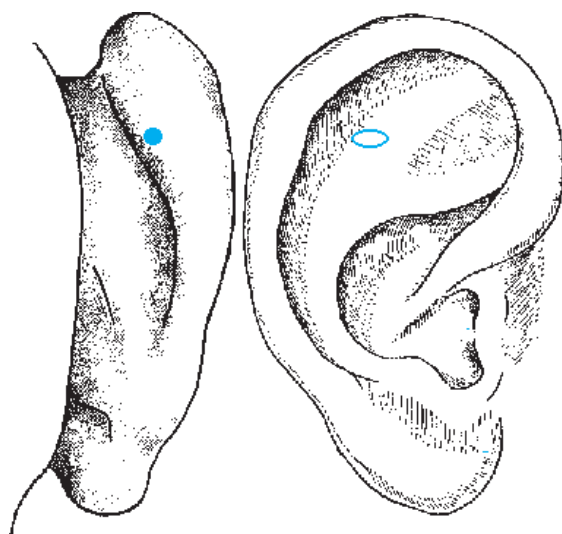


Fig. 2.39 Wrist

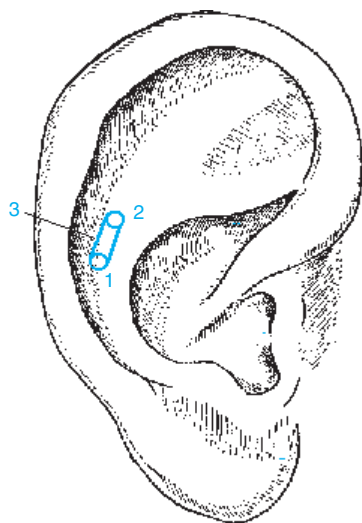


Fig. 2.40

- 1 Shoulder
- 2 Elbow
- 3 Upper arm

Forearm

Location:

- The forearm is represented between Wrist Zone and Elbow Joint Point, with the radius projecting medially and the ulna projecting laterally (near the body of the descending helix).

Finger

Location:

- The **metacarpal bones** and the **fingers** project onto the area between the Wrist Zone and the body of helix.
- For finding the reflex zones of the fingers it is helpful to know that the Thumb Zone meets the Big Toe Point beneath the medial third of the helical rim.
- The points for fingertips and fingernails, like those for the tips and nails of the toes, are partly hidden by the rim of the helix.
- The pulps of the fingers project onto a relatively large area of the ear, corresponding to the large area of representation in the brain.

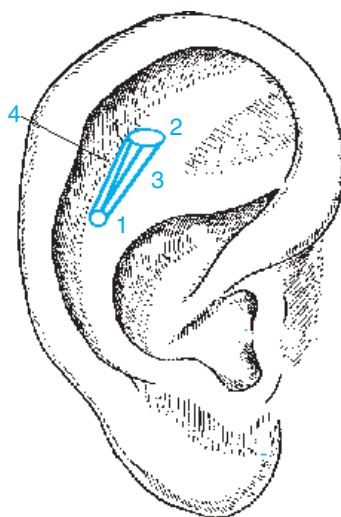


Fig. 2.41

- 1 Elbow joint
- 2 Wrist
- 3 Radius
- 4 Ulna

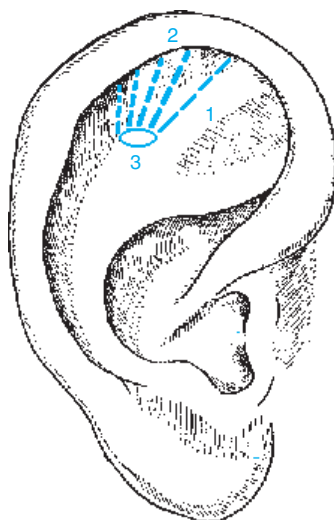


Fig. 2.42

- 1 Thumb
- 2 Other fingers
- 3 Wrist

Metacarpophalangeal Joint of Thumb

Location:

- Between the points for the head of the first metacarpal bone and the base of the proximal phalanx of the thumb.

Carpometacarpal Joint of Thumb

Location:

- Between the points for the first metacarpal bone of the thumb and the wrist (trapezium bone). Important location for treating arthrosis of the carpometacarpal joint.

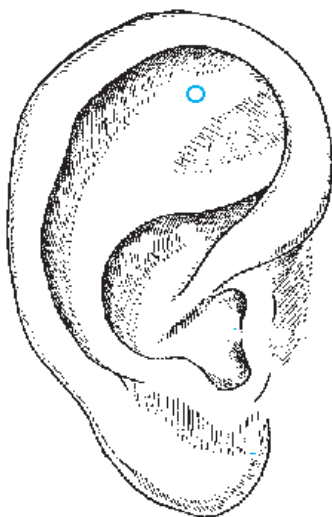


Fig. 2.43 Metacarpophalangeal joint of thumb

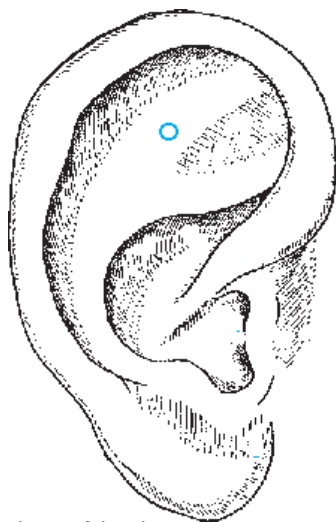


Fig. 2.44 Carpometacarpal joint of thumb

Internal Organs—Overview

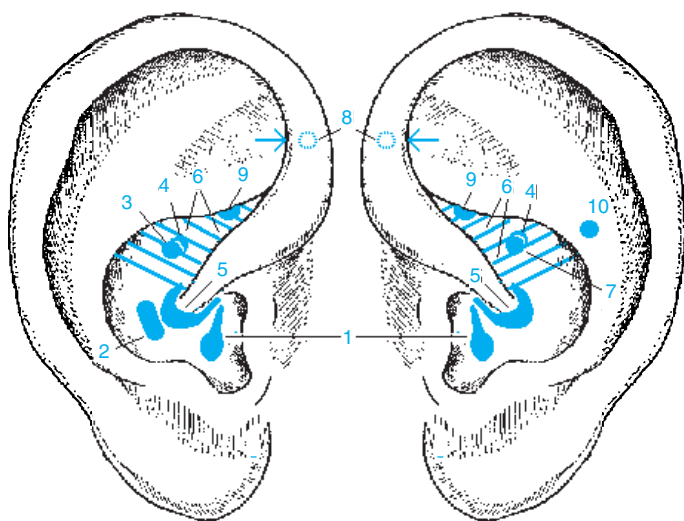
The solid areas are for description only; they do not represent Silver Points.

Location:

- Most of the internal organs project onto the concha.
- The organs of the **upper** half of the trunk (thoracic organs) are represented in the **inferior concha** (e.g., Lung Zone), while those of the **lower** half (abdominal organs) are represented in the **superior concha** (e.g., Intestine Zone).

Exceptions:

- The **heart** projects onto the antihelix (being a special type of striated muscle, it is represented in the reflex zone of the muscles).
- The reflex areas of **kidneys and genitals** are located underneath the ascending helix (their embryogenesis differs from that of other organs; the kidneys develop from the mesoderm).
- All **blood and lymph vessels** project basically near the structures that they supply (e.g., arm, leg).

**Fig. 3.1**

- 1 Lung
- 2 Liver
- 3 Gall bladder
- 4 Pancreas
- 5 Stomach, esophagus, and duodenum
- 6 Intestine
- 7 Spleen
- 8 Kidney
- 9 Bladder
- 10 Heart

Heart

Location:

- In contrast to most internal organs the heart does not project onto the concha but is represented in the reflex area of the thoracic spine muscles (4th–7th ICS) because it is a special type of striated muscle.
- The motor portion is represented on the back of the ear and the sensory portion on the front of the ear.

Contrary to the usual forceps method, the motor portion of the Heart Zone is pricked with a gold needle (e.g., concomitant acupuncture therapy in case of myocardial insufficiency).

Blood Vessels

Location:

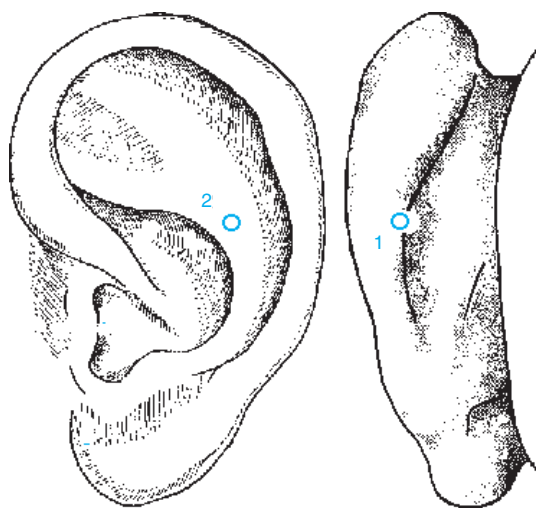
- The arteries and veins basically project near the structures that they supply (e.g., the femoral artery is represented in the Thigh Zone).

In ear acupuncture, the blood and lymph vessels rarely need to be treated. As the therapist can easily deduce their representations, these are not described here in detail. If a segment of a vessel is affected, the active points of the respective anatomical structure are searched for by means of point finder or VAS.

Lymph Vessels

Location:

- The lymph vessels basically project near the structures that they supply (e.g., arm, leg).

**Fig. 3.2**

- 1 Heart (motor portion)
- 2 Heart (sensory portion)

**Fig. 3.3**

- Blood vessels
- Lymph vessels

Respiratory System—Overview

Location:

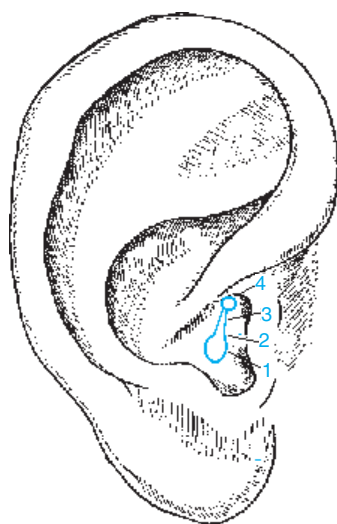
- The reflex zone of the lung is located in the middle of the deepest site of the inferior concha.
- The reflex zones of bronchi, trachea, and throat (pharynx) are located in an obliquely superior direction toward the supratragic notch, with the trachea projecting parallel to the reflex zone of the esophagus.

Lung

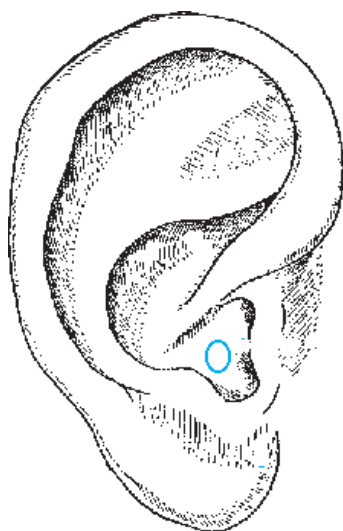
Location:

- At the deepest site of the inferior concha (approximately in the middle of the inferior concha).
- The right lung projects onto the right ear and the left lung onto the left ear.

When considering the lung as an organ or as a functional system in the sense of TCM, a gold needle is used on the dominant ear.

**Fig. 3.4**

- 1 Lung
- 2 Bronchi
- 3 Trachea
- 4 Throat (pharynx)

**Fig. 3.5** Lung

Bronchi

Location:

- In the inferior concha, superior to the Lung Zone. Merges with the Trachea Zone.

When considering the bronchi as an organ or as a functional system in the sense of TCM, a gold needle is used on the dominant ear.

Trachea

Location:

- Superomedial to the Bronchi Zone, parallel to the Esophagus Zone in the direction of the Throat Point (at the superior end of this zone).

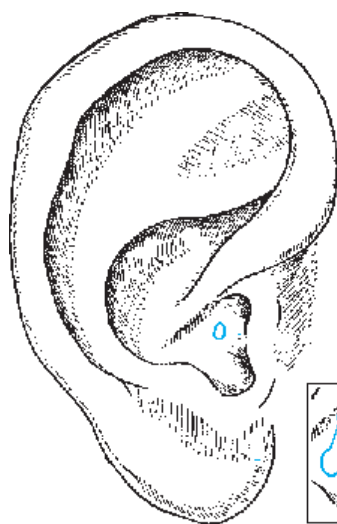
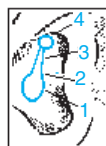


Fig. 3.6 Bronchi



- 1 Lung
- 2 Bronchi
- 3 Trachea
- 4 Throat (pharynx)

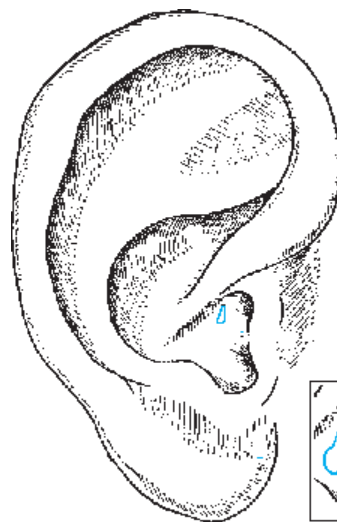
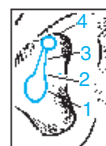


Fig. 3.7 Trachea



- 1 Lung
- 2 Bronchi
- 3 Trachea
- 4 Throat (pharynx)

Throat (Pharynx)

Location:

- At the superior end of the Trachea Zone. If the vocal cords are affected active points will also be found here.

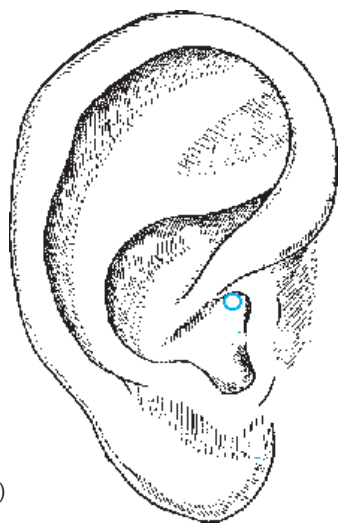


Fig. 3.8 Throat (pharynx)

Gastrointestinal System—Overview

The organs of the gastrointestinal system project onto the entire superior concha and the upper part of the inferior concha.

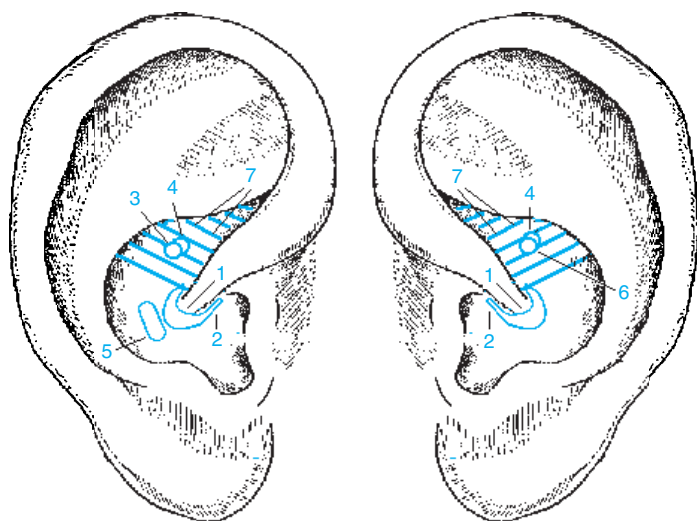
Depending on the position of the organs in the body, their reflex zones are found on either the right or the left ear, or on both ears (e.g., Stomach Zone, Pancreas Zone).

Again, sensory portions are represented on the front of the ear and motor portions on the back of the ear.

Esophagus

Location:

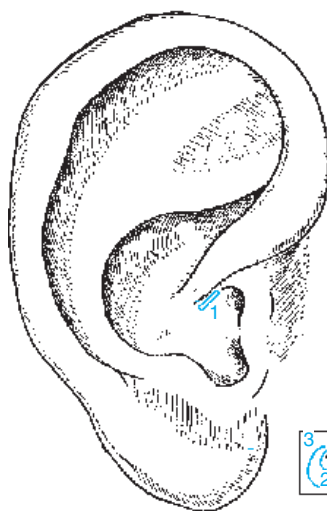
- At the medial end of the Stomach Zone (Cardia Zone) below the root of the helix. Tapering off in the direction of the Throat Point below the supratragic notch.

**Fig. 3.9**

1 Stomach, esophagus,
and duodenum

2 Esophagus
3 Gall bladder
4 Pancreas

5 Liver
6 Spleen
7 Small/large intestine

**Fig. 3.10**

1 Esophagus
2 Stomach
3 Duodenum



Stomach

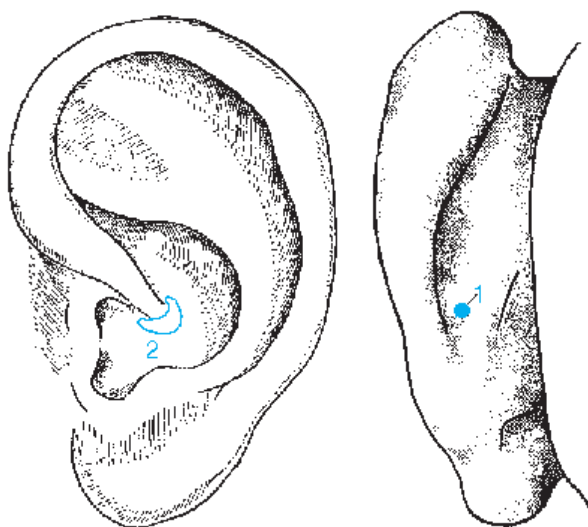
Location:

- The Stomach Zone forms a crescent-shaped area around the root of the helix.
- There is a zone on the right ear and a larger zone on the left ear.

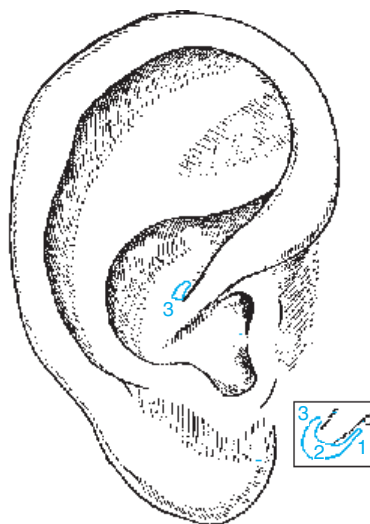
Duodenum

Location:

- In the superior concha adjacent to the reflex area of the posterior (inferior) end of the stomach, superolateral to the root of the helix.

**Fig. 3.11**

- 1 Stomach (motor portion)
- 2 Stomach (sensory portion)

**Fig. 3.12** Duodenum

- 1 Esophagus
- 2 Stomach
- 3 Duodenum

Jejunum/Ileum (Small Intestine)

Location:

- Upper and middle region of the superior concha, occupying the larger part of the superior concha (the Colon Zone and Rectum Zone are superolateral to it).

Colon (Large Intestine)

Location:

- In the upper part of the superior concha. On the back of the ear in the upper fifth of the auricle, about 3–4 mm away from the medial border.

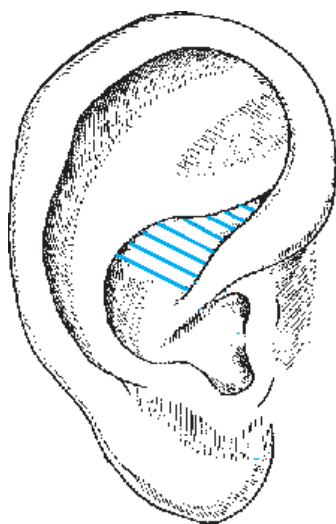


Fig. 3.13
Jejunum/ileum
(small intestine)

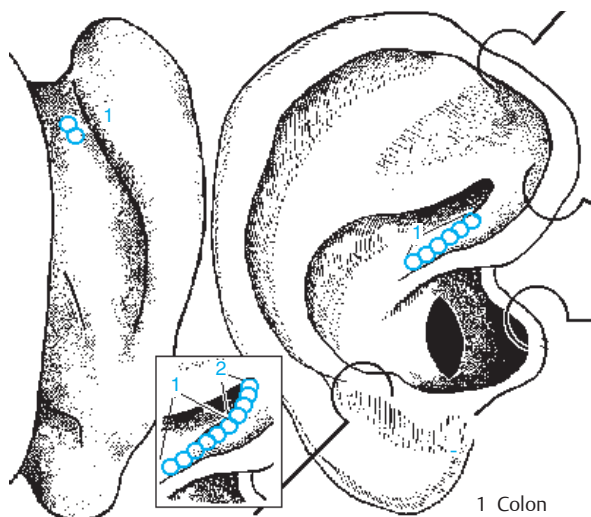


Fig. 3.14
Colon
(large intestine)

1 Colon
2 Rectum

Appendix

Location:

- Underneath the root of helix in the corner formed by the helical crus against the concha, at the medial border of the superior concha.
- The zone's location varies. This reflects the fact that the vermiform appendix extends upward behind the cecum in about 65% of people, while it reaches into the small pelvis in about 30%; it may also lie horizontally either in front of or behind the cecum.
- The size of the projection area depends on the size of the appendix in the body (2–20 cm in length).
- An important reflex area because it may indicate the activity of a focus. Following appendectomy, the area contains the projection area of both the internal and external (skin) scars.
- Depending on the part of the scar that exhibits focus activity, the respective active points will be found on the ear by using point finder or VAS.

Rectum

Location:

- A small area in the anterior medial part of the superior concha, hidden beneath the ascending helix.
- The area borders superiorly on the wall formed by the ascending helix.

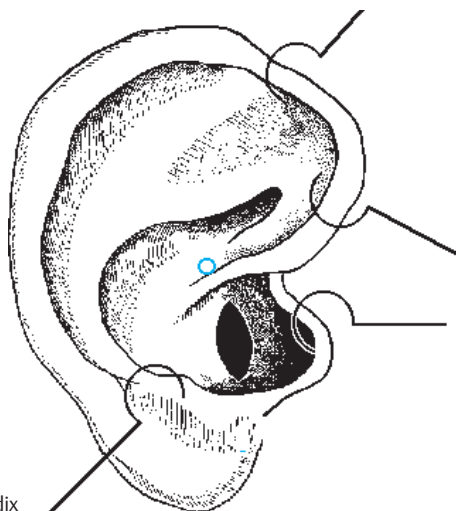


Fig. 3.15 Appendix

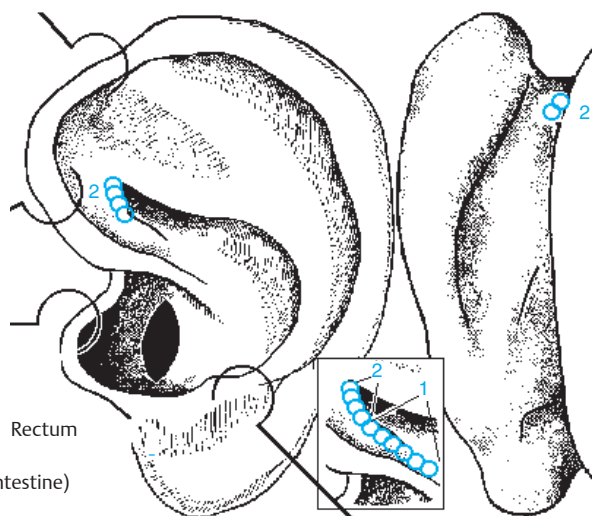


Fig. 3.16 Rectum

- 1 Colon
(large intestine)
- 2 Rectum

Anus

Location:

Internal anus (Hemorrhoid Point)

- The internal (**mucosal**) part of the anus projects near the Coccyx Point where the inferior antihelical crus begins underneath the ascending helix. An important point for treating hemorrhoids.

External anus

- The external part of the anus projects onto the anterior edge of the ascending helix (see inset). The point may be considered to be the forceps point to the Internal Anus Point (check it out if hemorrhoids are present).

Liver

Location:

- On the right ear in the mediolateral portion of the concha. A very small zone is also present on the left ear, but it is not used in therapy.

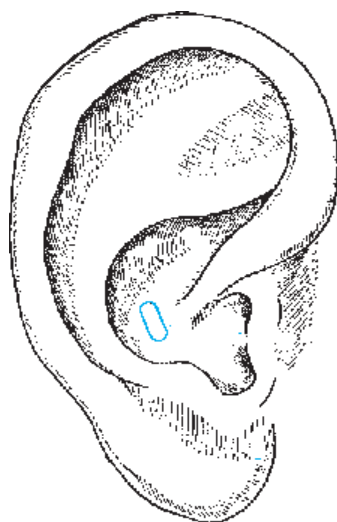
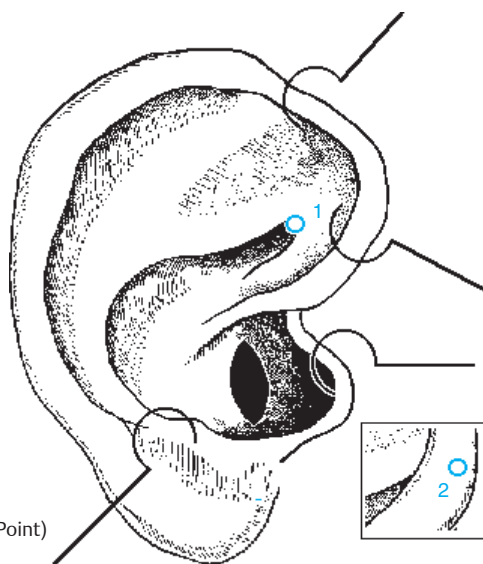


Fig. 3.18 Liver

Gall Bladder

Location:

- In the middle third of the superior concha.
- An important zone in the treatment of headache and migraine.

Pancreas

Location:

Parenchymal pancreas

- The reflex area of the parenchymal pancreas (**digestive enzymes**) is located medially to the Gall Bladder Point in the superior concha. The larger portion (head of pancreas) projects onto the right ear and the smaller portion onto the left ear.

Endocrine pancreas (Insulin Point)

- The reflex area of the endocrine pancreas (**islets, insulin production**) belongs to the series of points for endocrine organs (see Functional Points, p. 195).
- In the antihelical wall at the transition from the upper third to the middle third of the wall, approximately at the level of Point T12.

Corresponds to Point TB-4 of body acupuncture (p. 328).

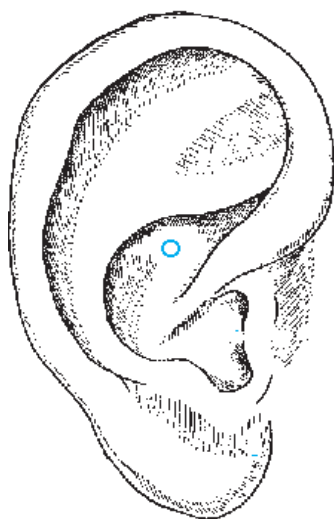


Fig. 3.19 Gall bladder

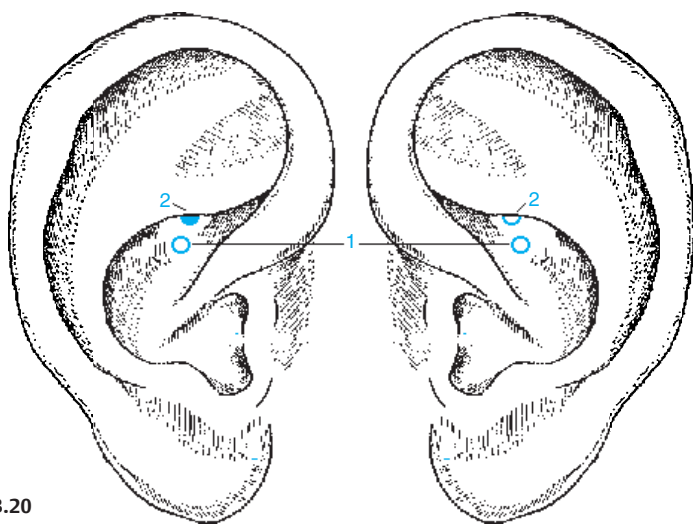


Fig. 3.20

- 1 Parenchymal pancreas
- 2 Endocrine pancreas

Spleen

Location:

- In the superior concha of the left ear, slightly below the Pancreas Zone.

Thymus Gland

Location:

- Belongs to the series of points for endocrine organs (see Functional Points, p. 195). In the antihelical wall at the transition of the upper third to the middle third of the wall, approximately at the level of Point T4.

The right ear bears the Silver Point (use gold needle on the left ear).

Application:

- Has an overriding effect on the body's defense system.

The most important anti-focal Functional Point in ear acupuncture. It has anti-inflammatory, antirheumatic, and antiallergic effects and stimulates the immune system.

Needling this point inactivates all other pathological points for a short period of time; therefore, this point should always be needled last.

Corresponds to Point TB-5 of body acupuncture (p. 328).

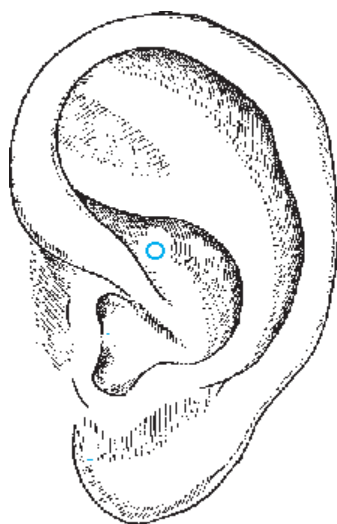


Fig. 3.21 Spleen

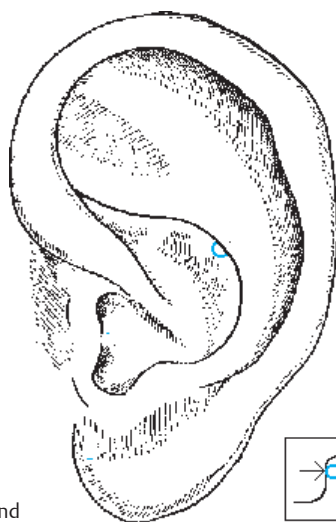


Fig. 3.22 Thymus gland



Tonsils

Location:

Palatine tonsil

- At the end of the scapha where the tail of the helical groove merges into the lobule. Near the TMJ Point, Molar Point, Depression Point, and Magnesium Point (see Head, p. 99).
- In order to distinguish between these reflex areas, specific points of body acupuncture and the cable method may be enlisted.

Pharyngeal tonsil

- Slightly superior to the Palatine Tonsil Point.

The pharyngeal tonsils are also known as polyps when enlarged; this is of importance in children.

Lymph Nodes

Location:

- The lymph vessels and also the lymph nodes basically project near the reflex areas of the structures they supply (see also Blood Vessels, p. 59).

In ear acupuncture, the lymph vessels and lymph nodes rarely need to be treated. As the therapist can easily deduce their representations, these are not described here in detail. If a lymph node or a segment of a lymph vessel is affected, the active points of the respective anatomical structure are searched for by using point finder or VAS.

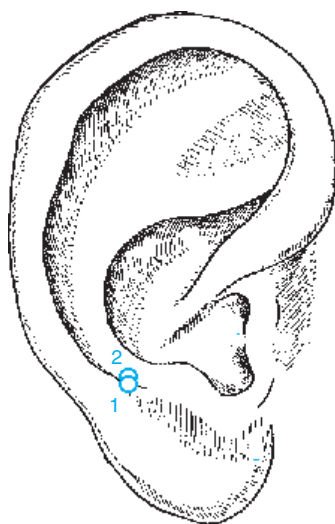


Fig. 3.23 Tonsils
1 Palatine tonsil
2 Pharyngeal tonsil



Fig. 3.24 Lymph nodes

Urogenital System—Overview

Location:

- The genital organs and the kidneys project onto, or underneath, the ascending helix, while the bladder and ureter are represented in the upper portion of the superior concha.

Kidney

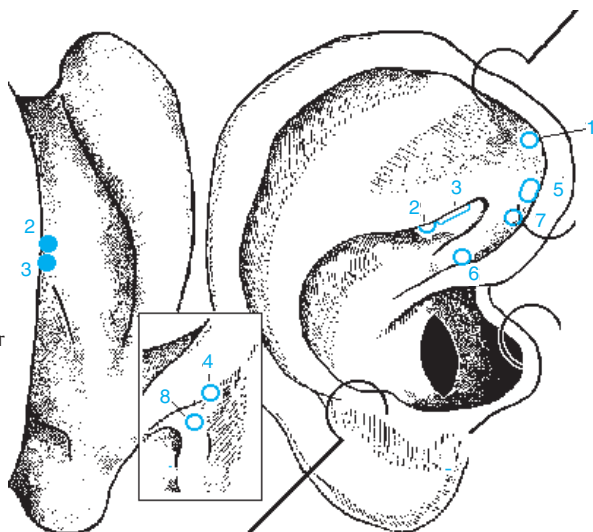
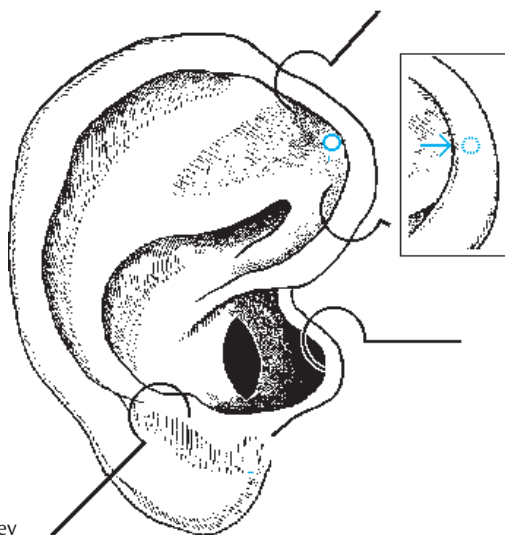
Location:

- On the inside of the ascending helix (right in the concavity) at the level of the triangular fossa (Knee Joint Point).

Auxiliary line: Where the horizontal line through the Knee Joint Point intersects the ascending helix. The kidneys are projected in the reflex area of the mesoderm because they are of mesodermal origin.

Fig. 3.25

- 1 Kidney
- 2 Urinary bladder
- 3 Ureter
- 4 Urethra
- 5 Uterus
- 6 Ovary/testis
- 7 Prostate/fallopian tube
- 8 Penis/clitoris

**Fig. 3.26** Kidney

Ureter

Location:

- **Sensory portion:** Bordering medially on the Urinary Bladder Point.
- **Motor portion:** On the back of the ear near the attachment of the ear, inferior to the Urinary Bladder Point. Usually present as Silver Point in case of ureter spasms.

Urinary Bladder

Location:

- **Sensory portion:** In the upper region of the superior concha adjacent to the antihelical wall, at the level of the upper points of the Lumbar Spine Zone.
- **Motor portion:** On the back of the ear in the upper third of the auricle, near the attachment of the ear to the facial skin.

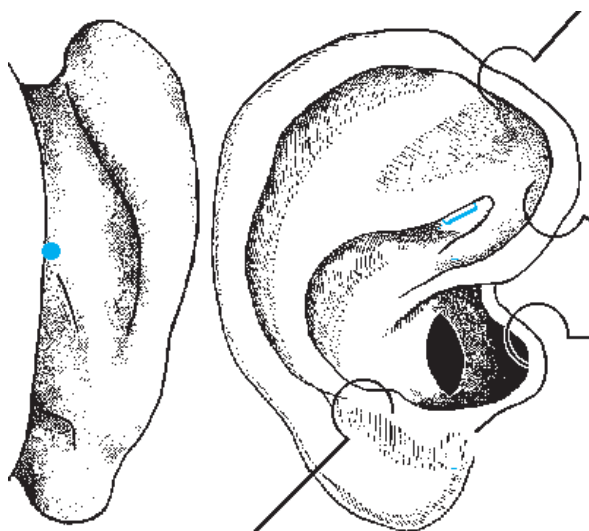


Fig. 3.27 Ureter

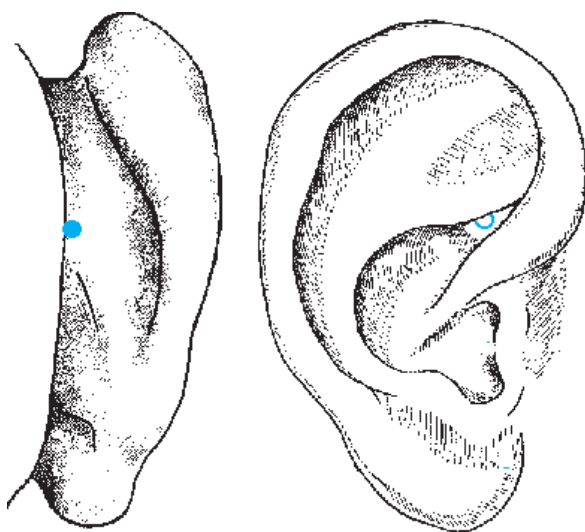


Fig. 3.28 Urinary bladder

Urethra

Location:

- The main portion of the urethra projects at the anterior edge of the ascending helix where the cartilaginous border is palpable under the skin. The point connects with the Urinary Bladder Point underneath the ascending helix in the superior concha.

Uterus

Location:

- On the inside of the ascending helix, near the Prostate Point and above the Ovary Point. The zone increases in size during pregnancy.

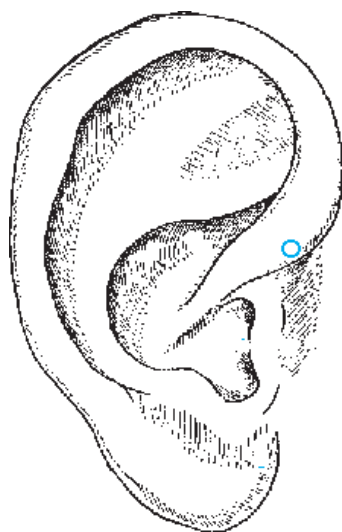


Fig. 3.29 Urethra

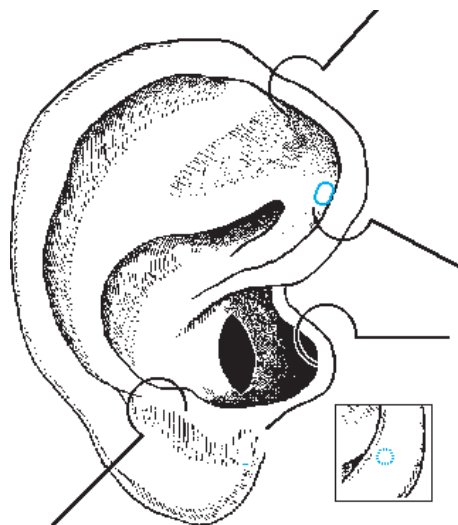


Fig. 3.30 Uterus

Ovary/Testis

Location:

- On the inside (underside) of the ascending helix about 2 mm away from the anterior edge, approximately midway between the Navel Point (Point Zero) and the Uterus Point.

Corresponds to Point SP-5 of body acupuncture (p. 322), has an effect similar to that of Point CV-7 of body acupuncture.

Vagina

Location:

- On the anterior edge of the ascending helix where the cartilaginous border is palpable below the skin, slightly superior to the Urethra Point.

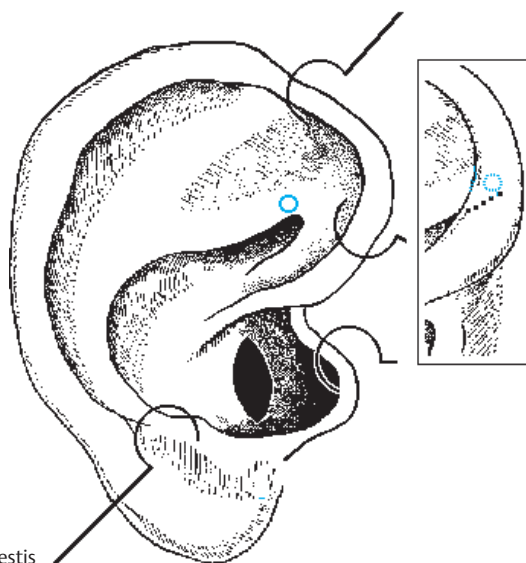


Fig. 3.31 Ovary/testis

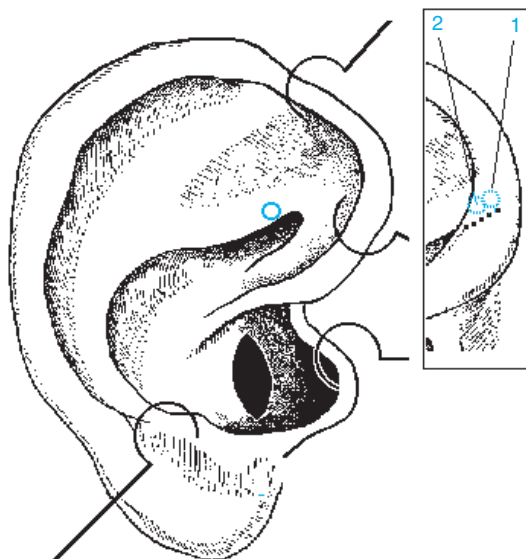


Fig. 3.32 Vagina

Prostate/Fallopian Tube

Location:

- On the inside of the ascending helix where the antihelix meets the ascending helix, about 2 mm away from the helical rim.

Penis/Clitoris

Location:

- In front of the supratragic notch at the anterior edge of the ascending helix where the cartilaginous border is palpable below the skin. Below the Urethra Point.

Identical with the Frustration Point.

Caution: This point is often mixed up with the Interferon Point in the supratragic notch (see Functional Points, p. 209 and Auxiliary Lines, p. 293).

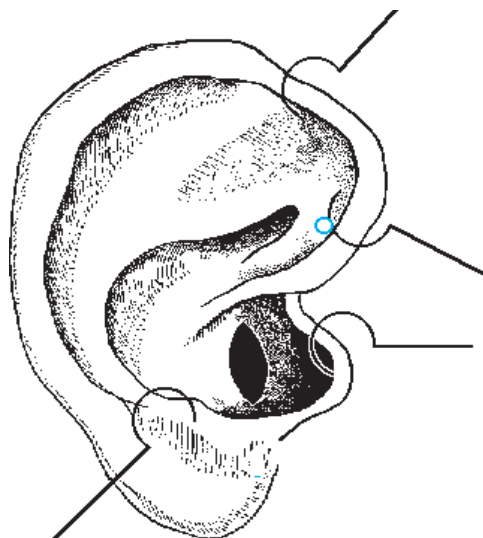


Fig. 3.33 Prostate/fallopian tube

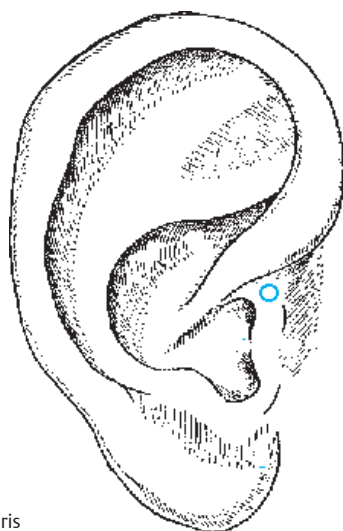


Fig. 3.34 Penis/clitoris

Endocrine System—Overview

The endocrine system includes the thyroid gland, the parathyroid gland, the cortex and medulla of the adrenal gland, the endocrine part of the pancreas, and the gonads. Those parts of the pituitary gland that belong to the endocrine system are discussed under Nervous System, p. 135.

The endocrine cells of the gastrointestinal tract and of the kidneys (juxtaglomerular apparatus) are discussed together with the respective organs.

Thyroid Gland

Location:

Parenchymal thyroid gland

- On the antihelix at the level of Points C5–C7. Not only the organ itself is represented here but also the skin scar caused by strumectomy (a possible focus).

Endocrine thyroid gland

- Belongs to the series of points for endocrine organs (see Functional Points, p. 203).
- In the antihelical wall at the transition from the upper third to the middle third of the wall, at the level of Point Zero.

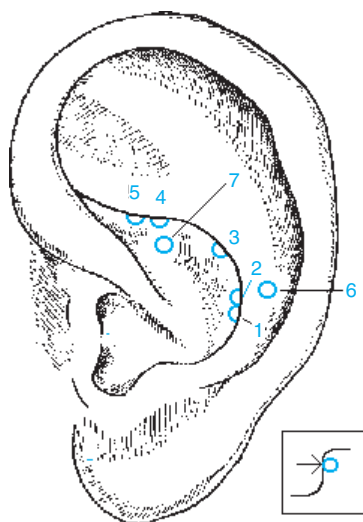
Auxiliary line: The horizontal line through Point Zero intersects with the antihelical wall at the indicated level (see Auxiliary Lines, p. 299).

The right ear normally bears the Silver Point (the needle metal needs to be decided depending on the functional state of the thyroid gland).

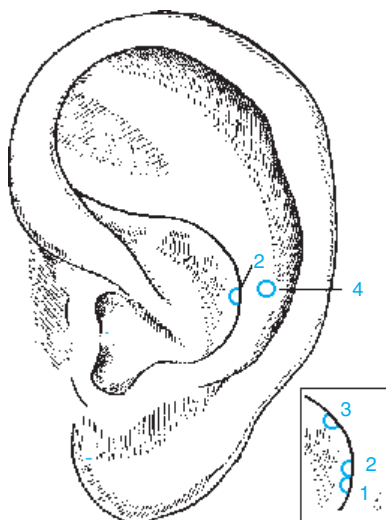
Application: Supportive effect in diseases of the thyroid gland. The goiter as well as the “normal” thyroid gland may function as a focus. Needling this point may substitute injections into the thyroid gland (neural therapy). Corresponds to Point TB-6 of body acupuncture.

Fig. 3.35

- 1 Endocrine parathyroid gland
- 2 Endocrine thyroid gland
- 3 Thymus gland
- 4 Endocrine pancreas (Insulin Point)
- 5 Adrenal gland (Cortisone Point)
- 6 Parenchymal thyroid gland
- 7 Parenchymal pancreas

**Fig. 3.36** Thyroid gland

- 1 Endocrine parathyroid gland
- 2 Endocrine thyroid gland
- 3 Thymus gland
- 4 Parenchymal thyroid gland



Parathyroid Gland

Location:

Parenchymal parathyroid gland

- Near the Thyroid Gland Point, on the antihelix at the level of Points C5–C7.

Endocrine parathyroid gland

- In the antihelical wall like the Thyroid Gland Point but at the level of Point C6.

The right ear bears the Silver Point (use gold needle on the left ear).

Supportive in functional disorders of the parathyroid gland.

Corresponds to Point TB-7 of body acupuncture (p. 328).

Thymus Gland

Location:

- In the antihelical wall like the Thyroid Gland Point, but approximately at the level of Point T4.

The right ear bears the Silver Point (use gold needle on the left ear).

The thymus gland is listed among the series of points for endocrine organs because it secretes, among other things, a humoral factor which stimulates the maturation of the reticular tissue of lymphatic organs. This, in turn, leads to the formation of immunocompetent cells (immunoblasts).

Application: The most important antifocal Functional Point in ear acupuncture. It has anti-inflammatory, antirheumatic, and antiallergic effects and stimulates the immune system.

Needling this point inactivates all other pathological points for a short period of time; therefore, this point should always be needled last.

Corresponds to Point TB-5 of body acupuncture (p. 328).

Fig. 3.37 Parathyroid gland
1 Endocrine parathyroid gland
2 Parenchymal parathyroid gland
3 Endocrine thyroid gland
4 Thymus gland

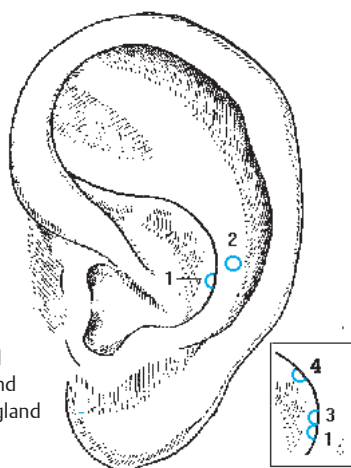
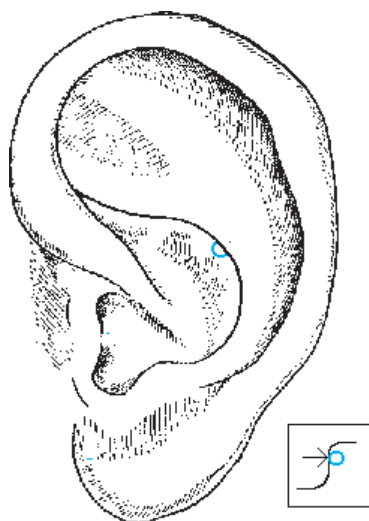


Fig. 3.38 Thymus gland



Pancreas

Location:

Parenchymal pancreas

- On the right ear medial to the Gall Bladder Point in the superior concha; on the left ear approximately in the area of the Spleen Point. The larger portion (representing the head of pancreas) is on the right ear, the smaller portion on the left ear.

Endocrine pancreas (Insulin Point)

- Belongs to the series of points for endocrine organs (see Functional Points, p. 205).
- In the antihelical wall at the transition from the upper third to the middle third of the wall, approximately at the level of Point T12.

The right ear bears the Silver Point (use gold needle on the left ear).

May be tried in diabetes mellitus.

Corresponds to Point TB-4 of body acupuncture (p. 328).

Adrenal Gland (Cortisone Point)

Location:

- In the antihelical wall like the Endocrine Pancreas Point and the Thymus Gland Point, but approximately at the level of Point L1.

The right ear bears the Silver Point (use gold needle on the left ear).

The point has a cortisone-like effect and is therefore indicated in all allergic disorders.

Corresponds to Point TB-3 of body acupuncture (p. 328).

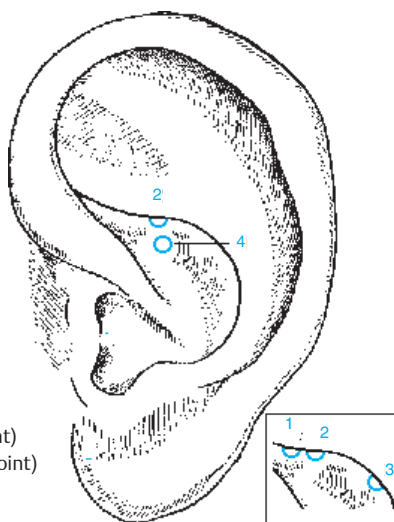


Fig. 3.39 Pancreas

- 1 Adrenal gland (Cortisone Point)
- 2 Endocrine pancreas (Insulin Point)
- 3 Thymus gland
- 4 Parenchymal pancreas

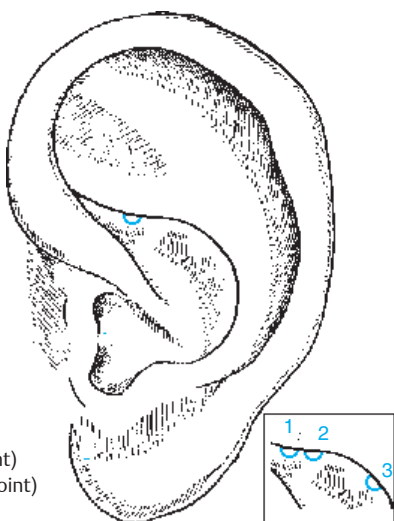


Fig. 3.40 Adrenal gland

- 1 Adrenal gland (Cortisone Point)
- 2 Endocrine pancreas (Insulin Point)
- 3 Thymus gland

Head—Overview

The reflex areas of the head are localized on the lobule and the descending helix. They include the projections of the nose, eyes, ears, tongue, teeth, tonsils, lymph nodes, maxillary sinuses, and frontal sinuses.

Also the brain projects onto the lobule; it is discussed under Nervous System (p. 121).

The entire head projection deviates from all other reflex areas on the ear. Instead of the usual upside-down arrangement, the head organs project in about the same arrangement as in an upright-standing person.

The upright arrangement of the head's reflex zones becomes clear when imagining the famous "inverted embryo" in an orientation similar to the face presentation within the uterus. In this position, the lower jaw necessarily projects **caudally**, not cranially, relative to the upper jaw. Likewise, the nose projects **caudally** to the mucous portion of the paranasal sinuses.

Bony Skull

Location:

- On the outer surface of the antitragus. The reflex area of the occipital bone (Occiput Point) borders immediately on that of the uppermost cervical vertebra (Point C1 or Atlas Point).

The postantitragal fossa forms the boundary between the projections of the uppermost cervical vertebra (Point C1) and the base of the skull. Here is the reflex area of the atlanto-occipital joint (Point C0/C1).

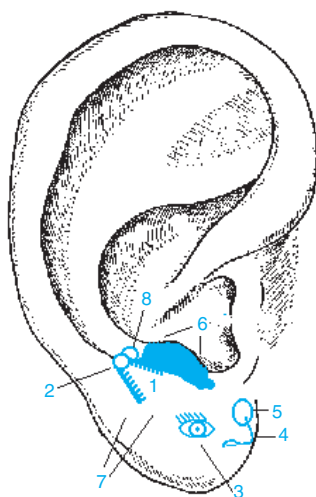
The other cranial bones project anteriorly and inferiorly to the occipital bone:

- the frontal bone and sphenoid bone in the anterior third,
- the parietal bone and temporal bone in the middle third, and
- the occipital bone in the posterior third of the zone.

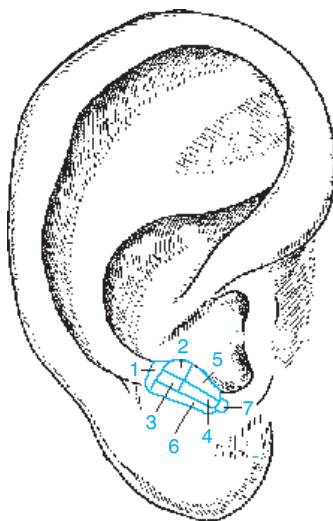
The reflex area of the maxilla (Upper Jaw Zone) forms the caudal border of the reflex zone of the cranial bones.

Fig. 4.1

- 1 Upper and lower jaws
- 2 Projection of throat, palatine tonsil, pharyngeal tonsil, and lymphoid ring
- 3 Eye
- 4 Nose
- 5 Maxillary and frontal sinuses
- 6 Bony skull
- 7 Tongue
- 8 Ear

**Fig. 4.2**

- 1 Occipital bone
- 2 Parietal bone
- 3 Temporal bone
- 4 Sphenoid bone
- 5 Frontal bone (Forehead Point)
- 6 Maxilla
- 7 Frontal sinus



Temporomandibular Joint

Location:

- At the end of the scapha where the helical groove merges into the lobule.

The following points are located very close to the Temporomandibular Joint (TMJ) Point and therefore indistinguishable from it when starting the diagnosis:

- Palatine Tonsil Point,
- Molar Point (for the molars of upper and lower jaws),
- Retromolar Cavity Point,
- Posterior Masseter Muscle Point (sensory portion),
- Depression Point, and
- Magnesium Point.

At the same location are also the points for

- the central portion of the deep and superficial **parotid gland** (affected only occasionally), and
- the insertion of the **lateral pterygoid muscle** (which adducts discus and lower jaw, causes tenseness during bruxism).

To distinguish between these reflex areas, distinct points of body acupuncture may be enlisted, or the assignment on the head or face may be made by means of the VAS.

See also B. Strittmatter, *Overcoming Blockages to Healing*, Thieme Publishers, Stuttgart–New York, 2004.

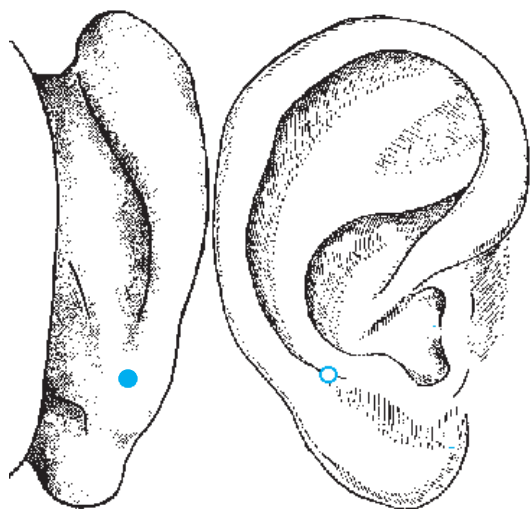


Fig. 4.3 Temporomandibular joint

Upper and Lower Jaws, Teeth

Location:

- Starting from the TMJ Point, the upper and lower jaws project onto the laterosuperior lobule.
- The location of the reflex area of the maxilla (**Upper Jaw Zone**) has been described above (p. 99).
- The teeth of the upper jaw project along the inferior border of the Upper Jaw Zone.
- The reflex area of the mandible (**Lower Jaw Zone**) opens in anteroinferior direction at an angle of about 40° relative to the Upper Jaw Zone (upright projection of the head structures), with the TMJ Point forming the hinge between the Upper Jaw Zone and Lower Jaw Zone.
- The teeth of the lower jaw project along the superior border of the Lower Jaw Zone (**Tooth Points**).
- Right next to every Tooth Point are the reflex areas of all structures belonging to the tooth (periodontium, gum, pulp, etc.)

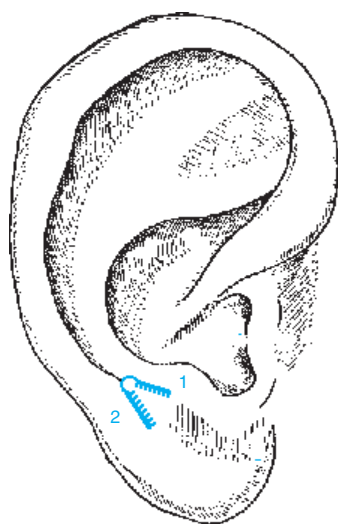


Fig. 4.4

- 1 Upper jaw
- 2 Lower jaw

Lips

Location:

- To be searched for in the region of the Tooth Points: near the projections of the 1st–5th teeth in front of both Upper Jaw Zone and Lower Jaw Zone.

Oral Cavity, Tongue

Location:

- The **Oral Cavity Zone** is located anterior to the TMJ Point between the Upper Jaw Zone and the Lower Jaw Zone.
- The **Tongue Zone** is also located here and extends across the lower edge of the lobule onto the back of the ear for almost 1 cm.
- As is the case with the Finger Zone, the size of this reflex area on the ear reflects the large number of sensory receptors and motor units.

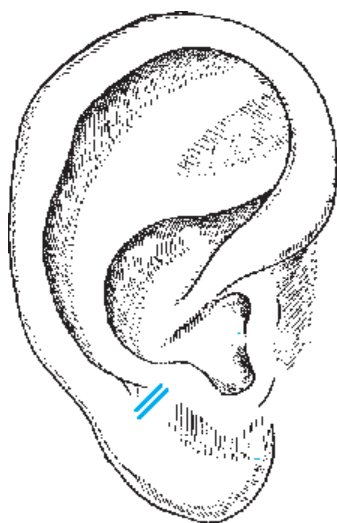


Fig. 4.5 Lips

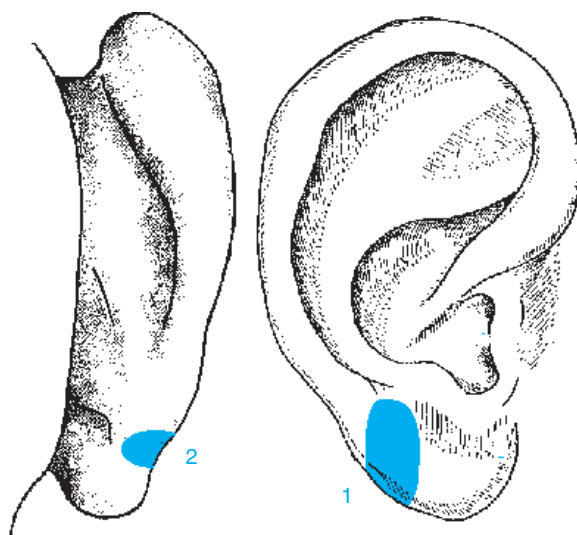


Fig. 4.6

- 1 Tongue (sensory portion)
- 2 Tongue (motor portion)

Tonsils

Location:

- At the lower end of the descending helix near the TMJ Point.
- The **Palatine Tonsil Point** is located slightly more caudally than the **Pharyngeal Tonsil Point** (the pharyngeal tonsils are known as polyps when enlarged).

Throat (Pharynx)

Location:

- Although the tonsils together with most of their draining lymph vessels project onto the lateral region of the lobule, the reflex area of the throat proper (Throat Point) is located in the inferior concha in the corner formed by the ascending helix and the upper end of the tragus.
- This is an ear region where a chain of reflex localizations (Stomach Zone → Esophagus Zone → Throat Point) stops abruptly and continues at another site.

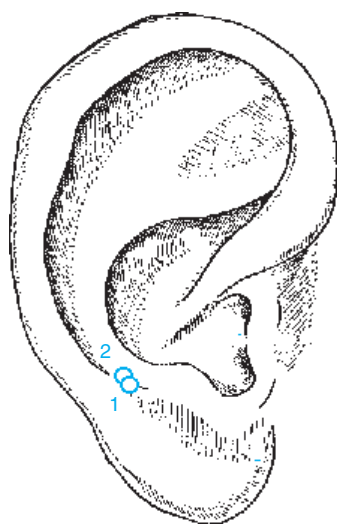


Fig. 4.7

- 1 Palatine tonsil
- 2 Pharyngeal tonsil

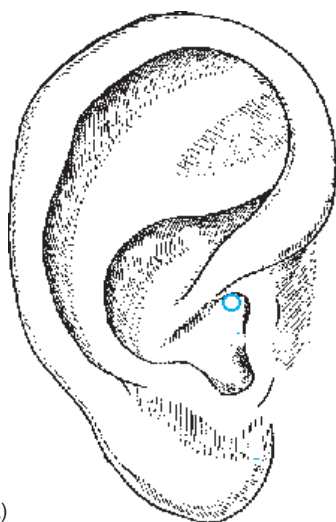


Fig. 4.8 Throat (pharynx)

Eye

Location:

- In the center of the lobule where many people wear an earring.

(Sometimes the piercing for an earring is also done at the Nose Point in the anterior region of the lobule.)

When imagining a circular area occupying the entire lobule, the Eye Point forms the center of the circle.

Near this point are also the reflex areas of all structures belonging to the eye, such as cornea, eye lashes, eyelids, retina, and also the extraocular muscles (with the motor portions represented, as always, on the back of the ear).

Nose

Location:

- At the front edge of the lobule, approximately at the level where the lobule attaches to the facial skin.

However, the projection of old fractures of the nasal bone (i.e. bone scars that may act as foci) are found more prominently on the anterior surface of the antitragus in the region of the Forehead Point (see Bony Skull, p. 99).

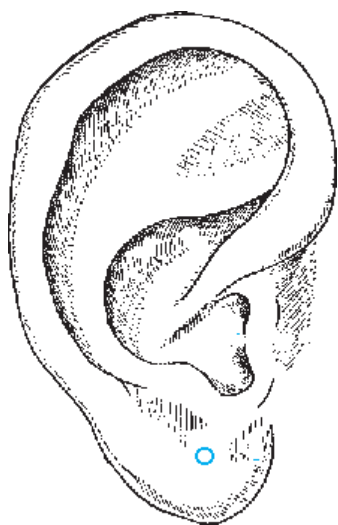


Fig. 4.9 Eye

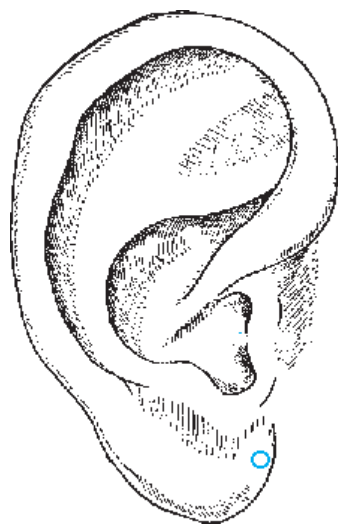


Fig. 4.10 Nose

Paranasal Sinuses

Location:

- Just superior to the Nose Point.
- Here are the reflex areas of the maxillary sinus, sphenoidal sinus, ethmoidal sinuses, and the frontal sinus. They all represent the mucosal portion of these sinuses.

Hence, in cases of inflammation or purulence, a far more distinct response will be obtained at this site than in the projection area of air cavities in the cranial bones (see Bony Skull, p. 99). In children and infants, focus activity primarily occurs in the region of the **ethmoidal sinuses**.

In newborns, the maxillary sinus and the ethmoidal sinuses are still small, and the frontal sinus and sphenoidal sinus have not yet developed. Growth of the sinuses is not complete before the ages 15–20. It is useful to know about this developmental sequence when treating the focus with a laser beam.

Ear

Location:

- At the end of the scapha, in the posterior region of the Auditory Line. All structures belonging to the ear project close to this point.

Fig. 4.11

- 1 Nose
- 2 Paranasal sinuses
(mucosal portion)
(Cranial: frontal sinus)
(Caudal: maxillary sinus)

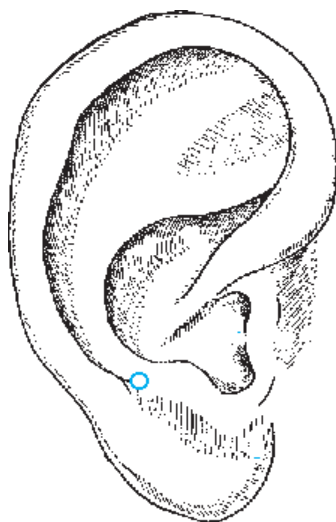
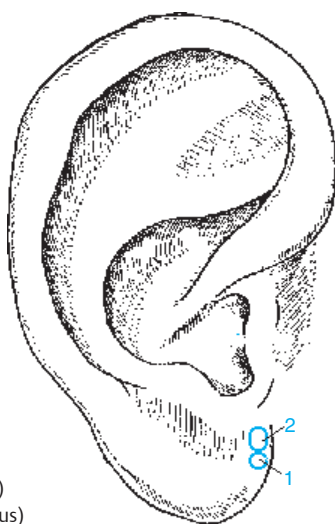


Fig. 4.12 Ear

Facial Muscles

Knowing the reflex localizations of the facial muscles can be important in the treatment of active trigger points and partial paralyses.

The projections have to be searched for in the reflex zone of the respective structure of the head or face.

(For example: the orbicular muscle of the mouth projects near the Oral Cavity Zone, the levator muscle of the upper eyelid projects near the Eye Point, and the lateral pterygoid muscle projects near the TMJ Point.)

Parotid Gland

Location:

- Near the Palatine Tonsil Point, slightly more lateral.

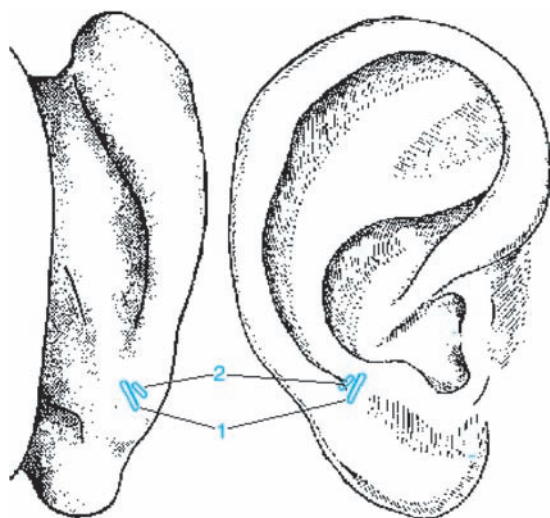


Fig. 4.13

1 Masseter muscle

2 Lateral pterygoid muscle

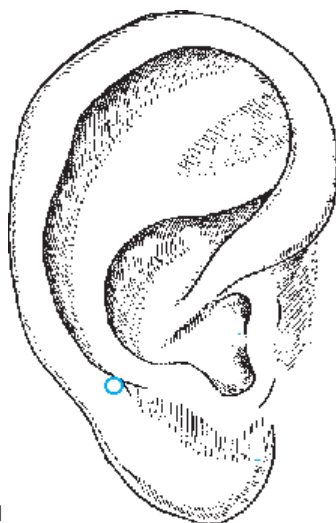


Fig. 4.14 Parotid gland

Occipital Trigger Points

The postantitragal fossa forms the boundary between the projections of the uppermost cervical vertebra (Point C1) and the base of the skull. Here is the reflex area of the atlanto-occipital joint (Point C0/C1).

In many cases of **acute** migraine the patient bears at this site an active point that needs treatment (use gold needle).

Trigger points of inserting neck muscles also project in the zone of the cranial base, and their reflex points are palpable during a migraine attack. If one or more such points are found (usually these are Gold Points because they are painful), one should also search for the corresponding Silver Points on the back of the ear; they indicate myogelosis (treat with silver needles).

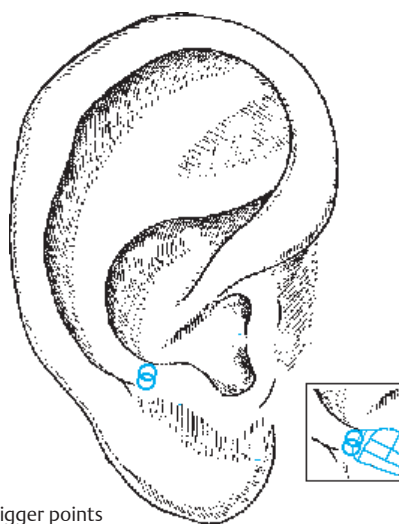


Fig. 4.15 Occipital trigger points

Because of the vagueness of the ear map in effect at that time, Bahr asked me in 1994 to check the accuracy of the localizations of the central nervous system, the cranial nerves, and the autonomic nervous system (VAS-controlled testing by means of resonance to the respective anatomical material, using Wala test ampoules).

This reevaluation resulted in new localizations on the ear for some structures of the nervous system (e.g., the hypothalamus, some cranial nerves, the nuclei of origin, and some ganglia of the autonomic nervous system).

Central Nervous System—Overview

Location:

- The **central nervous system** projects essentially onto the descending helix and the ear lobe (localization of the ectoderm, see Germ Layers, p. 8).
- Here, too, the general rule applies: the sensory portion of the reflex zone is located on the front of the ear and the motor portion on the back of the ear.

The projection of the head of the inverted embryo fills the entire lobule (Nogier). Accordingly, the main portions of the **brain** and the **head** (sensory organs, temporomandibular joint, teeth, maxillary sinuses, facial muscles) are represented on the ear lobe (see Germ Layers, p. 8, and Head, p. 99).

The **brain** projects onto the entire ear lobe.

The **bony skull** projects only onto the antitragus; the area is not identical with the projection of the brain.

The projected embryo lies with its back against the posterior edge of the ear (descending helix). Accordingly, the reflex zone of the **spinal cord** is in this area (sensory portion on the front of the ear, motor portion on the back of the ear).

Certain parts of the central nervous system are not represented on the descending helix and lobule, namely, the **commissural fibers**, the **thalamus**, and the **hypothalamus**. Their exact localizations will be described in the respective sections.

Note: Because many of the reflex zones will not come to mind easily in the daily clinical routine or practice, possible indications are also provided in this chapter.

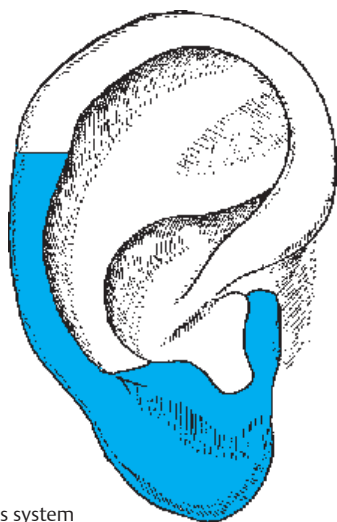


Fig. 5.1 Central nervous system

Cerebral Lobes—Overview

As a refresher and for a better understanding of the following description of auriculomedical details:

The **precentral area** is especially important in all neurological disorders that affect motor functions in one way or another (this area is often neglected in auriculotherapy).

Of equal importance is the **postcentral area** on the most anterior gyrus of the parietal lobe, because it is the terminal site of all sensory fibers.

Cortex

Location:

- The four large lobes of the telencephalon (frontal, temporal, parietal, and occipital lobes) project onto the anterior and middle ear lobe and, in part, onto the outer surface of the antitragus.

Indications:

- See discussion of the individual cerebral lobes.

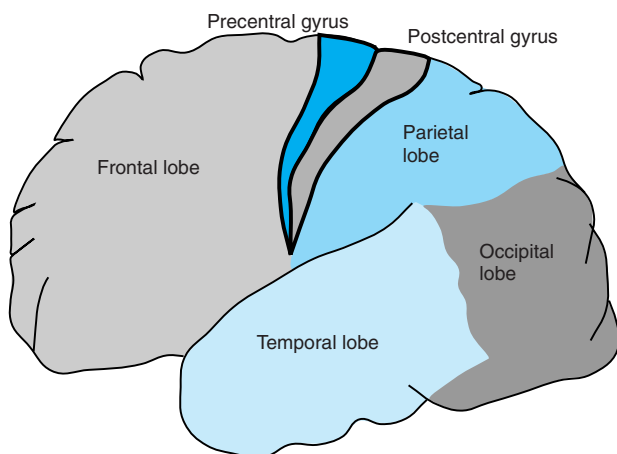


Fig. 5.2

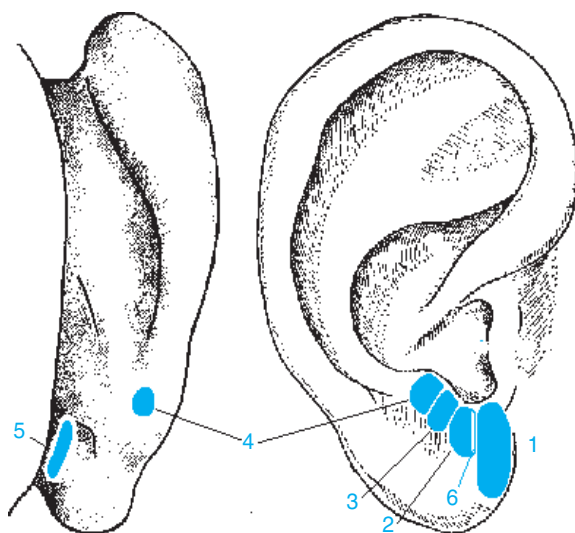


Fig. 5.3

- 1 Frontal lobe
- 2 Parietal lobe
- 3 Temporal lobe
- 4 Occipital lobe
- 5 Precentral gyrus
- 6 Postcentral gyrus

Frontal Lobe

Location:

- This is the area in the most anterior part of the ear lobe, below the intertragic notch and reaching down to the Main Omega Point.
- The **precentral gyrus** projects on the back of the ear near the attachment of the ear lobe.
- Broca's **motor speech area** projects on the back of the ear near the Precentral Gyrus Zone (search for the active point).

Indications:

- In case of hemorrhage in, or injury to, the frontal lobe it makes sense to try supportive therapy by ear acupuncture (especially when the motor speech area is affected).
- Combination of ear acupuncture with laser acupuncture on the skull (in the corresponding region) may be favorable.
- An affected area of the precentral gyrus can be directly influenced, for example, in infantile cerebral palsy and the like (use the reflex zone on the back of the ear).

Note: The **frontal lobe** includes:

- the **precentral area (precentral gyrus)**, hierarchically the highest motor center where the pattern of movement is designed and transmitted to deeper-lying centers (basal ganglia, cerebellum, etc., where the somatotopic arrangement results in an inverted homunculus);
- the **frontal eye field**, through which conjugated eye movements are controlled independently of the eye-muscle nuclei;
- Broca's **motor speech area** in the inferior frontal gyrus of the dominant hemisphere.

Damage (injury, hemorrhage) causes **motor aphasia**, i.e., the patient is no longer able to form words or to speak, although the speech muscles are not paralyzed and speech comprehension is intact.

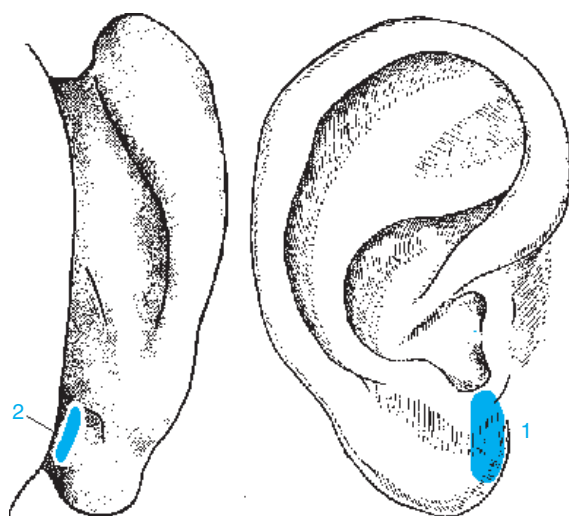


Fig. 5.4

- 1 Frontal lobe
- 2 Precentral gyrus

Parietal Lobe

Location:

- On the ear lobe, lateral to the Frontal Lobe Zone.
- The **postcentral gyrus** projects onto the boundary between these two zones.

Indications:

- For example, as supportive therapy in case of injury to this region.

Note: The **postcentral gyrus** is the most anterior gyrus of the parietal lobe where all sensory fibers terminate (somatosensory cortex).

Impairment in the region of the parietal lobe may result in various forms of **agnosia**. While sensations are still perceived, objects are no longer recognized as what they are.

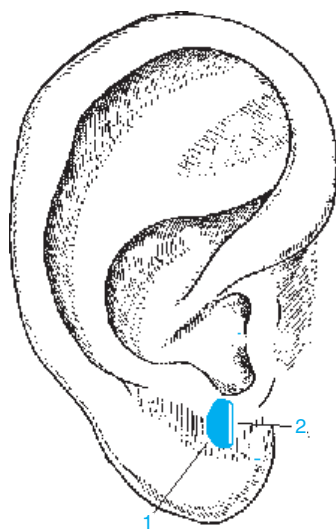


Fig. 5.5

- 1 Parietal lobe
- 2 Postcentral gyrus

Temporal Lobe

Location:

- On the ear lobe and lower edge of the antitragus, lateral to the Parietal Lobe Zone.

Of particular importance for the practical approach:

- The auditory area of the temporal lobe projects as Auditory Line on the ear lobe.

Indications:

- As supportive therapy in case of injury to this region, especially when **Wernicke's speech area** is affected.
- Special forms of tinnitus.

Note: Wernicke's speech area is located in the posterior region of the superior temporal gyrus of the dominant hemisphere. Injury causes deficit in word comprehension (**sensory aphasia**): words are used in senseless combinations (word salad, empty speech), language comprehension is impaired.

Injury to the angular gyrus causes word blindness, i.e., inability to read (**alexia**) and to write (**agraphia**).

The **acoustic radiation** (with origin in the lateral geniculate body) terminates in the transverse temporal gyrus, deep below the operculum.

Auditory Line

Location:

- A horizontal line on the antitragus (Nogier described this line before he even knew the exact projection of the cerebral lobes).
- The line needs to be pierced with several needles on the ear of the affected side, in most cases with silver needles (it is essential to decide on the needle metal by using the point finder).

Application:

- Tinnitus, Ménière's disease, sudden deafness.

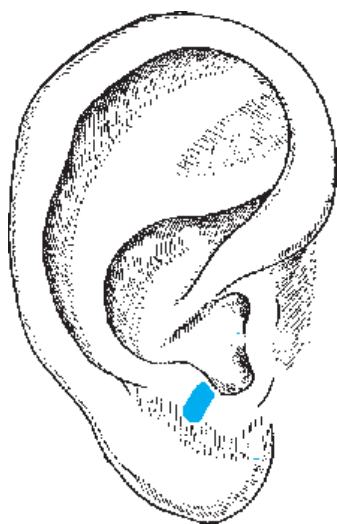


Fig. 5.6 Temporal lobe

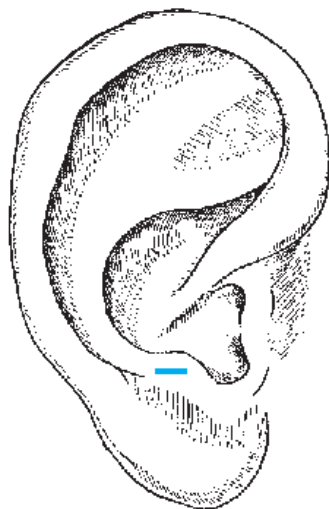


Fig. 5.7 Auditory Line

Occipital Lobe

Location:

- On the outer surface of the antitragus, also on the back of the ear in the corresponding area.

Indications:

- As accompanying therapy in **hemianopia**, for example, in case of infarction of the occipital lobe. The **optic radiation** terminates here in the calcarine sulcus.

Corpus Callosum

Location:

- The corpus callosum projects onto the entire tragus.

Indications:

- Especially important in all disorders associated with problems of laterality (e.g., bed-wetting, childhood asthma, learning difficulties, suppressed left-handedness).
- Laterality problems may result from disease, but may also cause disease.

Note: Physiologically, the corpus callosum plays an important role in **balancing the two cerebral hemispheres**.

It is related to **laterality** and laterality disorders because it is the most important and largest commissure of the cortex:

The commissural fibers crossing through the corpus callosum connect the cortices of both hemispheres (left–right connections).

These connections involve similar as well as different regions of the two hemispheres.

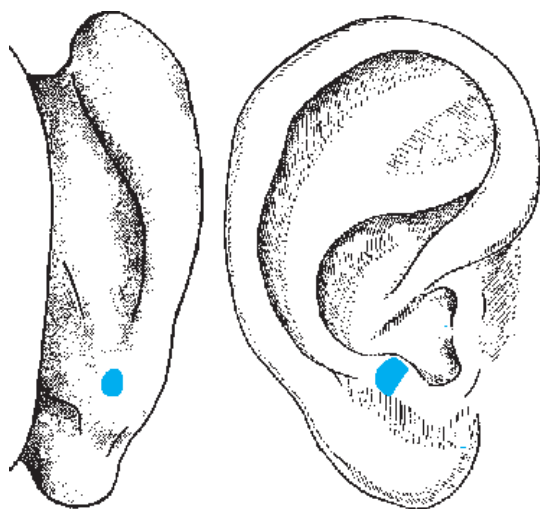


Fig. 5.8 Occipital lobe

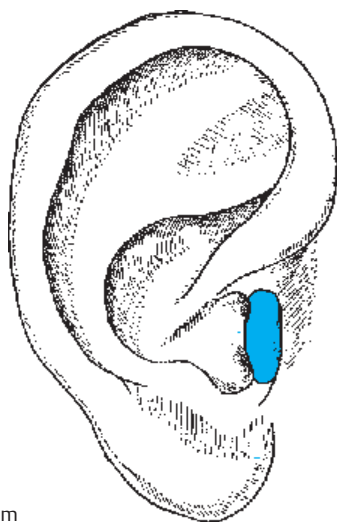


Fig. 5.9 Corpus callosum

Thalamus

Location:

- On the inside of the antitragus where the inferior concha merges with the ascending wall of the antitragus.
- When pulling the antitragus outside, a depression becomes visible on its inner wall. The Thalamus Point is located at its deepest point.

Indications:

- In all severe conditions of pain, possibly as a supplementary point.
- Corresponds to Point LI-4 of body acupuncture.

Note: The thalamus (Greek: *thalamos*, inner chamber) may be regarded as gateway and distribution station for all afferent systems to the phylogenetically younger telencephalon structures which permit targeted and conscious behavior.

It is therefore also known as the “gate to consciousness.” It plays a key role in processing and modulating pain.

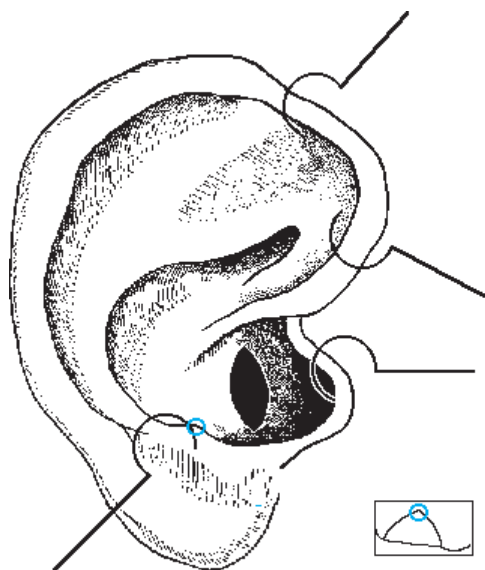


Fig. 5.10 Thalamus

Hypothalamus

Location:

- Superior and lateral to the Thalamus Point, in the wall ascending from the inferior concha to the antitragus.

Indications:

Still a neglected reflex area in auriculotherapy, it should be tested in the following conditions (the needle metal needs to be decided):

- arterial hypertension and hypotension,
- insomnia,
- overweight and eating disorders,
- fertility disorders and other hormonal problems,
- sexual problems,
- excessive perspiration,
- autonomic imbalance and everything associated with it.

Note: The hypothalamus contains the higher **control centers of the autonomic nervous system** which coordinate the essential **regulatory processes** of the body:

- thermoregulation,
- sleeping-waking rhythm,
- regulation of blood pressure,
- regulation of respiration,
- food intake (feeding center and satiety center),
- fat metabolism,
- fluid balance (water and electrolytes),
- sexual function,
- perspiration.

The hypothalamus is the center of all the essential homeostatic processes of the body.

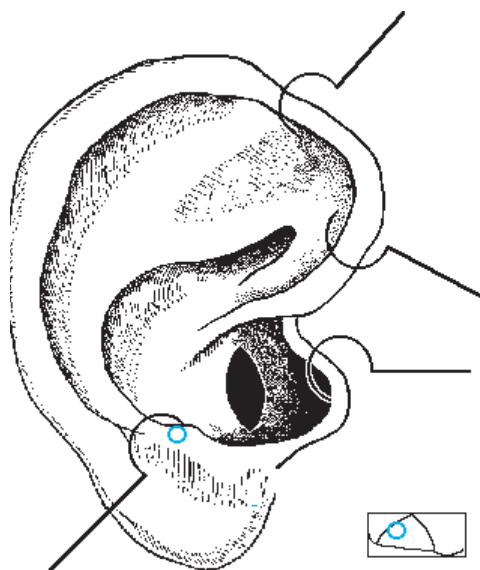


Fig. 5.11 Hypothalamus

Pituitary Gland

Location:

- The **adenohypophysis** (anterior pituitary gland) projects onto a relatively large area in and around the intertragic notch. Commonly used points, such as Prolactin Point, ACTH Point, TSH Point, Gonadotropin Point, are also found here.
- The **neurohypophysis** (posterior pituitary gland) projects lateral to the anterior part of the antitragus.

Indications:

- Analogous to the action of the individual hormones; for example, therapy when children are desired, hypermenorrhea, difficult menstruation, mastopathy, stimulation of lactation (Prolactin Point) or milk ejection (Oxytocin Point).
- The choice of needle metal is specified by the initial clinical situation and depends on the need for stimulation or sedation (use gold or silver needle, respectively).
- In any case, the point will need to be tested for its potential, as usual, by using the 3-volt-hammer or point finder.

Note:

- **Anterior lobe of pituitary gland (adenohypophysis):** production of FSH, LH, TSH, ACTH, GH (STH), prolactin.

With the exception of the two latter hormones, these are glandotropic hormones which control the function of other secondary endocrine organs. The secretion of hormones is controlled by the releasing hormones of the hypothalamus.

- **Posterior lobe of pituitary gland (neurohypophysis):** The hormones oxytocin (milk ejection) and vasopressin (regulation of blood pressure), which are produced by the hypothalamus, are stored here and released when needed (hypothalamic–pituitary system).

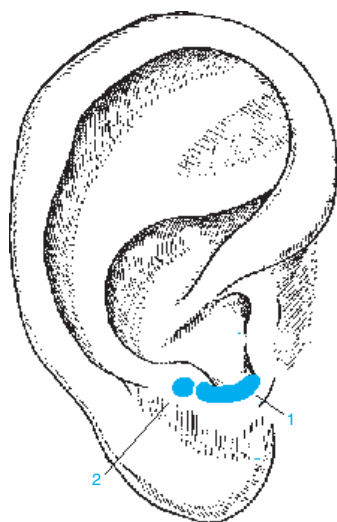


Fig. 5.12

- 1 Adenohypophysis
- 2 Neurohypophysis

Pituitary Gland Points

Prolactin Point

Location:

- When children are desired, the right ear usually bears the Silver Point (the needle metal needs to be decided).
- Just above the ACTH Point.

Indications:

- When children are desired (because hyperprolactinemia is often present, use silver needle on the right ear).
- Supports lactation (use gold needle).
- Hormonal disorders; for example, oligomenorrhea or amenorrhea associated with hyperprolactinemia (use silver needle).

ACTH Point

Location:

- The right ear normally bears the Gold Point.
- In the anterior corner of the intertragic notch where the notch ascends to the tragus.

Indications:

- ACTH stimulation; for example, in all cases of allergies.
- For supporting amalgam elimination use gold needle on the right ear.
- Corresponds to Point LR-13 of body acupuncture.

TSH Point

Location:

- Requires stimulation or sedation depending on the syndrome.
- Just behind the ACTH Point (in posterior direction toward the antitragus).

Indications:

- Predominantly diagnostic when latent hypothyroidism is suspected; supports the therapy in case of disease.

Gonadotropin Point (FSH/LH Point)

Location:

- The right ear normally bears the Gold Point (this may vary with the syndrome; hence, the needle metal needs to be decided).
- On the anterior outer bulge of the antitragus, about 2 mm inferior to the upper reflection. When the antihelix is imagined as a snake, the antitragus forms the head of the snake, while the Gonadotropin Point forms the eye.

Indications:

- Mainly during menopause in combination with the Estrogen Point.

Oxytocin Point

Location:

- On the antitragus, posterior to the Gonadotropin Point. The Vasopressin Point is also located very close to it.

Indications:

- Supports milk ejection during lactation.

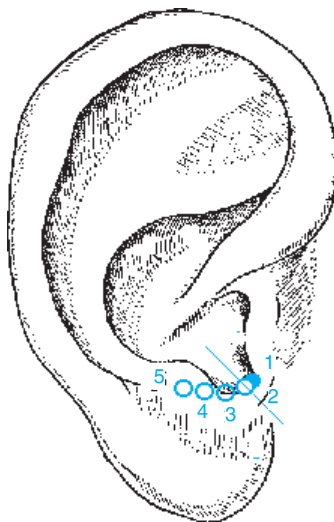


Fig. 5.13

- 1 Prolactin Point
- 2 ACTH Point
- 3 TSH Point
- 4 Gonadotropin Point (FSH/LH Point)
- 5 Oxytocin Point

Limbic System

Location:

- On the outside of the antitragus, about 2 mm below the upper edge (testing was done with the following substances in the ampoules: hippocampus, cingulate gyrus, amygdaloid body).

Indications:

- Anxiety, emotional depression associated with general autonomic hyperexcitability.

The reflex area of the limbic system is certainly worthy of more attention in auriculotherapy. (After needling is complete, search this area for low-energy points.)

Note: The entire limbic system is largely responsible for **emotional nuances of behavior in general as well as for emotional reactions (anger, fear, affection, anxiety, etc.)** and is probably also involved in cerebral functions such as **memory and learning**. The system includes the limbic cortex (hippocampus, parahippocampal gyrus, cingulate gyrus, amygdaloid gyrus, mamillary body, habenular nuclei), the limbic mesencephalon, and the extramural and intramural limbic fibers. It is the control center directly superior to the hypothalamus and regulates the endocrine and autonomic nervous systems (visceral brain).

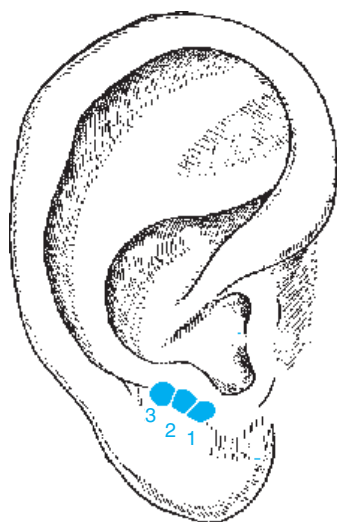


Fig. 5.14

- 1 Cingulate gyrus
- 2 Amygdaloid body
- 3 Hippocampus

Cerebellum

Location:

- On the back of the ear in the caudal fifth of the auricle.

Indications:

- Disturbed balance or vertigo as well as unsteadiness. Treatment requires prior diagnosis by a specialist.

Note: Disorders of the cerebellum cause disturbances in muscle coordination and muscular tone, resulting in unsteady, excessive movements (ataxia). Rapid sequential movements are no longer possible, and impeded course corrections lead to action tremor. Other symptoms are pathological eye movements (cerebellar nystagmus) and speech disturbance (dysarthria).

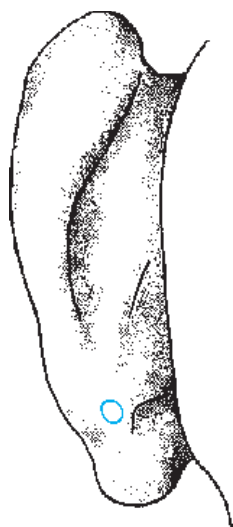


Fig. 5.15 Cerebellum

Brain Stem

Location:

- On the descending helix, in continuation of the Spinal Cord Zone.

Indications:

- This area represents the nuclei of origin of most cranial nerves as well as the vomiting center.

Note: The brain stem is divided into:

- the **medulla oblongata** (continuous with the spinal cord),
- the **pons**, and
- the **mesencephalon** (midbrain).

The brain stem gives rise to a number of true peripheral nerves (cranial nerves). Its main function is the maintenance of **balance** and normal **posture** in the field of gravity; autonomic control center (subjected to the hypothalamus). Injury to the centers of the brain stem lead to death in most cases.

Vomiting Point

Location:

- On the tail of the helix, in the reflex zone of the medulla oblongata.

Indications:

- Morning sickness (emesis), possibly pharyngeal reflex at the dentist.
- The point has an effect on the vomiting reflex.

Note: The **vomiting center** lies in the medulla oblongata near the respiratory center.

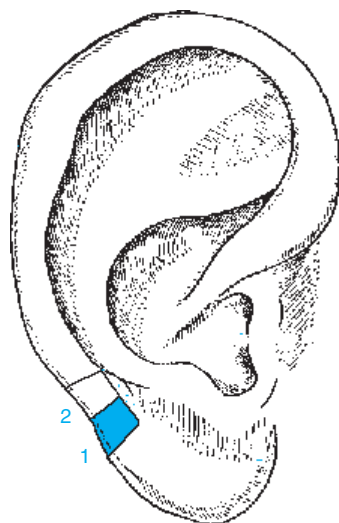


Fig. 5.16

- 1 Pons
- 2 Medulla oblongata

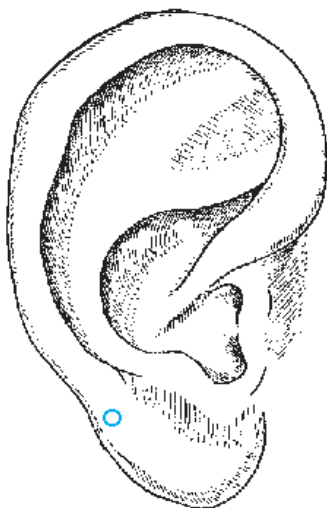


Fig. 5.17 Vomiting Point

Reticular Formation

Location:

- The main reflex area is underneath the tragus, more prominent on the left ear, i.e., in case of pathological activity in the reticular formation, a Gold Point is found on the left ear and a Silver Point on the right ear.
- This reflects an unstable reflex behavior or oscillation (in case of oscillation, the *Yin Tang* Point of body acupuncture is also found as Gold Point).

Indications:

- For example, insomnia. Treatment of disturbed circadian rhythms (e.g., jet lag after long flights) might be considered and should be tested on a larger scale.

Note: The reticular formation is a system of longitudinal and transverse fibers between the medulla oblongata and the diencephalon, with loosely interspersed ganglion cells which partly aggregate to form indistinctly circumscribed nuclei.

Transmission of vital reflexes, control of autonomic functions, coordination of motion reflexes, information processing of afferent excitations for the cerebral cortex. Important function in circadian rhythms, for example, sleeping–waking rhythm.

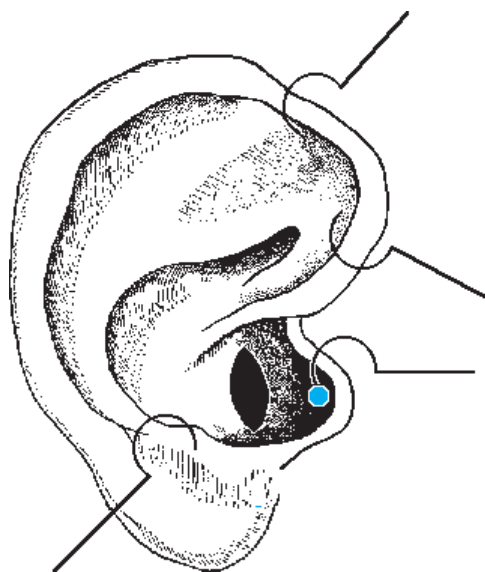


Fig. 5.18 Reticular formation

Spinal Cord

Location:

- On the descending helix.
- **Start:** Below Darwin's Point (caudal portion of the Spinal Cord Zone).
- **End:** approximately at the level of the postantitragal fossa (Point C0/C1).
- Sensory portions on the front of the ear, motor portions on the back of the ear.
- The nuclei of origin of the **sympathetic nervous system** are represented in the groove of the descending helix at the transition to the scapha.

Extension: The reflex areas of spinal cord segments C8–L2 reflect the arrangement of the sympathetic nuclei of origin in the spinal cord (intermediolateral nucleus).

Indications:

- Sensitive areas in herpes zoster.
- As accompanying therapy in cases of true root irritation or herniation of an intervertebral disk. Here, too, the reflex areas of the autonomic spinal cord portions of the affected segment should be searched for activity and, if necessary, included in the therapy (alleviation of concomitant autonomic reactions).

Note: The spinal cord is the part of the central nervous system that is enclosed in the vertebral canal. It extends from the exit of the first cervical nerve to the medullary cone (at the level of L1–L2). Caudal continuation as terminal filament (free of neurons), attachment to S1.

The spinal cord has a twofold purpose:

- As independent central nervous apparatus it generates reflexes (**reflex organ**, reflex arches).
- As conduction apparatus it connects the upper parts of the central nervous system (brain stem and brain) with the peripheral nervous system (**conduction organ**).

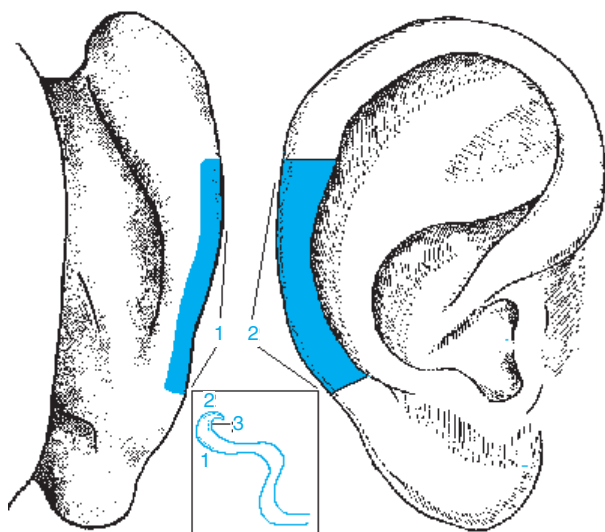


Fig. 5.19

- 1 Spinal cord, motor portion
- 2 Spinal cord, sensory portion
- 3 Nuclei of origin of sympathetic/parasympathetic nerves

Cranial Nerves—Overview

Location:

- The nuclei of origin of almost all cranial nerves project onto the Pons Zone and Medulla Oblongata Zone.

Some cranial nerves (VIII and X) have two reflex localizations. Cranial nerves I and II are represented on the anterior part of the ear lobe (Frontal Lobe Zone) as protruding parts of the brain.

- The front of the ear bears the reflex points of the nuclei of afferent (e.g., sensory) fibers.
- The back of the ear bears the reflex points of the nuclei of efferent (e.g., visceromotor) fibers.

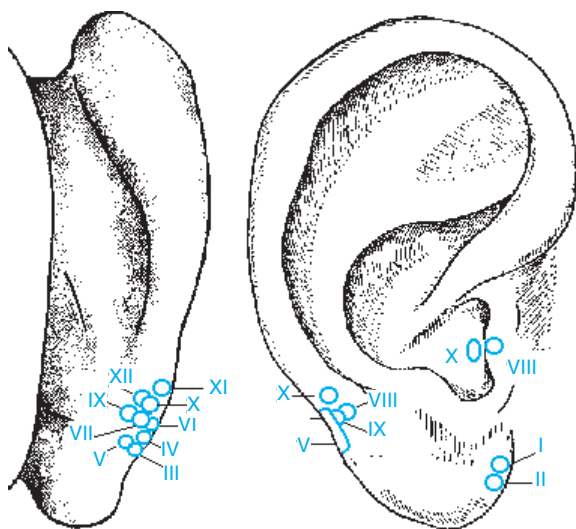


Fig. 5.20

- I Olfactory nerve
- II Optic nerve
- III Oculomotor nerve
- IV Trochlear nerve
- V Trigeminal nerve
- VI Abducens nerve
- VII Facial nerve
- VIII Vestibulocochlear nerve
- IX Glossopharyngeal nerve
- X Vagus nerve
- XI Accessory nerve
- XII Hypoglossal nerve

Olfactory Nerve (CN I)

Location:

- In the anterior part of the Frontal Lobe Zone.

Indications:

- Helpful in the common, but etiologically not always understood, olfactory anesthesia (anosmia). Neurological clarification is essential prior to therapy (possibility of a tumor).

Note: Although not a peripheral nerve, the olfactory nerve is included in the cranial nerves for historical reasons.

The olfactory nerve consists of the bundled processes of the sensory cells of the olfactory epithelium, which then enter the olfactory bulb. The olfactory bulb contains the olfactory tract and represents a protruding part of the brain.

Optic Nerve (CN II)

Location:

- In the anterior part of the Frontal Lobe Zone.

Indications:

- Possibly the condition after retrobulbar optic neuritis associated with MS (in the acute state use acupuncture only as an accompanying measure), degenerative disease of the papilla (prior examination by eye specialist or neurologist is mandatory).

Note: Like cranial nerve I, the optic nerve is a part of the brain. The optic nerve origin and the retina, together with the pigment epithelium of the eyeball, represent an evagination of the diencephalon.

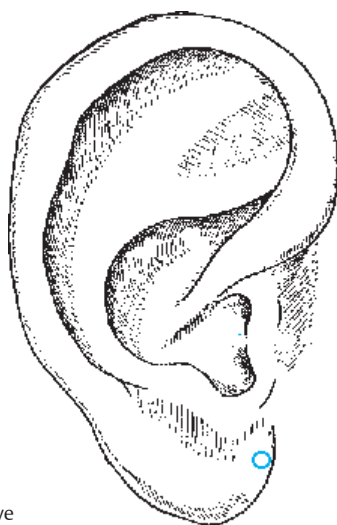


Fig. 5.21 Olfactory nerve

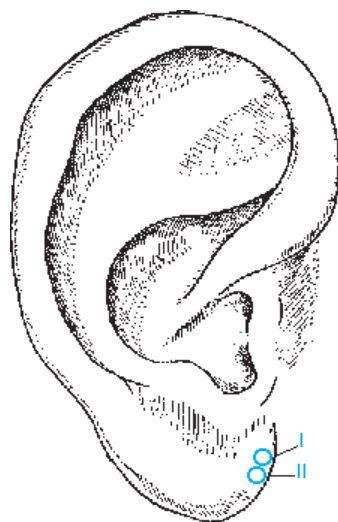


Fig. 5.22
 I Olfactory nerve
 II Optic nerve

Oculomotor Nerve (CN III)

Location:

- On the back of the ear, at the upper border of the Pons Zone. The points of the eye-muscle nerves (III, IV, VI) are located on a line along the edge of the ear, like pearls on a string.

Indications:

- Possibly isolated peripheral oculomotor paralysis caused, for example, by diabetes; Adie's syndrome, disturbed accommodation, insufficient convergence.

Note: Cranial nerves III, IV, and VI are referred to as eye-muscle nerves; they form a functional unit.

The oculomotor nerve contains somatomotor and visceromotor fibers. It supplies all eye muscles except the superior oblique muscle (trochlear nerve) and the lateral rectus muscle (abducens nerve). Its visceromotor fibers supply the inner eye muscles.

Trochlear Nerve (CN IV)

Location:

- On the back of the ear, superior to the Oculomotor Nerve Point.

Indications:

- Possibly isolated paralysis of the superior oblique muscle.

Note: The trochlear nerve supplies the superior oblique muscle of the eye.

Fig. 5.23 Oculomotor nerve

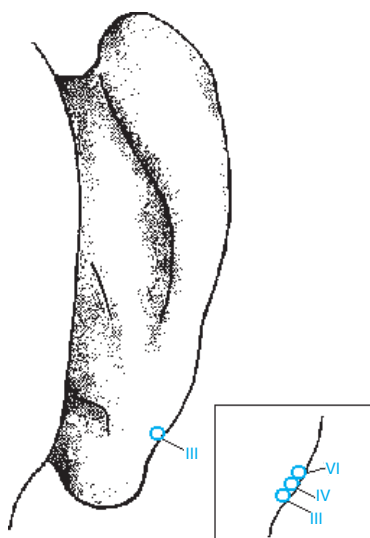
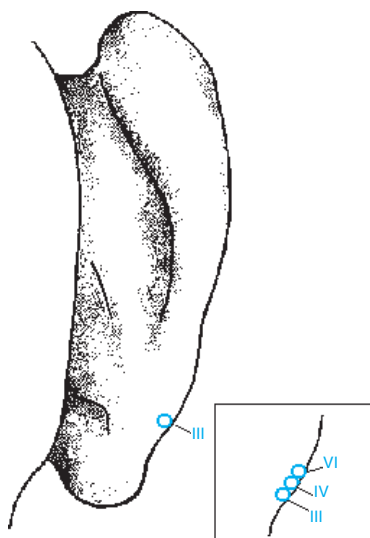


Fig. 5.24 Trochlear nerve



Trigeminal Nerve (CN V)

Location:

- A relatively long area along the edge of the ear, on the front of the ear in the Pons Zone.
- The different branches of the trigeminal nerve are projected in inverted sequence in the same way as the vertebral column and spinal cord are projected upside down.
- The motor branch is represented as a point on the back of the ear.

Indications:

- Trigeminal neuralgia or affections related to each of its different branches (use gold needle).
- In case of symptoms, the precise localization of the affected branch is searched for by checking the pulse (VAS) or by using the point finder.

Note: Cranial nerve V contains sensory fibers for the facial skin and mucosa and motor fibers for the muscles of mastication. It exits as a thick trunk from the pons with a sensory root (greater portion) and a motor root (lesser portion). In Gasser's ganglion, it divides into its three main branches, the **ophthalmic nerve**, **maxillary nerve**, and **mandibular nerve**, and into the **motor root**.

Abducens Nerve (CN VI)

Location:

- On the back of the ear at the transition of the Pons Zone to the Medulla Oblongata Zone.

Indications:

- Isolated abducens paralysis, often caused by diabetes.

Note: The abducens nerve is a somatomotor nerve that supplies the lateral rectus muscle of the eye.

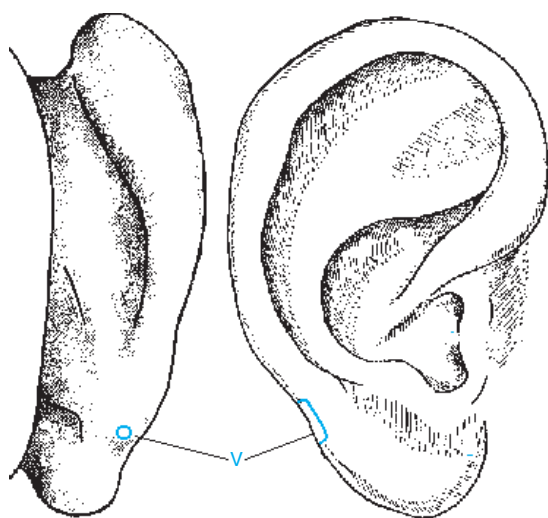


Fig. 5.25 Trigeminal nerve

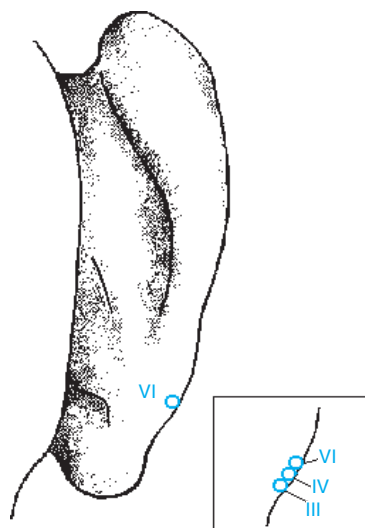


Fig. 5.26 Abducens nerve

Facial Nerve (CN VII)

Location:

- On the back of the ear at the transition from the Pons Zone to the Medulla Oblongata Zone, near the Abducens Nerve Point.

Indications:

- Facial paralysis (use gold needle on the affected side); glandular disorders, for example, dry eye and insufficient production of saliva associated with internal diseases (Sjögren's syndrome); faulty innervation after facial paralysis (crocodile tears syndrome).

Note: Cranial nerve VII contains motor fibers for the mimic muscles and, in a nerve bundle emerging from the brain stem separately (as intermediate nerve), also gustatory fibers and visceromotor (secretory) fibers.

Vestibulocochlear Nerve (CN VIII)

Location:

- Apart from the well-known localization on the apex of the tragus, there is also one at the transition from the Pons Zone to the Medulla Oblongata Zone. This second point has been found during testing by means of VAS resonance.
- It is assumed that the second point represents the nucleus of origin in the brain stem, while the point on the apex of tragus probably represents the vestibulocochlear nerve fibers.
- The therapeutic effect of the original point, which has been used for over 20 years, also points in this direction.

Indications:

- Sudden deafness, Ménière's disease, peripheral vestibular neuronitis (very common), all types of vestibular vertigo, certain types of tinnitus.

Note: Cranial nerve VIII is an afferent nerve consisting of two components: the cochlear root for the organ of hearing and the vestibular root for the organ of balance.

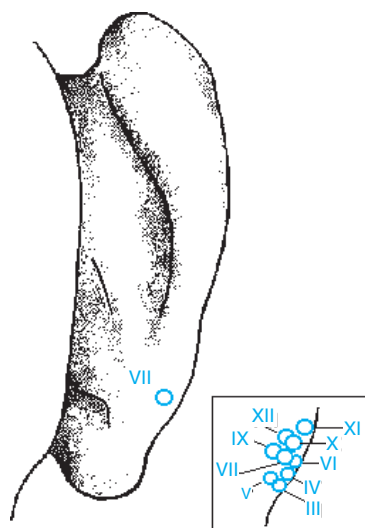


Fig. 5.27 Facial nerve

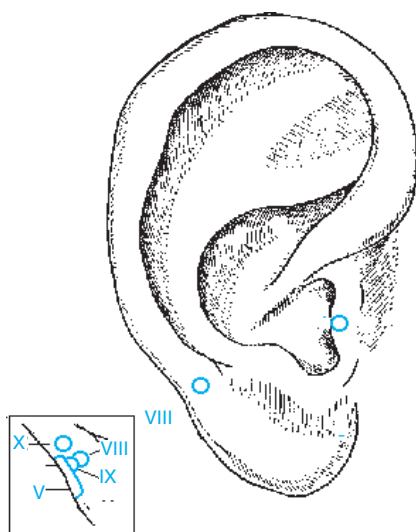


Fig. 5.28 Vestibulocochlear nerve

Glossopharyngeal Nerve (CN IX)

Location:

- Sensory portion at the upper border of the Medulla Oblongata Zone.
- Motor portion on the back of the ear in the same region.

Indications:

- Difficulty in swallowing, globus sensation, glossodynia.

Note: Cranial nerve IX provides sensory innervation to the middle ear, some regions of the tongue and the pharynx, and motor innervation to the pharyngeal muscles. It contains motor, visceromotor, and viscerosensory fibers as well as gustatory fibers.

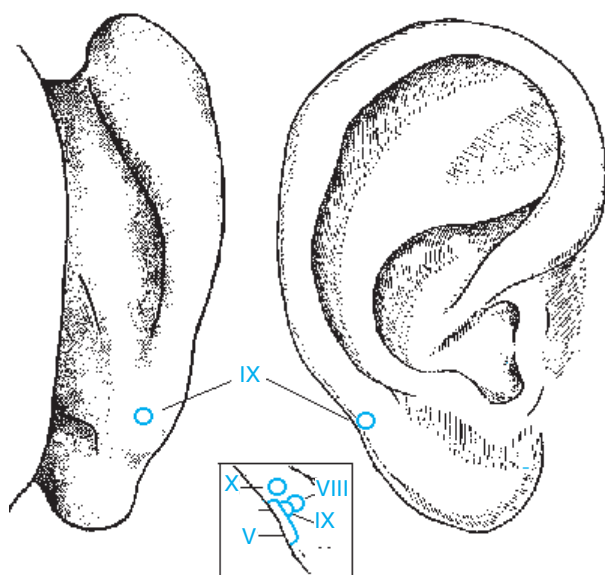


Fig. 5.29 Glossopharyngeal nerve

Vagus Nerve (CN X)

Location:

- In the Medulla Oblongata Zone, on both the front and the back of the ear (projection of the origins of the nerve's cranial and thoracic portions).
- The other localization near the external acoustic meatus, used so far and described as effective by many therapists, is the Vagus Nerve Zone. It yielded a distinct resonance VAS when testing with the substance "vagus nerve, cervical part."
- Because a branch of the vagus nerve enters the concha at this site, this is understood as a direct resonance response to the anatomical material "vagus nerve."

Indications:

- Autonomic hyperexcitability, tendency to sweat, tachyarrhythmia (**caution:** heart rate), bronchial catarrh, bronchitis, recurrent gastritis or ventricular ulcer (use sedation), disturbed secretion of gall bladder and pancreas, obstruction, intestinal colic, possibly also micturition disorders.

Note: Cranial nerve X supplies not only regions of the head, like the other cranial nerves, but descends also into the thoracic and abdominal regions where it forms plexus-like ramifications in the intestines.

It is the largest parasympathetic nerve of the autonomic nervous system (see p. 142).

In everyday usage, the term "vagus nerve" is often used as a synonym for "parasympathetic system." However, this nerve is only part of the parasympathetic system.

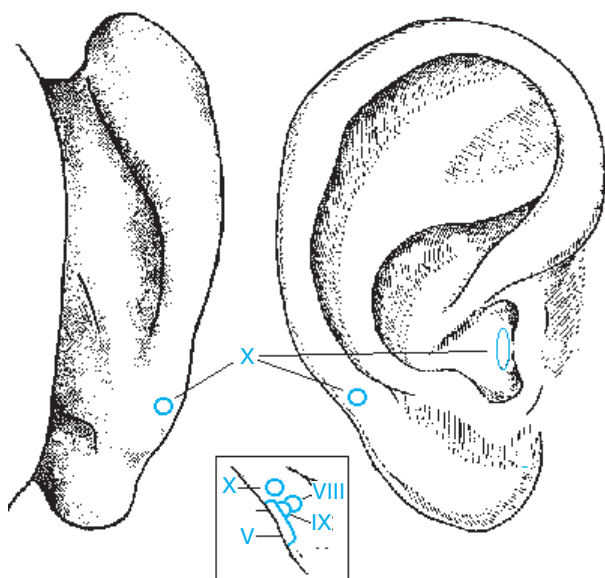


Fig. 5.30 Vagus Nerve

Accessory Nerve (CN XI)

Location:

- On the back of the ear in the Medulla Oblongata Zone.

Indications:

- Myogenic torticollis, wryneck, dystonia.

Note: Cranial nerve XI is a pure motor nerve that supplies the sternocleidomastoid muscle and the trapezius muscle.

Hypoglossal Nerve (CN XII)

Location:

- On the back of the ear in the Medulla Oblongata Zone. Remarkably, this location is slightly **caudal** to Point CN XI. However, this is understandable when considering the anatomical site of the nerve's origin.

Indications:

- Isolated cases of paralysis of the tongue.

Note: Cranial nerve XII is a pure somatomotor nerve for the tongue muscles.

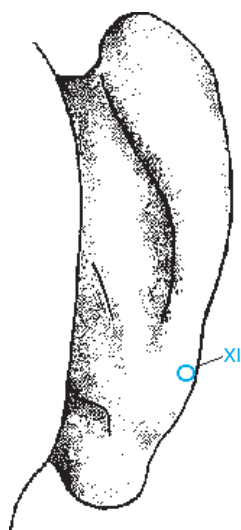


Fig. 5.31 Accessory nerve

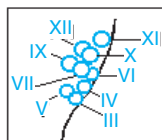
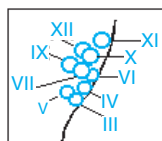


Fig. 5.32 Hypoglossal nerve



Autonomic Nervous System—Overview

The peripheral autonomic nervous system consists of two divisions:

the **sympathetic nervous system** and the **parasympathetic nervous system**.

This anatomical and functional unit regulates organ function through inhibition or stimulation.

The two divisions have different origins:

The sympathetic system originates from the thoracic spinal cord and the upper two to three segments of the lumbar spinal cord (thoracolumbar system, C8–L2). The parasympathetic nervous system originates from the brain stem and the sacral spinal cord (craniosacral system).

The peripheral autonomic nervous system is **efferent** and consists of two groups of neurons arranged in tandem. (A neuron is the morphological and functional unit of the nervous system, consisting of a cell body and its fiber, or axon.)

The cell body of the first neuron (**preganglionic neuron**) is situated in the spinal cord. Its axon runs to an autonomic ganglion (either sympathetic trunk ganglion or another ganglion).

In the ganglion, the first neuron synapses with the second neuron (**postganglionic neuron**), the axon of which terminates on an effector organ.

The higher control center of both systems (sympathetic and parasympathetic systems) is located in the **hypothalamus** (an important region for auriculotherapy, see Hypothalamus, p. 132). Knowledge of the respective reflex localizations on the ear is essential and may be helpful when treating the autonomic aspects of many diseases by ear acupuncture.

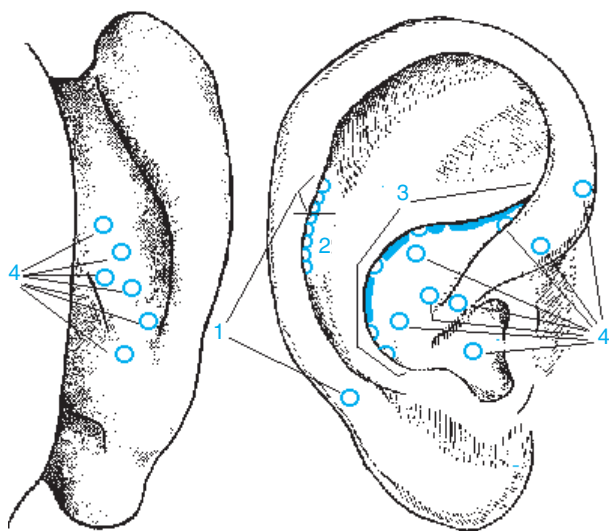


Fig. 5.33

- 1 Parasympathetic nuclei of origin
- 2 Sympathetic nuclei of origin
- 3 Sympathetic trunk
- 4 Prevertebral ganglia

Sympathetic Nervous System—Overview

The sympathetic nuclei of origin are situated in the lateral horn of the spinal cord, in the intermediolateral nucleus (expansion between C8 and L2 **only**).

Nerve fibers connect the nuclei with the sympathetic trunk (paired chain of sympathetic ganglia, running anterolateral to the vertebral column: paravertebral ganglia, in contrast to the large unpaired prevertebral ganglia such as the celiac ganglion).

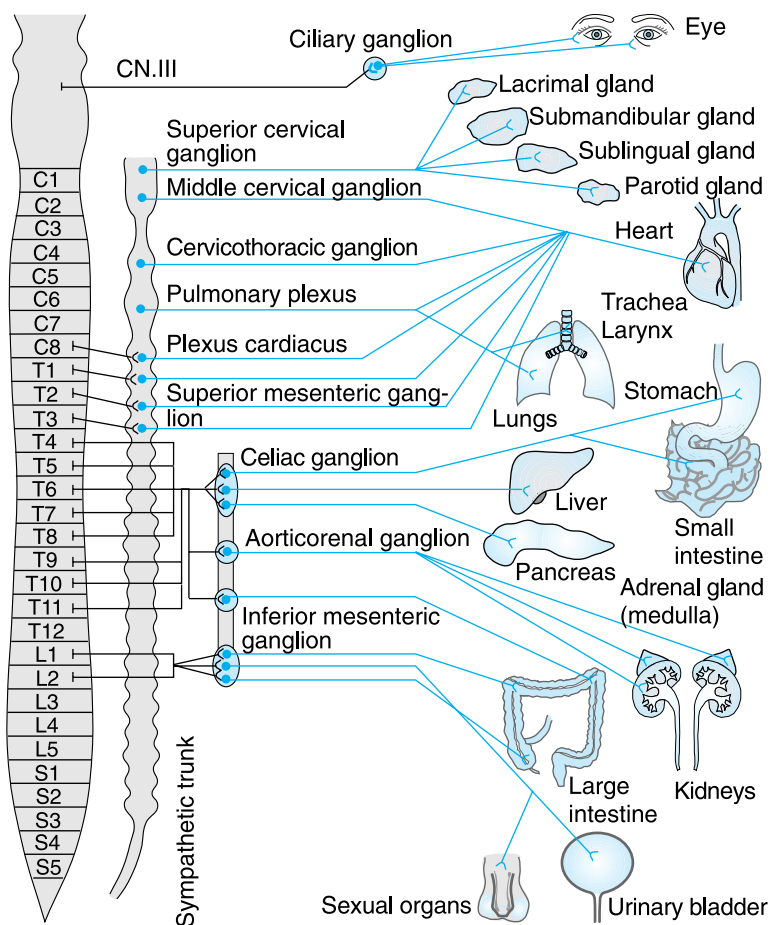


Fig. 5.34 System of sympathetic innervation of the organs
(Taken from: Neurologie des vegetativen Systems, R. Schiffter, Springer Verlag, Berlin-Heidelberg-New York 1985)

Parasympathetic Nervous System—Overview

The parasympathetic nuclei of origin are located in the brain stem and in the sacral spinal cord (cell bodies of the **preganglionic** parasympathetic neurons). The axons of the neurons are very long, unlike those of the preganglionic sympathetic neurons.

The preganglionic parasympathetic fibers for the inner eye muscles and for the glands in the head region exit from the brain stem together with the oculomotor nerve, the facial nerve, and the glossopharyngeal nerve.

The preganglionic parasympathetic fibers for the organs in the thoracic and abdominal regions run in the **vagus nerve**.

The sacral parasympathetic fibers for the pelvic organs run in the **pelvic nerve**.

In everyday usage, the term “vagus nerve” is often used as a synonym for the “parasympathetic system.” However, this nerve is only part of the parasympathetic system.

The parasympathetic nervous system supplies the smooth muscles and the glands of the gastrointestinal tract, excretory organs, sexual organs and lungs (but, with a few exceptions, not the vascular smooth muscles). Furthermore, it supplies the atria of the heart, the lacrimal and salivary glands in the head region, and the inner eye muscles.

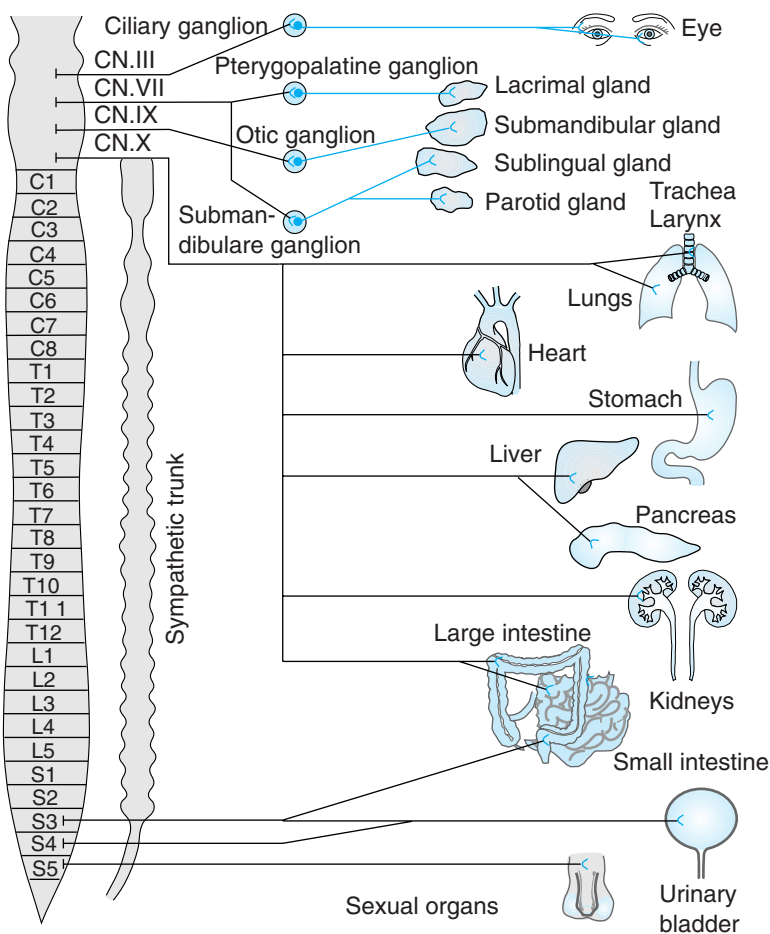


Fig. 5.35 System of parasympathetic innervation of the organs
(Taken from: Neurologie des vegetativen Systems, R. Schiffter, Springer Verlag, Berlin-Heidelberg-New York 1985)

Sympathetic Nuclei of Origin (Preganglionic Sympathetic Nerves)

Location:

- The group of sympathetic nuclei of origin (intermediolateral nucleus) is represented in the groove of the descending helix, in the Spinal Cord Zone between Points C8 and L2.

Indications:

- The **effector organs** of the sympathetic nervous system are the smooth muscles of all organs (intestine, excretory organs, lung, hair follicles, pupils, and some blood vessels), the heart, and also some glands (sweat glands, salivary glands, digestive glands).
- Fat cells, liver cells, and, possibly, renal tubules are additionally innervated by sympathetic postganglionic fibers. The various indications thus may be deduced from this fact.
- Concomitant auriculotherapy of the autonomic aspects of many diseases may be essential and helpful.

Diagnosis and therapy will consider both groups of active points: those related to organ functions and located in the **Zone of Sympathetic Nuclei of Origin** and those located in the **Zone of Sympathetic Trunk**.

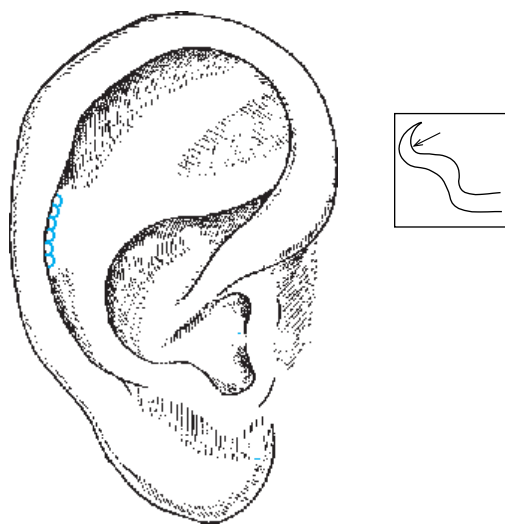


Fig. 5.36 Sympathetic nuclei of origin (spinal cord between C8 and L2)

Sympathetic Trunk (Paravertebral Chain of Sympathetic Ganglia, Postganglionic Sympathetic Nerves)

Location:

- The chain of sympathetic ganglia is represented parallel to the projection of the vertebral column, at the foot of the antihelical wall where the wall merges into the concha.

The area includes the narrow band of the concha that is not innervated by the auricular branch of the vagus nerve and, thus, belongs to the projection of the mesoderm.

In total, the sympathetic trunk consists of 23 ganglia: three in the cervical region, 12 in the thoracic region, and four each in the lumbar and sacral regions. In the cervical region, the original eight ganglia have combined to form three. In front of the coccyx, the two sympathetic trunks unite to form a single ganglion (ganglion impar).

Indications:

- The zone should always be considered for concomitant therapy in all organic affections (internal organs, vertebral column, head region).
- Indications can easily be deduced from the descriptions above. Especially those neurovegetative disorders which occur, for example, as a result of vertebral blockage or herniation of an intervertebral disk can be influenced.
- Migraine also calls for the testing of activity, and possibly needling, in the upper Zone of Sympathetic Trunk.

Note: Not all fibers that originate in the spinal cord synapse in the sympathetic trunk.

Some preganglionic sympathetic fibers pass through the sympathetic trunk without forming synapses and reach the larger **prevertebral ganglia** (e.g., celiac plexus) where they synapse with postganglionic neurons.

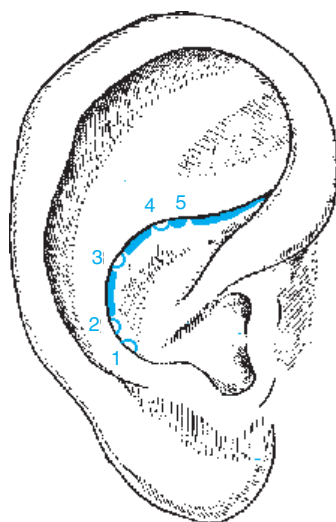


Fig. 5.37

- 1 Superior cervical ganglion
- 2 Middle cervical ganglion
- 3 Inferior ganglion (stellate ganglion)
- 4 Nervous duodenum
- 5 Nervous liver

Superior Cervical Ganglion

Location:

- At the level of Point C2 in the Zone of Sympathetic Trunk.

Indications:

- Complaints in the head region, for example, migraine, cephalgia, facial pain.

Note: The **sympathetic nerve fibers for the head** originate in the spinal cord segments C8–T2.

They synapse in the superior cervical ganglion and accompany the large blood vessels for the head as parietal plexus until they reach their effector organs (lacrimal glands, salivary glands, smooth muscles, and mucosa).

Knowledge about these relationships is **essential for auriculotherapy** because, particularly in case of symptoms in the head region, consideration of the autonomic aspects favors the therapeutic success.

Middle Cervical Ganglion

Location:

- At the level of Point C3 in the Zone of Sympathetic Trunk.

Indications:

- Similar to Superior Cervical Ganglion Point.

Note: The neck is supplied by the three cervical ganglia.

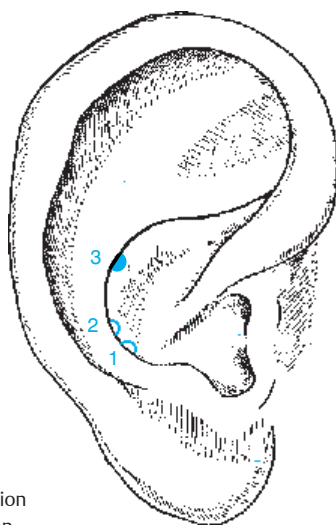


Fig. 5.38

- 1 Superior cervical ganglion
- 2 Middle cervical ganglion
- 3 Inferior cervical ganglion (stellate ganglion)

Inferior Cervical Ganglion (Stellate Ganglion)

Location:

- At the level of Point C7 in the Zone of Sympathetic Trunk.

Indications:

- For example, whiplash syndrome after a car accident, cervical rib syndrome, and, in general, all indications listed for the stellate ganglion block, such as:
- migraine, one-sided headache, postconcussion syndrome, cervical syndrome, various disorders of the shoulder (the fibers for the arm are mainly supplied by the stellate ganglion), hyperemesis gravidarum, trigeminal neuralgia and zoster neuralgia, reflex sympathetic dystrophy (Sudeck's atrophy).
- Blocking of the 1st rib, which is usually accompanied by irritation of the stellate ganglion, can be well treated via the corresponding points on the ear.
- Testing of this point and, possibly, inclusion in the therapy are worth a try also in case of severe focal activity in the upper quarter of the body.

Note: The stellate ganglion is the major site of distribution through which all sympathetic fibers of the **upper quarter of the body** run.

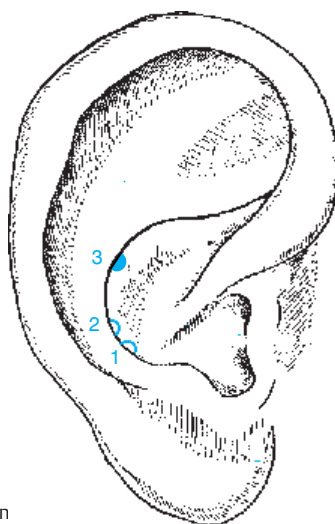


Fig. 5.39

- 1 Superior cervical ganglion
- 2 Middle cervical ganglion
- 3 Inferior cervical ganglion (stellate ganglion)



Nervous Organ Points

These are the projections of the nerve fibers to the organs.

Location:

Nervous Liver Point (syn. Anger Point) and **Nervous Gall Bladder Point**

- In the Zone of Sympathetic Trunk at the level of Points T5–T9. The two points are located very close to each other, each one appearing as a Silver Point on the right ear.

Nervous Stomach Point and **Nervous Duodenum Point**

- In the Zone of Sympathetic Trunk at the level of Points T6–T9. The Nervous Stomach Point appears as a Gold Point on the left ear, while the Nervous Duodenum Point appears as a Gold Point on the right ear.

Despite their obviously strong effect on the psyche, the nervous organ points usually reflect more their anatomical location, namely, their assignment to the Zone of Sympathetic Trunk. This means that the needle metal is the same for left-handed and right-handed persons (no switching of Gold Points to the other ear).

Indications:

- Dysfunction in the respective organ region.

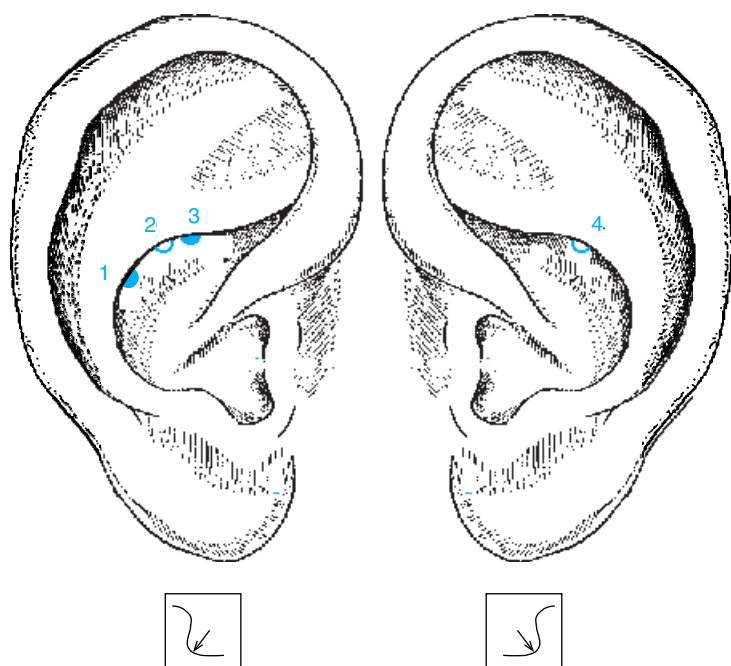


Fig. 5.40

- 1 Stellate Ganglion Point
- 2 Nervous Duodenum Point
- 3 Nervous Liver Point and Nervous Gall Bladder Point
- 4 Nervous Stomach Point

Parasympathetic Nuclei of Origin

Location:

- The parasympathetic nuclei of origin project in the Sacral Spinal Cord Zone in the groove of the descending helix and in the Brain Stem Zone on the tail of the helix at the transition to the lobule.

Indications:

- Autonomic hyperexcitability, tendency to sweat, tachyarrhythmia (**caution:** heart rate), recurrent gastritis or ventricular ulcer (use sedation), obstipation, intestinal colic (see also indications for Vagus Nerve Point, p. 160).

Prevertebral Ganglions—Overview

Whereas the paravertebral ganglia (sympathetic trunk) are of a purely sympathetic nature, the prevertebral and periarterial ganglia are **functionally of a mixed nature**; they contain predominantly parasympathetic portions.

The ganglia are arranged in large networks and form several **prevertebral plexus** which are projected on the ear as the following points:

- Bronchopulmonary Plexus Point,
- Cardiac Plexus Point,
- Celiac Plexus Point (Solar Plexus Point),
- Superior and Inferior Mesenteric Plexus Points (with each plexus having a twofold representation on the ear), and
- Hypogastric Plexus Point.

In principle, all disorders of the associated regions can be treated by needling these points in addition to the reflex areas for the main symptoms.

Every condition of pain, every functional disorder (e.g., of the heart and in the gastrointestinal region) is always accompanied by sympathetic and/or parasympathetic symptoms, or may be actually caused by them.

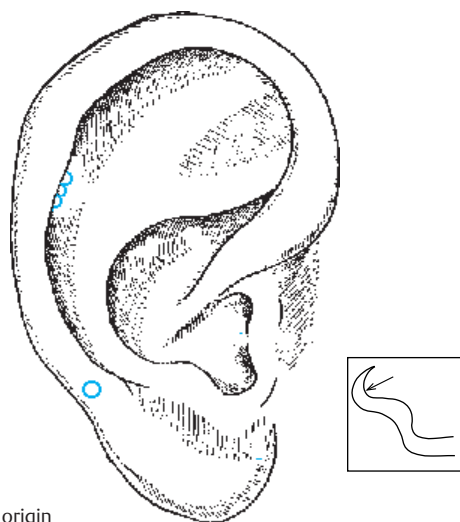


Fig. 5.41
Parasympathetic nuclei of origin

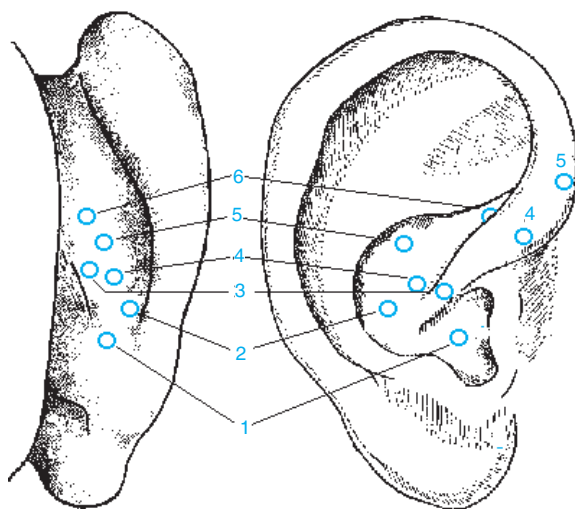


Fig. 5.42

- 1 Bronchopulmonary plexus
- 2 Cardiac plexus
- 3 Celiac plexus (solar plexus)

- 4 Superior mesenteric plexus
- 5 Inferior mesenteric plexus
- 6 Hypogastric plexus

Bronchopulmonary Plexus

Location:

- In the upper portion of the inferior concha. Also on the back of the ear at the corresponding site.

Indications:

- Bronchospasmic conditions, general diseases of the lung and bronchi.

Cardiac Plexus

Location:

- In the concha just anterior to the Middle Cervical Ganglion Point, approximately at the level of Point C4. Also on the back of the ear at the corresponding site.

Indications:

- Hypertension, also autonomic regulatory disorders in the form of heart sensation; helpful in various forms of arrhythmia.

Nogier has described this area as Zone of Fearfulness (cardiophobia, cardiac functional symptoms).

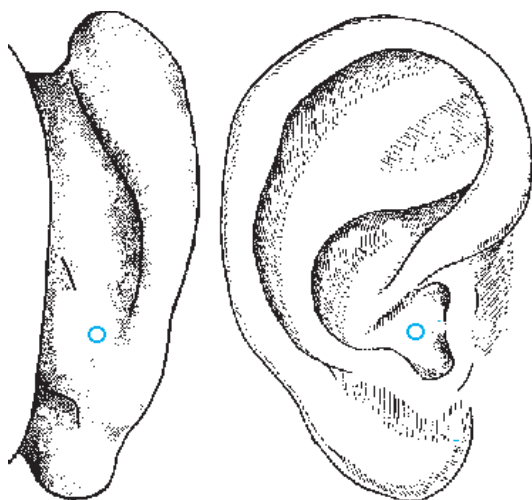


Fig. 5.43 Bronchopulmonary plexus

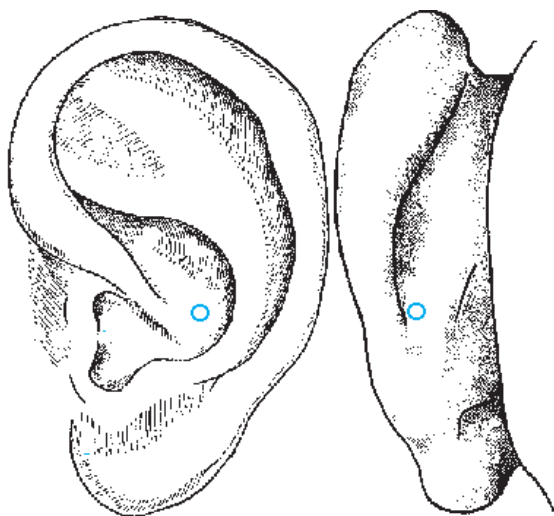


Fig. 5.44 Cardiac plexus

Celiac Plexus (Solar Plexus)

Location:

- At the well-known location of Point Zero (identical localization); on the root of helix, in a cartilage depression that can be clearly palpated with the stirrup-shaped ear probe.
- On the back of the ear, the point is located on the auricle approximately 3 mm away from the attachment of the ear, at the level of Point Zero.

Indications:

- Painful intestinal spasms, singultus, intestinal focus, for example, through malnutrition (in this case, ear acupuncture surely must not be the only form of therapy).

Because of the mixed sympathetic and parasympathetic nerve components of the solar plexus, the point is suitable for treating autonomic imbalance caused by severe nervous tension. In classical Chinese acupuncture, Point Zero is used to strengthen the whole body (the effect is intensified when used in combination with the Ear Meridian Point GV-4 on the back of the ear (see Ear Meridians, p. 332).

The Celiac Plexus Point on the back of the ear (Retro-Point Zero) has the same location as the Ear Meridian Point SI-3 (p. 324) and the same spasmolytic effect.

Many other ear points are located on various axes through Point Zero, which clearly indicates the key role that this point plays in auriculotherapy.

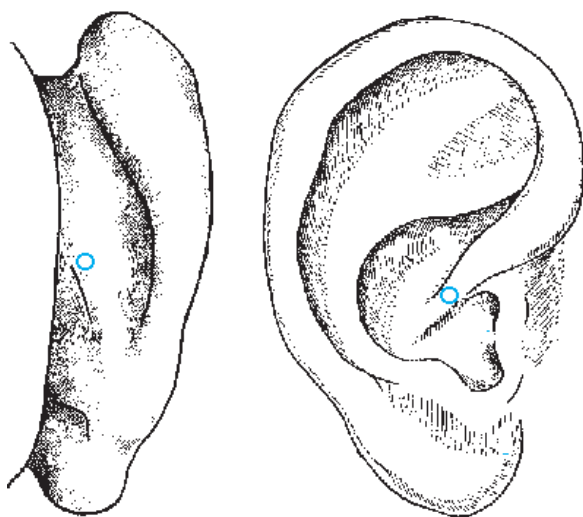


Fig. 5.45 Celiac plexus (solar plexus)

Superior Mesenteric Plexus

Location:

- In the caudal part of the superior concha slightly above the root of helix. Also on the back of the ear at the corresponding location.
- However, testing yielded a resonance VAS also on the anterior edge of the ascending helix, slightly caudal to the Estrogen Point (localization of the current ear map).

Here, we are dealing with the phenomenon of twofold representation, with the location in the concha being easy to comprehend in anatomical terms.

Indications:

- As concomitant therapy in all functional diseases of the small intestine, ascending colon and transverse colon (irritable bowel syndrome), tenesmus, meteorism, inflammatory diseases.

Inferior Mesenteric Plexus

Location:

- In the superior concha but more in the middle, slightly superior to the Gall Bladder Zone. Also on the back of the ear at the corresponding location.
- Here, too, testing resulted in a distinct resonance VAS on the anterior edge of the ascending helix at the level of the antihelix (twofold representation).

Indications:

- As concomitant therapy in all symptoms and functional diseases in the region of descending colon, sigmoid colon, and rectum.

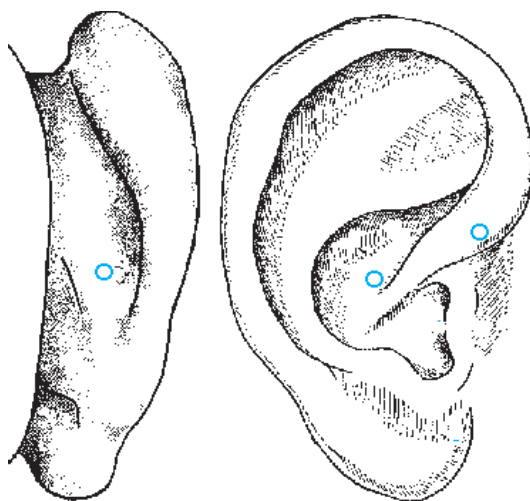


Fig. 5.46 Superior mesenteric plexus

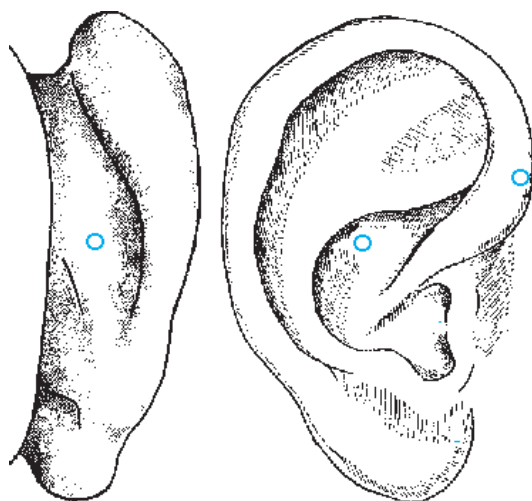


Fig. 5.47 Inferior mesenteric plexus

Hypogastric Plexus

Location:

- In the superior concha (same location as Omega Point I). Also on the back of the ear at the corresponding site.

This reflects a connection also known from body acupuncture: Point CV-12 (which corresponds to Omega Point I) is the Front Collecting Point of the Stomach.

Indications:

- Urogenital disorders in men and women (e.g., micturition disorders, irritable bladder) as well as disorders in the region of the descending colon, sigmoid colon, and rectum (e.g., obstipation, intestinal colic).

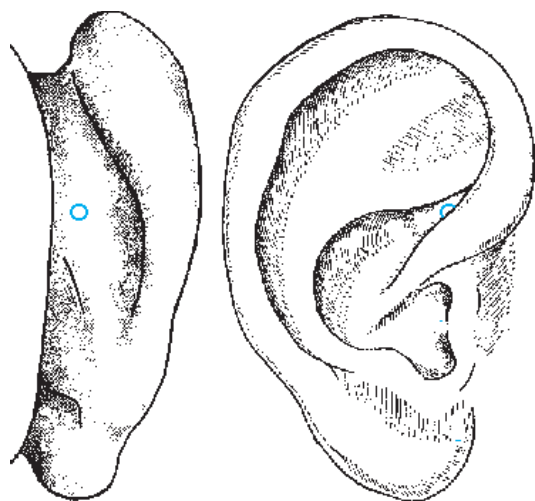


Fig. 5.48 Hypogastric plexus

Peripheral Nervous System

In principle, all structures belonging to the same body region (e.g., to a joint) are also represented in the same region of the ear.

For example, the following points are all found on the ear in the area around the Hip Joint Point: those for the inguinal region (inguinal strain, inguinal hernia), the buttock muscles, the upper portions of sciatic nerve and femoral artery, as well as the points for the pelvic bones and the sacroiliac joint.

This means that blood vessels and nerves are also represented near the structures that they supply.

The points are not identical; they rather lie close together, and each must be carefully searched for by using point finder or VAS.

When searching for the reflex area of a peripheral nerve (e.g., in case of stump pain after surgery), one will first contemplate the surrounding anatomical structures and then search for the active point of the nerve in the respective reflex area on the ear (by using point finder or VAS). Sensory nerves project on the front of the ear and motor nerves on the back of the ear.

In contrast to the forceps method applied to the reflex zone of the muscles, where usually a silver needle is used on the back of the ear for sedation or relaxation, it may be necessary here to use a gold needle for stimulation (e.g., when a nerve is damaged).

Indications:

- Peripheral compression syndromes, such as carpal tunnel syndrome, sulcus ulnaris syndrome, meralgia paraesthetica (compression of the femoral cutaneous nerve),
- neuroma pain after transection,
- damage to a plexus, and
- possibly polyneuropathy.



Fig. 5.49 Diagram of the projection of the locomotor system, used as orientation tool when searching for peripheral nerve points

Functional Points—Overview

In principle, there are two different groups of points among the reflex areas of the ear:

- **Anatomical points** and
- **Functional Points.**

Anatomical points are called **Organ Points**; they are pain points associated with certain organs (organ-specific Pain Points). They are **always** found on the ipsilateral ear, i.e., on the same side on which the affected organ is located in the body (e.g., the Appendix Point and Gall Bladder Zone are on the right ear). **This rule applies to both right-handed and left-handed persons, independent of lateral dominance** (in a left-handed person, the Gall Bladder Zone is still on the **right** ear, while the zone for the left knee is on the left ear, etc.).

Functional Points are those that have not only an effect on a local disorder but may also influence the entire system. They include the points listed on the following pages.

In contrast to the organ-specific points, **Functional Points** are always closely associated with handedness. This means that the lateral dominance decides on which ear a certain Functional Point will be found, i.e., on which ear it will be a Gold Point (on the contralateral ear there is always a Silver Point).

For example:

- In a right-handed person the Gold Point is on the right ear;
- for the same reflex area, the Gold Point is on the left ear in a left-handed person.

For each Gold Point, the corresponding Silver Point lies on the contralateral ear at the **same** location.

For example:

The Diazepam Analogue Point (Valium Point according to Bahr) is found

- in a right-handed person on the right ear with a silver needle, and on the left ear with a gold needle;
- in a left-handed person on the left ear with a silver needle, on the right ear with a gold needle.

In the present book, only the location for the **right** (dominant) ear of a **right-handed** person is described. Further information can be deduced (see above). Because of the better effect, the Gold Point is preferred; even permanent needles are preferably inserted in Gold Points.

An exact classification of ear points into Medication Analogue Points, Hormone Points, and Metabolite Points is often not possible, because some medications are in fact identical or similar to substances produced by the body, or they have been designed to mimic the effects of the body's own glands (e.g., estrogen).

Note: If steel needles are used instead of gold and silver needles, always needle the Gold Point because this is the point that needs to be stimulated. For example, in case of the Diazepam Analogue Point, the Gold Point would be on the left ear in a right-handed person but on the right ear in a left-handed person.

Exemption: Anxiety Point, Worry Point, Aggression Point, Nicotine Analogue Point, Nervous Liver Point—here use Silver Point for steel needles (see also p. 271).

Hormone and Metabolic Points—Overview

All points for endocrine glands are located in the wall ascending from the concha to the antihelix, at the transition from the upper third to the middle third of the antihelical wall. The series of points form a line running along the antihelical wall parallel to the concha. They all are normally found as Gold Points on the left ear; in some cases (e.g., Thyroid Gland Point) the type of needle metal needs to be decided on an individual basis, taking the nature of the syndrome into account.

Estrogen Point

Location:

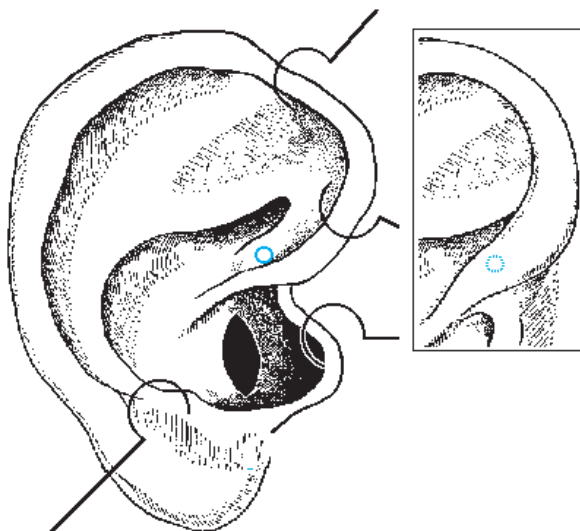
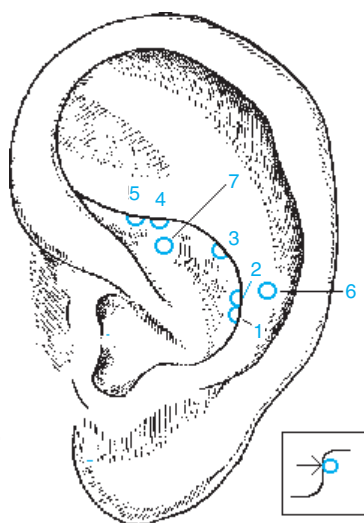
- The right ear bears the Gold Point (tonification is normally required).
- On the inside (underside) of the ascending helix and about 2 mm away from the reflection, approximately midway between Navel Point (Point Zero) and Uterus Point.
- Similar location as that of the Ovary/Testis Point.

Application:

- In all conditions of hormone deficiency, such as menopausal syndrome.
- During pregnancy this point is definitely contraindicated for nongynecologists.
- Corresponds to Point SP-5 of body acupuncture (p. 322).

Fig. 6.1

- 1 Parathyroid Gland Point
- 2 Thyroid Gland Point
- 3 Thymus Gland Point
- 4 Endocrine Pancreas Point (Insulin Point)
- 5 Adrenal Gland Point (Cortisone Point)
- 6 Parenchymal thyroid gland
- 7 Parenchymal pancreas

**Fig. 6.2** Estrogen Point

Progesterone Point

Location:

- The right ear normally bears the Silver Point. This may vary with the syndrome and from person to person; hence, the needle metal of choice needs to be decided.
- About 4 mm above the Kidney Point in the fold of the ascending helix.

Application:

- In the presence of hormonal and menopausal symptoms.
- May be combined with other points of the Hormonal Axis (see Auxiliary Lines, p. 291).
- During pregnancy this point is definitely contraindicated for nongynecologists.

Gonadotropin Point

Location:

- The right ear normally bears the Gold Point. This may vary with the syndrome and from person to person; hence, the needle metal of choice needs to be decided.
- On the anterior outer bulge of the antitragus, about 2 mm inferior to its upper edge. When the antihelix is imagined as a snake, the antitragus forms the head of the snake, while the Gonadotropin Point forms the eye.

Application:

- Mainly during menopause in combination with the Estrogen Point.

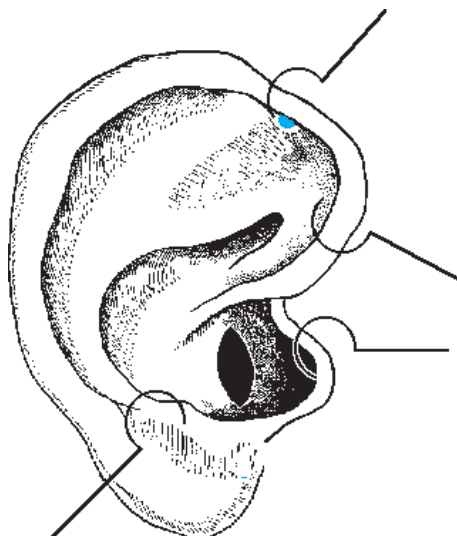


Fig. 6.3 Progesterone Point

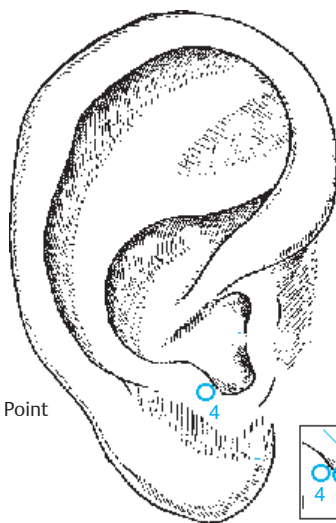
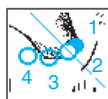


Fig. 6.4 Gonadotropin Point

- 1 Prolactin Point
- 2 ACTH Point
- 3 TSH Point
- 4 Gonadotropin Point



ACTH Point

Location:

- The right ear normally bears the Gold Point. In the anterior corner of the intertragic notch where the notch ascends to the tragus.

Application:

- ACTH stimulation; for example, in all cases of allergies.
- For supporting amalgam elimination use gold needle on the right ear.
- Corresponds to Point LR-13 of body acupuncture (p. 330).

TSH Point

Location:

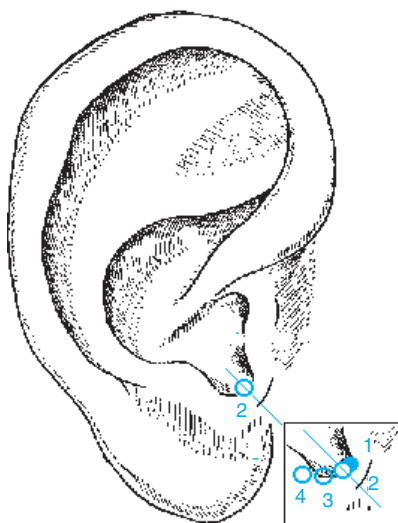
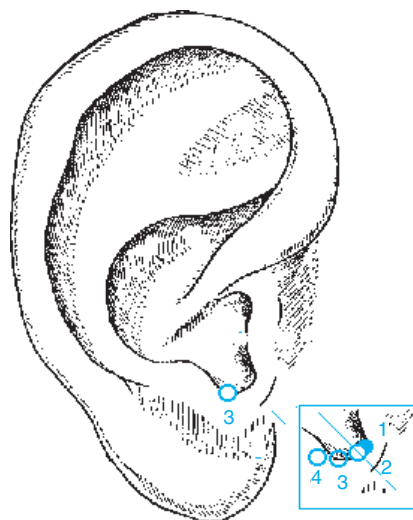
- Just behind the ACTH Point (in posterior direction toward the antitragus).
- Requires stimulation (gold needle) or sedation (silver needle), depending on the syndrome.

Application:

- Predominantly diagnostic when latent hypothyroidism is suspected, supports the therapy in case of disease.

Fig. 6.5 ACTH Point

- 1 Prolactin Point
- 2 ACTH Point
- 3 TSH Point
- 4 Gonadotropin Point

**Fig. 6.6** TSH Point

Prolactin Point

Location:

- The right ear normally bears the Silver Point if children are desired (the needle metal of choice needs to be decided).
- Immediately above (superior to) the ACTH Point.

Application:

- When children are desired; because hyperprolactinemia is often present, use silver needle on the right ear.
- Supports lactation (use gold needle).
- Hormonal disorders, for example, oligomenorrhea or amenorrhea associated with hyperprolactinemia (use silver needle).

Endocrine Parathyroid Gland Point

Location:

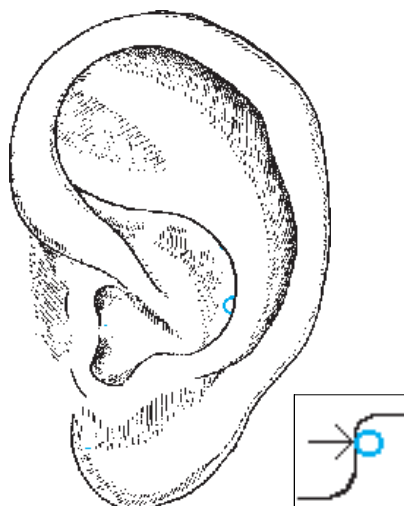
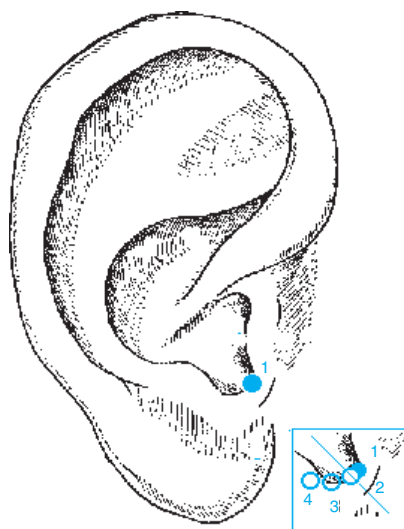
- The right ear bears the Silver Point (use gold needle on the left ear).
- At the level of Point C5 in the antihelical wall, at the transition from the upper third to the middle third of the wall.

Application:

- Disorders of the parathyroid gland.
- Corresponds to Point TB-7 of body acupuncture (p. 328).

Fig. 6.7 Prolactin Point

- 1 Prolactin Point
- 2 ACTH Point
- 3 TSH Point
- 4 Gonadotropin Point

**Fig. 6.8** Endocrine Parathyroid Gland Point

Thyroid Gland Point

Location:

- The right ear bears the Silver Point (for stimulation, use gold needle on the left ear).
- At the level of Point Zero in the antihelical wall, at the transition from the upper third to the middle third of the wall (at the intersection of the antihelical wall and a horizontal line through Point Zero; see Auxiliary Lines, p. 299).

Application:

- Disorders of the thyroid gland (use gold needle in case of hypothyroidism).
- Corresponds to Point TB-6 of body acupuncture (p. 328).

Thymus Gland Point

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- At the level of Point T4 in the antihelical wall, at the transition from the upper third to the middle third of the wall.

Application:

- The most important Functional Point in ear acupuncture with an antifocal effect.
- Has anti-inflammatory, antirheumatic, and antiallergic effects.
- Stimulates the immune system.
- Needling this point inactivates all other pathological points for a short period of time; therefore, this point should always be needled last.
- Corresponds to Cardinal Point TB-5 of body acupuncture (p. 328).

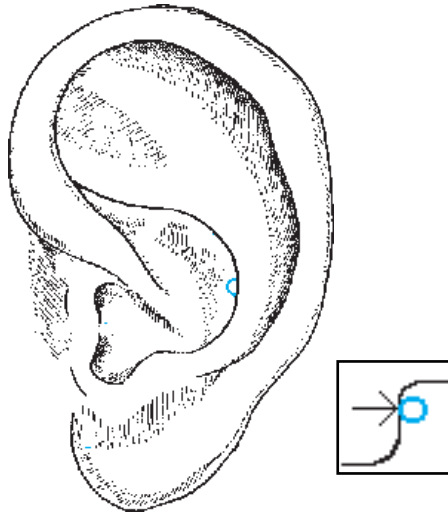


Fig. 6.9 Thyroid Gland Point

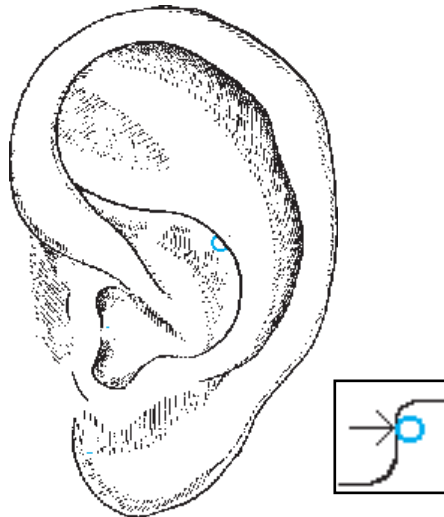


Fig. 6.10 Thymus Gland Point—corresponds to Cardinal Point TB-5 of body acupuncture

Endocrine Pancreas Point (Insulin Point)

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- At the level of Points T10–T12 in the antihelical wall, at the transition from the upper third to the middle third of the wall.

Application:

- May be tried in diabetes mellitus (Insulin Point).
- Corresponds to Point TB-4 of body acupuncture (p. 328).

Adrenal Gland Point (Cortisone Point)

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- At the level of Points T12–L1 in the antihelical wall, at the transition from the upper third to the middle third of the wall.

Application:

- The point has a cortisone effect and is, therefore, indicated in all allergic disorders.
- Corresponds to Point TB-3 of body acupuncture.

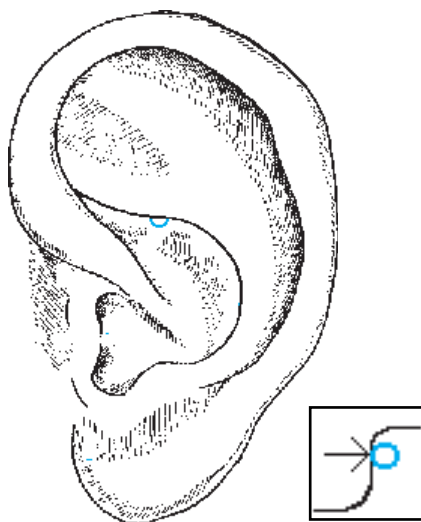


Fig. 6.11 Endocrine Pancreas Point (Insulin Point)

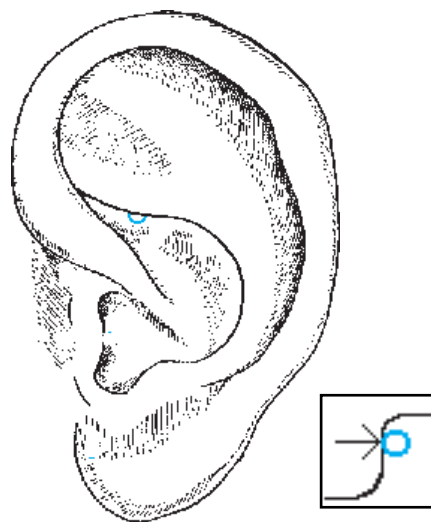


Fig. 6.12 Adrenal Gland Point (Cortisone Point)

Histamine Point (Allergy Point 1)

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- At the most superior point of the ear (apex of the helix).

Application:

- Allergies, hay fever, asthma, etc.
- To be needled from the inner or outer side of the helix.
- Double function: a Focus Indicator Point for a severe focus of Type 1 (Histamine Type) and a point corresponding to Point BL-40 of body acupuncture (p. 326).

Prostaglandin E1 Point (PGE1 Point)

Location:

- The right ear bears the Gold Point.
- On the back of the ear, about 2 mm away from the attachment site of the lobule.

Application:

- Rheumatoid diseases, hypotensive therapy, etc.
- Triple function: a Focus Indicator Point for a focus of Type 3 (PGE1 Type), a point with antirheumatic effect, and a Cardinal Point corresponding to Point GB-41 of body acupuncture (p. 330).

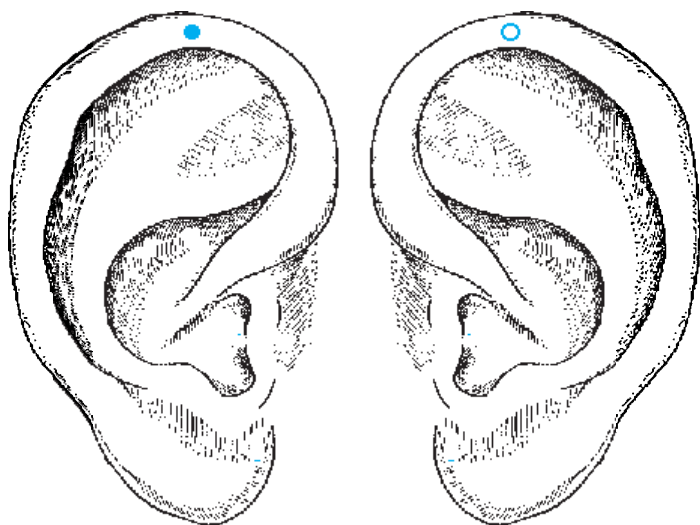


Fig. 6.13 Histamine Point (Allergy Point 1)



Fig. 6.14 Prostaglandin E1 Point (PGE1 Point)
—corresponds to Cardinal Point
GB-41 of body acupuncture

Prostaglandin E2 Point (PGE2 Point)

Location:

- The right ear bears the Gold Point.
- Near the PGE1 Point.

Application:

- Effect similar to that of the PGE1 Point.

Interferon Point

Location:

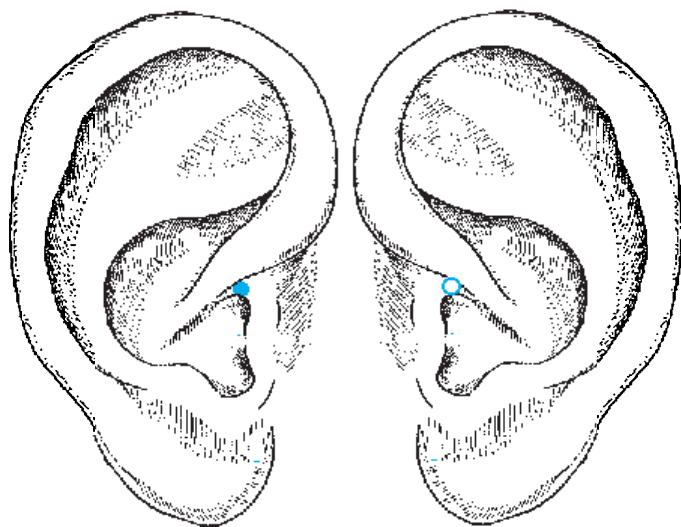
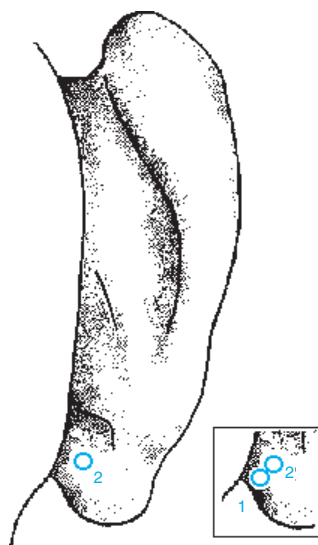
- The right ear bears the Silver Point (use gold needle in the left ear).
- In the corner of the supratragic notch.

Application:

- Anti-inflammatory.
- Increases immune defense.
- The most important point in children with fever (use laser frequency D for treatment).
- Corresponds to Cardinal Point SP-4 of body acupuncture (p. 322) and is therefore effective against diarrhea.

Fig. 6.15

- 1 Prostaglandin E1 Point (PGE1 Point)
- 2 Prostaglandin E2 Point (PGE2 Point)

**Fig. 6.16** Interferon Point—corresponds to Cardinal Point SP-4 of body acupuncture

Renin/Angiotensin Point

Location

- The right ear bears the Silver Point in patients with hypertension.
- The right ear bears the Gold Point in patients with hypotension.
- Located 2 mm above the Kidney Point in the rim of the ascending helix.

Application:

- Antihypertensive effect when used in combination with the Beta-1-Receptor Point (use silver needle on the right ear, gold needle on the left ear) and the Hypothalamus Point (use silver needle on the right ear).

Beta-1-Receptor Point (Beta-Blocker Point According to Bahr)

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- Partly hidden in the groove of the ascending helix, on the Auxiliary Line through Point Zero and Point C7/T1 (p. 297).

Application:

- Reduces activity of beta-1-receptors, has an antihypertensive effect.
- Pharmaceutical analogue: metoprolol.
- During antihypertensive therapy, it makes sense to use it in combination with the Renin/Angiotensin Point (use silver needle on the right ear) and with the Hypothalamus Point (use silver needle on the right ear), perhaps also with the Diazepam Analogue Point or with the Depression Point.

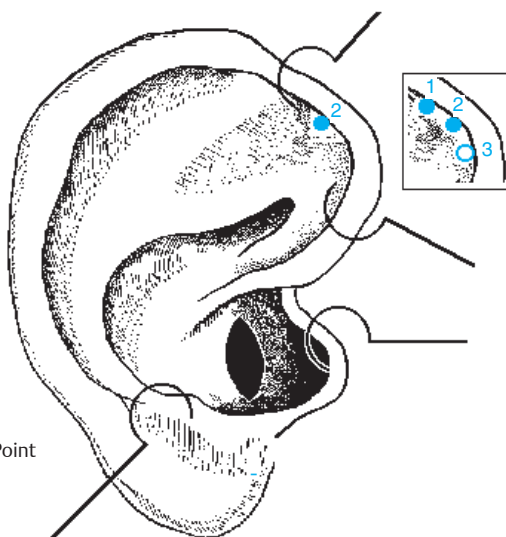


Fig. 6.17 Renin/Angiotensin Point

- 1 Progesterone Point
- 2 Renin/Angiotensin Point
- 3 Kidney Point

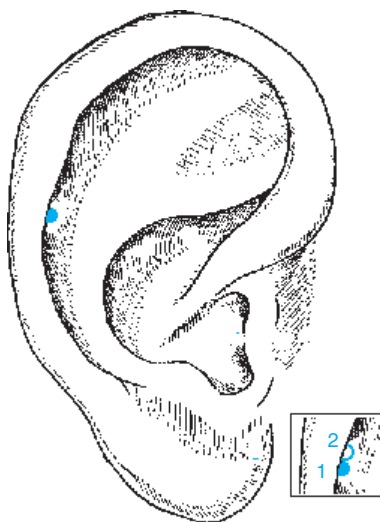


Fig. 6.18 Beta-1-Receptor Point

- 1 Beta-1-Receptor Point
- 2 Beta-2-Receptor Point

Beta-2-Receptor Point (Beta-Mimetic Point According to Bahr)

Location:

- The right ear bears the Gold Point.
- Immediately superior to the Beta-1-Receptor Point.

Application:

- Bronchospasmic conditions, bronchial asthma.
- Stimulates beta-2-receptors.
- Has a broncholytic effect.
- Pharmaceutical analogue: fenoterol.
- Corresponds to Point ST-40 of body acupuncture (p. 322).



Fig. 6.19 Beta-2-Receptor Point

1 Beta-1-Receptor Point

2 Beta-2-Receptor Point

Diazepam Analogue Point (Valium Point According to Bahr)

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- On the tragus, about 2 mm away from the edge of the tragus and just inferior to the middle of the tragus.

Application:

- Sedative and relaxing effects.
- Often leading in the treatment of lower abdominal and urinary bladder symptoms (irritable bladder); useful in combination with the Spleen Point as a second Anxiety Point (use gold needle).
- Effect identical to that of Cardinal Point KI-6 of body acupuncture (p. 326).

Barbiturate Analogue Point

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- Partly hidden in the groove of the descending helix at the level of Point C7 (horizontal line through Point C7, see Auxiliary Lines, p. 287); located within the Zone of Origin of Sympathetic Nuclei.
- Localization identical to that of the Caffeine Analogue Point (use the opposite needle metal).

Application:

- Important point, for example, in insomnia.
- May be combined with all other Psychotropic and Medication Analogue Points.
- Corresponds to Point ST-36 of body acupuncture (p. 322).

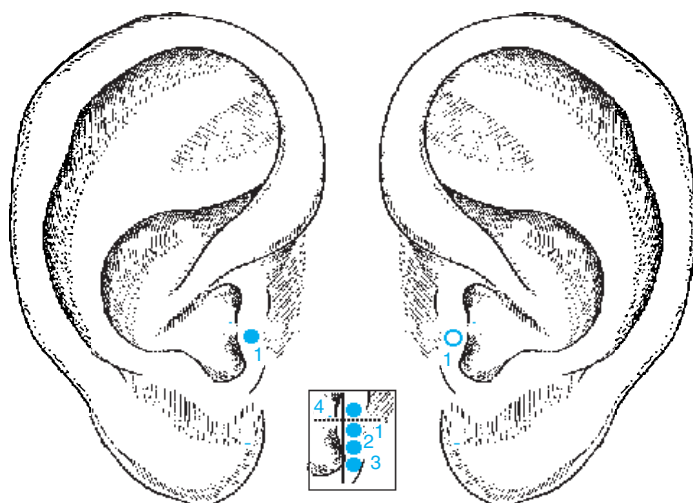


Fig. 6.20

- 1 Diazepam Analogue Point—corresponds to Cardinal Point KI-6 of body acupuncture
- 2 Nicotine Analogue Point
- 3 Pineal Gland Point
- 4 Phosphate Analogue Point

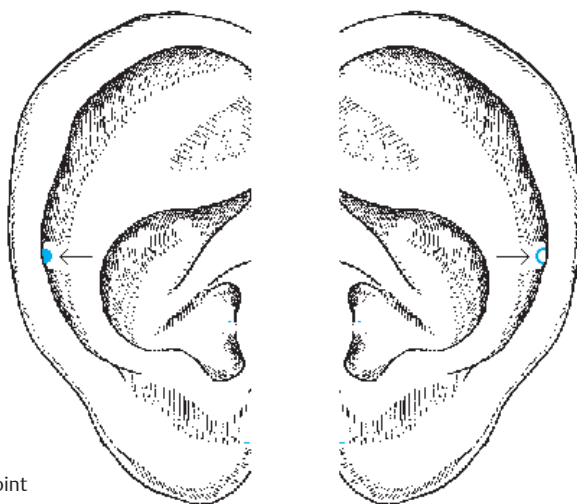


Fig. 6.21

Barbiturate
Analogue Point

Caffeine Analogue Point

Location:

- The right ear bears the Gold Point.
- Localization identical to that of the Barbiturate Analogue Point (use opposite needle metal).

Application:

- Corresponds to Point ST-36 of body acupuncture (p. 322).

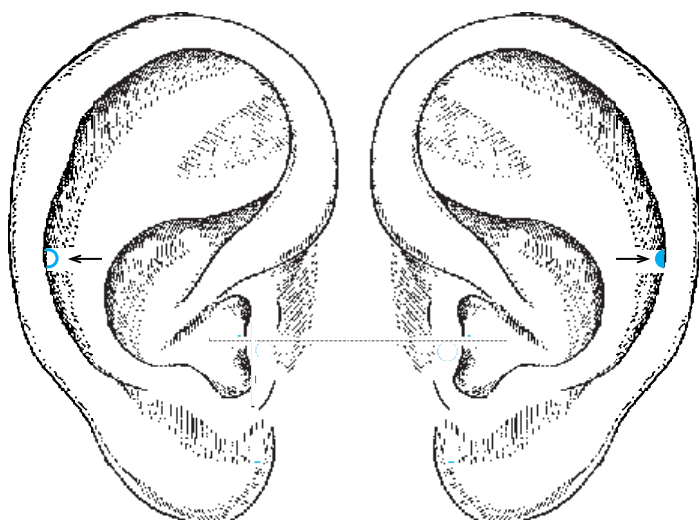


Fig. 6.22 Caffeine Analogue Point

Nervous Organ Points—Overview

Location:

The **sympathetic nerve fibers of the liver and gall bladder** are represented in the Zone of Sympathetic Trunk at the level of Points T5–T9. The Nervous Liver Point and the Nervous Gall Bladder Point are also located here, very close together (for both points, use silver needle on the right ear).

The **sympathetic nerve fibers of the stomach and duodenum** are represented in the Zone of Sympathetic Trunk at the level of Points T6–T9. The Nervous Stomach Point (use gold needle in the left ear) and the Nervous Duodenum Point (use gold needle in the right ear) are also located here.

Application:

- Syndromes affecting individual digestive organs (e.g., gastritis, duodenal ulcer).
- The Nervous Liver Point corresponds to Point LR-3 of body acupuncture (p. 330) and is also called Anger Point.

Warning: As a rule, the Nervous Points—despite their strong mental effects—follow more the anatomical localization of the organ (assigned to the sympathetic trunk). This means, **the choice of metal for a left-handed person is the same as for a right-handed person.**

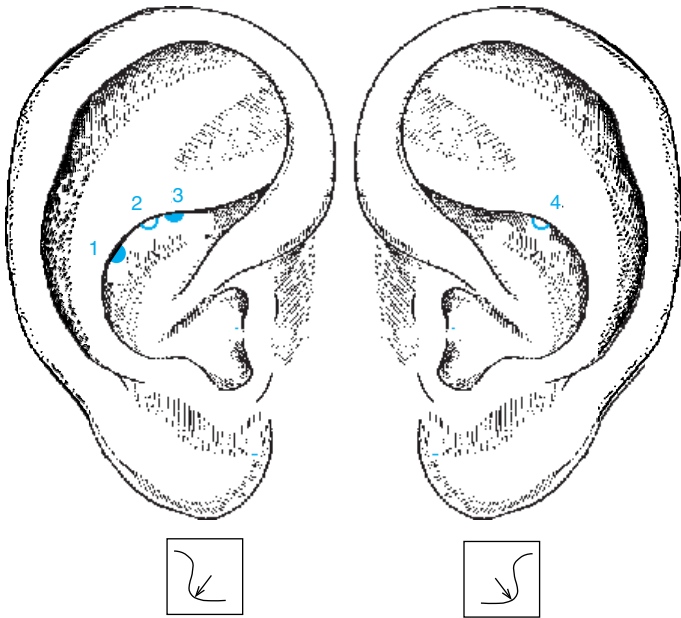
Anger Point Syn. Nervous Liver Point

Location:

- In the Sympathetic Trunk Zone at the level of Points T5–T9 (use silver needle on the right ear).

Application:

- Corresponds to the Nervous Liver Point (representing the sympathetic trunk fibers that innervate the liver), and also to Point LR-3 of body acupuncture (p. 330; in TCM there is a special connection between the liver and the emotional state of anger). Needling this point will support any treatment that aims at strengthening the liver as an organ.

**Fig. 6.23**

- 1 Stellate Ganglion Point
- 2 Nervous Duodenum Point
- 3 Nervous Liver Point (Anger Point) and Gall Bladder Point
- 4 Nervous Stomach Point

Psychotropic Points—Overview

There is, of course, some overlap between the Psychotropic Points and the Medication Analogue Points.

All Psychotropic Points as well as all Medication Analogue Points may be combined with one another without incurring side effects.

For a better understanding, those points found as Silver Points on the right (dominant) ear are illustrated there as Silver Points. However, most of the points are pricked preferably on the left (nondominant) ear with a gold needle (all descriptions are for right-handed persons; needle the opposite ear in a left-handed person).

Master Omega Point (Bromazepam Analogue Point)

Location:

- The right ear bears the Gold Point.
- On the inferior portion of the lobule, about 4 mm away from the edge.
- Forms the Omega Axis together with Omega Point I and Omega Point II (see Auxiliary Lines, p. 289).

Application:

- Possibly indicating a field of mental disturbance.
- Medication assigned to this point: bromazepam.
- Corresponds to Points SP-21 (p. 322) and CV-17 (p. 332) of body acupuncture.

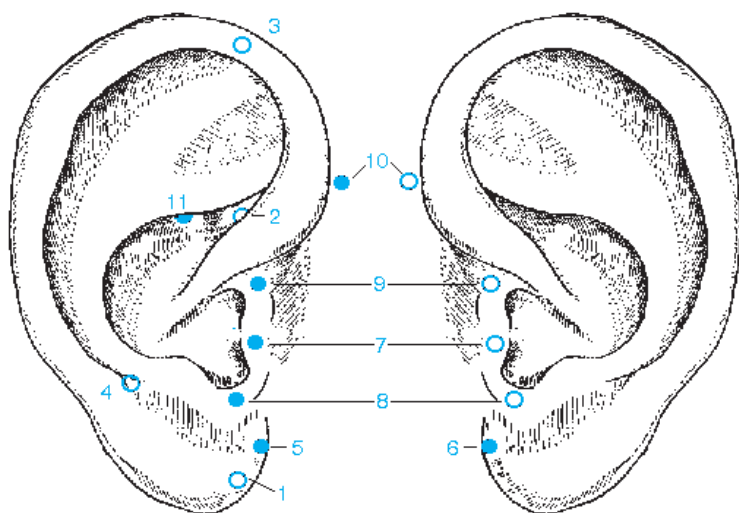


Fig. 6.24

- 1 Master Omega Point
- 2 Omega Point I
- 3 Omega Point II
- 4 Depression Point
- 5 Anxiety Point
- 6 Worry Point
- 7 Diazepam Analogue Point
- 8 Aggression Point
- 9 Frustration Point
- 10 Psychotherapy Point according to Bourdiol
- 11 Anger Point

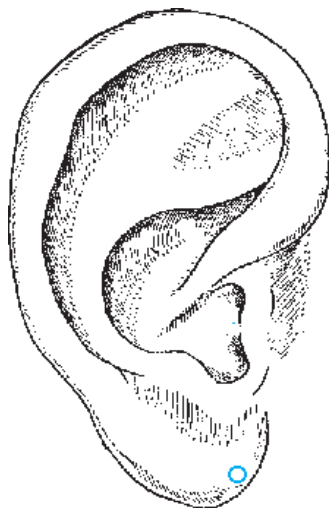


Fig. 6.25 Master Omega Point (Bromazepam Analogue Point)

Omega Point I

Location:

- The right ear bears the Gold Point, possibly indicating a mercury load.
- In the superior concha, in the middle of the triangle formed by ascending helix and antihelix.

Application:

- Prick with gold needle as a supporting measure in case of mercury load or during amalgam elimination (perhaps in combination with ACTH Point, Thy-mus Gland Point).
- An autonomic Functional Point (Hypogastric Plexus Point).
- Corresponds to Point CV-12 of body acupuncture (p. 332, Focus Indicator Point for mercury load).

Omega Point II

Location:

- The right ear bears the Gold Point.
- On the ascending helix, on the line running through Omega Point I and Master Omega Point.

Application:

- Mental problems related to a person's interaction with other people and the environment.
- Also an area of Indicator Points for vitamins B1, B3, B6, and E (p. 267).
- Corresponds to Point CV-5 of body acupuncture (p. 332).

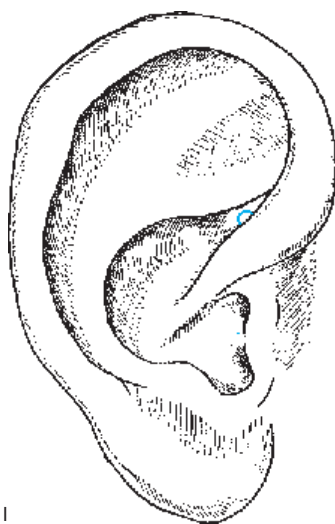


Fig. 6.26 Omega Point I

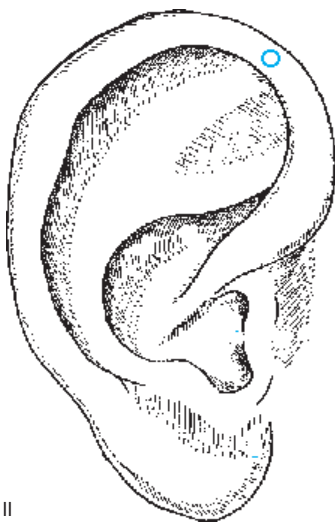


Fig. 6.27 Omega Point II

Depression Point

Location:

- The right ear bears the Gold Point (synonyms: Joy Point, Antidepression Point).
- On a horizontal line at the level of the intertragic notch.
- Auxiliary Line: Depression Point–Point C0–Point Zero (see Auxiliary Lines, p. 295).
- Localization identical to that of Magnesium Point.
- Differential diagnosis required because this localization is almost identical with the reflex areas of the temporomandibular joint, palatine tonsils, upper and lower wisdom teeth, and retromolar space.

Application:

- General mood-enhancing effect.
- For support during menopausal syndrome.
- Pointing in the direction of a field of mental disturbance (masked depression) in case of spinal and other complaints that cannot be controlled through local points and classical focus therapy.
- Corresponds to Point HT-9 of body acupuncture (p. 324).

Anxiety Point

Location:

- The right ear bears the Silver Point (use silver needle on right ear).
- On the lobule where the ear lobe attaches to the facial skin. If the lobule is flat or short, approximately at the level of the Eye Point.

Application:

- Corresponds to Point LU-1 of body acupuncture (p. 320).
- May be combined with the Spleen Point to increase its effect (use gold needle on the left ear for “classical Chinese organ coupling”). The latter point corresponds to Point SP-2 of body acupuncture (p. 322).
- Is needled as Silver Point because the identical localization on the contralateral ear is the Sorrow Point (also needled as Silver Point).

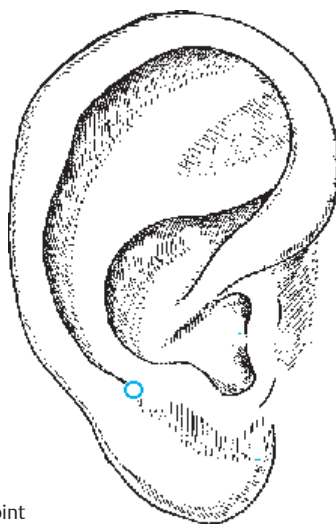


Fig. 6.28 Depression Point

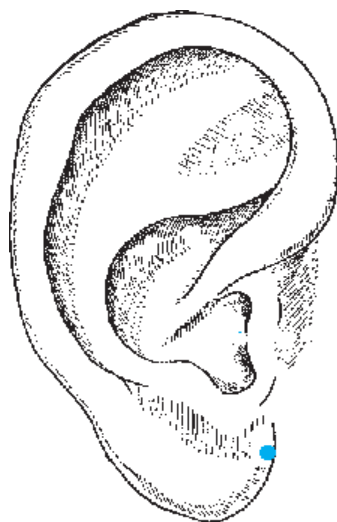


Fig. 6.29 Anxiety Point

Worry Point

Location:

- The left ear bears the Silver Point (use silver needle on the left ear).
- Location similar to that of the Anxiety Point but on the left ear (at the attachment site of the lobule, approximately at the level of the Eye Point).

Application:

- Corresponds to Point LU-1 of body acupuncture (p. 320).
- May be combined with the Gall Bladder Point to increase its effect (use gold needle on the right ear for “classical Chinese organ coupling”). The Gall Bladder Point corresponds to Point GB-43 of body acupuncture (p. 330, gold needle on the right ear). Often essential during migraine therapy.

Anger Point

Location:

- In the Sympathetic Trunk Zone, at the level of Points T5 to T9.
- The right ear bears the Silver Point. Even in a left-handed person, the Anger Point is found and needled on the right ear using a silver needle.
- The location is identical to that of the Nervous Liver Point.

Application:

- The point corresponds to Point LR-3 of body acupuncture.
- As the name implies, it is used to treat anger, liver *qi* stagnation, and aggressive tendencies.

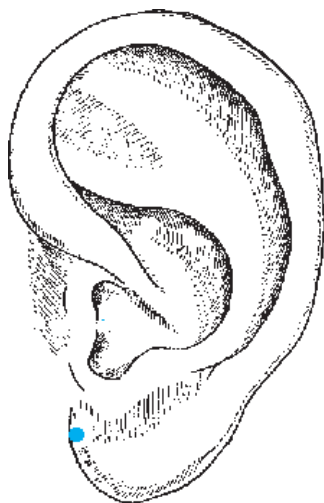


Fig. 6.30 Worry Point

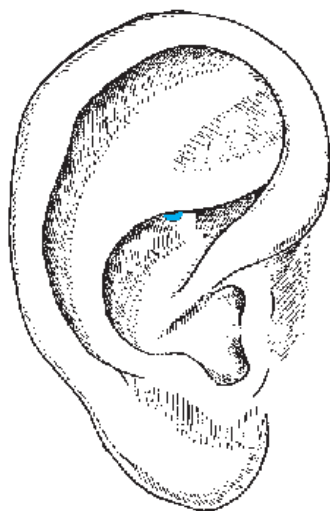


Fig. 6.31 Anger Point

Nicotine Analogue Point According to Bahr

Location:

- The right ear bears the Silver Point.
- On the tragus about 2 mm away from the edge, on a line between Diazepam Analogue Point and Pineal Gland Point.

Application:

- If the Silver Point is very prominent on the right ear, an exception is made by needling on the right ear during the antiaddiction program.
- The point becomes more pronounced if the patient has not smoked for several hours prior to needling.

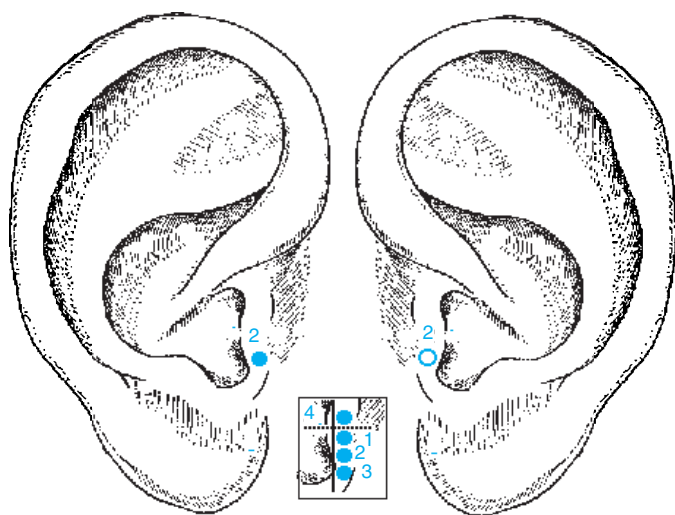


Fig. 6.32

- 1 Diazepam Analogue Point
- 2 Nicotine Analogue Point
- 3 Pineal Gland Point—corresponds to Cardinal Point BL-62 of body acupuncture
- 4 Phosphate Analogue Point

Aggression Point

Location:

- The right ear bears the Silver Point.
- About 2 mm inferior to the intertragic notch. The Aggression Point is located as much **below** the intertragic notch as the Pineal Gland Point is located **in front** of it (see Auxiliary Lines, p. 307).

Application:

- If the Silver Point is very pronounced on the right ear, an exception is made by needling on the right ear during the antiaddiction program.
- May be combined with the Master Omega Point via the Bridging Point (use the latter point for permanent needling). Helpful in all addictions.
- Corresponds to Point LR-14 of body acupuncture (p. 330).

Bridging Point According to Bahr

Location:

- Midway between Master Omega Point and Aggression Point.

Application:

- Used in combination with Master Omega Point and Aggression Point for treating addictions.
- All three points are treated with permanent needles.

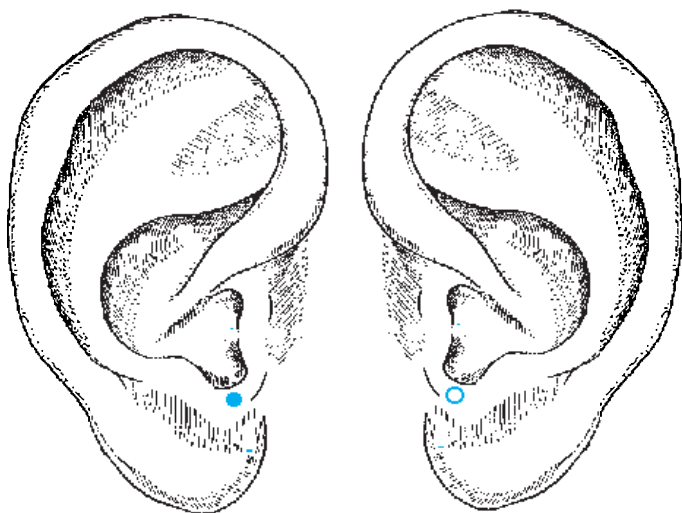


Fig. 6.33 Aggression Point

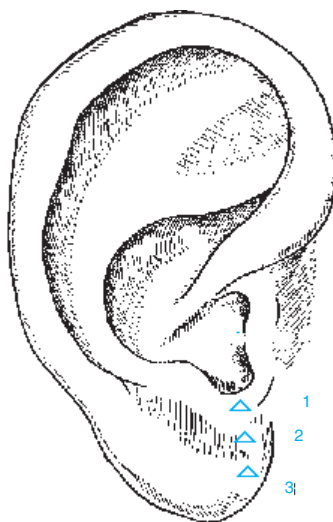


Fig. 6.34

- 1 Aggression Point
- 2 Bridging Point
- 3 Master Omega Point

Frustration Point

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- About 0.5 mm **anterior** to and slightly above the Interferon Point.

Application:

- Include point in the diagnostic test and needle permanently in cases of addictions.

Psychotherapy Point According to Bourdiol

Location:

- The right ear bears the Silver Point (use gold needle on the left ear, if necessary).
- On the facial skin in front of the ascending helix.

Application:

- Supportive in all psychosomatic disorders, especially in globus hystericus without organic cause.
- A certain resonance with haloperidol.

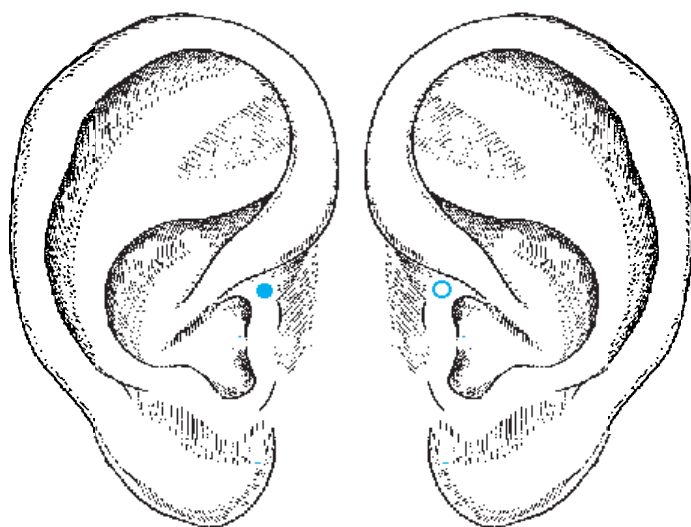


Fig. 6.35 Frustration Point

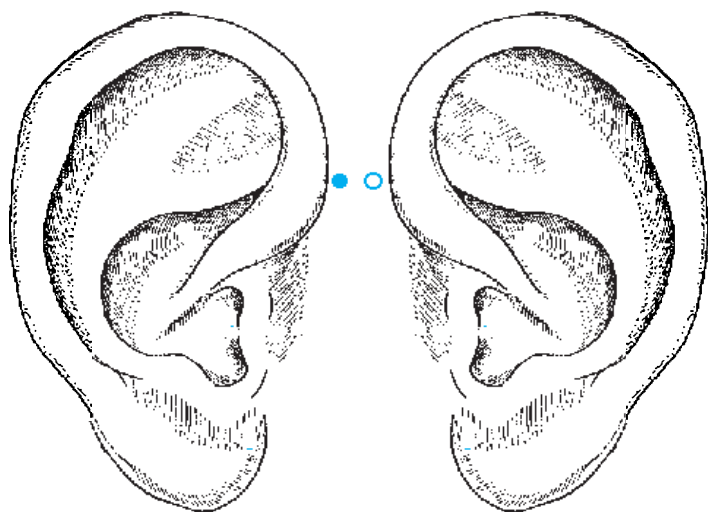


Fig. 6.36 Psychotherapy Point according to Bourdiol

Thalamus Point

Location:

- The right ear bears the Gold Point.
- On the inside of the antitragus where the inferior concha merges with the ascending wall of the antitragus.
- When pulling the antitragus toward the outside, a depression becomes visible on its inner wall. The Thalamus Point is located at the deepest point of this depression.

Application:

- The most important analgesic point of auriculotherapy.
- Corresponds to Point LI-4 of body acupuncture (p. 320).

Prostaglandin E1 Point (PGE1 Point)

Location:

- The right ear bears the Gold Point.
- On the back of the ear, about 2 mm away from the attachment site of the lobule.

Application:

- Effective pain control, with the effect being similar to that of the endogenous hormone, prostaglandin, especially in rheumatoid diseases.
- Multiple function: an Indicator Point for a focus of Type 3 (PGE1 Type) and a Cardinal Point corresponding to Point GB-41 of body acupuncture (p. 330).

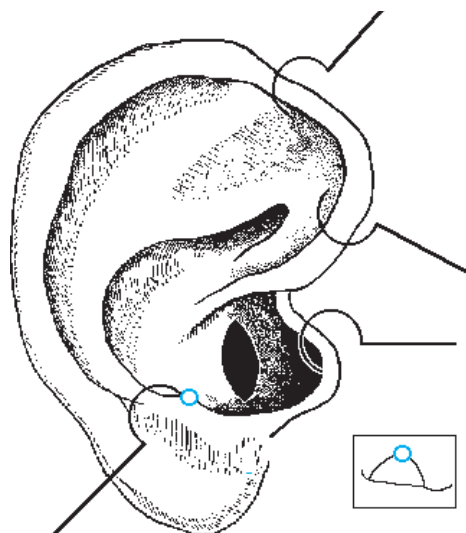


Fig. 6.37 Thalamus Point



Fig. 6.38 Prostaglandin E1 Point (PGE1 Point)
—corresponding to Cardinal Point GB-41 of body acupuncture

Laterality Point According to Bahr

The Laterality Point may be classified as a Psychotropic Point as well as a Medication Analogue Point (it has the same effect as ginseng root extract: it stabilizes laterality).

This is an important point in diagnosis and therapy, not only for determining handedness but also for diagnosing and treating unstable laterality (oscillation).

Location:

- The right side bears the Gold Point.
- Approximately 3 cm in front of the right tragus.

Application:

- Powerful analgesic effect.
- Stabilizing effect on the psyche.
- Can be detected only in case of unstable laterality (e.g., during pain) or, in its second function, as a Focus Indicator Point.
- In left-handed persons, especially in those persuaded to use their right hand, almost always present as Gold Point on the left side. This indicates unstable laterality which may give rise to autonomic symptoms or even disease.
- May be used for confirmation of the established handedness (only in right-handed persons is it present as Gold Point on the right side).
- Corresponds to Point LI-1 of body acupuncture.



Fig. 6.39 Laterality Point according to Bahr

Shen Men Point

The best-known analgesic point of the Chinese system.

Location:

- On the superior antihelical crus, just above the Hip Point.
- The right ear bears the Gold Point.

Indications:

- This primary Master Point has a general analgesic effect; it stabilizes the mind.

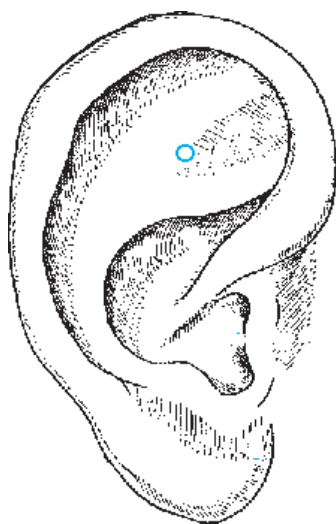


Fig. 6.40 *Shen Men Point*

Pain Memory Points According to Bahr

According to current knowledge, the term “pain memory” includes the following:

- efferent and afferent inhibitory systems of the spinal cord (noradrenalin, serotonin),
- the limbic system (affective evaluation of pain) (see p. 138),
- the thalamus (general pain modulation) (see p. 130),
- the somatosensory cortex (postcentral gyrus: cognitive evaluation and perception of pain) (see p. 124).

It is therefore not surprising that the Pain Memory Points—which Bahr discovered in 2002—are localized on the ear in the reflex zone of the limbic system. He found them to be Points of Excess, that is, they should be calmed down using silver needles. This tallies well with the finding that chronic pain is associated with excessive cerebral metabolism especially in the limbic system and also with pathological cell growth.

Location:

- On the antitragus in the zone of the limbic system. The affected side is treated with a silver needle.

Indications:

- All chronic pain conditions in which a developing pain memory is suspected on clinical grounds.

Before the symptom point in question is needled, it is best to needle first the associated Pain Memory Point of the affected side. It does make sense to search for auricular reflex zones of other parts of the pain memory (thalamus, somatosensory cortex) and to include these in the therapy, if applicable.

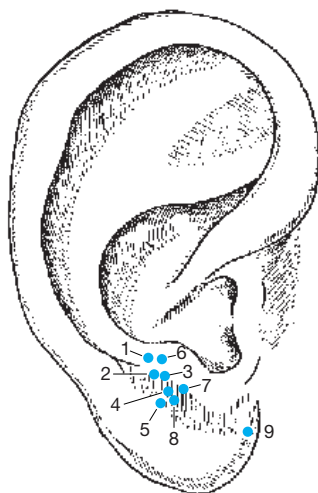


Fig. 6.41 Pain Memory Points according to Bahr

1 Vertebral column

2 Elbow

3 Shoulder

4 Ankle joint

5 Knee

6 Wrist

7 Heel

8 Hip

9 General Analgesic Point

Cardinal Points—Overview

For each of the eight Cardinal Points of body acupuncture (in Traditional Chinese Medicine also called the eight Opening Points or Confluent Points), Bahr has discovered a corresponding point on the ear. These ear points have been given **the same names** as their body counterparts and are located on special meridians on the ear (see Meridians of the Ear, p. 318).

This means that, also in auriculotherapy, the specific effects of the Cardinal Points can easily be employed for supplementary treatment.

Furthermore, comparison of the activity spectra of the newly discovered ear points with those of the already known body points yielded interesting new insights into the Cardinal Points of both ear and body.

The following pairs of Cardinal Points on the ear are often found to appear together (the names of the corresponding body acupuncture points are given in parentheses):

- PGE1 Point (GB-41) on the right ear as Gold Point ↔ Thymus Gland Point (TB-5) on the left ear as Gold Point.
- Lung Zone (LU-7) on the right ear as Gold Point ↔ Diazepam Analogue Point (KI-6) on the left ear as Gold Point.
- Stellate Ganglion Point (PC-6) on the right ear as Gold Point ↔ Interferon Point (SP-4) on the left ear as Gold Point.
- Retro-Celiac (Retro-Solar) Plexus Point (SI-3) on the right ear as Gold Point ↔ Pineal Gland Point (BL-62) on the left ear as Gold Point.

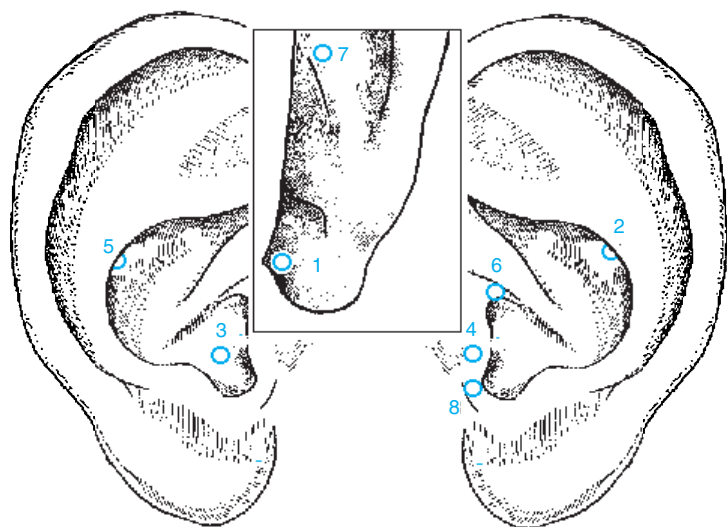
Prostaglandin E1 Point (PGE1 Point)—Corresponds to Cardinal Point GB-41 of Body Acupuncture

Location:

- The right ear bears the Gold Point.
- On the back of the ear, approximately 2 mm away from the attachment site of the lobule.

Application:

- Master Point for treating affections of the joints.
- Same indications as for the endogenous hormone, prostaglandin, especially rheumatic diseases, hypertension, etc.

**Fig. 6.42**

- 1 GB-41 –Prostaglandin E1 Point (PGE1 Point)
- 2 TB-5 –Thymus Gland Point
- 3 LU-7 –Lung Zone
- 4 KI-6 –Diazepam Analogue Point
- 5 PC-6 –Stellate Ganglion Point
- 6 SP-4 –Interferon Point
- 7 SI-3 –Retro-Celiac (Retro-Solar) Plexus Point
- 8 BL-62 –Pineal Gland Point

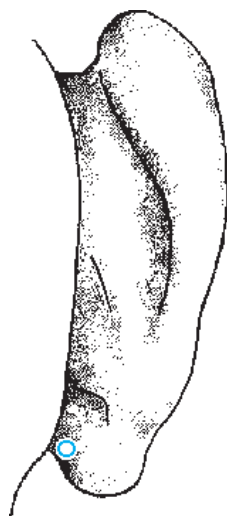


Fig. 6.43 Prostaglandin E1 Point (PGE1 Point)
 –corresponding to Cardinal Point GB-41 of body acupuncture

Thymus Gland Point—Corresponds to Cardinal Point TB-5 of Body Acupuncture

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- At the level of Point T4 in the antihelical wall.

Application:

- Master Point for treating all inflammatory and rheumatoid processes, above all when joints are affected.
- Ear acupuncture's most important Functional Point against focal disturbance.
- Has anti-inflammatory, antirheumatic, and antiallergic effects.
- Stimulates the immune system.
- Needling this point inactivates all other pathological points for a short period of time; therefore, this point should always be needled last.

Lung Zone—Corresponds to Cardinal Point LU-7 of Body Acupuncture

Location:

- The right ear bears the Gold Point.

Application:

- Master Point for treating all affections of the respiratory tract, also Master Point for treating congestion (or fullness, in Traditional Chinese Medicine terms); antidepressant effect, effective in conditions of weakness.
- According to Traditional Chinese Medicine, this area is important for Defense Energy (*Wei Qi*).

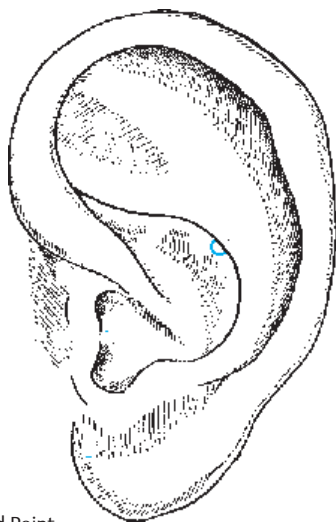


Fig. 6.44 Thymus Gland Point
—corresponding to Cardinal Point TB-5 of body acupuncture

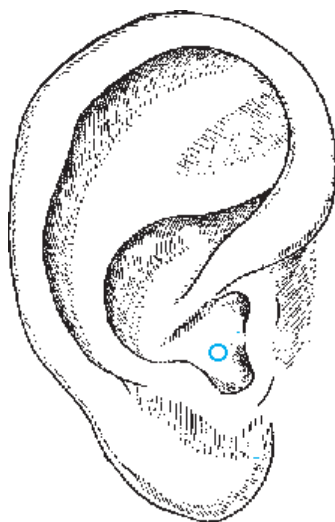


Fig. 6.45 Lung Zone—corresponding to Cardinal Point LU-7 of body acupuncture

Diazepam Analogue Point (Valium Point According to Bahr) —Corresponds to Cardinal Point KI-6 of Body Acupuncture

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- On the tragus, about 2 mm away from the edge and just below the middle of the tragus.

Application:

- Master Point for treating insomnia and all symptoms that worsen during menstruation; also effective against menopausal symptoms.
- Pharmaceutical analogue: diazepam (Valium).
- Called also Tranquilizer Point because of its anxiolytic effect.

Stellate Ganglion Point —Corresponds to Cardinal Point PC-6 of Body Acupuncture

Location:

- The right ear bears the Gold Point.
- At the level of Point C7 at the transition of the concha to the antihelical wall.

Application:

- Local effect on the stellate ganglion when the first rib is blocked (use silver needle).
- Regulating effect on circulation; hormone-like activity.

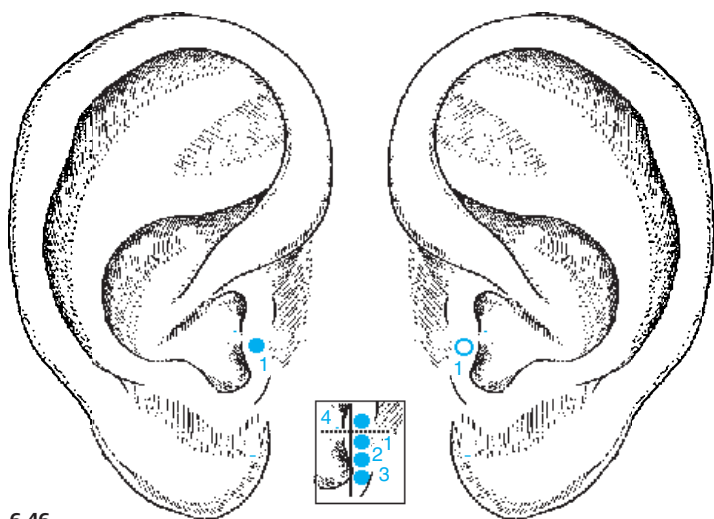


Fig. 6.46

- 1 Diazepam Analogue Point—corresponding to Cardinal Point KI-6 of body acupuncture
- 2 Nicotine Analogue Point
- 3 Pineal Gland Point—corresponding to Cardinal Point BL-62 of body acupuncture
- 4 Phosphate Analogue Point

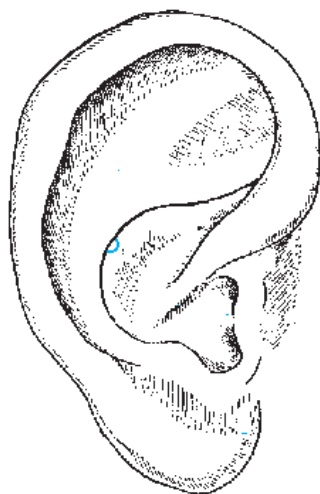


Fig. 6.47 Stellate Ganglion Point
—corresponding to Cardinal Point PC-6
of body acupuncture

Interferon Point—Corresponds to Cardinal Point SP-4 of Body Acupuncture

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- In the supratragic notch.

Application:

- Master Point for treating all forms of diarrhea.
- Anti-inflammatory effect.
- Increases the immune defense.
- Most important point for treating fever in children.

Retro-Celiac Plexus Point (Retro-Solar Plexus Point, Retro-Point Zero)—Corresponds to Cardinal Point SI-3 of Body Acupuncture

Location:

- The right ear bears the Gold Point.
- On the back of the ear, 2–3 mm away from the attachment of the ear.

Application:

- Master Point for spasmolytic treatment.
- Has an effect on the celiac (solar) plexus.
- General point for treating mucosal ailments.
- Especially effective in torticollis and ankylosing spondylitis (Bechterew's disease) in combination with the Pineal Gland Point (which corresponds to Point BL-62 of body acupuncture).

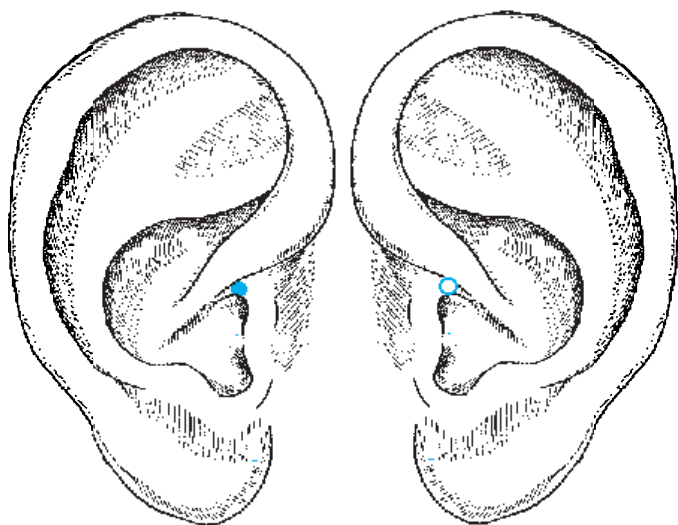


Fig. 6.48 Interferon Point
—corresponding to Cardinal Point SP-4 of body acupuncture

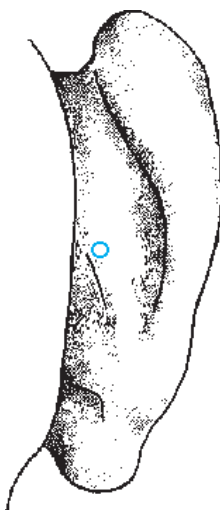


Fig. 6.49 Retro-Celiac (Retro-Solar) Plexus Point
—corresponding to Cardinal Point SI-3 of body acupuncture

Pineal Gland Point—Corresponds to Cardinal Point BL-62 of Body Acupuncture

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- Mirror image to the Aggression Point but anterior (rather than inferior) to the intertragic notch.

Application:

- Master Point for treating nervous insomnia.
- Premenstrual syndrome and other conditions of tension.

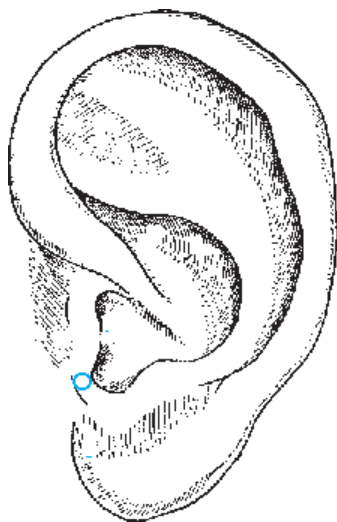


Fig. 6.50 Pineal Gland Point
—corresponding to Cardinal Point BL-62 of body acupuncture

Master Point of Oscillation According to Bahr

Location:

- Approx. 1 cm in front of the lobule at the level of the Eye Point; at the same level as the Master Point of Regulation. The right ear bears the Gold Point.

Indications:

- Highly effective in treating oscillation; highly effective in energizing the whole body as it corresponds to the Source Point of the Kidney (Point KI-3) of body acupuncture. When needling this point, the intensity of focal disturbances detected so far usually drops by one or two levels. In a patient showing general exhaustion, it makes sense to combine this point with the Kidney Point (corresponding to Point KI-7 of body acupuncture).

Master Point of Regulation According to Bahr

Location:

- The Master Point of Regulation lies in front of the ear on the auxiliary line running through Laterality Point and Endoxan Point. The right ear bears the Gold Point.

Indications:

- All conditions calling for general stimulation of nutrition and regulation.

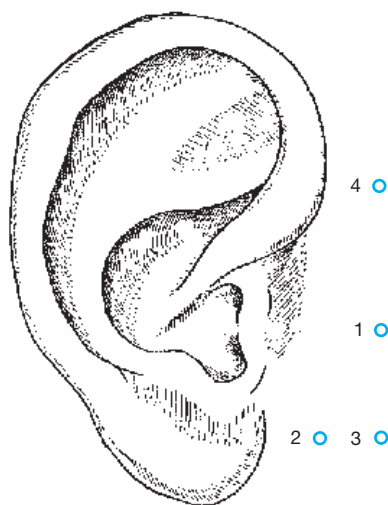


Fig. 6.51

- 1 Laterality Point
- 2 Master Point of Oscillation
- 3 Master Point of Regulation
- 4 Endoxan Point

Super Omega Point According to Bahr

This newly detected point has a broad effect on other Omega Points.

Location:

- Approx. 1 cm above the ear on the scalp, on the auxiliary line running through the other Omega Points. The right ear bears the Gold Point.

Indications:

- When an illness is largely affected by emotional factors, or by mental or unconscious imbalances. By needling the Super Omega Point, treatment of the other Omega Points becomes unnecessary (thus making room for other needles, if necessary).

Master Point of Qi Flow According to Bahr

The flow of *qi* obviously influences nutrition. It is therefore not surprising that Bahr found the Master Point of *qi* Flow as a satellite point to the Zone-Dominating Point B1 (Point ZB1, see pp. 274 f.).

Location:

- Almost identical to Point ZB1, in front of the ear in continuation of the inferior antihelical crus. The right ear bears the Gold Point.

Indications:

- All conditions that Traditional Chinese Medicine (TCM) identifies as *qi* stagnation (deficient flow of *qi*). These include the easily identifiable Weakness of the Middle Burner (see below). If the point is electrically active, it should be included in the treatment plan.

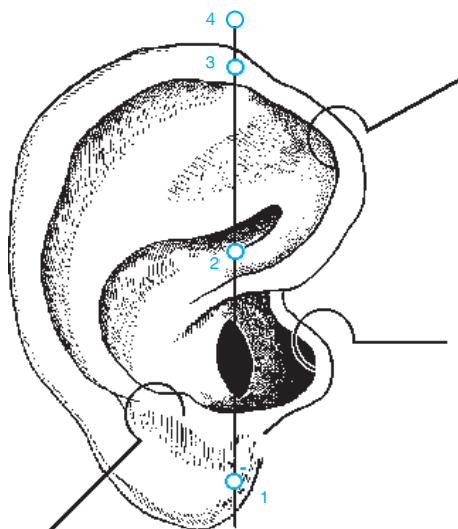


Fig. 6.52

- 1 Master Omega Point
- 2 Omega Point I
- 3 Omega Point II
- 4 Super Omega Point



Fig. 6.53 Master Point of *Qi* Flow

Point Zero

The transverse auxiliary line is indicated for treating Weakness of the Middle Burner (TCM: *qi* stagnation), a disturbed exchange between the upper and lower body. Symptoms include Excess in the upper body associated with a swollen tongue (impressions of the teeth), hypertension, red face, arrhythmia, migraine, sometimes also anxiety, and Cold/Emptiness below the navel, associated with cold feet, genitourinary problems, and sometimes impotence.

Location:

- On the root of helix, in a cartilage depression that can be clearly palpated with the stirrup-shaped ear probe. The right ear usually bears the Gold Point.

Indications:

- Weakness of the Middle Burner (see above) as a result of eating refrigerated foods and having too much stress. The point is identical with the Celiac Plexus Point (Solar Plexus Point) and thus is indicated for imbalances due to severe nervous tension. Many auxiliary lines run through Point Zero (see pp. 291 ff.).
- Point Zero works well in combination with the organ point of the spleen on the left ear (Point SP-2; the spleen is an organ of the Middle Burner, see p. 322).

Point GV-4

Location:

- At the attachment site on the back of the ear, near the central posterior sulcus. The Gold Point is found on the left ear.

Indications:

- When combined with Point Zero, this point stabilizes the Middle Burner (see above). Point GV-4 alone has a strong energizing effect on the lumbar spine by strengthening the kidney, which—according to TCM—dominates the bones. The point is identical with the DNA Point, the Master Point for potency (see p. 332).

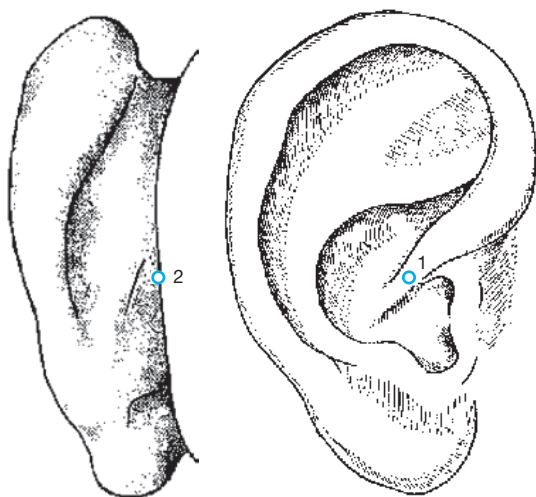


Fig. 6.54

1 Point Zero

2 Point GV-4

Focus Indicator Points According to Bahr—Overview

The Focus Indicator Points are a system for quick and easy diagnosis:

- Is there a focal disturbance (or even several foci)?
- How many foci are present?
- Which of several scars is a focal disturbance?
- How strong are these foci?

To be able to use this system successfully, one should be familiar with Nogier's reflex, or vascular autonomic signal (VAS) (knowledge level 2).

The Focus Indicator Points are used exclusively for diagnosis (knowledge level 2 and above); they are not needed as such. However, some of them may be used therapeutically because of their dual function (Histamine Point, PEG 1 Point, Laterality Point).

For further information and detailed work instructions, see B. Strittmatter, *Identifying and Treating Blockages to Healing—New Approaches to Therapy-resistant Patients*, Thieme Publishers, Stuttgart–New York, 2004.

Histamine Point (Allergy Point 1)

Location:

- The right ear bears the Silver Point (use gold needle on the left ear).
- At the apex of the helix.

Application:

- Indicates a very intense focus of Type 1 (Histamine Type).
- Corresponds to Point BL-40 of body acupuncture (p. 326).
- Double function: Indicator Point for true allergies in asthmatic or allergic persons.
- The point is not needed in its function as an Indicator Point.

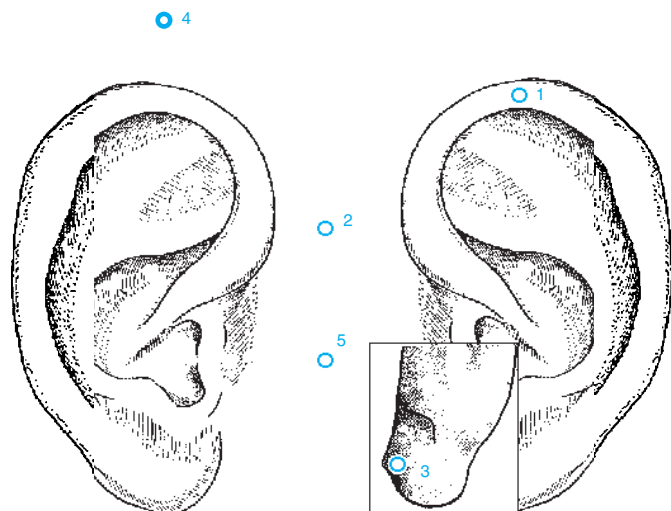


Fig. 6.55 Five Focus Indicator Points:

- 1 Histamine Point (Allergy Point 1)
- 2 Cyclophosphamide Analogue Point (Allergy Point 2), on the right side
- 3 Prostaglandin E1 Point (PGE1 Point), on the back of the right ear
- 4 Vitamin C Point
- 5 Laterality Point, on the right side

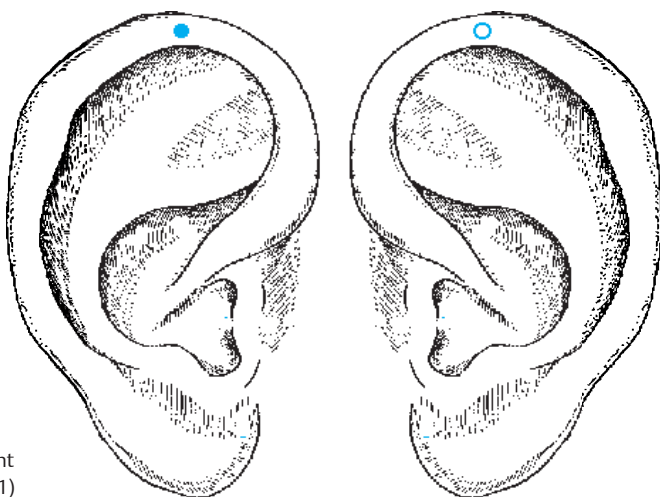


Fig. 6.56

Histamine Point
(Allergy Point 1)

Cyclophosphamide Analogue Point (Endoxan Point According to Bahr, Allergy Point 2)

Location:

- The right side bears the Gold Point.
- In front of the ear in continuation of the antihelix. In most persons this site is in the hair region.

Application:

- Indicates an intense focus of Type 2 (Endoxan Type).
- Corresponds to Point PC-9 of body acupuncture (p. 328).
- Pharmaceutical analogue: cyclophosphamide (Endoxan).
- The point is not needled as an Indicator Point.

Prostaglandin E1 Point (PGE1 Point)

Location:

- The right ear bears the Gold Point.
- On the back of the ear, about 2 mm away from the attachment site of the lobule.

Application:

- Indicates a focus of Type 3 (PGE1 Type).
- Frequently indicates a focus in rheumatoid disease.
- Multiple function: prostaglandin-like effect, corresponds to Cardinal Point GB-41 of body acupuncture (p. 330).

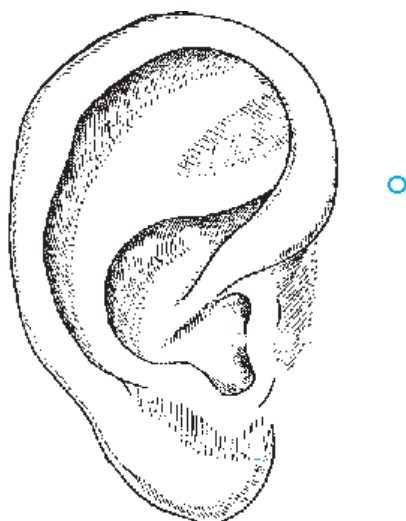


Fig. 6.57 Cyclophosphamide Analogue Point (Allergy Point 2)



Fig. 6.58 Prostaglandin E1 Point (PGE1 Point)
—corresponding to Cardinal Point GB-41 of body acupuncture

Vitamin C Point

Location:

- The right side bears the Gold Point.
- About 2 cm above the apex of the ear in the hair region.

Application:

- Indicates a weak focus (Type 4).
- Corresponds to Point TB-1-1 of body acupuncture (p. 328).

Laterality Point According to Bahr

Location:

- The right side bears the Gold Point.
- About 3 cm in front of the tragus.

Application:

- Indicates a very weak focus (Type 5).
- Multiple functions: analgesic effect, stabilizing effect on laterality.
- Corresponds to Point LI-1-1 of body acupuncture (p. 320).

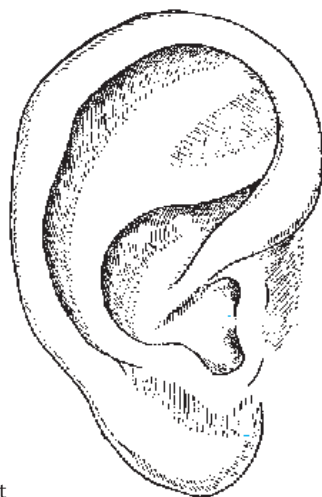


Fig. 6.59 Vitamin C Point

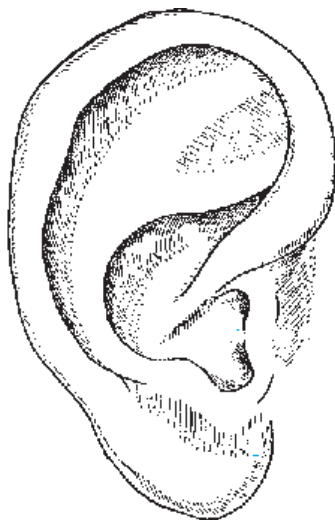


Fig. 6.60 Laterality Point according to Bahr

Tissue Layer Control Points—Overview

These points differ in their sensitivity to pressure depending on the auricular tissue layers they control (deep, middle, and superficial tissue layers). They were discovered by Nogier and were originally used for diagnosis.

Today, they are used for practicing VAS but are not needed.

An exception is the Control Point of the Superficial Tissue Layer (the same location as Point Zero, or Umbilical Cord Point), which may be used therapeutically because of its double function as Point Zero.

All three control points are present as Gold Points on the dominant ear.

Control Zone of the Deep Tissue Layer

Location:

- A small area near the superior attachment site of the ear, located on the scalp. It is partially covered by the ascending helix.

Application:

- Just for practice (formerly diagnostic). The beginner can use this zone for practicing VAS, provided the 9-Volt bars are properly positioned for the deep tissue layer.

Control Zone of the Intermediate Tissue Layer

Location:

- On the back of the dominant ear at the upper edge of the lobule.

Application:

- Just for practice (formerly diagnostic). The beginner can use this zone for practicing VAS, provided the 9-Volt bars are properly positioned for the intermediate tissue layer.

For diagnosis only. As the zone is active in the intermediate tissue layer only, the beginner can check his/her own performance in practicing VAS and positioning the 9-Volt bars.

Control Point of the Superficial Tissue Layer

Location:

- On the root of helix in a palpable cartilage depression (same location as Point Zero or Umbilical Cord Point), as Gold Point on the dominant ear.

Application:

- Just for practice or, because of its double function, as Point Zero (see pp. 184, 291–301).

As Indicator Point only active in the superficial tissue layer; the beginner can therefore use this point for practicing VAS, provided the 9-Volt bars are properly positioned for the superficial tissue layer.

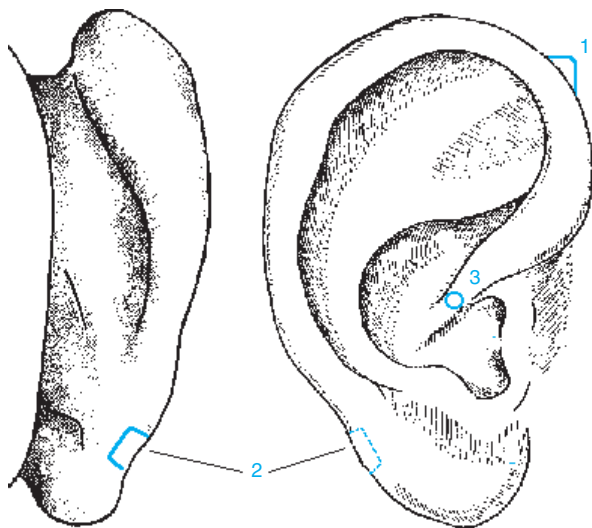


Fig. 6.61

- 1 Control Zone of the Deep Tissue Layer
- 2 Control Zone of the Intermediate Tissue Layer
- 3 Control Point of the Superficial Tissue Layer

Indicator Points for Vitamins According to Bahr—Overview

Location:

- The right ear bears the Gold Points.

Application:

- Normally for the diagnosis of relative vitamin deficiencies.
- The importance of some Vitamin Indicator Points is now known in detail:

The Vitamin B5 Indicator Point corresponds to Point ST-45 of **body acupuncture** (Sedation Point of Stomach Meridian) and, thus, may be needled in case of stomach complaints.

Furthermore, this finding explains the positive effect of administering vitamin B5 (pantothenic acid) in acute and chronic gastritis.

Indicator Points for Trace Elements According to Bahr—Overview

Location:

- Most of the resonance points of trace elements known so far are normally found **as Gold Points on the right ear** if there is a deficiency in the particular trace element.

By contrast, an overload normally presents itself as an energy-rich point (Silver Point). When an essential trace element (e.g., iron) is present in excess, a Silver Point is present in the respective reflex area on the ear.

An exception is Omega Point I which appears as Gold Point in case of a mercury load. This is a sign of mercury-induced weakness of the hypogastric plexus.

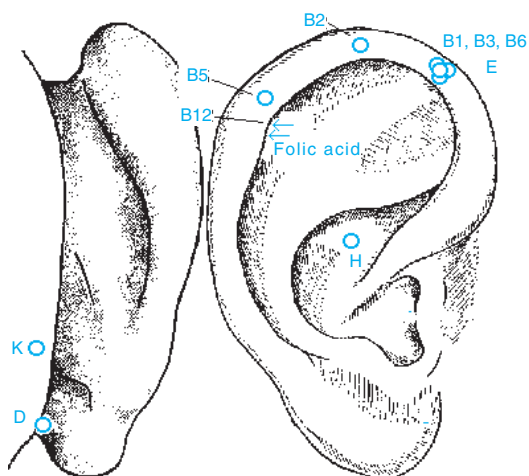


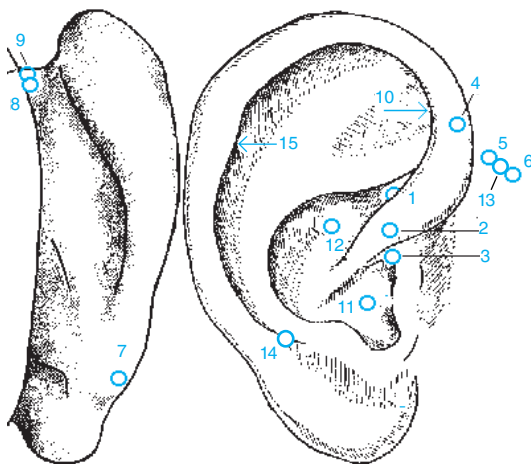
Fig. 6.62

Indicator Points for
Vitamins according to Bahr

Fig. 6.63

Indicator Points for Trace
Elements according to Bahr

- 1 Mercury Indicator Point
- 2 Copper Indicator Point
- 3 Iron Indicator Point
- 4 Stannum Indicator Point
- 5 Aluminum Indicator Point
- 6 Lead Indicator Point
- 7 Silver Indicator Point
- 8 Gold Indicator Point
- 9 Platinum Indicator Point
- 10 Zinc Indicator Point
- 11 Cadmium Indicator Point
- 12 Palladium Indicator Point
- 13 Nickel Indicator Point
- 14 Magnesium Indicator Point
- 15 Formaldehyde Indicator Point



The Indicator Points for trace elements may be included in the therapy and then have a supportive function (e.g., Omega Point I in amalgam load and elimination).

The rule that a **deficiency** is expressed as Gold Point and a load as Silver Point applies also to the corresponding body acupuncture points for trace elements (on the forehead).

Important Silver Points on the Dominant Ear

Knowing by heart the correct needle metal for all Functional Points pays off. Only about 50 points are strictly dependent on the handedness of a person.

Out of these, only about 20 points do **not** appear as Gold Points on the dominant ear but as Silver Points. (In case of the points of the locomotor system, their motor portions on the back of the ear appear normally also as Silver Points.)

It is best to memorize just these Silver Points, that is, those points which appear as Silver Points on the **right** side in a right-handed person. **All other points can be assumed to be Gold Points.**

The Gold Point is preferred for therapy—and for each Silver Point in question, this is the Gold Point on the opposite ear.

For example:

Diazepam Analogue Point

- in a right-handed person: Silver Point on the **right** ear → needle the Gold Point on the **left** ear,
- in a left-handed person: Silver Point on the **left** ear → needle the Gold Point on the **right** ear.

The only thing to learn is therefore: Diazepam Analogue Point—the dominant ear bears the Silver Point.

Some points are not associated with a distinct skin potential or needle metal. This is true, for example, for the points of hormones and metabolites.

Depending on the disease, either sedation or stimulation will be required (e.g., Endocrine Thyroid Gland Point, Progesterone Point).

The teaching material only lists the needle metal that is required **most often** for a particular point. It is recommended to explore in each patient whether a gold or silver needle is required.

Important Silver Points on the right (dominant) ear are:

- Interferon Point (corresponding to Cardinal Point SP-4)
- Diazepam Analogue Point (Valium Point according to Bahr—corresponding to Cardinal Point KI-6)
- Pineal Gland Point (corresponding to Cardinal Point BL-62)
- Thymus Gland Point (corresponding to Cardinal Point TB-5)
- Nicotine Analogue Point according to Bahr
- Phosphate Analogue Point according to Bahr
- Aggression Point
- Frustration Point
- Psychotherapy Point according to Bourdiol
- Histamine Point (Allergy Point 1)
- Adrenal Gland Point (Cortisone Point)
- Endocrine Pancreas Point (Insulin Point)
- Anxiety Point (identical location but Silver Point on the left ear: Worry Point)
- Barbiturate Analogue Point
- Beta-1-Receptor Point
- Nervous Liver Point (syn. Anger Point) and Nervous Gall Bladder Point (but not the Duodenum Point)
- Fig. 6.64, p. 270

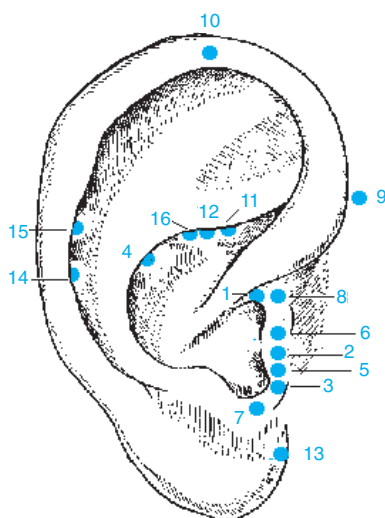


Fig. 6.64 Important Silver Points on the right (dominant) ear:

- 1 Interferon Point (corresponding to Cardinal Point SP-4)
- 2 Diazepam Analogue Point (corresponding to Cardinal Point KI-6)
- 3 Pineal Gland Point (corresponding to Cardinal Point BL-62)
- 4 Thymus Gland Point (corresponding to Cardinal Point TB-5)
- 5 Nicotine Analogue Point according to Bahr
- 6 Phosphate Analogue Point
- 7 Aggression Point
- 8 Frustration Point
- 9 Psychotherapy Point according to Bourdiol
- 10 Histamine Point (Allergy Point 1)
- 11 Adrenal Gland Point (Cortisone Point)
- 12 Endocrine Pancreas Point (Insulin Point)
- 13 Anxiety Point (identical location but Silver Point on the left ear: Worry Point)
- 14 Barbiturate Analogue Point
- 15 Beta-1-Receptor Point
- 16 Nervous Liver Point (synonym: Anger Point) and Nervous Gall Bladder Point (does not apply to Duodenum Point)

In case of the following points, the possibility should be considered that they may appear **as both Gold and Silver Points**, depending on the disease:

- Thyroid Gland Point (normally as Silver Point on the right ear)
- Parathyroid Gland Point
- Progesterone Point
- Renin-Angiotensin Point
- TSH Point
- Prolactin Point

Even though most of the Silver Points are needled as Gold Points on the opposite ear, there are a few reflex areas where it is preferable to needle the Silver Point on the dominant ear:

- Anxiety Point
 - Worry Point
 - Aggression Point
 - Nicotine Analogue Point according to Bahr
 - Nervous Liver Point (synonym: Anger Point)
- 

Zone-Dominating Points—Overview

In the 1970s, Nogier experimented with colored light produced by a stroboscope. He noticed that certain clearly defined ear zones—and thus certain regions of the body—responded to specific light frequencies with a resonance in the vascular autonomic signal (VAS). In other words, irradiation of pathologically altered ear points with specific light frequencies resulted in a distinct pulse at the radial artery.

This discovery that ear zones and body regions resonate with specific light frequencies provided the foundation for designing the fully developed lasers used today in diagnosis and therapy in both ear acupuncture and body acupuncture.

Nogier discovered seven distinct frequencies to which the different ear zones responded with a resonance. He found different frequencies for the locomotor system, internal organs, neurological structures, etc. Frequency A is especially important for focal therapy.

- **Frequency A:** Frequency of unrest and disorganization; resonance with reticular formation, resonance with all focal disturbances.
- **Frequency B:** Nutritional frequency; resonance with internal organs.
- **Frequency C:** Mesenchymal frequency related to the locomotor system, orthopedic frequency.
- **Frequency D:** Frequency of laterality, related to the tragus.
- **Frequency E:** Frequency of spinal cord; resonance with the spinal cord (motor and sensory portions).
- **Frequency F:** Emotional frequency; related to structures in the dental-maxillary region and to the subcortical parts of the brain (such as diencephalon and mesencephalon, hypothalamus).
- **Frequency G:** Psychosomatic frequency; related to the cortex and also to the eyes and maxillary sinuses.

Knowledge about the ear zones allows us to specifically influence each zone as a whole by treating the respective Zone-Dominating Point. While working with the laser, the figure and table (see previous page) can be consulted for the zone-specific frequency of the ear point to be treated. If the active ear point of a zone exhibits a different frequency, further disturbances may be deduced from this. For example, if frequency F is found on a Vertebral Column Point within Zone C, one may presume a psychosomatic illness possibly with masked depression).

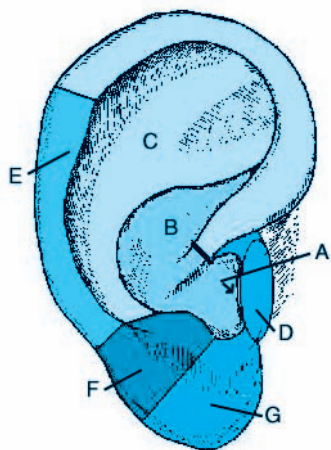


Fig. 6.65 Nogier's resonance zones on the ear

Nogier's frequencies	A	2.28 Hz	A'	292 Hz	Corresponding laser frequencies used today
	B	4.56 Hz	B'	584 Hz	
	C	9.12 Hz	C'	1168 Hz	
	D	18.25 Hz	D'	2336 Hz	
	E	36.50 Hz	E'	4672 Hz	
	F	73.00 Hz	F'	9344 Hz	
	G	146.00 Hz	G'	146 Hz	

Nogier has found one Functional Point for each resonance zone of the ear (for details on resonance zones for different laser frequencies, see Advanced Knowledge Level). Each of these points exhibits a particularly strong resonance with the laser frequency of the respective zone and has an overruling (dominating) effect on this zone of the ear. Through his own research, Bahr has found a counterpart for each of these Zone-Dominating Points, which further intensifies the effect of the original Zone-Dominating Point. (**ZA2** and **ZB1-ZG1** are the original points discovered by Nogier. The other points, **ZA1**, **ZB2-ZG2**, stem from Bahr's research.)

Needling one or two Zone-Dominating Points has an overruling effect on the entire respective zone. This is primarily of interest to therapists working without a laser device. The therapist may deduce the effect of each Zone-Dominating Point by general consideration of the respective frequency range.

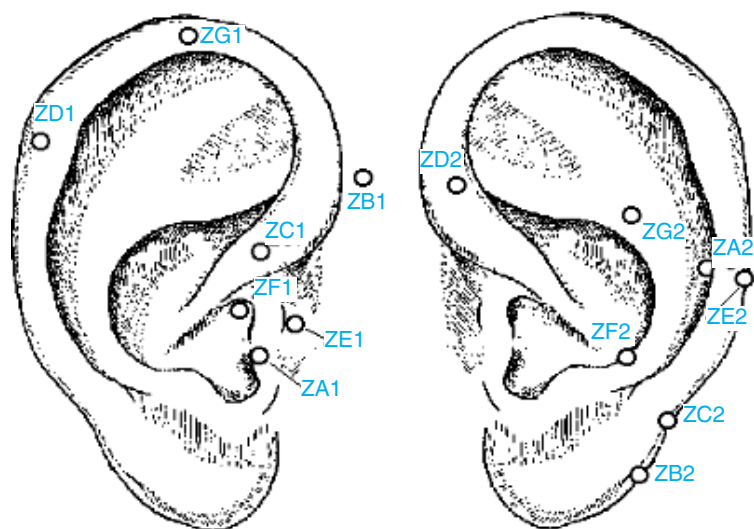


Fig. 6.66 Zone-Dominating Points—Overview

Points Dominating Zone A (ZA Points)

Location:

- **ZA1:** The right ear bears the Gold Point. Just below the middle of the tragus near its edge.
- **ZA2:** The left ear bears the Gold Point. Partly covered in the groove of the descending helix, in the Zone of Sympathetic Nuclei of Origin, approximately at the level of Point C6.

Application:

- In all problems associated with resonance zone A, the ZA Points have a stabilizing effect on this zone and support the treatment. For example, allergic disorders, possibly also severe focal activity.

Points Dominating Zone B (ZB Points)

Location:

- **ZB1:** The right side bears the Gold Point. In front of the upper attachment of the ear, just below the Psychotherapy Point according to Bourdiol.
- **ZB2:** The left ear bears the Gold Point. At the lower lateral edge of the lobule.

Application:

- In all problems associated with resonance zone B, the ZB Points have a stabilizing effect on this zone and support the treatment. For example, diseases of the internal organs, defective wound healing, edema, periodontitis, some forms of allergy.

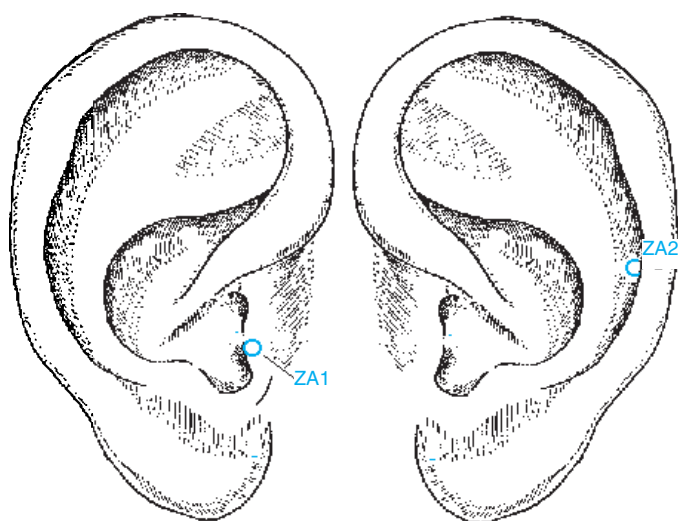


Fig. 6.67 Points Dominating Zone A

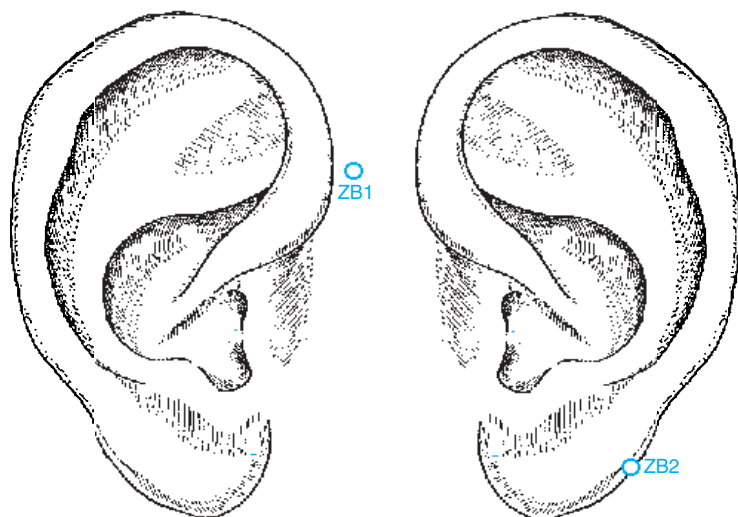


Fig. 6.68 Points Dominating Zone B

Points Dominating Zone C (ZC Points)

Location:

- **ZC1:** The right ear bears the Gold Point. On the ascending helix, anterosuperior to Point Zero.
- **ZC2:** The left ear bears the Gold Point. At the lateral edge of the lobule, about 1 cm above Point ZB2.

Application:

- In all problems associated with resonance zone C, the ZC Points have a stabilizing effect on this zone and support the treatment. For example, disorders of the locomotor system, kidney disorders.

ZC1 corresponds to Point LI-15 of body acupuncture (Master Point of the upper limb, p. 320).

Points Dominating Zone D (ZD Points)

Location:

- **ZD1:** The right ear bears the Gold Point. Near Darwin's Point.
- **ZD2:** The left ear bears the Gold Point. On the ascending helix, approximately at the level of the intersection with the antihelix.

Application:

- In all problems associated with resonance zone D, the ZD Points have a stabilizing effect on this zone and support the treatment. For example, diseases associated with severe unstable laterality, such as neurodermatitis, general skin problems, all addictions; also mental disorders in which the Diazepam Analogue Point (irritable bladder) or the Pineal Gland Point play a leading role.

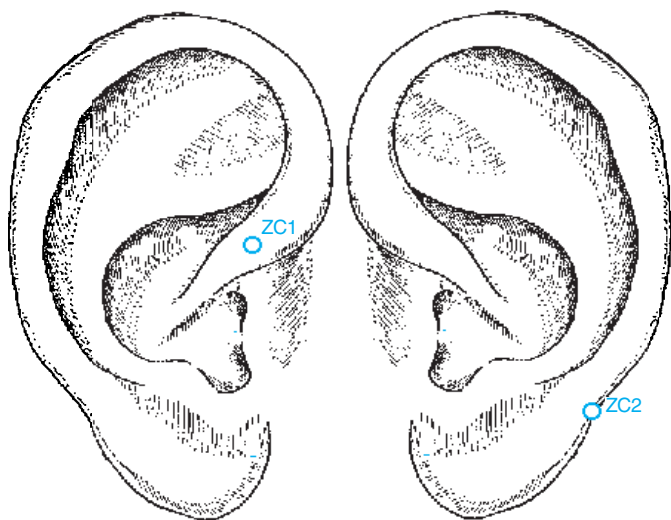


Fig. 6.69 Points Dominating Zone C

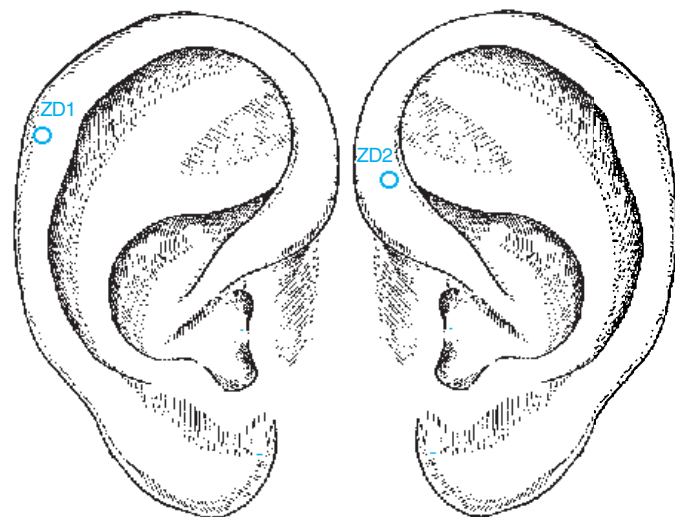


Fig. 6.70 Points Dominating Zone D

Points Dominating Zone E (ZE Points)

Location:

- **ZE1:** The right ear bears the Gold Point. About one finger width in front of the upper half of the tragus, at the transition to the cheek.
- **ZE2:** The left ear bears the Gold Point. On the descending helix at the lateral edge of the ear, just below the middle of the ear.

Application:

- In all problems associated with resonance zone E, the ZE Points have a stabilizing effect on this zone and support the treatment. For example, as accompanying therapy for root compression (herniation of intervertebral disk), neuralgia, herpes zoster.

Points Dominating Zone F (ZF Points)

Location:

- **ZF1:** The right ear bears the Gold Point. In the Throat Zone.
- **ZF2:** The left ear bears the Gold Point. Near Point C1.

Application:

- In all problems associated with resonance zone F, the ZF Points have a stabilizing effect on this zone and support the treatment. For example, all diseases of the oral region (teeth, mouth, jaws), possibly also masked depression (Depression Point in frequency zone F).

Supports the treatment of allergies (use ZF Points for the inherited portion).

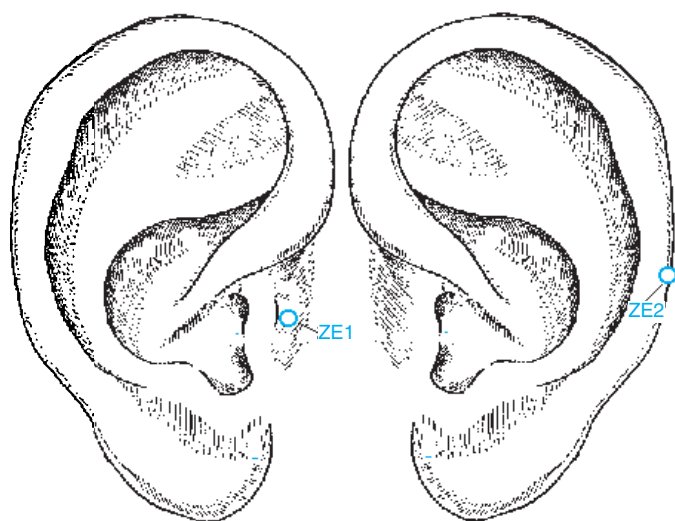


Fig. 6.71 Points Dominating Zone E

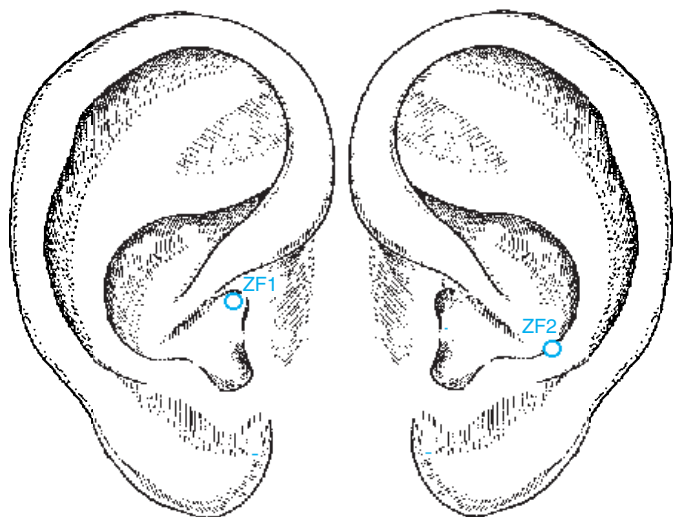


Fig. 6.72 Points Dominating Zone F

Points Dominating Zone G (ZG Points)

Location:

- **ZG1:** The right ear bears the Gold Point. Near Allergy Point 1.
- **ZG2:** The left ear bears the Gold Point. Slightly lateral to the middle of the Thoracic Spine Zone.

Application:

- In all problems associated with resonance zone G, the ZG Points have a stabilizing effect on this zone and support the treatment. For example, as accompanying treatment in mental disorders with an active Master Omega Point.

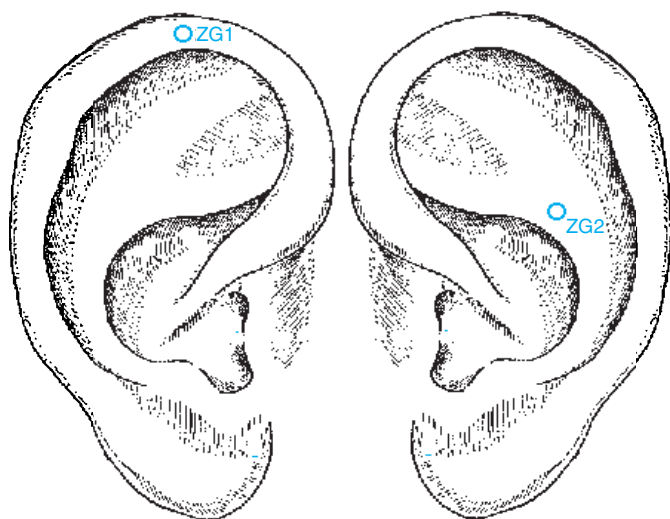


Fig. 6.73 Points Dominating Zone G

Auxiliary Lines of Ear Acupuncture

Every ear is of a slightly different shape so that the beginner often finds it difficult to search for points. However, there are certain common features of proportion that are independent of the overall features of the ear relief, and these features seem to be the same on every ear.

Based on these regularities it is thus possible to use **auxiliary lines** that are **generally applicable**, thus making it easier to find a number of important points.

This fact is particularly impressive in the case of the various lines running through Point Zero. It is always surprising to see that these points are indeed arranged exactly on a straight line. Hence, if two of the points are known, the other points on the line can be easily found (with the aid of a ruler).

Surprisingly, this applies to ears of all shapes. Likewise, many structures on the ear have fixed anatomical or geometric relationships to each other.

The auxiliary lines described below are extremely helpful in finding the accurate location of the searched-for areas on the ear. However, the lines are only useful if time is first taken to establish them on the ear. Beginners, in particular, should mark the lines with a fine marker prior to searching for points. The experienced therapist works by eye.

Barbiturate Analogue Point, Caffeine Analogue Point

Auxiliary line:

The horizontal line running through Point C7. The line first crosses the Shoulder Joint Point in the scapha and then the Barbiturate Analogue Point in the groove of the descending helix (Zone of Sympathetic Nuclei of Origin).

Barbiturate Analogue Point and Caffeine Analogue Point have identical locations but differ in the needle metal required:

The Barbiturate Point appears as Silver Point on the right ear and as Gold Point on the left ear. The Caffeine Point appears as Gold Point on the right ear and as Silver point on the left ear.

Note: Two points are required for establishing a straight line, but a single point is sufficient for establishing a horizontal or vertical line.

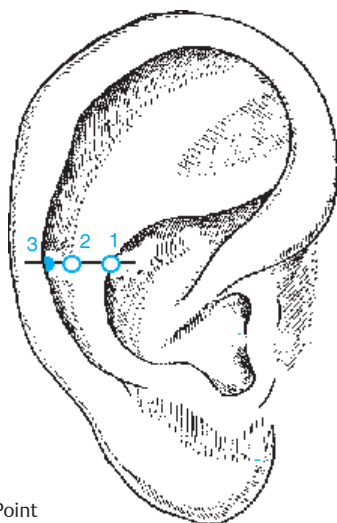


Fig. 7.1

- 1 Point C7
- 2 Shoulder Joint Point
- 3 Barbiturate Analogue Point

Omega Points

Auxiliary line:

A vertical line runs through all three Omega Points.

Aggression Point

Auxiliary line:

The vertical line through the three Omega Points runs also through the Aggression Point. (For another auxiliary line through the Aggression Point, see Auxiliary Lines Near the Tragus, p. 307).

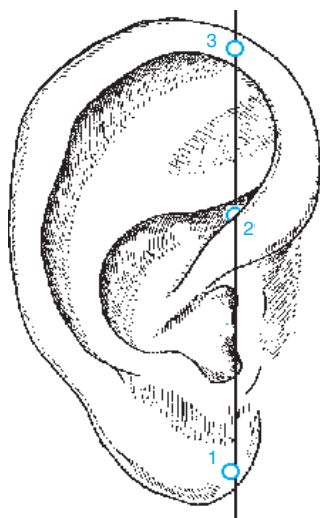


Fig. 7.2

- 1 Master Omega Point
- 2 Omega Point I
- 3 Omega Point II

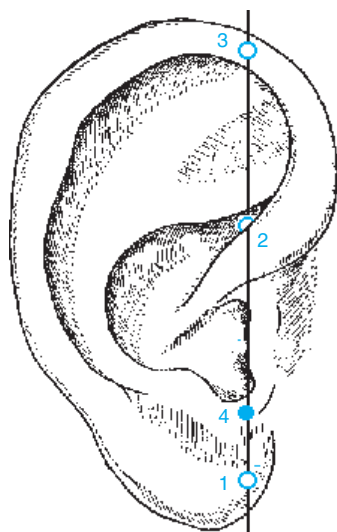


Fig. 7.3

- 1 Master Omega Point
- 2 Omega Point I
- 3 Omega Point II
- 4 Aggression Point

Hormone Points

Auxiliary line:

A straight line through Point Zero and one of the three points for hormones: Estrogen Point, Gonadotropin Point, Progesterone Point. If only one of these points is known (because it was found to be active), the other two points can easily be located by means of the auxiliary line.

As always, the rule applies that only active points should be needled.

By means of the auxiliary lines, however, the reflex areas to be examined are precisely located or circumscribed.

As a refresher:

The **Estrogen Point** is located on the inside (underside) of the ascending helix, about 2 mm away from the helical rim, halfway between Point Zero (Umbilical Cord Point) and Uterus Zone.

The **Progesterone Point** is located 4 mm away from the Kidney Point in the fold of the ascending helix.

The location of the **Gonadotropin Point** and further help in finding it are described in detail elsewhere (see pp. 137, 197).

Cortisone Point, ACTH Point

Auxiliary line:

Cortisone Point and ACTH Point are located on a straight line running through Point Zero.

If only one of the two points is known, the other one can be easily located by means of the line through Point Zero. (For precise locations of the Endocrine Points in the antihelical wall, see p. 194).

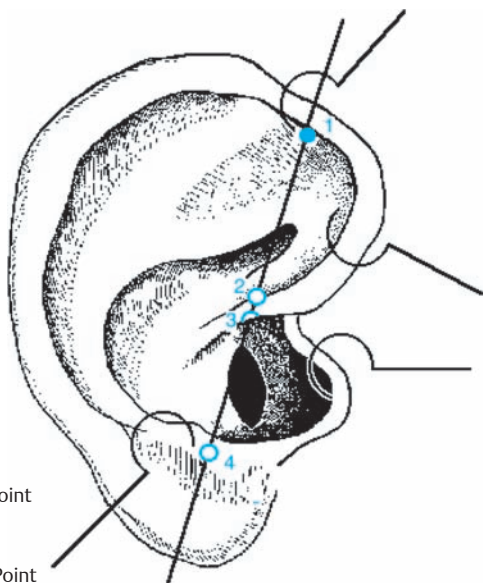


Fig. 7.4

- 1 Progesterone Point
- 2 Estrogen Point
- 3 Point Zero
- 4 Gonadotropin Point

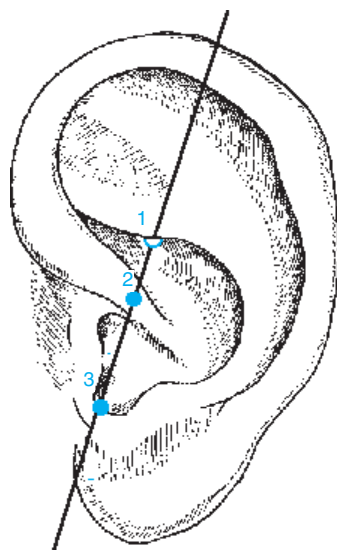


Fig. 7.5

- 1 Cortisone Point
- 2 Point Zero
- 3 ACTH Point

Thymus Gland Point, Interferon Point, Laterality Point

Auxiliary line:

Thymus Gland Point and Interferon Point are located on the straight line running through Point Zero and the Laterality Point.

Spleen Point, Diazepam Analogue Point

Auxiliary line:

The Spleen Point is located on a straight line running through Point Zero and the Diazepam Analogue Point, on the left ear.

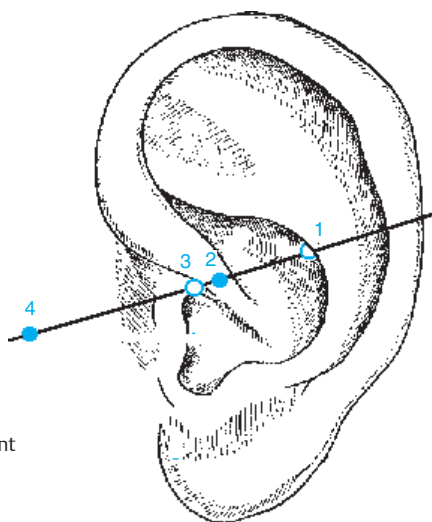


Fig. 7.6

- 1 Thymus Gland Point
- 2 Point Zero
- 3 Interferon Point
- 4 Laterality Point

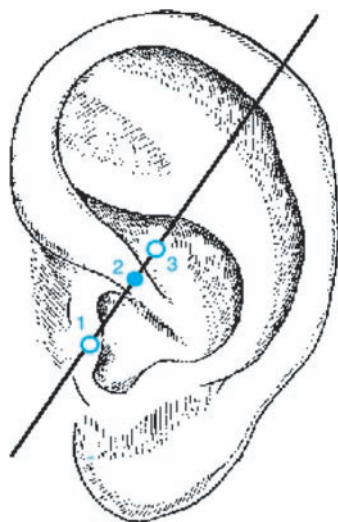


Fig. 7.7

- 1 Diazepam Analogue Point
- 2 Point Zero
- 3 Spleen Point

Depression Point, Tonsil Point, TMJ Point

Auxiliary line:

A straight line running through Point Zero and the postantitragal fossa, just anterior to Point C1 (Point C0/C1). At the end of the scapha, the line runs through the following reflex areas: **Depression Point, Tonsil Point, Retromolar Space Point, and TMJ Point**, all of which lie very close together.

The location of the Depression Point (Antidepression Point, Joy Point) is defined as “at the end of the helical groove” (where scapha and helix meet).

The end of the helical groove, however, may differ widely in its location and is often found further up. The auxiliary line is therefore important, especially for the beginner, because it ensures that this major reflex area of symptoms or foci will not be overlooked.

Trigeminal Nerve

Auxiliary line:

The same line described above which runs through Point Zero, postantitragal fossa, Depression Point, Tonsil Point, Retromolar Space Point, and TMJ Point.

On the extension of this auxiliary line at the edge of the ear lies the **Trigeminal Nerve Zone** (sensory portion on the front of the ear, motor portion on the back of the ear).

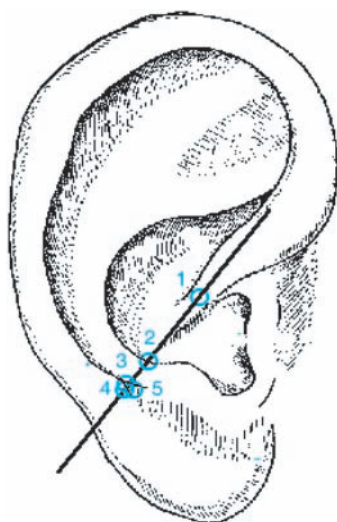


Fig. 7.8

- 1 Point Zero
- 2 Point C0/C1
- 3 Tonsil Point
- 4 TMJ Point
- 5 Depression Point

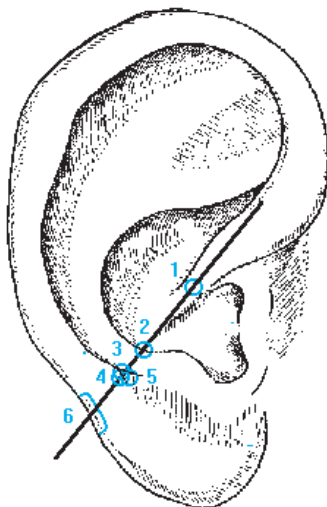


Fig. 7.9

- 1 Point Zero
- 2 Point C0/C1
- 3 Tonsil Point
- 4 TMJ Point
- 5 Depression Point
- 6 Trigeminal Nerve Zone

First Rib Point

Auxiliary line:

A straight line through Point Zero and Point C7/T1.

Both structures are easily palpable with the stirrup-shaped ear probe.

First Rib Point, Stellate Ganglion Point

Auxiliary line:

The straight line through Point Zero and Point C7/T1 runs through the **Stellate Ganglion Point** which is located at the transition between antihelical wall and concha (Zone of Sympathetic Trunk).

The extension of this line beyond Point C7/T1 runs through the **First Rib Point** on the scapha.

Both points are important in case of inversion or simple blocking of the first rib.

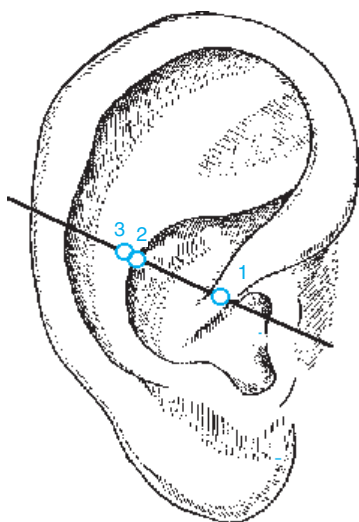


Fig. 7.10

- 1 Point Zero
- 2 Point C7/T1
- 3 First Rib Point

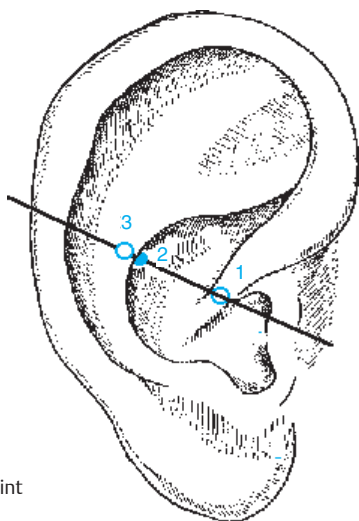


Fig. 7.11

- 1 Point Zero
- 2 Stellate Ganglion Point
- 3 First Rib Point

Beta-1-Receptor Point

Auxiliary line:

The straight line through Point Zero and Point C7/T1 runs through the partly covered **Beta-1-Receptor Point** in the groove of the descending helix (blocks beta-1-receptors, lowers blood pressure; the right ear bears the Silver Point).

Just superior to it lies the **Beta-2-Receptor Point** (bronchospasmic conditions, bronchial asthma; the right ear bears the Gold point).

Endocrine Thyroid Gland Point

Auxiliary line:

The horizontal line through Point Zero. Its lateral extension runs through the Endocrine Thyroid Gland Point which is located in the antihelical wall in the reflex area of the endocrine organs (the left ear normally bears the Gold Point).

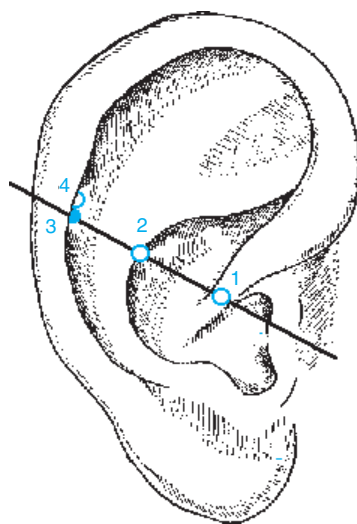


Fig. 7.12

- 1 Point Zero
- 2 Point C7/T1
- 3 Beta-1-Receptor Point
- 4 Beta-2-Receptor Point

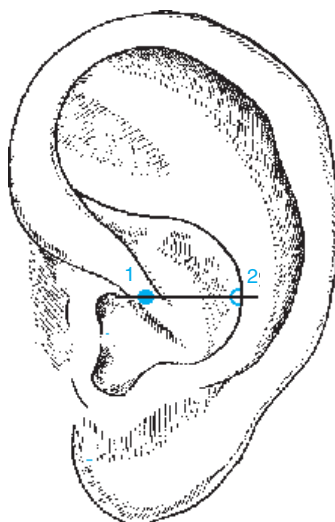


Fig. 7.13

- 1 Point Zero
- 2 Endocrine Thyroid

Diazepam Analogue Point

The Diazepam Analogue Point is located on the tragus, about 2 mm in front of the edge of the tragus and just below the middle of the tragus (the right ear bears the Silver Point and the left ear the Gold Point).

Auxiliary line:

The horizontal line through the apex of tragus runs tangentially above the Diazepam Analogue Point.

Note: Two points are required for establishing a straight line, but a single point is sufficient for establishing a horizontal or vertical line.

Phosphate Analogue Point

Auxiliary line:

The Phosphate Analogue Point mirrors the Diazepam Analogue Point on the superior side of the horizontal line described above.

Both points appear as Silver Points on the right ear and as Gold Points on the left ear.

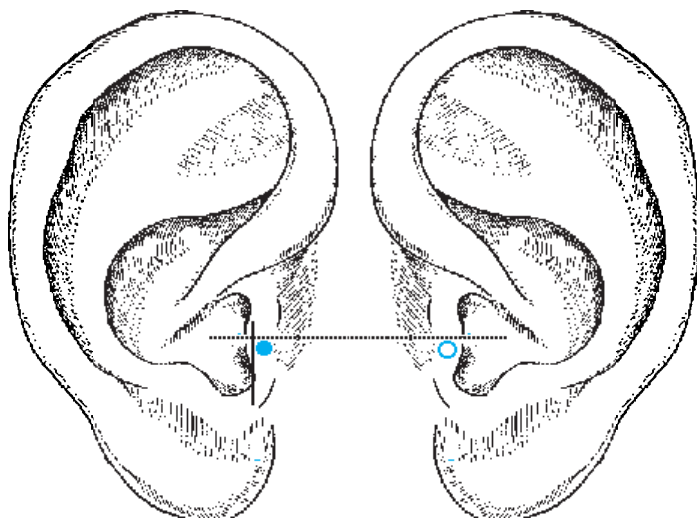


Fig. 7.14 Gland Point (on the left ear)
Diazepam Analogue Point

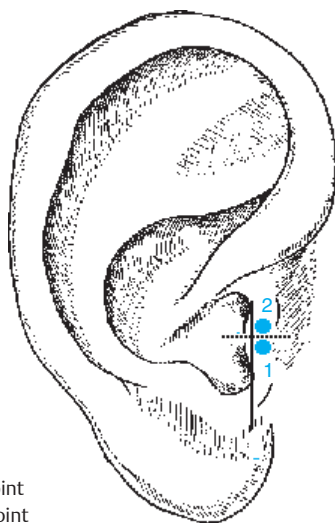


Fig. 7.15
1 Diazepam Analogue Point
2 Phosphate Analogue Point

Nicotine Analogue Point, Pineal Gland Point

Vertically below the **Diazepam Analogue Point**, but still on the tragus, are the **Nicotine Analogue Point** and the **Pineal Gland Point**.

The three points are positioned about the same distance from one another.

Auxiliary line:

The tangential vertical line just posterior to the Diazepam Analogue Point.

Frustration Point

Auxiliary line:

The auxiliary line described above for Diazepam Analogue Point, Nicotine Analogue Point, and Pineal Gland Point may also be used for the Frustration Point. All points shown appear as Silver Points on the right ear and as Gold Points on the left ear.

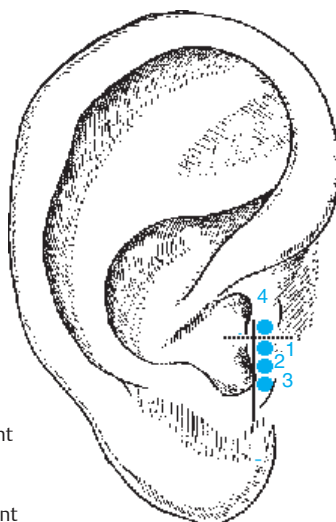


Fig. 7.16

- 1 Diazepam Analogue Point
- 2 Nicotine Analogue Point
- 3 Pineal Gland Point
- 4 Phosphate Analogue Point

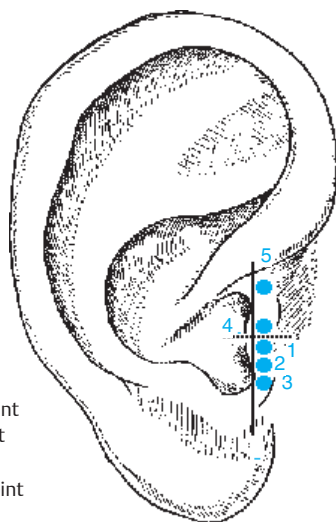


Fig. 7.17

- 1 Diazepam Analogue Point
- 2 Nicotine Analogue Point
- 3 Pineal Gland Point
- 4 Phosphate Analogue Point
- 5 Frustration Point

Interferon Point

Auxiliary line:

The auxiliary line described above for Diazepam Analogue Point, Nicotine Analogue Point, and Pineal Gland Point may also be used for the Interferon Point. All points shown appear as Silver Points on the right ear and as Gold Points on the left ear.

The Frustration Point and the Interferon Point are sometimes mixed up. It is therefore recommended to memorize their locations relative to each other.

Nose Point

Auxiliary line:

The auxiliary line described above for Diazepam Analogue Point, Nicotine Analogue Point, and Pineal Gland Point may also be used for the Nose Point as well as for the Maxillary Sinus Zone and Frontal Sinus Point (mucosal portions).

Thus, the following points are all located on the same vertical line on the tragus:

- Diazepam Analogue Point
- Nicotine Analogue Point
- Pineal Gland Point
- Phosphate Analogue Point
- Frustration Point
- Nose Point
- Maxillary Sinus Zone
- Frontal Sinus Point

Fig. 7.18

- 1 Diazepam Analogue Point
- 2 Nicotine Analogue Point
- 3 Pineal Gland Point
- 4 Phosphate Analogue Point
- 5 Frustration Point
- 6 Interferon Point

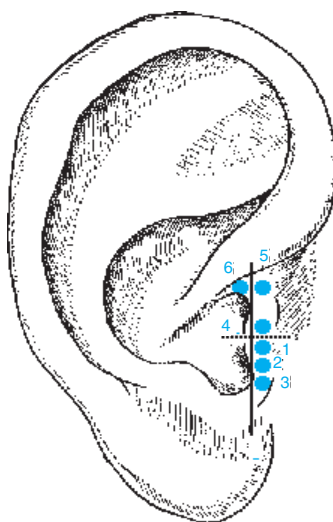
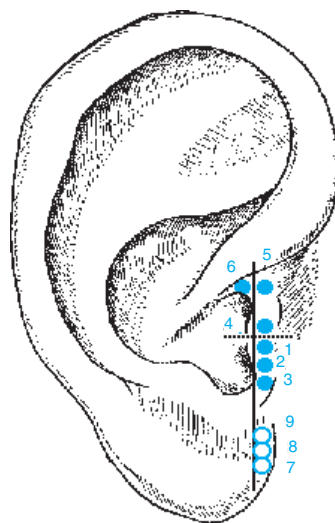


Fig. 7.19

- 1 Diazepam Analogue Point
- 2 Nicotine Analogue Point
- 3 Pineal Gland Point
- 4 Phosphate Analogue Point
- 5 Frustration Point
- 6 Interferon Point
- 7 Nose Point
- 8 Maxillary Sinus Zone (mucosal portion)
- 9 Frontal Sinus Point



Aggression Point

The **Aggression Point** is located below the intertragic notch, at the same distance from the notch (about 2 mm) as the Pineal Gland Point is in front of the notch.

Auxiliary lines:

The horizontal line below the intertragic notch and, on the tragus, the vertical line described above.

Both lines make it possible to find the precise locations of the two points.

ACTH Point

The **ACTH Point** is located in the anterior corner of the intertragic notch (reflex area of the pituitary gland).

Auxiliary lines:

The two lines on and below the tragus (as described for the Aggression Point) form a right angle.

The respective bisector (45°) passes through the ACTH Point in the anterior corner of the intertragic notch.

The direction of the bisector is also the direction in which the point is needed.

Establishing the ACTH Point by means of the bisector provides a good start for locating the other points of the pituitary gland (Prolactin Point, TSH Point, Gonadotropin Point).

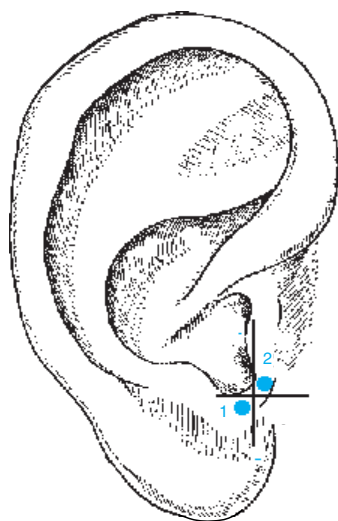


Fig. 7.20

- 1 Aggression Point
- 2 Pineal Gland Point

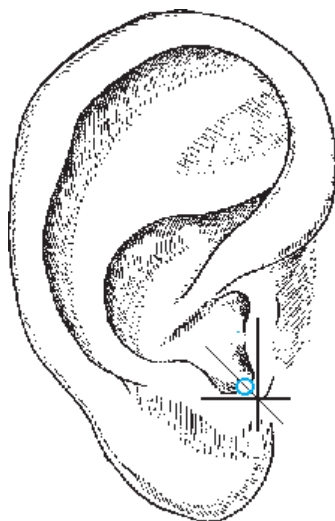


Fig. 7.21 ACTH Point

Points of the Pituitary Gland Zone

Establishing the ACTH Point by means of the bisector provides a good start for locating the other points of the pituitary gland (Prolactin Point, TSH Point, Gonadotropin Point).

Auxiliary line:

These points are located on a **waving line** starting at the intertragic notch and ending in the middle of the antitragus. The **Prolactin Point** (normally as Silver Point on the right ear) is just above the **ACTH Point**, and the **TSH Point** (normally as Gold Point on the right ear) is just posterior to the ACTH Point.

The fourth reflex point of the pituitary gland, the **Gonadotropin Point**, is located posterior to the TSH Point and already on the antitragus (normally as Gold Point on the right ear). The **Oxytocin Point** follows just behind the Gonadotropin Point.

Gonadotropin Point

Closer inspection of the location of the Gonadotropin Point in relation to the already known (horizontal) auxiliary line below the intertragic notch shows that this point lies as far **above** the horizontal line as the Aggression Point lies **below** it.

Gonadotropin Point and Pineal Gland Point are located at the same level.

Auxiliary line:

As previously mentioned, the point's exact location is easily found by imagining the edge of antihelix and antitragus as a **snake**, whereby the antitragus is the head.

The Gonadotropin Point is located at the eye of this snake (about 2 mm away from the upper edge of the antitragus).

Fig. 7.22

- 1 Prolactin Point
- 2 ACTH Point
- 3 TSH Point
- 4 Gonadotropin Point
- 5 Oxytocin Point

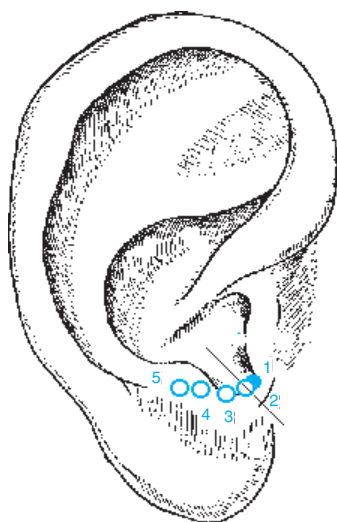
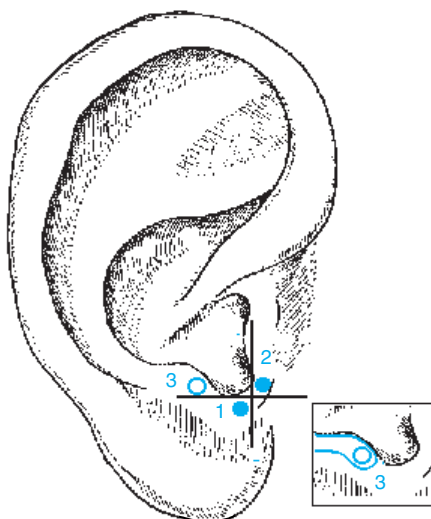


Fig. 7.23

- 1 Aggression Point
- 2 Pineal Gland Point
- 3 Gonadotropin Point



Points of the Hip Joint, Knee Joint, Ankle Joint

Auxiliary line:

A straight line through the intersection of superior and inferior antihelical crura (**Hip Joint Point**) and the deepest point of the triangular fossa (**Knee Joint Point**).

The extension of this line in the direction of the ascending helix intersects with the helical rim at the **Ankle Joint Point**.

Wrist Point, Knee Joint Point

Auxiliary line:

The horizontal line through the **Knee Joint Point** (deepest point of the triangular fossa).

The horizontal line passes through the **Wrist Zone** midway between the edge of the ear and the superior crus of the antihelix.

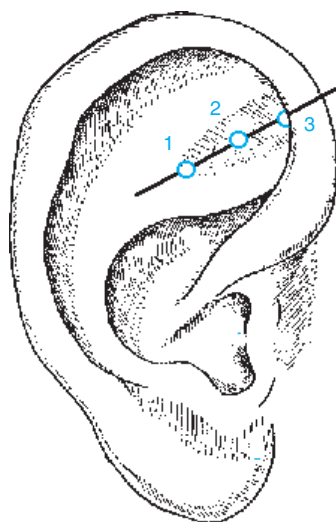


Fig. 7.24

- 1 Hip Joint Point
- 2 Knee Joint Point
- 3 Ankle Joint Point

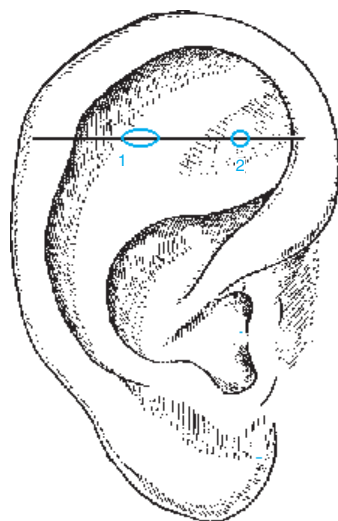


Fig. 7.25

- 1 Wrist Point
- 2 Knee Joint Point

Points of the Shoulder Joint, Elbow Joint, Wrist

Auxiliary lines:

The tangential line to the antihelix of the Lumbar Spine Zone runs through the Elbow Joint Point in the scapha.

A slightly curved line along the groove of the scapha connects the Shoulder Joint Point (at the level of Point C7 in the groove of the scapha) with the Wrist Zone. It intersects the other line at the Elbow Joint Point.

Kidney Point

Auxiliary line:

The horizontal line through the Knee Joint Point.

This line runs through the Kidney Point in the reflection of the ascending helix.

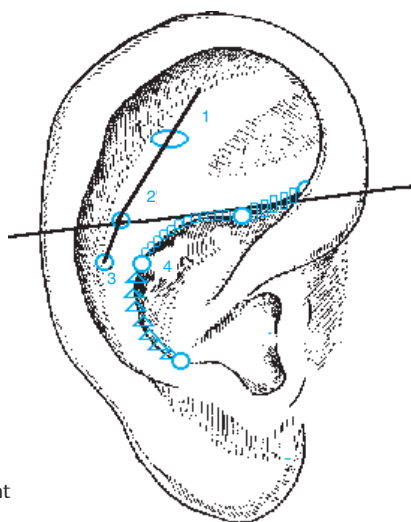


Fig. 7.26

- 1 Wrist Point
- 2 Elbow Joint Point
- 3 Shoulder Joint Point
- 4 Point C7

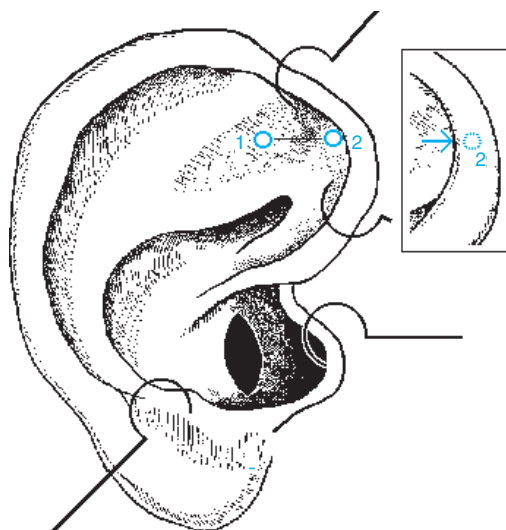


Fig. 7.27

- 1 Knee Joint Point
- 2 Kidney Point

Laterality Point

Auxiliary line:

The Laterality Point is located on the horizontal line through the apex of tragus, about 3 cm in front of the ear on the cheek. In the right-handed person it appears as Gold Point on the right side and as Silver Point on the left side.

The same auxiliary line is also used for finding the Diazepam Analogue Point.

Cyclophosphamide Analogue Point (Allergy Point 2)

Auxiliary lines:

The vertical line through the Laterality Point.

The Cyclophosphamide Analogue Point (Endoxan Point) is located on the scalp at the intersection of this line with the horizontal line that extends the antihelix.

The **Psychotherapy Point according to Bourdiol** is also located on the scalp on the horizontal line that extends the antihelix.

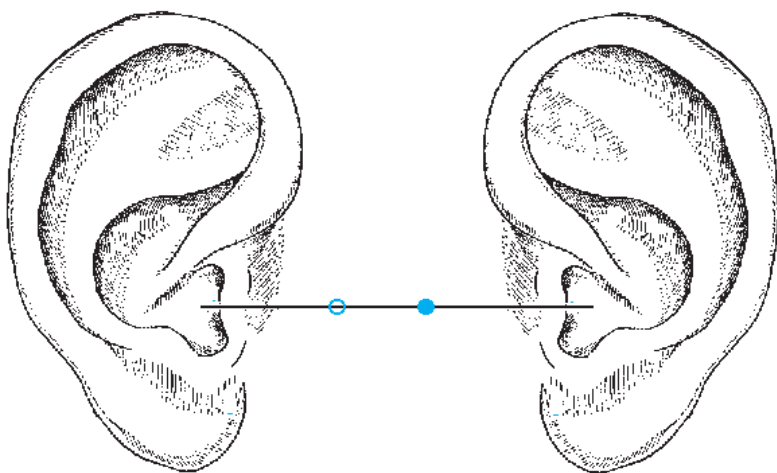


Fig. 7.28 Laterality Point

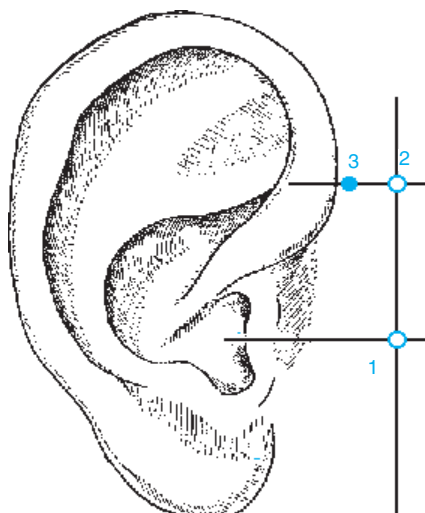


Fig. 7.29

- 1 Laterality Point
- 2 Cyclophosphamide Analogue Point (Allergy Point 2)
- 3 Psychotherapy Point

Histamine Point (Allergy Point 1), Vitamin C Point

Auxiliary line:

The **Histamine Point** is located on the apex of the helix (as Silver Point on the right ear).

The **Vitamin C Point** is located 2 cm superior to it on the scalp. When searching for its exact location, the vertical line through the Histamine Point is helpful.



Fig. 7.30

- 1 Histamine Point (Allergy Point 1)
- 2 Vitamin C Point

Meridians on the Ear

The following symbols are used to identify the different types of points along each meridian:

- (+) Tonification Point (Mother Point)
- (-) Sedation Point (Son Point)
- (L) *Luo* Point (Connecting Point, Passage Point)
- (Y) *Yuan* Point (Source Point, Primary *Qi* Point)
- (M) Master Point (Influential Point)
- (C) Cardinal Point (Opening Point)
- (H) *He* Point (Sea Point)

In the 1990s, Bahr and coworkers were able to determine the projection of all body meridians on the surface of the ear. This not only led to new exciting insights into the significance of certain body acupuncture points, it also revealed that many well-known ear acupuncture points have a far wider spectrum of activity than previously assumed. For example:

Point HT-3 of body acupuncture projects onto the Heart Point (motor portion) on the back of the ear. Consequently, we advise patients with irregular heart beat (arrhythmia) or heart failure (cardiac insufficiency) to stimulate this body acupuncture point gently by acupressure or transcutaneous nerve stimulation (concomitant with their normal medication, if necessary), in order to support the ear acupuncture treatment. Point HT-3 is easily accessible in the crease of the elbow.

Point SI-3 of body acupuncture projects onto the Celiac Plexus Point on the back of the ear (Retro Point Zero). This ear point has a spasmolytic effect—just like Point SI-3 of body acupuncture—and is also one of the eight Cardinal Points on the ear. It is highly effective for treating Bechterew's disease (ankylosing spondylitis) when used in combination with the ear point corresponding to Point BL-62.

In almost all cases, the Tonification Point (+) of the body meridian projects onto the respective Organ Point on the ear. For example, ST-41 (+) corresponds to the Stomach Point on the ear, LR-8 (+) corresponds to the Liver Point on the ear, and so forth. The only exception is H9 (+) which does **not** correspond to the Heart Point on the ear. Both LU-7 and LU-9 project within the Lung Zone on the ear, which represents the parenchyma of the lung.

The real benefit for ear acupuncture is that the projection of body meridians onto the ear can be used to access essential body acupuncture points via the ear. This way, classical concepts of the flow of energy can now be included in ear acupuncture treatment (e.g., KI-7 for all allergies, the Cardinal Points SI-3 and BL-62 for Bechterew's disease, the Spleen Point on the ear for treating a disturbance of the Middle Burner). It is now possible to strengthen an organ not just by means of its Organ Point (Tonification Point) on the ear, but by including the Source Point as well, and either the transverse or the longitudinal *Luo* Point if needed. All this can be accomplished without having to switch between ear acupuncture and body acupuncture, because treatment is exclusively performed on the ear. For example, the ear point corresponding to the Source Point of an organ, is indeed found to be electrically active when the symptoms suggest that it may be in need of treatment.

In this book, the most important points of each ear meridian are described together with their multiple functions. They carry the same names as the corresponding body acupuncture points. Points ending with “-1” are located close to the original point (e.g., TB-1-1 lies close to TB-1).



Lung Meridian

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

- LU-1** Anxiety Point (Silver Point on the right ear) or Worry Point (Silver Point on the left ear).
- LU-7** Located in the Lung Zone; Cardinal Point indicated for strengthening the defense mechanism, Master Point for all affections of the respiratory tract.
- LU-9** Located in the Lung Zone; Tonification Point, Master Point of the blood vessels.
- LU-11** Tonsil Point.

Large Intestine Meridian

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

- LI-4** Thalamus Point; Source Point, Master Point for pain.
- LI-11** Large Intestine Point (motor portion) on the back of the left ear, Tonification Point, *He* Point.
- LI-15** Master Point of the upper limb. It is located at Point ZC1 and, as such, has an effect on the entire Zone C (locomotor system). Nogier also detected this point much earlier. Unaware that it is the projection of body acupuncture Point LI-15, he already named this point with great intuition the “Master Point of the upper limb.”
- LI-20** Nose Point; Master Point of the nose.

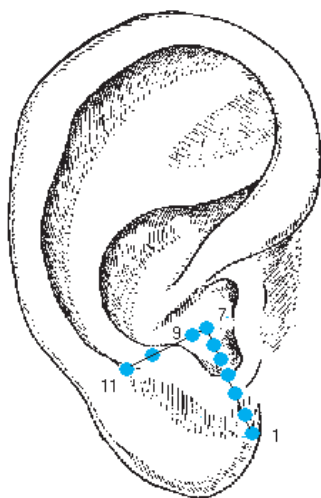


Fig. 8.1 Lung Meridian

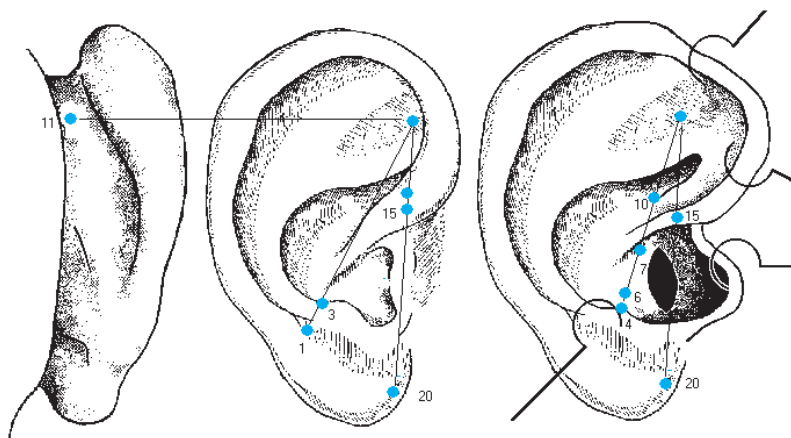


Fig. 8.2 Large Intestine Meridian

Stomach Meridian

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

ST-36 Caffeine Analogue Point (Gold Point on the right ear) or Barbiturate Analogue Point (Gold Point on the left ear). We may expect the same intensive, broad-spectrum effect from these ear points as from Point ST-36 of body acupuncture, for example, as He Point of the Stomach Meridian through which all meridians can be energetically influenced.

The projection of the Stomach Meridian onto the ear has revealed an immensely broad spectrum of possible activities for many ear points that have been neglected so far.

ST-40 Beta-2-Receptor (Beta-Mimetic) Point (Gold Point on the right ear) or Beta-1-Receptor (Beta-Blocker) Point (Gold Point on the left ear).

ST-41 Stomach Zone; Tonification Point.

ST-45 Vitamin B5 Point; Sedation Point. (It was through the projection of Point ST-45 onto the ear that we learned about the excellent effect of vitamin B5 lozenges in patients with gastritis.)

Spleen Meridian

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

SP-1 Indicator Point for folic acid on the right ear.

SP-2 (right ear)–Pancreas Point on the right ear; Tonification Point, Indicator Point for palladium load.

SP-2 (left ear)–Spleen Point; Indicator Point for titanium load.

SP-4 Interferon Point; Luo Point, Cardinal Point, Master Point for diarrhea.

SP-5 Ovary Point (Estrogen Point); Sedation Point, Master Point of the connective tissue.

SP-6 Uterus Point; Master Point for affections of the lesser pelvis.

SP-21 Master Omega Point.

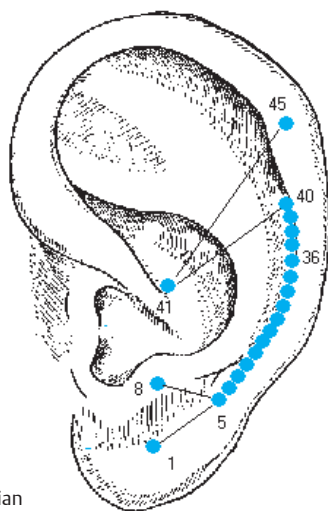


Fig. 8.3 Stomach Meridian

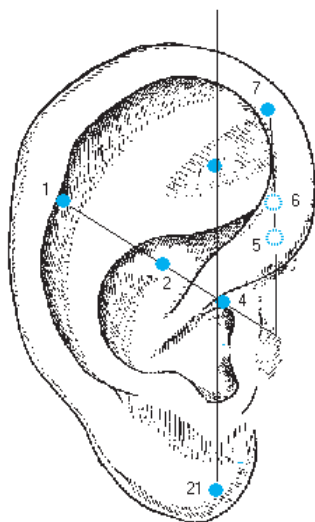


Fig. 8.4 Spleen Meridian

Heart Meridian

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

- HT-3** Heart Point (motor portion) on the back of the ear (Gold Point on the left ear); Digitalis Analogue Point.
- HT-4** Heart Point (sensory portion) on the front of the ear (Gold Point on the left ear).
- HT-5** Cardiac Plexus Point.
- HT-9** Depression Point (Gold Point on the right ear); Indicator Point for magnesium.

Small Intestine Meridian

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

- SI-3** Celiac Plexus Point on the back of the ear (Retro Point Zero); Tonification Point, Cardinal Point, Master Point for spasmolysis, general point for mucosa.

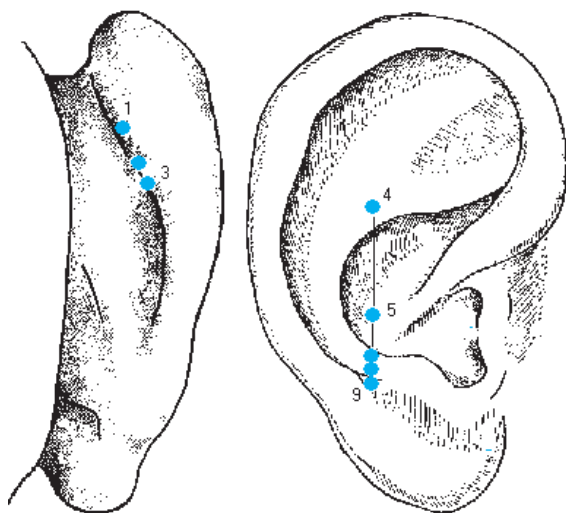


Fig. 8.5 Heart Meridian

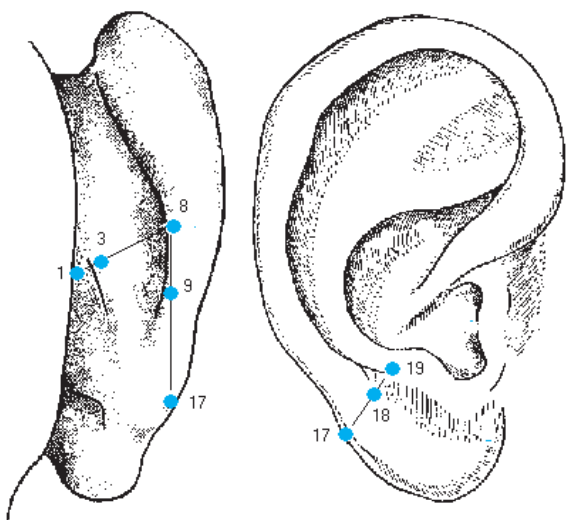


Fig. 8.6 Small Intestine Meridian

Bladder Meridian

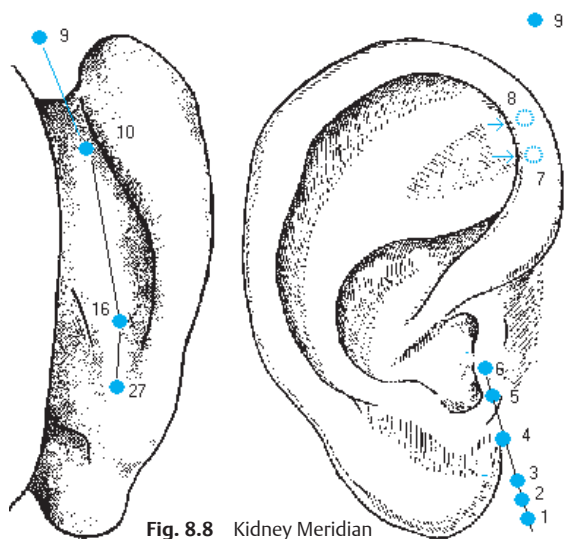
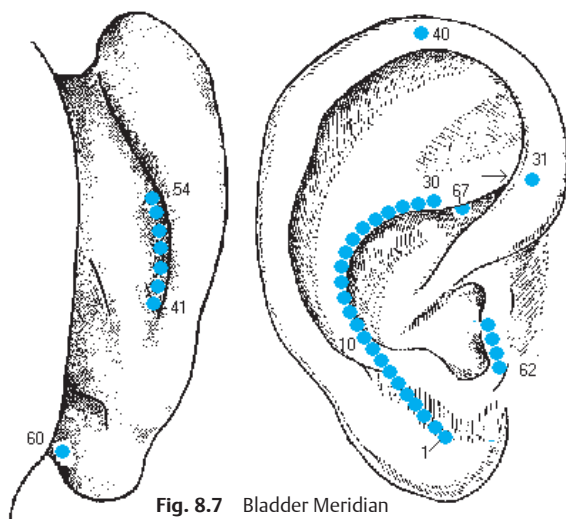
Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

- BL-31** Uterus Point (see also SP-6); Master Point for menopause.
- BL-40** Histamine Point (Gold Point on the left ear); Master Point for allergies.
- BL-60** Located on the back of the ear near the inferior attachment of the earlobe; Master Point for all symptoms in the region of the vertebral column.
- BL-62** Pineal Gland Point (Gold Point on the left ear); Cardinal Point, Master Point for insomnia.
- BL-67** Urinary Bladder Point; Tonification Point.

Kidney Meridian

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

- KI-3** Located in front of the earlobe; Yuan Point, Master Point for oscillation.
- KI-6** Valium Analogue Point; Cardinal Point, Master Point for sleep disturbances.
- KI-7** Kidney Point; Tonification Point, Indicator Point for zinc.



Pericardium Meridian

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

PC-6 Stellate Ganglion Point; Cardinal Point.

PC-9 Adrenaline Point (Adrenal Gland Point, cortex portion) (Gold Point on the right ear); Tonification Point.

Triple Burner Meridian

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

TB-3 Cortisone Point (Adrenal Gland Point, medulla portion) (Gold Point on the left ear); Tonification Point.

TB-4 Insulin Point (Gold Point on the left ear).

TB-5 Thymus Gland Point (Gold Point on the left ear); Cardinal Point, Master Point for rheumatic symptoms.

TB-6 Endocrine Thyroid Gland Point.

TB-7 Endocrine Parathyroid Gland Point.

TB-18 Located at the transition of antitragus and antihelix; Indicator Point for fluoride.

TB-21 Located in the postantitragal fossa, Master Point of the ear.

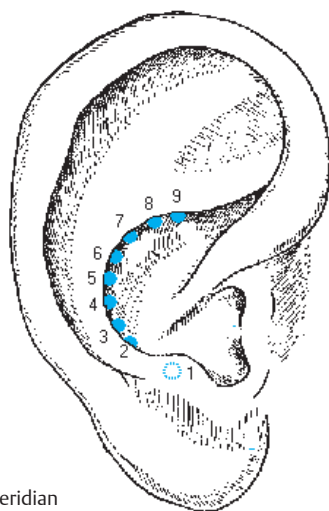


Fig. 8.9 Pericardium Meridian

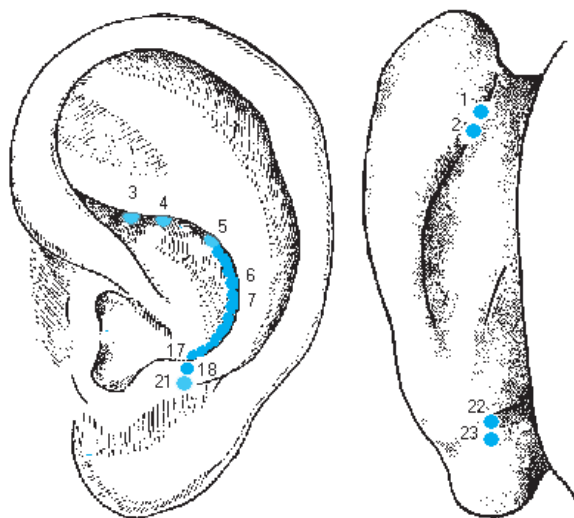


Fig. 8.10 Triple Burner Meridian

Gall Bladder Meridian

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

GB-1 Eye Point.

GB-3 Located on the baseline of the antitragus; Migraine Point (Sumatriptane Analogue Point).

All those points of the Gall Bladder Meridian that are located on the head are projected onto the ear between the Eye Point and the apex of the antitragus. On this short stretch, the practitioner will often find, and treat, active points in patients with acute migraine or headache.

GB-21 Hypothalamus Point.

GB-40 Located on the back of the ear, just above the Prostaglandin PGE1 Point; Indicator Point for vitamin D, *Yuan* Point.

GB-41 Prostaglandin PGE1 Point; Cardinal Point.

GB-43 Gall Bladder Point; Tonification Point.

Liver Meridian

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

LR-1 Indicator Point for vitamin B12.

LR-3 Nervous Liver Point, Anger Point (Silver Point on the right ear), Source Point, Master Point for spasmodic.

LR-8 Located in the Liver Zone; Tonification Point.

LR-13 ACTH Point.

LR-14 Aggression Point (Silver Point on the right ear)

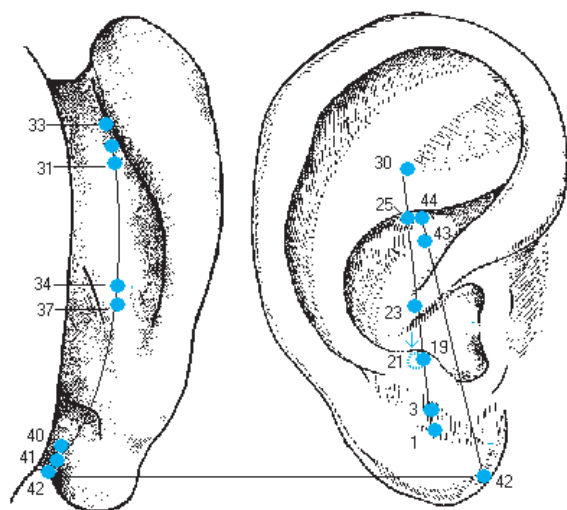


Fig. 8.11 Gall Bladder Meridian

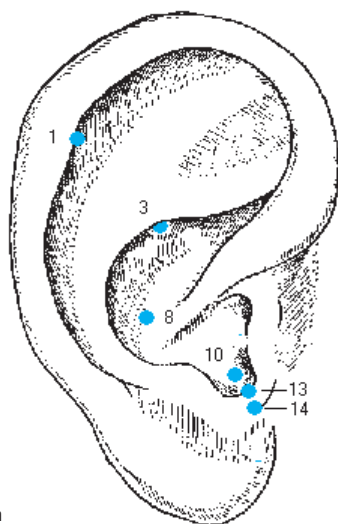


Fig. 8.12 Liver Meridian

Conception Vessel (*Ren Mai*)

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

- CV-5** Omega Point II; Indicator Point for vitamins B1, B3, B6, E.
- CV-8** Point Zero (Celiac Plexus Point).
- CV-12** Omega Point I; Indicator Point for amalgam, Master Point of the hollow visceral organs.
- CV-17** Master Omega Point.
- CV-22** Located on the neck below the earlobe; Indicator Point for vitamin A, Indicator Point for formaldehyde.
- CV-24** Located on the neck below the earlobe; it has an effect on the Laterality Point when combined with GV-20.

Governor Vessel (*Du Mai*)

Note: The points indicate the course of the ear meridian but not the metal type to be used for treatment.

- GV-4** Located on the back of the ear near the central posterior sulcus; DNA Point, Master Point for potency.
- GV-14** Located on the back of the ear at the inferior attachment of the earlobe; the “Spider” of body acupuncture (with an effect on all *Yang* meridians in the neck–shoulder region).
- GV-16** Located on the neck below the inferior attachment of the earlobe; Counter Point to the *Yin Tang* Point on the transverse cranial axis of body acupuncture.
- GV-20** Located on the neck anteroinferior to the earlobe; Master Point of mental energy (with an effect on the Laterality Point when combined with CV-24).

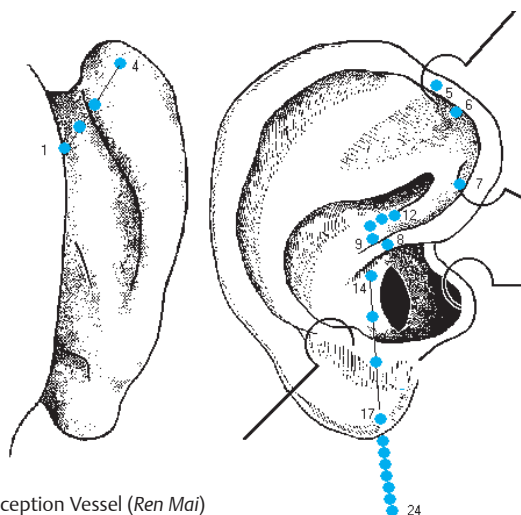


Fig. 8.13 Conception Vessel (*Ren Mai*)

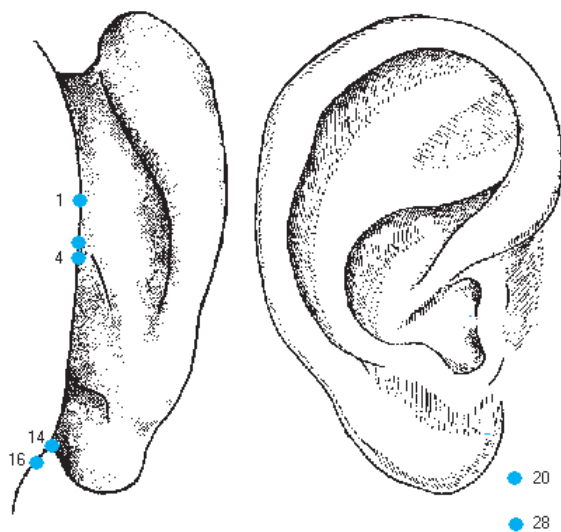


Fig. 8.14 Governor Vessel (*Du Mai*)

Indications

Indications—Overview

The points suggested here should be checked with the point finder for electrical activity. Only active points will be needled. All descriptions refer to right-handed persons.

As the more experienced reader will already know, there are no treatment plans or recipes in ear acupuncture (except for the auxiliary lines and the addiction programs). The only points to be needled are those actually found to be active on the ear. The various combinations of points are only listed to give the practitioner an idea of how to develop his/her own concepts of acupuncture treatment in relation to clinical considerations.

Pain points are usually treated with gold needles on the ear of the affected body side (of course, the final decision about the needle metal to be selected depends on the point finder). So-called Functional Points are described for the right-handed person. The needle metal to be used, or the ear to be treated, in the left-handed person can easily be deduced from this: the Functional Points lie on the **other** (contralateral) ear. After the treatment, it is of advantage to insert a permanent needle into the most important point(s).

 **An important note for therapists wishing to work exclusively with steel needles:** always needle the point on the same ear for which the Gold Point is described, because this is the point that needs to be stimulated.

In all stubborn cases that do not respond to therapy—and particularly when a good ear acupuncture treatment did not lead to early and significant alleviation of symptoms—one should consider the presence of a focal disturbance (see B. Strittmatter, *Overcoming Blockages to Healing—A Manual for Health Care Professionals*, Thieme Publishers, Stuttgart–New York, 2004).

The following foci are common and should be taken into account:

- Scars, especially those that healed secondarily (the size of the scar is not important), also drainage scars and internal scars resulting from surgery (including the scar from perineotomy).
- Chronic inflammation (tonsils, paranasal sinuses, hemorrhoids, pelvic inflammatory disease).
- Devalitized teeth or inflammation in the dental–maxillary region.

If focal activity is suspected, one can simply examine the ear reflex zones belonging to potential foci detected in the patient's history and check them with the point finder for electric activity (indicating focal disturbance). A gold needle placed exactly into this point (i.e., the point for a disturbing scar) alleviates the focal activity, reduces inflammation, and heals it completely after a few repeat treatments. (Dental foci must usually be cleaned up by the dentist; however, the focal activity of a tooth is normally not discovered without ear acupuncture diagnostics in the first place.)

Disorders of the Locomotor System

Auriculotherapy of symptoms of the locomotor system (joints, vertebral column) always proceeds according to the same principle:

- 1 Local pain point (e.g., reflex point of a particular joint or vertebra)
- 2 The corresponding motor point on the back of the ear (forceps method according to Bahr), using a silver needle
- 3 Points in the corresponding sympathetic trunk segment, usually with silver needles
- 4 Anti-inflammatory, analgesic, and/or anti-rheumatic points
- 5 Points of focal disturbances, if present

This treatment plan is valid for both acute and chronic symptoms. Local pain points may also be pierced with two or three needles if the ear point occupies a wider area. Points for joints or vertebrae are needled, like all organ points, on the ear of the affected body side. So-called Functional Points are needled as described in detail (always use the other ear for left-handed persons).

Upper Cervical Syndrome (Craniocervical Syndrome)

Chief complaints: Occipital headache, migraine. In most cases, the syndrome is caused by myogelosis of the upper neck muscles and blockage of the upper cervical spine. As a rule, myogelosis will at some time cause blockages of facet joints. A properly placed needle in the corresponding ear point will prompt the body to release these blockages on its own within a short period of time.

Note: For each blockage there are usually **counter blockages** of facet joints at other sites. These must be included in the therapy; their points should be located on the ear and then needled.

Therapy:

- Local pain points of the upper cervical spine: for example, Point C0 as Gold Point on the ear of the affected body side.
- The corresponding motor points as Silver Points on the back of the ear (forceps method); in this case also Point C0/C1.
- Points of the counter blockage: for example, Point C7 as Gold Point.
- Points with spasmolytic and analgesic effects:
Point SI-3 as Gold Point on the right ear;
Point LR-3 as Silver Point on the right ear (synonym: Anger Point);
Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Psychotropic points, if indicated:
Valium Analogue Point as Gold Point on the left ear;
Depression Point as Gold Point on the right ear;
Master Omega Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
C0	15	○ or	○
Retro C0/C1	17	● or	●
C7	15	○ or	○
Stellate ganglion	177	●	
SI-3 (Retro-Celiac Plexus Point)	325	○	
LR-3 (Anger Point)	331	● Always on the right	
LI-4 (thalamus)	321	○	
Diazepam Analogue Point	215		○
Depression Point	225	○	
Master Omega Point	221	○	

Middle Cervical Syndrome

Chief complaint: Frequent pain in the shoulder, or in shoulder and neck, without any findings at the joints themselves.

Therapy:

- Local points of the cervical spine: usually Points C3–C5 as Gold Points on the ear of the affected side.
- The corresponding motor points as Silver Points on the back of the ear (forceps method).
- Points of the counter blockage: for example, Point L4/L5 as Gold Point.
- Points with spasmolytic and analgesic effects:
Point SI-3 as Gold Point on the right ear;
Point LR-3 as Silver Point on the right ear (synonym: Anger Point).
Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Points with antirheumatic effect: PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.
- Psychotropic points, if indicated:
Valium Analogue Point as Gold Point on the left ear;
Depression Point as Gold Point on the right ear;
Master Omega Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
C3–C5	15	○ or	○
L4/L5	17	○ or	○
Retro C3–C5	17/19	● or	●
SI-3 (Retro-Celiac Plexus Point)	325	○	
Stellate ganglion	177	● or	●
LR-3 (Anger Point)	331	● Always on the right	
LI-4 (thalamus)	321	○	
PGE1 Point	261	○	
Diazepam Analogue Point	215		○
Depression Point	225	○	
Master Omega Point	221	○	

Lower Cervical Syndrome (Cervicobrachial Syndrome)

Chief complaints: Swollen hands, pain in the trapezius muscle.

Therapy:

- Local points of the lower cervical spine (see Cervical Spine Zone), and points listed under middle cervical syndrome.

Note: The trapezius muscle is supplied by cranial nerve XI (accessory nerve), represented by the Accessory Nerve Point as Gold Point on the ear of the affected side.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Points of the lower cervical spine	15	○ or	○
Accessory nerve (C0/C1)	163	○ or	○

Blockage of the First Rib

Chief complaint: Pain while lifting the arm, or even in resting position, caused by unilateral (rarely bilateral) blockage of the 1st rib. Treatment of the points along the auxiliary line of the First Rib Point releases the blockage.

Therapy:

First Rib Point, Stellate Ganglion Point, Point Zero (synonym: Celiac Plexus Point).

Note: Repeated blockage may indicate a focal disturbance.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
First rib	297	○ or	○
Stellate ganglion	297	●	●
Point Zero	185	○	

Disk Prolapse

It is self-evident that there should be a neurological examination of the lower limbs prior to every treatment of lumbago or sciatica (Lasègue's sign, reflexes, weakness in the dorsiflexion of big toe and foot, epicritic and proprioceptive sensibilities of the dermatomes). A disk prolapse justifying surgical intervention is not a primary indication for acupuncture, although pain treatment may be performed prior to surgery.

Therapy:

- Symptom points:
 - Points for painful muscles in the Zone of Paravertebral Muscles and Ligaments;
 - point of the prolapsed disk in the Zone of Intervertebral Disks;
 - point for the irritated segment in the Spinal Cord Zone (sensory portion).
- All points are needled as Gold Points on the ear of the affected side.
- The corresponding motor points as Silver Points on the back of the ear (forceps method).
- Points for the respective segment in the Sympathetic Trunk Zone as Gold Points on the ear of the affected side (sedation of pain-producing sympathetic activity).
- Spasmolytic points:
 - Point SI-3 as Gold Point on the back of the right ear;
 - LR-3 as Silver Point on the right ear (synonym: Anger Point).
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Antirheumatic points:
- PGE1 Point as Gold Point on the right ear;
- Thymus Gland Point as Gold Point on the left side.
- Psychotropic points, if indicated:
 - Valium Analogue Point as Gold Point on the left ear;
 - Depression Point as Gold Point on the right ear;
 - Master Omega Point as Gold Point on the right ear;
 - Psychotherapy Point (synonym: Bourdiol Point) as Silver Point on the right ear;
 - Anxiety Point as Silver Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom points: Point of the painful muscles		○ or	○
Corresponding retro point	11	● or	●
Point of the intervertebral disk		○ or	○
Point of the spinal cord (sensory portion)	147	●/○ or	●/○
Points of the corresponding sympathetic trunk segment	173	●/○ or	●/○
SI-3 (Retro-Celiac Plexus Point)	325	○	
LR-3 (Anger Point)	331	● Always on the right	
LI-4 (thalamus)	321	○	
PGE1 Point	261	○	
Diazepam Analogue Point	215		○
Depression Point	225	○	
Master Omega Point	221	○	
Bourdiol Point	233	●	
Anxiety Point	225	●	

Also a small prolapse or a true irritation of the nerve root that should be treated conservatively can be treated as described above.

Note: All therapy of the spinal column should include physiotherapy, and weight loss if necessary, to stabilize the anterior and posterior trunk muscles.

Lumbago, Sciatica

Pain in the lower back, often caused by blockages of the small vertebral joints and/or concomitant myogelosis. It always makes sense to search for active points in the respective segment of the Sympathetic Trunk Zone (sedation of the pain-producing sympathetic activity).

 **Caution:** If resistant to therapy, check the Depression Point for activity and needle if indicated. Symptoms of pain are often caused by a masked depression.

Therapy:

- Symptom point: Point L5/S1 (affected in most cases) and the corresponding motor point on the back of the ear (forceps method).
- Points in the respective segment in the Sympathetic Trunk Zone as Gold Points on the ear of the affected side.
- Spasmolytic points:
Point SI-3 as Gold Point on the right ear;
LR-3 as Silver Point on the right ear (synonym: Anger Point).
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Antirheumatic points:
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.
- Point with a general effect on the entire zone of the locomotor system: Zone Dominating Point ZC 2 (for the lower limb).
- Psychotropic points, if indicated:
Valium Analogue Point as Gold Point on the left ear;
Depression Point as Gold Point on the right ear;
Master Omega Point as Gold Point on the right ear.

Note: Repeated lumbago or lumbago-sciatica may indicate a focal disturbance.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom points: L5/S1	21	○ or	○
Retro L5/S1	21	● or	●
Points of the corresponding sympathetic trunk segment	173	●/○ or	●/○
SI-3 (Retro-Celiac Plexus Point)	325	○	
LR-3 (Anger Point)	331	● Always on the right	
LI-4 (thalamus)	321	○	
PGE1 Point	261	○	
Thymus gland	95		○
ZC2	279		○
Diazepam Analogue Point	215		○
Depression Point	225	○	
Master Omega Point	221	○	

Blockage of the Sacroiliac Joint

Pain radiating into the leg (pseudoradicular irritation) caused by blockage of one or both sacroiliac joints. This certainly is the most misdiagnosed (unrecognized) cause of back pain. A crude test: direct pressure on the affected sacroiliac symphysis is painful; when bending forward the upper edge of the affected (blocked) ala of the ileum (posterior superior iliac spine) is asymmetrically drawn upward. Ear acupuncture may release the blockage.

Therapy:

- Sacroiliac Joint Point as Gold Point on the ear of the affected side, and the corresponding motor point on the back of the ear (forceps method).
- Points of the counter blockage: for example, Point C7 as Gold Point on the ear of the affected side.
- Cardinal Point SI-3 (spasmolytic effect; synonym: Retro-Celiac Plexus Point, Retro-Point Zero), as Gold Point on the right ear;
Cardinal Point BL-62 as Gold Point on the left ear (synonym: Pineal Gland Point).
Both Cardinal Points have a relaxing effect, especially on the sacroiliac joint, because they connect to two special meridians (Point SI-3 is the Opening Point of the Governor Vessel, and Point BL-62 is the Opening Point of the Yang Heel Vessel).
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).

Note: Repeated blockage of the sacroiliac joint may indicate a focal disturbance.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Sacroiliac joint (on the affected side)	31	●/○ or	●/○
Corresponding retro point	31	● or	●
C7	15	○ or	○
SI-3 (Retro-Celiac Plexus Point)	325	○	
BL-62	327		○
LI-4 (thalamus)	321	○	

Bechterew's Disease (Ankylosing Spondylitis)

In patients with this chronic disease, the two Cardinal Points SI-3 and BL-62 on the ear should be tried in any case, even if the loss of spinal motion is advanced. These two Cardinal Points have a relaxing effect especially on the sacroiliac joint because they connect to two special meridians (Point SI-3 is the Opening Point of the Governor Vessel, Point BL-62 is the Opening Point of the *Yang* Heel Vessel). This treatment results in pain reduction and, surprisingly, often in a wider range of spinal motion.

Therapy:

- Sacroiliac Joint Point as Gold Point on each ear.
- The respective motor point (for muscle relaxation) on the back of each ear.
- Point SI-3 as Gold Point on the right ear, and Point BL-62 as Gold Point on the left ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Sacroiliac joint	31	○ and	○
SI-3 (Retro-Celiac Plexus Point)	325	○	
BL-62	327		○

Coccygodynia

Unexplained pain in the coccyx; more common in females.

Therapy:

- Coccyx Point. A permanent needle in the painful ear point is often sufficient; the ear may be pre-pierced with a gold needle, if necessary.

Note: Resistance to therapy may indicate a focal disturbance.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Coccyx (permanent needle)	23	○ and	○

Shoulder Problems

It should first be clarified whether the symptoms are only due to pain radiating into the shoulder from the cervical spine region (see under Cervicobrachial Syndrome). Frequently, it is not the joint itself that causes pain in the shoulder but insertion tendinopathies (supraspinatus and infraspinatus muscles, subscapular muscle); they are associated with a characteristic pain spot at the greater tubercle of humerus and pain radiating into the arm.

Therapy:

- Symptom point: Shoulder Joint Point as Gold Point on the ear of the affected side, and the corresponding motor point as Silver Point on the back of the ear (forceps method). It is essential to consider also the root of the problem (motor nerve supply of the shoulder): Point C3 in the Cervical Spine Zone.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Antirheumatic and anti-inflammatory points:
Interferon Point as Gold Point on the left ear;
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.

Note: Resistance to therapy may indicate a focal disturbance. Potential foci with an effect on the shoulder are scars along the Large Intestine Meridian of the body or devitalized inferior molars.

 The same therapy is also used for periarthritis of the shoulder. Ear acupuncture leads to better mobilization during physiotherapy.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom point: Shoulder	47	○ or	○
Corresponding retro point	47	● or	●
Pain Memory Point of the shoulder (on the painful side)	241	● or	●
C3	15	○	○
LI-4 (thalamus)	321	○	
Interferon Point	209		○
PGE1 Point	261	○	
Thymus gland	95		○

Tennis Elbow (Lateral Epicondylitis)

Pain at the lateral epicondyle caused by myogelosis in the extensor muscles of the forearm. Apart from mechanical strain of the elbow, symptoms are often caused by disturbed laterality (resulting from working with both hands, such as typing or piano playing).

Therapy:

- Symptom points: Elbow Joint Point as Gold Point on the ear of the affected side, and the corresponding motor point on the back of the ear (forceps method).
- Points in the Cervical Spine Zone (usually Points C7 and T1) as Gold Points on the ear of the affected side.
- Analgesic points: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Antirheumatic and anti-inflammatory points:
Interferon Point as Gold Point on the left ear;
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.
- Laterality Point as Gold Point on the right side. (Attention: in left-handed persons as Gold Point on the left ear.)

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Elbow	49	○ or	○
Retro elbow	49	● or	●
Pain Memory Point of the elbow (on the painful side)	241	●	●
C7	15	○ or	○
LI-4 (thalamus)	321	○	
Interferon Point	209		○
PGE1 Point	261	○	
Thymus Gland Point	95		○
Laterality Point	237	○	

Carpal Tunnel Syndrome

To avoid surgery it is always worthwhile to try conservative therapy by ear acupuncture.

Therapy:

- Symptom points: local pain points or points of the inflamed tendons in the Wrist Zone as Gold Points on the ear of the affected side, and the corresponding motor points as Silver Points on the back of the ear (forceps method; for relaxation of the muscles).
- Points in the Cervical Spine Zone (with special attention to Points C4–C6) as Gold Points on the ear of the affected side.
- Anti-inflammatory point: Interferon Point as Gold Point on the left ear.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Local pain point	51	○ or	○
Corresponding retro point	51	● or	●
Pain Memory Point for the wrist (on the painful side)	241	● or	●
Points of the cervical spine (C4–C6)	15	○ or	○
Interferon Point	209		○
LI-4 (thalamus)	321	○	

Sudeck's Atrophy

Dystrophy of the hand and forearm as a sympathetic reflex response resulting in disturbed circulation and metabolism of soft tissues and bones, usually after surgery (post-traumatic osteoporosis). The clinical symptoms tend to become chronic. Because this is dangerous for the patient, all possible therapies (including focal diagnosis and therapy) should be exhausted. It is always worthwhile to try local symptom points and also points that stabilize the autonomic nervous system and sedate sympathetic overactivity. Apart from the usual physical measures and medication, treatment with auriculotherapy is possible at any stage.

Therapy:

- Symptom points: points in the Wrist Zone as Gold Points on the ear of the affected side. Use permanent needles; the ear may be pre-pierced with a gold needle.
- Points of the respective segment in the Sympathetic Trunk Zone as Gold Points (or Silver Points) on the ear of the affected side (important to sedate sympathetic overactivity). (The needle metal, or the ear to be needled, must be determined with the point finder).
- Points that stabilize the autonomic nervous system:
Valium Analogue Point as Gold Point on the left ear;
Spleen Point as Gold Point on the left ear.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Antirheumatic and anti-inflammatory points:
Interferon Point as Gold Point on the left ear;
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom point: Wrist	51	○ or	○
Diazepam Analogue Point	215		○
Spleen	79		○
LI-4 (thalamus)	321	○	
Interferon Point	209		○
PGE1 Point	261	○	
Thymus gland	95		○

Polyarthrosis of the Hand

Therapy:

- Symptom point: Metacarpophalangeal Joint Point as Gold point on the ear of the affected side.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Antirheumatic and anti-inflammatory points:
Interferon Point as Gold Point on the left ear;
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant ear
Symptom point: Metacarpophalangeal joint of thumb	55	○ or	○
LI-4 (thalamus)	321	○	
Interferon Point	209		○
PGE1 Point	261	○	
Thymus gland	95		○

Coxarthrosis (Osteoarthritis of the Hip Joint)

Even in cases of advanced coxarthrosis, soothing of the symptoms can be achieved with auriculotherapy, and the ability to walk can be maintained for a long time in older persons (especially when surgery is no longer possible).

Differential diagnosis: pain on palpation in the inguinal region. Athletes (especially soccer players) frequently suffer from a sudden onset of insertion tendinopathy of the adductor muscle at the inferior ramus of pubis. Here, a permanent needle in the corresponding ear zone (close to the Hip Joint Point) is often sufficient.

Therapy:

- Symptom points: Hip Joint Point as Gold Point on the ear of the affected side, and the corresponding motor point as Silver Point on the back of the ear (forceps method, because of myogelosis of the adductor muscles).
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Antirheumatic and anti-inflammatory points:
Interferon Point as Gold Point on the left ear;
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom point: Hip joint	33	○ or	○
Retro hip joint	33	●	●
Pain Memory Point of the hip (on the painful side)	241	● or	●
LI-4 (thalamus)	321	○	
Interferon Point	209		○
PGE1 Point	261	○	
Thymus gland	95		○

Knee Joint Problems

All diseases of the knee joint (gonarthrosis, meniscopathy, chondropathy of the patella) require the same therapy: symptom points as well as anti-inflammatory, antirheumatic, and analgesic points.

Therapy:

- Symptom point: Knee Joint Point as Gold Point on the ear of the affected side (may require several needles next to each other).
- Anti-inflammatory point: Interferon Point as Gold Point on the left ear.
- Antirheumatic points:
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom point: Knee joint	35	○ or	○
Pain Memory Point of the knee (on the painful side)	241	● or	●
Interferon Point	209		○
PGE1 Point	261	○	
Thymus gland	95		○
LI-4 (thalamus)	321	○	

Pain underneath the patella (chondropathy of patella) is especially difficult to treat with conventional methods. Auriculotherapy is usually successful within a few sessions and may bring about complete freedom from symptoms (same therapy as for gonarthrosis, see above).

Concomitant exercise of the quadriceps muscle is essential because instability in the knee joint may produce pain.

It is essential to clarify whether “pain in the knee” is not just a simple irritation of the pes anserinus. (Irritation of the muscle insertion of the adductor muscle of the thigh at the medial superior tibial ridge is interpreted by the patient as pain in the knee.) A thin steel needle placed directly in the muscle

insertion (primary pressure spot) so that it has contact with the bone often results in immediate freedom from pain (and thus excludes the knee joint itself as the cause of pain). Another needle in the reflex point on the ear is often no longer necessary.

Ankle Joint Problems

Pain in the ankle joint caused by arthropathy, ligament lesions, or distortions. Fresh distortions are especially responsive to emergency needling; a permanent needle placed exactly in the reflex point on the ear is often sufficient. But also pain caused by chronic osteoarthritis of the ankle joint can be treated, and nutrition in this region improved, by needles in the ear.

Therapy:

- Symptom point: Ankle Joint Point as Gold Point on the ear of the affected side (this may require a few needles next to each other).
- Anti-inflammatory point: Interferon Point as Gold Point on the left ear.
- Antirheumatic points:
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom point: Ankle joint	35	○ or	○
Pain Memory Point of the ankle joint (on the painful side)	241	● or	●
Interferon Point	209		○
PGE1 Point	261	○	
Thymus gland	95		○
LI-4 (thalamus)	321	○	

Achillodynia

Pain in the Achilles tendon. Auriculotherapy yields excellent results, with immediate freedom from pain in most cases.

Therapy:

- Local point: Achilles Tendon Point as Gold Point on the ear of the affected side. (A permanent needle is inserted exactly into the active point; this often requires two gold needles for pre-piercing.)
- Anti-inflammatory point: Interferon Point as Gold Point on the left ear.
- Antirheumatic points:
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Reflex point: Achilles tendon	41	○ or	○
Interferon Point	209		○
PGE1 Point	261	○	
Thymus gland	95		○
LI-4 (thalamus)	321	○	

Calcaneal Spur

Pain in the heel bone. It can be successfully treated with auriculotherapy, even if the radiographs show a calcaneal spur. (Many calcaneal spurs accidentally discovered on radiographs have been free from symptoms for years.)

Therapy:

- Local point: Heel Point as Gold Point on the ear of the affected side. (A permanent needle is inserted exactly at the active point. Due to the point's hidden location, the permanent needle is difficult to apply; it is therefore inserted into the hole left behind by the gold needle treatment.)
- Anti-inflammatory point: Interferon Point as Gold Point on the left ear.
- Antirheumatic points:
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Heel Point	41	○ or	○
Interferon Point	209		○
PGE1 Point	261	○	
Thymus gland	95		○
LI-4 (thalamus)	321	○	

Burning Feet

Burning sensation in the soles of the feet, usually on the lateral side. This is mostly a female complaint. It is difficult to treat by conventional medicine.

Therapy:

- Local points in the Foot Zone as Gold Points on the ear of the affected side.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Local point of the foot	39	○ or	○
LI-4 (thalamus)	321	○	

Gouty Toe

Auriculotherapy in addition to the usual medical approach regarding the underlying disease.

Therapy:

- Symptom point: point of the big toe's metatarsophalangeal joint in the Foot Zone as Gold Point on the ear of the affected side.
- Anti-inflammatory points: Interferon Point as Gold Point on the left ear; Thymus Gland Point as Gold Point on the left ear.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- A point that is energetically stabilizing: Kidney Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom point: Big toe	43	○ or	○
Interferon Point	209		○
Thymus gland	95		○
LI-4 (thalamus)	321	○	
Kidney	83	○	

Rheumatoid Arthritis (Chronic Polyarthritis)

It is not recommend that the beginner treats patients with true rheumatoid arthritis. There are usually multiple focal activities that support the inflammatory activity of the disease. In addition, once the disease has been going on for a long time, there is a tendency for concomitant emotional reactions that can enhance the sensitivity to pain via changes in the neurotransmitters.

Therapy:

- Symptom points: Points of the affected joints as Gold Points on the ear of the affected side.
- Anti-inflammatory point: Interferon Point as Gold Point on the left ear.
- Points stimulating the body's own cortisone production:
Cortisone Point as Gold Point on the left ear;
ACTH Point as Gold Point on the right ear.
- Antirheumatic points:
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Points that may indicate focal disturbances.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom points: Points of the affected joints	33 ff.	○ or	○
Interferon Point	209		○
Cortisone Point	205		○
ACTH Point	199	○	
PGE1 Point	261	○	
Thymus gland	95		○
LI-4 (thalamus)	321	○	

Fibromyalgia

Defined as chronic generalized pain in the region of muscles, connective tissue, and bones with typical extra-articular pain points (the upper part of the trapezius muscle, iliac crest, 2nd rib, posterior part of elbow, medial part of knee joint). The cause of the disease is still unknown. In practice, however, it is often impressive to observe how elimination of focal disturbances by auriculotherapy (or by restoration of teeth with focal activity) cannot only bring the process to a halt but can often also achieve freedom from pain for many years. Sometimes, heavy metal poisoning is involved, for example due to amalgam fillings. Though treatment should be the privilege of an experienced practitioner, even the beginner can achieve remarkable results if he/she patiently searches for foci and includes these in the therapy. (After careful analysis of the patient's history for potential foci, the ear is screened with the point finder for active points.)

Therapy:

- Symptom points: Reflex points of painful muscle areas as Gold Points on the ear of the affected side; for example, the point of the upper trapezius region in the Trunk Muscle Zone or the point of the iliac crest in the Pelvic Zone.
- Anti-inflammatory point: Interferon Point as Gold Point on the left ear.
- Antirheumatic points:
 - PGE1 Point as Gold Point on the right ear;
 - Thymus Gland Point as Gold Point on the left ear.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Points stimulating the body's own cortisone production:
 - Cortisone Point as Gold Point on the left ear;
 - ACTH Point as Gold Point on the right ear.
- Psychotropic points (a careful search for active points is essential):
 - Depression Point as Gold Point on the right ear;
 - Aggression Point as Silver Point on the left ear;
 - Psychotherapy Point (synonym: Bourdiol Point) as Silver Point on the right ear;
 - Valium Analogue Point as Gold Point on the left ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Points of affected muscles (gold needle in right or left ear)		○ or	○
Interferon Point	209		○
PGE1 Point (GB-41)	261	○	
Thymus gland (TB-5)	95		○
LI-4 (thalamus)	321	○	
Cortisone Point	205		○
ACTH Point	199	○	
Depression Point	225	○	
Aggression Point	231	●	
Bourdiol Point	233	●	
Diazepam Analogue Point	215		○
SI-3 (Retro-Celiac Plexus Point)	325	○	
BL-62	327		○

Phantom Limb Pain

Chief complaint: Although the limb is no longer present, pain may still be perceived at the site of the absent limb. The local symptom points on the ear should be located (e.g., those for the hand) because the respective ear zone will influence the corresponding brain areas. (Every effect on the limbs via ear points is probably mediated in this way.) Active ear points will also be found in the **contralateral** Postcentral Gyrus Zone, where the sensation of pain in the absent body part is projected. It is important to search also for active points in the Postcentral Gyrus Zone of the **ipsilateral** hemisphere because of the commissural tracts (connections between the right and left hemispheres).

Therapy:

- Local pain point: Exact insertion of a gold needle into the respective point on the ear of the affected side (followed by a permanent needle, if necessary).
- Active point in the Postcentral Gyrus Zone as Gold Point on the contralateral ear and, perhaps, also on the ipsilateral ear.
- Analgesic points:
Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Laterality Point as Gold Point on the right side.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Local pain point (on the affected side)		○ or	○
Active point of the postcentral gyrus (on the painful side)	121	○ or	○
LI-4 (thalamus)	321	○	
Laterality Point	237	○	
Pain Memory Point (General Analgesic Point)	241	● or	●
Depression Point	225	○	

Headaches and Facial Pain

Almost 20 years of experience in the field of pain therapy have taught me that there is a gradual transition from one type of headache to another one (e.g., from tension headache to migraine). This is demonstrated clearly by migraine patients: under the influence of auriculotherapy they report that, initially, the migraine is perceived as being less intense and less frequent, while at the end of the therapy it occurs only once in a while with the typical symptoms of tension headache. If the migraine was usually associated with menstruation, the patient often reports—as a sign of improvement under auriculotherapy—that instead of a migraine, tension headache occurred exactly at the usual time. The typical migraine patient is very familiar with the symptoms of tension headache. Myogelosis in the upper part of the trapezius muscle and in the occipital region is often the precursor of a migraine attack, or the attacks are triggered by prolonged, unilateral, and tensed body posture, for example, at a writing desk.

Examination by ear acupuncture has revealed that many cases of migraine are caused by a weakness of the Gall Bladder Meridian. When asked to describe the spread of pain during an attack, most patients accurately describe with their finger the course of this meridian on the head: at the temples, beside and behind the eyes, at the forehead, at the occiput, and in the neck region. It is also interesting to note that many patients wake up very early in the morning with a beginning migraine, its onset coinciding with the tonification period of the Gall Bladder Meridian (between 11 p.m. and 1 a.m.).

For all types of migraine, the Gall Bladder Point should therefore be included in the examination (synonym: Point GB-43, **Tonification Point of the Gall Bladder Meridian** on the ear).

Tension Headache

Frequently caused by myogelosis of shoulder and neck muscles (incorrect posture at the work place, stress-induced myogelosis).

Therapy:

- Local points: reflex points of the muscles in the cervical spine region (in the Zone of Paravertebral Muscles and Ligaments), sensory portion as Gold Points on the front of the ear, and motor portion as Silver Points on the back of the ear.
- Points of the cervical region of the sympathetic trunk: Superior, Medium, and Inferior (Stellate) Cervical Ganglion Points as Gold Points on the right ear.
- Spasmolytic points: Point SI-3 as Gold Point on the right ear (synonym: Retro-Celiac Plexus Point, Retro-Point Zero); LR-3 as Silver Point on the right ear (synonym: Anger Point).
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Psychotropic points, if indicated: Valium Analogue Point as Gold Point on the left ear; Depression Point as Gold Point on the right ear; Master Omega Point as Gold Point on the right ear; Psychotherapy Point (synonym: Bourdiol Point) as Silver Point on the right ear.
- Gall Bladder Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Active points of the cervical muscles	15	○ or	○
Corresponding retro points	11	● or	●
Active points of the cervical sympathetic trunk	175	○ or	○
SI-3 (Retro-Celiac Plexus Point)	325	○	
LR-3 (Anger Point)	331	●	
LI-4 (thalamus)	321	○	
Diazepam Analogue Point	215		○
Depression Point	225	○	
Master Omega Point	221	○	
Bourdiol Point	233	●	
Gall bladder	77	○	

Migraine

In the majority of cases, an active Gall Bladder Point will be found on the ear. From this it may be deduced that there is an underlying weakness of the Gall Bladder Meridian. Many patients report the spreading of pain before and during the attack as following exactly the course of the Gall Bladder Meridian on the head, and the start of the attack often overlaps with the tonification period of the Gall Bladder Meridian (very early in the morning or immediately after waking up). The pain behind the eyes can be explained by the internal and external course of the Gall Bladder Meridian behind and near the eyes: Point GB-1 projects onto the Eye Point on the ear.

Scientific studies also indicate that the system of bile ducts contributes to migraine attacks: ultrasound examination of 30 migraine patients revealed that 29 patients had previously unknown alterations in the bile duct area (stones, thickening of the gall bladder wall, etc.; M. Strittmatter, Department of Neurology, Saarland University Hospitals, unpublished results).

Strengthening of the Gall Bladder Meridian in such cases is surprisingly quick and effective. During a migraine attack, therefore, the Gall Bladder Point on the ear, which corresponds to Point GB-43 of body acupuncture, and other points of the Gall Bladder Meridian projected onto the ear should **always** be checked for activity and included in the treatment, if active. The energetic partner of the Gall Bladder Point is the Liver Zone; it should also be checked for active points and treated if necessary.

Focal disturbances may support a migraine, but they may also cause a new migraine (in persons with the respective disposition). For example, surgery on the lateral malleolus will cut through the Gall Bladder Meridian. As a result, migraine may occur after the surgery for the first time.

Clinically, there are different types of migraine that are relevant to auriculotherapy:

Gall Bladder Migraine

There is an obvious weakness of the gall bladder and its meridian in these patients: they worry about everything (coupling of Gall Bladder and Worry in the Chinese sense). The migraine typically begins in the early morning or in the middle of the night (tonification period of the Gall Bladder Meridian: between 11 p.m. and 1 a.m.).

Therapy:

- Gall Bladder Point as Gold Point on the right ear (synonym: Point GB-43, tonification point of the meridian).
- Perhaps also Point GB-40 (Source Point of the meridian) as Gold Point on the right ear.
- Worry Point as Silver Point on the left ear.
- Eye Point (synonym: Point GB-1) as Gold Point on the ear of the affected side, if the patient reports severe pain behind the eyes.
- Points for vegetative stabilization:
Hypothalamus Point as Gold Point on the right ear (synonym: Point GB-21);
Stellate Ganglion Point as Gold Point on the right ear.
- Point C0/C1 as Gold Point on the ear of the affected side (frequent blockage of the atlanto-occipital joint).
- All active points found on the antitragus during a migraine attack. **All** points of the Gall Bladder Meridian on the head (Points GB-1–GB-20) project on the antitragus.
- Testing for focal disturbances (check patient's history: ask the patient about all new or more intense migraine attacks).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Gall bladder	77	○	
GB-40	331	○	
Worry Point	229		●
GB-1 (eye)	331	○ or	○
GB-21 (hypothalamus)	331	○	
C0/C1	17	○ or	○

All medications and methods that strengthen the function of the gall bladder may be used; for example, taking mustard in the evening, magnesium at noon, plant extracts (e.g., cholagogue) in the evening.

Hormonal Migraine

In the case of an underlying weakness of either Liver Meridian or Gall Bladder Meridian, the decrease in hormones just before menstruation seems to have such a strong stimulating effect that it triggers a migraine attack. Here, too, the Gall Bladder Point is often active.

Therapy:

- Hormone points:
- Estrogen Point as Gold Point on the right ear, perhaps also Progesterone Point and Gonadotropin Point as Gold or Silver Points (use the point finder to decide on the needle metal).
- Other points: essentially those listed for gall bladder migraine, perhaps Liver Zone instead of Gall Bladder Point as Gold Point (use point finder to decide).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Estrogen Point	195	○	○
Progesterone Point	197		● or ○
Gonadotropin Point	197		● or ○

Cervical Migraine

- Points in the Cervical Spine Zone as Gold Points on the ear of the side with blockage; search for blockages along the cervical spine.
- Other points: essentially those listed for gall bladder migraine.

Cluster Headache

Here, ear acupuncture can yield excellent results. Once the patient is free from pain, it makes sense to repeat a few sessions of auriculotherapy every six months to keep the situation stable. Here, too, the key to therapy often lies in a weakness of the Gall Bladder Meridian. In some cases, intolerance to food components may trigger the attacks (a detailed patient's history is essential).

Therapy:

- Examination of all points listed under Gall Bladder Migraine.

Trigeminal Neuralgia

Therapy:

- Symptom points: points in the Trigeminal Nerve Zone as Gold Points on the ear of the affected side.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).
- Anti-inflammatory points:
Interferon Point as Gold Point on the left ear;
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.
- Psychotropic points, if indicated:
Valium Analogue Point as Gold Point on the left ear;
Depression Point as Gold Point on the right ear;
Master Omega Point as Gold Point on the right ear;
Psychotherapy Point (synonym: Bourdiol Point) as Silver Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Trigeminal nerve	155	○ or	○
LI-4 (thalamus)	321	○	
Interferon Point	209		○
PGE1 Point	261	○	
Thymus gland	95		○
Diazepam Analogue Point	215		○
Depression Point	225	○	
Master Omega Point	221	○	
Bourdiol Point	233	●	

Temporomandibular Joint Syndrome

In a wider context, myoarthropathy of the temporomandibular joint (TMJ) is not only a type of headache, it is also a disease of the locomotor system. Malfunction of this joint (e.g., inflammatory changes due to overstrain because of malocclusion, bruxism caused by stress or malocclusion) leads to tension of the masticatory muscles, and it also plays a decisive role in the pathogenesis of muscular tension in the upper and lower back. Furthermore, patients often complain about headache and facial pain, diffuse toothache, or morning pain in the shoulder, neck, or back.

Therapy aims primarily at achieving relaxation of the respective masticatory muscles, which is possible by ear acupuncture. Based on the principle that soft tissues shape hard tissues, effective relaxation of the masticatory and neck muscles will lead to a change in occlusion. (Other worthwhile therapies are transcutaneous nerve stimulation, occlusion control by a dentist, wearing an occlusion brace, and relaxation therapy.)

Inflammation of the TMJ is easy to influence: TMJ Point on the affected side, or on both sides, Interferon Point, as well as points in the Cervical Spine Zone, especially with incorrect posture as a potential contributing cause. Furthermore, points for general relaxation (Retro-Celiac Plexus Point) and emotional balance (Anger Point), because bruxism is frequently caused by stress and anger. Also, for the same reason, Valium Analogue Point.

Therapy:

- Symptom points:
TMJ Point as Gold Point on the ear of the affected side (or on both ears); points for the tensed masticatory muscles (Masseter Muscle Point; Temporal Muscle Point; perhaps also the Digastric Muscle Point) as Gold Points on the ear of the affected side.
- In particular, the corresponding motor points on the back of the ear.
- Points of the spinal column; for example, points in the Cervical Spine Zone.
- Spasmolytic points:
Retro-Celiac Plexus Point as Gold Point on the back of the right ear; Point LR-3 as Silver Point on the right ear (synonyms: Anger Point, Nervous Liver Point); Valium Analogue Point as Gold Point on the left ear.

- Anti-inflammatory point: Interferon Point as Gold Point on the left ear.
- Antirheumatic points:
PGE1 Point as Gold Point on the right ear;
Thymus Gland Point as Gold Point on the left ear.
- Analgesic point: Point LI-4 as Gold Point on the right ear (synonym: Thalamus Point).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom point: TMJ	101	○ or	○
Retro point of the masseter muscle	113	● or	●
Active points of the cervical spine	15	○ or	○
Points of the vertebral column	13		
SI-3 (Retro-Celiac Plexus Point)	325	○	
LR-3 (Anger Point)	331	● Always on the right	
Diazepam Analogue Point	215		○
Interferon Point	209		○
PGE1 Point	261	○	
Thymus gland	95		○
LI-4 (thalamus)	321	○	

Allergic Disorders

Hay Fever

Here, even the beginner can usually obtain surprising and quick results (relief from discomfort) with ear acupuncture. The Histamine Point (synonym: Allergy Point 1) is the most important point for all allergic diseases, together with Zone-Dominating Point ZA2. Depending on the affected organ, additional points are selected: Nose Point, Eye Point, points in the Bronchi Zone and Lung Zone. For example, the Eye Point may be needled on both sides as Gold Point. However, if several other needles need to be placed on the ear, these symptom points will be needled on the ear of the dominant body side. The therapy may be effectively supported by daily administration of zinc.

Therapy:

- Histamine Point as Gold Point on the left ear (synonym: Allergy Point 1).
- Local points: depending on the affected organ, Nose Point, Eye Point, points in the Bronchi Zone or Lung Zone as Gold Points on the ear of the affected side.
- Other antiallergic or anti-inflammatory points:
 - Cortisone Point as Gold Point on the left ear;
 - ACTH Point as Gold Point on the right ear;
 - Interferon Point as Gold Point on the left ear;
 - Thymus Gland Point as Gold Point on the left ear;
 - Zone-Dominating Point ZA2 as Gold Point on the left ear.
- Points that strengthen the constitution:
 - Kidney Point as Gold Point on the right ear (to strengthen Kidney Energy and Hereditary Energy);
 - Point Zero as Gold Point on the right ear (synonym: Celiac Plexus Point).
- Points that may indicate focal disturbances.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Histamine (Allergy) Point	207		○
Local points	61/63/109	○ or	○
Cortisone Point	205		○
ACTH Point	199	○	
Interferon Point	209		○
Thymus gland	95		○
ZA2	277		○
Kidney	83	○	
Point Zero	185	○	

Allergic Asthma

Therapy:

- Local points in the Lung Zone as Gold Points on the right ear, or in the Bronchi Zone as Gold Point on the right ear.
- Bronchospasmolytic points:
Beta-2-Receptor Point as Gold Point on the right ear.
- Histamine Point as Gold Point on the left ear (synonym: Allergy Point 1).
- Bronchopulmonary Plexus Point as Gold Point on the right ear.
- Other antiallergic and anti-inflammatory points:
Cortisone Point as Gold Point on the left ear;
ACTH Point as Gold Point on the right ear;
Interferon Point as Gold Point on the left ear;
Thymus Gland Point as Gold Point on the left ear;
Zone-Dominating Point ZA2 as Gold Point on the left ear.
- Points that strengthen the constitution:
Kidney Point as Gold Point on the right ear;
Perhaps Point KI-3 as Gold Point on the right ear.

- Psychotropic points, if indicated:
Valium Analogue Point as Gold Point on the left side;
Anxiety Point as Silver Point on the right ear;
Aggression Point as Silver Point on the right side.
- Points than may indicate focal disturbances.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Local points of the lung and bronchi	61/63	○	
Beta-2-Receptor Point	213	○	
Histamine (Allergy) Point	207		○
Bronchopulmonary plexus	183	○	
Cortisone Point	205		○
ACTH Point	199	○	
Interferon Point	209		○
Thymus gland	95		○
ZA2	277		○
Kidney	83	○	
KI-3	327	○	
Diazepam Analogue Point	215		○
Anxiety Point	225	●	
Aggression Point	231	●	

Neurodermatitis, Allergic Eczema

Neurodermatitis is included here as an allergic disease. If there is a predisposition to atopic disorders, the eczema itself is often induced or supported by food intolerance. Parallel to auriculotherapy, it is definitely recommended that an intestinal cleansing is performed with preparations containing, for example, *Lactobacillus*. It has been shown that a deficient intestinal flora causes a “leaky gut,” an increased permeability of the intestinal wall for food components. In an already compromised patient this may cause the eczema. Testing for food intolerance (blood test, energetic testing via kinesiology, or polarization filter) and consequent avoidance of certain food components for several months will certainly be beneficial and will support the acupuncture treatment.

Therapy:

- Symptom points: reflex zones of the exacerbated skin areas as Gold Points on the ear of the affected side.
- Histamine Point as Gold Point on the left ear (synonym: Allergy Point 1).
- Other antiallergic and anti-inflammatory points:
 - Cortisone Point as Gold Point on the left ear;
 - ACTH Point as Gold Point on the right ear;
 - Interferon Point as Gold Point on the left ear;
 - Thymus Gland Point as Gold Point on the left ear;
 - Zone-Dominating Point ZA2 as Gold Point on the left ear.
- Local points in the Lung Zone as Gold Points on the right ear (energetic connection to the skin through coupling of the Large Intestine and Lung Meridians).
- Laterality Point as Gold Point on the right side (in left-handed persons on the left side).
- Points that strengthen the constitution:
 - Kidney Point as Gold Point on the right ear (for strengthening Kidney Energy and Hereditary Energy);
 - Point Zero as Gold Point on the right ear (synonym: Celiac Plexus Point).
- Points that may indicate focal disturbances.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Reflex zones of exacerbated skin areas (on the affected side)		○ or	○
Histamine (Allergy) Point	207		○
Cortisone Point	205		○
ACTH Point	199	○	
Interferon Point	209		○
Thymus gland	95		○
ZA2	277		○
Lung	61	○	
Laterality Point	239	○	
Kidney	83	○	
Point Zero	185	○	

Unfortunately, the mercury load from amalgam fillings often has a disease-supporting effect because of its proven inhibition of enzymes. It is always worthwhile to administer zinc and selenium in cases of neurodermatitis.

Respiratory Disorders

Bronchial Asthma

See under Allergic Asthma; the treatment guidelines are the same (with the exclusion of antiallergic points).

Bronchitis, Bronchopulmonary Infection

Therapy:

- Local points in the Bronchi Zone as Gold Points on both ears, if necessary.
- Anti-inflammatory point: Interferon Point as Gold Point on the left ear.
- Mucolytic point: Point ST-40 as Gold Point on the left ear.
- Points in the Lung Zone as Gold Points on the right ear (strengthening of the lung and its meridian).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Bronchi	63	○ or	○
Interferon Point	209		○
Beta-2-Receptor Point	213	○	
Lung	61	○	

Sinusitis

If a patient suffers from chronic or recurrent sinusitis, searching for focal disturbances is essential. They are often the cause of resistance to treatment.

Therapy:

- Local points in the Maxillary Sinus Point or the Frontal Sinus Point as Gold Points on the ear of the affected side, possibly on both ears.
- Anti-inflammatory points:
Interferon Point as Gold Point on the left ear;
Thymus Gland Point as Gold Point on the left ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Maxillary/frontal sinus	111	○ or	○
Interferon Point	209		○
Thymus gland	95		○

Tonsillitis

Therapy:

- Tonsil Point as Gold Point on the ear of the affected side, possibly on both ears.
- Anti-inflammatory points:
Interferon Point as Gold Point on the left ear;
Thymus Gland Point as Gold Point on the left ear.
- Points in the Lung Zone as Gold Points on the right ear (energetic connection to the tonsils: the Tonsil Point corresponds to Point LU-11).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Tonsil Point (LU-11)	81	○ or	○
Interferon Point	209		○
Thymus gland	95		○
Lung	61	○	○

Parotitis

Therapy:

- Parotid Point as Gold Point on the ear of the affected side, usually on both sides.
- Anti-inflammatory points:
Interferon Point as Gold Point on the left ear;
Thymus Gland Point as Gold Point on the left ear.
- Points in the Lung Zone as Gold Points on the right ear (energetic connection of the parotid gland to the Lung Meridian).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Parotid gland	81	○ or	○
Interferon Point	209		○
Thymus gland	95		○
Lung	61	○	

Gastrointestinal Disorders

Gastritis and Peptic Ulcer

Therapy:

- Points in the Stomach Zone as Gold Points on the left ear and in the Duodenum Zone as Gold Points on the right ear.
- It is essential to locate the corresponding points in the Sympathetic Trunk Zone: Nervous Stomach Point as Gold Point on the left ear, or Nervous Duodenum Point as Gold Point on the right ear (use the point finder to decide on the needle metal).
- Points for controlling excessive activity of the parasympathetic nervous system:
 - Superior Mesenteric Plexus Point as Gold Point on the right or left ear;
 - Vagus Nerve Point in the cranial portion of the Zone of Parasympathetic Nuclei of Origin as Gold/Silver Point on the right/left ear (for the parasympathetic supply of stomach and duodenum by the vagus nerve, see p. 168).
- Point Zero (synonym: Celiac Plexus Point) as Gold Point on the right ear.
- Anti-inflammatory point: Interferon Point as Gold Point on the left ear.
- Spleen Point (synonym: Point SP-2) as Gold Point on the left ear (to control concurrent weakness of the Middle Burner, if present).
- Psychotropic points, if indicated:
 - Anger Point as Silver Point on the right ear;
 - Valium Analogue Point as Gold Point on the left ear;
 - Depression Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Stomach (sensory portion)	69		○
Duodenum	69	○	
Nervous stomach	179		○ Always on the left
Nervous duodenum	179	○ Always on the right	
Superior mesenteric plexus	187	○ or	○
Point Zero	185	○	
Interferon Point	209		○
SP-2 (Spleen)	323		○
LR-3 (Anger Point)	331	● Always on the right	
Diazepam Analogue Point	215		○
Depression Point	225	○	

Hepatitis, Hepatopathy

Here, too, auriculotherapy is very successful. An improvement of laboratory data (and of the patient's condition) may be observed from one treatment to the next, especially in the case of undefined hepatopathy with increased levels of transaminases (sometimes discovered by accident), but also hepatopathy concurrent with hepatitis A or B, or non-A, non-B hepatitis.

Therapy:

- Points in the Liver Zone as Gold Point on the right ear.
- The corresponding points in the Sympathetic Trunk Zone; for example, Nervous Liver Point as Silver Point on the right ear.
- Points that may indicate focal disturbances.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Points of the liver	75	○	
Corresponding points of the sympathetic trunk (e.g., LR-3)	173 (331)	●	

Inflammatory and Functional Intestinal Disorders

Irritable Bowel Syndrome, Diarrhea

Following a clinical examination of the causes (and the exclusion of clinical therapeutic options that are not just of a suppressing nature), it certainly makes sense to try body acupuncture or auriculotherapy. One of the most important points is the Interferon Point (synonym: Point SP-4, Master Point for diarrhea).

Therapy:

- Symptom points: points in the Small/Large Intestine Zones as Gold Points on the right or left ear (to reduce the often inflammatory irritation).
- The motor points of the Large Intestine Zone as Silver Points on the back of the ear (to calm the hyperactive muscles).
- Interferon Point as Gold Point on the left ear (synonym: Point SP-4).
- If active, also Superior and Inferior Mesenteric Plexus Points, Hypogastric Plexus Point as Gold Points on the right ear.
- Points in the Sympathetic Trunk Zone that represent the intestine.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom points: Points of the intestine	71	○ or	○
Retro points of the intestine	71	● or	●
Interferon Point	209		○
Superior/inferior mesenteric plexus	187	○	
Hypogastric plexus	189	○	
Points of the sympathetic trunk	173	○ or	○

Obstipation

Obstipation also justifies trying treatment with ear acupuncture. The points listed above (except for the sensory points of the Large Intestine Zone) should be checked for activity.

It is important to remind the patient to drink plenty of water. The needles in the ear will have very little effect without sufficient fluid intake, exercise, and a change in diet to increase the fiber portion.

Crohn's Disease, Ulcerative Colitis

Both are functional intestinal diseases and, as such, can be easily treated with ear acupuncture. It is certainly possible to achieve freedom from symptoms without medication. The above points should be checked for activity.

It is often beneficial to test for food intolerance and completely avoid the corresponding food for a few weeks.

Hemorrhoids

Acupuncture needles will not remove the hemorrhoids, but they can certainly eliminate inflammation, pain, itching, and/or the tendency for bleeding. Especially here, surprising and prompt therapeutic results can be observed.

Therapy:

- Symptom points: Internal Anus Point (synonym: Hemorrhoid Point) and/or External Anus Point as Gold Points on the right ear.
- Anti-inflammatory points: Interferon Point as Gold Point on the left ear; PGE1 Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Symptom points: Internal anus (hemorrhoid) and/or external anus	75	○	
Interferon Point	209		○
PGE1 Point	261	○	

Cardiovascular Diseases

Arterial Hypertension

Ear acupuncture allows us to gain access to the body's important center of homeostasis, the hypothalamus. For this purpose, the active Hypothalamus Point in the inner wall of the antitragus must be located as precisely as possible; there may be more than one Gold Point. Auriculotherapy can achieve surprising results—even normal blood pressure without medications. If patients can then be persuaded to change their diet according to the latest scientific research to include predominantly vegetables, fish, and unsaturated oils (no meat, reduction in animal protein), success will be more certain and, most importantly, will persist long after completion of the acupuncture treatment.

Initially, drug therapy will be continued; it should then be reduced in consultation with a specialist.

Therapy:

- Hypothalamus Point as Gold Point on the right ear.
- Renin/Angiotensin Point as Silver Point on the right ear.
- Barbiturate Analogue Point as Gold Point on the left ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Hypothalamus	133	●	
Renin/Angiotensin Point	211	●	
Barbiturate Analogue Point	215		○

Arterial Hypotension

In my experience, treatment of hypotension is less successful than that of hypertension—unless the patient decides to add regular physical exercise.

Therapy:

- Hypothalamus Point as Gold Point on the right ear (stimulation of homeostasis).
- Renin/Angiotensin Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Hypothalamus	133	●	
Renin/Angiotensin Point	211	○	

Cardiac Arrhythmia

To my own surprise, irregular heart beat is an ideal indication for auriculotherapy—in particular, when the arrhythmia is troublesome but not life-threatening, such as frequent ventricular or extraventricular premature systoles.

It is essential to continue any prescribed antiarrhythmic drug therapy and then reduce the dosage after a few sessions in consultation with a specialist. Quite often, patients do have irregularities despite the medication so that the therapeutic success of acupuncture can be observed at the very beginning without any change in medication.

The patient should be asked about blockages in the thoracic spine region (pain upon rotating the trunk). Because of the autonomic synapsing of the sympathetic trunk or the postganglionic supply, such pain is a common trigger for cardiac arrhythmia (but is ignored by most physicians). One of my patients repeatedly ended up in the intensive care unit with severe arrhythmia precisely because of this trigger. When telling the attending physicians about his observation that thoracic blockages acted as triggers, they only smiled at him. When he finally arrived at my practice, I only had to treat his tendency for blockages, which was due to a focal disturbance caused by a scar crossing a meridian. The arrhythmia did not return in the intervening five years.

Therapy:

- The motor portion of the Heart Point as Gold Point on the back of the left ear (independent of the handedness).
- Cardiac Plexus Point as Gold Point on the left ear (important for homeostasis).
- Beta-1-Receptor Point (Beta-Blocker Point) according to Bahr as Gold Point on the right ear.
- Magnesium Point (Depression Point) as Gold Point on the right ear.
- Points in the Sympathetic Trunk Zone at the level of Points C1–T4 as Gold Points on the left ear (see also sympathetic nerve supply of the heart).
- Perhaps also the Vagus Nerve Point as Gold Point on the left ear (the vagus nerve is the most important parasympathetic nerve).
- Points in the Thoracic Spine Zone as Gold/Silver Points on both ears.
- Points that may indicate focal disturbances should be located and treated.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Retro heart	59		○
Cardiac plexus	183		○
Beta-1-Receptor Point	211	○	
Depression Point (Magnesium Point)	225	○	
Active points of the sympathetic trunk (C1–T4)	173		○
Vagus nerve	161		○
Active points of the thoracic spine	15	● or	●

Angina Pectoris

Here, acupuncture is only used together with drug therapy. From the projection of the Heart Meridian onto the ear, we know that the motor portion of the Heart Point on the ear corresponds to Point HT-3, while the sensory portion corresponds to Point HT-4 of body acupuncture. Patients should therefore be advised to use this knowledge and massage the two points between individual treatments (Point HT-3 in the medial elbow, Point HT-4 at the distal medial forearm).

Therapy:

- The sensory portion of the Heart Point as Gold Point on the left ear.
- The motor portion of the Heart Point also as Gold Point on the left ear (do not use a silver needle to avoid weakening the heart).
- Cardiac Plexus Point as Gold Point on the left ear.
- Points in the Sympathetic Trunk Zone at the level of Points C1–T4 as Gold Points on the left ear (see also sympathetic nerve supply of the heart).
- Psychotropic points:
Anxiety Point as Silver Point on the right ear;
Master Omega Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Heart	59		○
Retro heart	59		○
Active points of the sympathetic trunk (C1–T4)	173		○
Anxiety Point	225	●	
Master Omega Point	221	○	

Disorders of the Urogenital System

Irritable Bladder, Recurrent Urinary Tract Infections

Many women with an irritable bladder undergo hysterectomy on the suspicion that a slightly prolapsed uterus is the mechanical cause for the steady urge to pass urine. I have several such patients visiting my practice: in each case surgery did not lead to the desired improvement because there was already another cause for the irritated bladder, usually a focal disturbance. For example, defective or devitalized frontal teeth have an effect primarily and specifically on the urinary bladder. Dental treatment can perform miracles in these cases.

Therapy:

- Sensory portion of the Urinary Bladder Point as Gold Point on the right ear.
- Motor portion of the Urinary Bladder Point as Silver Point on the back of the right ear (forceps method, especially effective in cases of irritable bladder).
- Inferior Mesenteric Plexus Point as Gold Point on the right ear.
- Points in the Sympathetic Trunk Zone at the level of Point L1/L2 as Gold Points on the right ear.
- Anti-inflammatory points: Interferon Point as Gold Point on the left ear; Thymus Gland Point as Gold Point on the left ear.
- Valium Analogue Point as Gold Point on the left ear (this important point is found to be active more often in patients with bladder problems than in the average population).
- Points that may indicate focal disturbances.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Urinary bladder (sensory portion)	85	○	
Urinary bladder (motor portion)	85	●	
Inferior mesenteric plexus	187	○	
Points of the sympathetic trunk (L1/L2)	173	○	
Interferon Point	209		○
Thymus gland	95		○
Diazepam Analogue Point	215		○

Even conventional medicine now **acknowledges that administering preparations** containing *Escherichia coli* effectively supports the patient's immune defenses during the common cold as well as during recurrent cystitis. To support this therapy, the acupuncturist should also be well versed in general intestinal cleansing; the principles are simple and highly effective.

Enuresis

Here, my experience with the sole use of acupuncture is not particularly good. Auriculotherapy is certainly worthwhile but, if possible, in combination with other methods (homeopathy, Bach flower remedies, psychotherapy, counseling of the parents).

Therapy:

- The motor portion of the Urinary Bladder Point as Gold Point on the back of the right ear.
- Laterality Point as Gold Point on the right side.
- Psychotropic points:
 - Anxiety Point as Silver Point on the right ear;
 - Depression Point as Gold Point on the right ear;
 - Master Omega Point as Gold Point on the right ear;
 - Valium Analogue Point as Gold Point on the left ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Retro urinary bladder	85	○	
Laterality Point	237	○	
Point Zero	185	○	
Anxiety Point	225	●	
Depression Point	225	○	
Master Omega Point	221	○	
Diazepam Analogue Point	215		○

Impotence

As with all pathological processes in the region of the pelvis, regulation of the sympathetic/parasympathetic supply also makes sense here (sympathetic trunk at the level of L1/L2; inferior mesenteric ganglion, see p. 167; and caudal part of the parasympathetic nervous system, see p. 169). In any case, the patient should be advised that regular physical activity (stamina sports) has been proved to improve erections through increased blood supply to the genitals.

Therapy:

- Penis Point as Gold Point on the right ear (corresponds to the Frustration Point, as Gold Point on the left ear, which may also be needed).
- Urethra Point as Gold Point on the right ear.
- Points in the Sympathetic Trunk Zone at the level of Point L1/L2 as Gold Points on the right ear.
- Inferior Mesenteric Plexus Point as Gold Point on the right ear.
- Psychotropic points:
 - Depression Point as Gold Point on the right ear;
 - Master Omega Point as Gold Point on the right ear;
 - Psychotherapy Point (synonym: Bourdiol Point) as Silver Point on the right ear;
 - Anxiety Point as Silver Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Penis	91	○	
Spongy body (urethra)	87	○	
Points of the sympathetic trunk (L1/L2 level)	173	○	
Inferior mesenteric plexus	187	○	
Depression Point	225	○	
Master Omega Point	221	○	
Bourdiol Point	233	●	
Anxiety Point	225	●	
Omega Point I	223	○	
Organ point of the liver (LR-8)	75	○ Always on the right	

Hormonal Disorders

Premenstrual Syndrome/Menstrual Disorders

Since these disorders are purely functional, they are easy to treat with ear acupuncture (as long as organic diseases have been excluded). Auriculotherapy leads to the relief of symptoms in most cases.

Premenstrual syndrome (PMS) is associated with progesterone deficiency; estrogen levels are usually normal. (In laboratory tests, the progesterone level is supposed to be 10% of the estrogen level.) A gold needle in the Progesterone Point is always in order, while a silver needle is more appropriate for achieving hormonal balance. The Uterus Point is energetically weak; use a gold needle to subdue the pain, preferably in combination with a silver needle (forceps method according to Bahr). The gold needle is inserted on the inside of the ascending helix, and the silver needle into the corresponding point on the outside of the helix. (The outside of the helix represents the back of the ear; during embryogenesis, the margin of the auricle folds to form the helix.)

Therapy:

- Examine the three hormone points for activity. The Estrogen Point often requires a silver needle, while the Progesterone Point needs a gold needle. Check whether the Gonadotropin Point needs a gold needle or a silver needle.
- Uterus Point: as Gold Point on the right ear. If there are severe cramps, use a silver needle instead (particularly with severe cramps during menstruation). Make sure to determine the needle metal individually because the energetically weak Uterus Point may require a gold needle even in this case.
- Spasmodic points: Point SI-3 (synonym: Retro-Celiac Plexus Point) as Gold Point on the right ear, LR-3 as Silver Point always on the right ear because of the anatomical position of the liver (synonym: Anger Point).
- Psychotropic points: Master Omega Point as Gold Point on the right ear, possibly Depression Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Estrogen Point	195	●	
Progesterone Point	197	○	
Gonadotropin Point	197	○	
Uterus	87	○/●	
SI-3 (Retro-Celiac Plexus Point)	325	○	
LR-3 (Anger Point)	331	● Always on the right	
Master Omega Point	221	○	
Depression Point	225	○	

Menopausal Symptoms

Therapy:

- Points on the Auxiliary Line for Hormone Points:
Estrogen Point usually as Gold Point on the right ear;
Progesterone Point as Silver Point on the right ear;
Gonadotropin Point as Gold Point on the right ear.
- Points that stabilize the autonomic nervous system:
Valium Analogue Point as Gold Point on the left ear;
Spleen Point as Gold Point on the left ear;
Hypothalamus Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Estrogen Point	195	○	
Progesterone Point	197	●	
Gonadotropin Point	197	○	
Diazepam Analogue Point	215		○
Spleen	79		○
Hypothalamus	133	○	

Childlessness

Here, therapy will certainly not be completed in three to four sessions as in the case with many other indications. Patience is needed; usually 10 sessions are required.

Unfortunately, there is increasing evidence from university studies that mercury from amalgam fillings can reduce fertility in women. Patients are therefore advised to consider a dental clean-up.

Therapy:

- Points on the Auxiliary Line for Hormone Points, as previously described. (Use the point finder to decide on the needle metal. The Estrogen Point is usually treated with a gold needle on the right ear, which is followed by a permanent needle.)
- Points that may indicate focal disturbances (especially important since foci might disturb homeostasis).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Hormone Points	291	○	
Prolactin Point	201	●	

Hyperemesis Gravidarum

Most cases of excessive vomiting during pregnancy can be easily treated via the vomiting center in the brain.

Therapy:

- Vomiting Point as Gold Point on the right ear.
- Laterality Point as Gold Point on the right side.
- Point Zero as Gold Point on the right ear and as Silver Point on the left ear.
- Motor point of the stomach, as Gold Point on back of the left ear.
- Barbiturate Analogue Point (synonym: Point ST-36) as Gold Point on the left ear.
- Perhaps also: Gall Bladder Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Vomiting Point	143	○	
Laterality Point	237	○	
Gall bladder	77	○	
Point Zero	185	○	●
Retro stomach	69		○ Always on the left
Barbiturate Analogue Point	215		○

Autonomic Symptoms

Vertigo

The false sensation that the environment or one's own body is rotating may have many causes (e.g., inner ear disorders, cervical spine or TMJ problems, malocclusion of the teeth, eye problems, problems with circulation, arrhythmia and other internal diseases, disorders of the cerebellum, brain tumors). Scars acting as focal disturbances can also cause vertigo (e.g., a scar in the area of the Gall Bladder or Urinary Bladder Meridians, since both meridians supply the entire head).

Ear acupuncture should only be started after a specialist's diagnosis and, in particular, after tumors have been excluded (brain, brain stem, inner ear). Vertigo triggered by problems in the cervical spine is especially accessible to auriculotherapy.

Therapy:

- Points in the Cervical Spine Zone as Gold Points on both ears.
- Points in the cervical portion of the Sympathetic Trunk Zone.
- Points on the antitragus: all points of the head portion of the Gall Bladder Meridian project here.
- Perhaps also: Cerebellum Point and Brain Stem Point as Gold Points on the right ear.
- Perhaps also: Gall Bladder Point as Gold Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Points of the cervical spine	15	○ or	○
Points of the cervical sympathetic trunk	173	○ or	○
Points on the antitragus (Gall Bladder Meridian)	331	○	
Cerebellum	141	○	
Brainstem	143	○	
Gall bladder	77	○	

Sleep Disorders

Excellent therapeutic results are possible here. The difficulty in staying asleep is sometimes triggered by a weakness in the liver and its meridian (tonification period of the Liver Meridian: between 1 and 3 a.m.). Patients are advised to take the strain off the liver (no alcohol) and to support the liver, for example, with preparations of milk thistle (*Silybum marianum*).

If there are focal disturbances, the physiological balance is often controlled by their treatment. Patients who undergo acupuncture treatment for other complaints (including focus therapy) frequently mention in passing that they no longer have sleeping problems. Some scars specifically disturb the sleep rhythm because they affect meridians with tonification periods during the night, for example, scars crossing the Liver Meridian or Gall Bladder Meridian.

Therapy:

- Hypothalamus Point as Gold Point on the right ear.
- Points in the Liver Zone as Gold Points on the right ear.
- Nervous Liver Point as Silver Point on the right ear.
- Sedating points:
Barbiturate Analogue Point as Gold Point on the left ear;
Valium Analogue Point as Gold Point on the left ear.
- Pineal Gland Point (synonym: Point BL-62) (effect on the rhythm of sleeping and waking).
- Laterality Point as Gold Point on the right side.
- Points that may indicate focal disturbances.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Hypothalamus	133	○	
Liver	75	○	
Nervous Liver Point	219	●	
Barbiturate Analogue Point	215		○
Diazepam Analogue Point	215		○
Epiphysis (BL-62)	251		○
Laterality Point	237	○	

Tinnitus, Sudden Deafness, Ménière's Disease

If new cases of tinnitus, or sudden deafness with tinnitus, are treated within the first few days of occurring, freedom from symptoms is achieved quickly in most cases. When the symptoms are older, particularly when tinnitus has persisted for years, my own experience with auriculotherapy is not as good as compared to other indications (healing or alleviation only in about 30% of cases).

Therapy:

- Points on the Auditory Line (reflex zone of the auditory radiation of the temporal lobe) usually as Silver Points on the ear of the affected side (several needles in a row; use the point finder to decide on the needle metal).
- Ear Point as Gold Point on the ear of the affected side.
- Vestibulocochlear Nerve Point as Gold Point on the ear of the affected side.
- Superior Cervical Ganglion Point usually as Gold Point on the ear of the affected side (use the point finder to decide on the needle metal). Possibly other active points in the Sympathetic Trunk Zone.
- Points in the Cervical Spine Zone as Gold Points on both ears.
- Psychotropic Points:
 - Valium Analogue Point as Silver Point on the right ear;
 - Master Omega Point as Gold Point on the right ear;
 - Psychotherapy Point (synonym: Bourdiol Point) as Silver Point on the right ear.
- Points that may indicate focal disturbances, especially scars in the region of the Triple Warmer Meridian, which supplies the ear (e.g., scars resulting from face lifting or ear surgery).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Auditory Line	127	●	
Ear	111	○ or	○
Vestibulocochlear nerve	157	○ or	○
Superior cervical ganglion	175	○ or	○
Point of the cervical spine	15	○ or	○
Diazepam Analogue Point	215		○
Master Omega Point	221	○	
Bourdiol Point	233	●	

In the case of recent tinnitus, the therapy should be performed at least once a week, preferably more often.

Mental Health Disorders

Anxiety Syndrome

It should be carefully considered whether or not ear acupuncture should be used here. However, concurrent with drug therapy that may be indicated, it could be worthwhile. In the case of sudden and explained anxiety attacks, ear acupuncture is the method of choice (e.g., examination phobia).

Therapy:

- Symptom point: Anxiety Point as Silver Point on the right ear.
- Spleen Point (see Auxiliary Line) as Gold Point on the left ear; points in the Lung Zone as Gold Points on the right ear (energetic connection between the Spleen and Lung Meridians).
- Hypothalamus Point as Gold Point on the right ear (for stabilization of the autonomic nervous system).
- Other psychotropic points:
Barbiturate Analogue Point as Gold Point on the left ear;
Valium Analogue Point as Gold Point on the left ear;
Master Omega Point as Gold Point on the right ear.
- Laterality Point as Gold Point on the right side (disturbed laterality due to acute stress). It is essential to determine handedness accurately because a needle inserted on the wrong side may weaken laterality and, hence, further disturb coordination of the two hemispheres.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Anxiety Point	225		●
Spleen	77		○
Lung	61	○	
Hypothalamus	133	○	
Barbiturate Analogue Point	215		○
Diazepam Analogue Point	215		○
Master Omega Point	221	○	
Laterality Point	237	○	

Depression

It should be carefully considered whether or not ear acupuncture should be used here. However, concurrent with drug therapy that may be indicated, it could be worthwhile.

Therapy:

- Symptom point: Depression Point as Gold Point on the right ear.
- Points in the Lung Zone as Gold Points on the right ear (energetic connection between the Lung Meridian and Sadness).
- Other psychotropic points:
 Barbiturate Analogue Point as Gold Point on the left ear;
 Valium Analogue Point as Gold Point on the left ear;
 Master Omega Point as Gold Point on the right ear;
 Psychotherapy Point (synonym: Bourdiol Point) as Silver Point on the right ear.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Depression Point	225	○	
Lung	61	○	
Barbiturate Analogue Point	215		○
Diazepam Analogue Point	215		○
Master Omega Point	221	○	
Bourdiol Point	223	●	

Lack of Concentration

A common cause of lack of concentration is a disturbed laterality, often triggered by stress, lack of sleep, left-handedness re-educated to use the right hand, or focal disturbances. It is essential to assess the life-style in addition to performing auriculotherapy (primarily the patient's eating habits; white sugar, artificial food colors and food preservatives can contribute to the symptom, especially in children).

Therapy:

- Laterality Point as Gold Point on the right side.
- Point Zero as Gold Point on the right ear (synonym: Celiac Plexus Point).
- Points in the Frontal Lobe Zone.

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Laterality Point	237	○	
Point Zero	185	○	
Active points of the frontal lobe	123	○	

Addictions

Obesity, Eating Disorders

Most obese persons are already experts in counting calories and have undergone different diets. Most diets involve restrictions that cannot be maintained, or the diet later causes a yo-yo effect of losing and gaining weight through various mechanisms. Thanks to ear acupuncture we know that most obese persons exhibit a disturbed laterality (similar to all addicted patients). They try to stabilize themselves by eating (or smoking); the Laterality Point disappears for a short while after food intake (or a cigarette).

The patient can easily be stabilized by needling the Laterality Point so that the mechanism mentioned above no longer works. Treatment of other ear points will further stabilize the autonomic nervous system and emotional state.

Of course, all this does not replace a detailed discussion about other potential reasons for obesity, such as the mechanisms of compensation or rewards, reaction to stress, or simply intolerance to certain food, in the Chinese sense of energy. (Stressing the spleen with too much dairy products, sugar, or cold food/drinks may result, for example, in chronic abnormal distribution of fat and water.) It is important to inform the patient that acupuncture will reduce the appetite and that they should eat only as needed and not out of habit.

Gold needles may be initially inserted into the active points, or permanent needles are used right away (up to six needles are possible without problems). The patient needs to be informed in detail about permanent needles (see B. Strittmatter, *Overcoming Blockages to Healing—A Manual for Health Care Professionals*, Thieme Publishers, Stuttgart–New York, 2004)

Therapy:

- Most important point: Laterality Point as Gold Point on the right side (caution: it is essential first to determine the handedness; in left-handed persons on the left side).
- Hypothalamus Point as Gold Point on the right ear (locating and treating this point is essential: the hypothalamus is the center of food intake control).
- Psychotropic points:
 - Depression Point as Gold Point on the right ear;
 - Master Omega Point as Gold Point on the right ear;
 - Psychotherapy Point as Silver Point on the right ear (synonym: Bourdiol Point);
 - Aggression Point as Silver Point on the right ear;
 - Frustration Point as Gold Point on the left ear (frequently active).
- Point Zero as Gold Point on the right ear (synonym: Celiac Plexus Point).
- Perhaps also: Throat Point as Gold Point on the right ear (synonym: Pharynx Point).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Laterality Point	237	○	
Hypothalamus	133	○	
Depression Point	225	○	
Master Omega Point	221	○	
Bourdiol Point	233	●	
Aggression Point	231	●	
Frustration Point	233		○
Point Zero	185	○	
Throat	65	○	

Anorexia Nervosa

Acupuncture can provide an excellent support parallel to therapy by a specialist. The combination of Aggression Point, Bridge Point and Master Omega Point seems to be especially effective.

Therapy:

- Points to be treated with permanent needles on the dominant ear:
Aggression Point;
Bridge Point;
Master Omega Point.
- Laterality Point as Gold Point on the right side (caution: it is essential first to determine the handedness; in left-handed persons on the left side).
- Hypothalamus Point as Gold Point on the right ear.
- Other psychotropic points:
Depression Point as Gold Point on the right ear;
Psychotherapy Point as Silver Point on the right ear (synonym: Bourdiol Point).
Frustration Point as Gold Point on the left ear (frequently active).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Aggression Point	231	▲	
Bridging Point	231	△	
Master Omega Point	221	△	
Laterality Point	237	○	
Hypothalamus	133	○	
Depression Point	225	○	
Master Omega Point	221	○	
Bourdiol Point	233	●	
Frustration Point	233		○

Nicotine Addiction

In my practice, it proved best to identify prior to treatment the most sensitive periods and situations when the patient will crave for a cigarette (reward after work, for relaxation during stress at work, self-esteem in social settings, after meals). Patients are advised to think of other activities (substitute habits) for such situations before they undergo auriculotherapy.

Thanks to ear acupuncture we know that the great majority of smokers (like most obese persons and all other addicted patients) exhibit a severely disturbed laterality. They try to stabilize themselves by smoking (eating); the Laterality Point disappears for a short time after smoking a cigarette. Hence, one of the most important points of the treatment (apart from the Nicotine Analogue Point) will be the Laterality Point as Gold Point on the dominant side.

Gold needles may be initially inserted into the active points, or permanent needles are used right away (up to six needles are possible without problems). The patient needs to be informed in detail about permanent needles (see B. Strittmatter, *Overcoming Blockages to Healing—A Manual for Health Care Professionals*, Thieme Publishers, Stuttgart–New York, 2004).

Therapy:

(Use only the active points.)

- Most important point: Laterality Point as Gold Point on the right side (caution: it is essential first to determine the handedness; in left-handed persons on the left side).
- Nicotine Analogue Point as Silver Point on the right ear (insert permanent needle in the Silver Point).
- Psychotropic points:
 - Depression Point as Gold Point on the right ear;
 - Master Omega Point as Gold Point on the right ear;
 - Aggression Point as Silver Point on the right ear;
 - Frustration Point as Gold Point on the left ear (frequently active).
- Point Zero as Gold Point on the right ear (synonym: Celiac Plexus Point).
- Points in the Lung Zone as Gold Points on the right ear.
- Hypothalamus Point as Gold Point on the right ear (stabilization of the autonomic nervous system).

Acupuncture Point	Page	Dominant Ear	Non-dominant Ear
Laterality Point	237	○	
Nicotine Analogue Point	227	●	
Depression Point	225	○	
Master Omega Point	221	○	
Aggression Point	231	●	
Frustration Point	233		○
Point Zero	185	○	
Lung	61	○	
Hypothalamus	133	○	

Patient Information Leaflet: Permanent Needles

Permanent needles are sterile disposable needles. Your doctor will insert them into some of the points found on your ears after individual inspection. Prior to needling, both external ears (auricles) will be carefully examined because they bear reflex points of the entire body.

Once the short permanent needles have been positioned correctly into the auricle, they have to be stimulated in order to be fully effective. This is done with the small dipole magnet contained at the end of the plastic case of the permanent needle. The small black magnet is easily visible. The stimulation is best achieved by holding the magnet very close to the needle (or gently touching it) and then turning the plastic case quickly back and forth between thumb and index fingers. This acts like a small generator—like a bicycle dynamo—and produces a weak electric current that enhances the effect of the needle.

The stimulation may be repeated several times during the day without reservation, each time for about 15–20 seconds per needle. In addition, you may use this stimulation every time when symptoms occur for which you have received the permanent needle (such as pain or the urge to smoke). The faster the magnet is turned, the more effective the stimulation will be.

The stimulating effect of the magnet penetrates the adhesive patch that covers the needle. The protective patch therefore remains in place when using the magnetic needle stimulator.

If the protective patch comes off or gets dirty, you should replace it. You can make a new patch by cutting out a piece of hypoallergenic adhesive tape (available in any pharmacy or drug store).

The permanent needles project only about 1 mm above the surface of the skin and thus do not show up. Nevertheless, you should avoid washing your ears in the areas where the needles have been inserted. You may temporarily use eau de toilette or aftershave to clean the area, if necessary.

-  The needle should not cause any pain or inflammation.
-  The skin around the needle should not be red.

If the skin does become inflamed (reddened), the needles must be removed with forceps. After removal, the area should be cleaned with ethanol or wound disinfectant. If in doubt, ask your doctor to inspect the site.

You should keep the permanent needles in the ear for about 1 week. As long as the skin does not reject the needles and the skin around the needles is not reddened, you may keep the needles longer than a week. Usually, it is not necessary to see your doctor for removal of the needles. You can easily remove the needles yourself with forceps.



Ear Topography

Helix:

The helix is the brim that forms the superior and posterior margin of the auricle. It ascends from the concha as helical root, arches around the apex of the ear to form the helical body, and merges into the lobule as helical tail.

Root of helix:

Point Zero is located at the notch in the cartilage of the helical root.

Ascending helix:

The reflex zone of the sex organs lies below the ascending helix.

Descending helix:

Reflex zone of the spinal cord (between Darwin's tubercle and helical tail).

Tail of helix:

Reflex zone of the trigeminal nerve.

Darwin's tubercle:

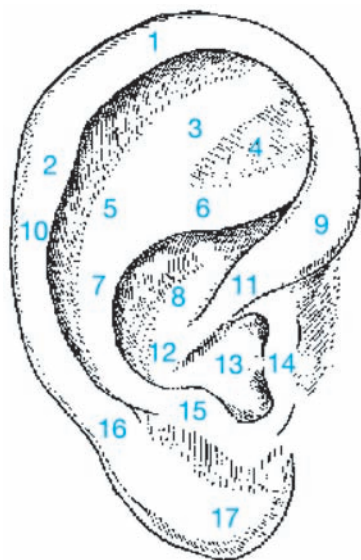
Reflex zone of the caudal end of the spinal cord.

Antihelix:

The semicircular ridge between scapha and concha. Reflex zone of the vertebral column.

Superior antihelical crus:

The ridge along the superior side of the triangular fossa. It forms the border between the reflex zones of upper limb (hand) and lower limb.



- 1 Helix
- 2 Darwin's tubercle
- 3 Superior antihelical crus
- 4 Triangular fossa
- 5 Scapha
- 6 Inferior antihelical crus
- 7 Antihelix
- 8 Superior concha
- 9 Ascending helix
- 10 Descending helix
- 11 Root of helix
- 12 Concha
- 13 Inferior concha
- 14 Tragus
- 15 Antitragus
- 16 Tail of helix
- 17 Lobule

Inferior antihelical crus:

The ridge along the inferior side of the triangular fossa. Reflex zones of lumbar spine and lower thoracic spine. The reflex zone of the hip lies where superior and inferior antihelical crura intersect.

Tragus:

The cartilaginous protrusion between ascending helix and lobule. Reflex zone of many psychotropic reflex points.

Antitragus:

The cartilaginous projection opposite the tragus, bordering on the concha and continuing as antihelix. Reflex zone of the bony skull.

Intertragic notch:

The incision between tragus and antitragus. Reflex zone of the hypophysis.

Triangular fossa:

The cavity between the two crura of the antihelix. Reflex zone of the lower limb.

Lobule:

The inferior cartilage-free part of the auricle. Reflex zone of all head structures and the brain.

Postantitragal fossa:

Incision between antitragus and antihelix. Reflex point of the atlanto-occipital joint (C0/C1).

Scapha:

The long curved depression between helix and antihelix; it merges caudally into the lobule. Reflex zone of the upper limb.

Concha:

The cavity between tragus and antihelix; it is subdivided into an upper and a lower part by the root of helix.

Superior concha:

Reflex zone of the abdominal organs.

Inferior concha:

Reflex zone of the thoracic organs except the heart.

Conchal wall:

The hidden part of the concha that rises from the floor of the concha toward the antihelix and antitragus. The reflex zones of endocrine glands and sympathetic trunk lie in the antihelical part of the wall.

Projection Zones, Points, and Meridians on the Ear

Page references in **bold** refer to the major coverage of topics

A

Abducens Nerve Point 149, **154**, 155
 Accessory Nerve Point 149, **162**, 163
 Achilles Tendon Point 39, **40**, 41
 Acromioclavicular Joint **48**, 49
 ACTH Point **136**, 137, 197, **198**, 199, 201, 291, **306**, 307, 308, 309, 330
 Adenohypophysis Zone **134**, 135
 Adrenal Gland Point 93, **96**, 97, 195, **204**, 205, 269, 270, 328
 Adrenaline Point 328
 Aggression Point 221, **230**, 231, 269, 270, 271, **288**, 289, **306**, 307, 309, 330
 Allergy Point 1 **206**, 207, **258**, 259, 269, 270, **316**, 317
 Allergy Point 2 259, **260**, 261, **314**, 315
 Aluminium Indicator Point 267
 Amygdaloid Body Zone 139
 Anger Point 178, 219, 221, **218**, **226**, 227, 269, 270, 271, 330
 Angiotensin Point **210**, 211
 Ankle Joint Point **34**, 35, 39, **310**, 311
 Anus Zone **74**, 75
 Anxiety Point 221, **224**, 225, 269, 270, 271, 320
 Appendix Zone **72**, 73
 Atlanto-occipital Joint Point **16**, 17
 Auditory Line **126**, 127
 Autonomic Nervous System 164–165
 Auxiliary Line 82, 92, 285–317

B

Barbiturate Analogue Point **214**, 215, 269, 270, **286**, 287, 322
 Beta-1-Receptor Point **210**, 211, 213, 269, 270, **298**, 299, 322
 Beta-2-Receptor Point 211, **212**, 213, 298, 299, 322
 Beta-Blocker Point **210**, 211, 322

Beta-Mimetic Point **212**, 213, 322
 Big Toe Point 38, 42, 43
 Bladder Meridian **326**, 327
 Bladder Point see Urinary Bladder Point
 Blood Vessel Zones 56, **58**, 59
 Bony Skull Zone **98**, 99, 118
 Brain Stem Zone **142**, 143
 Brain Zone 118
 Bridging Point **230**, 231
 Bromazepam Analogue Point **220**, 221
 Bronchi Zone 61, **62**, 63
 Bronchopulmonary Plexus Point 181, **182**, 183
 Buttock Point **36**, 37

C

Cadmium Indicator Point 267
 Caffeine Analogue Point **216**, 217, **286**, 287, 322
 Cardiac Plexus Point 181, **182**, 183, 324
 Cardinal Points 242–251
 see also specific points
 Carpometacarpal Joint of Thumb Zone **54**, 55
 Celiac Plexus Point 181, **184**, 185, 318, 324, 332
 Central Nervous System 118–163
 Cerebellum Zone **140**, 145
 Cerebral Lobes Zone **120**, 121
 Cervical Spine Zone 12, 13, **14**, 15, 25
 Cingulate Gyrus Zone 139
 Clavicle Zone **26**, 27, 45
 Clitoris Point 83, **90**, 91
 Coccyx Point **20**, 21
 Colon Zone **70**, 71, 73
 Commissural Fibers Zone 118
 Conception Vessel (*Ren Mai*) **332**, 333
 Control Zone of the Deep Tissue Layer **264**, 265

Control Zone of the Intermediate Tissue Layer **264**, **265**
 Control Zone of the Superficial Tissue Layer **265**
 Copper Indicator Point **267**
 Corpus Callosum Zone **128**, **129**
 Cortex Zone **120**, **121**
 Cortisone Point **93**, **96**, **97**, **195**, **204**, **205**, **269**, **270**, **290**, **291**, **328**
 Cranial Nerves **148–163**
 Cyclophosphamide Analogue Point **259**, **260**, **261**, **314**, **315**

D

Depression Point **100**, **221**, **224**, **225**, **294**, **295**, **324**
 Diazepam Analogue Point **193**, **214**, **215**, **221**, **229**, **242**, **243**, **246**, **247**, **268**, **269**, **270**, **292**, **293**, **300**, **301**, **303**, **304**, **305**
 Digitalis Analogue Point **324**
 Duodenum Zone **57**, **67**, **68**, **69**

E

Ear Point **99**, **110**, **111**
 Ear terraces **6**, **7**
 Ectoderm **8**, **9**
 Elbow Joint Point **44**, **45**, **46**, **47**, **48**, **49**, **51**, **53**, **312**, **313**
 Endocrine Pancreas Point **76**, **77**, **93**, **96**, **97**, **195**, **204**, **205**, **269**, **270**
 Endocrine Parathyroid Gland Point **93**, **94**, **95**, **200**, **201**, **328**
 Endocrine System **92–97**
 Endocrine Thyroid Gland Point **92**, **93**, **95**, **298**, **299**, **328**
 Endoderm **8**, **9**
 Endoxan Point **253**, **260**, **261**
 Episiotomy **30**
 Episiotomic scar
 see perineum
 Erector Spinae Muscle Zone **27**
 Esophagus Zone **57**, **66**, **67**, **69**
 Estrogen Point **194**, **195**, **290**, **291**, **322**
 Ethmoidal Sinuses **110**, **111**
 External Anus Point **74**, **75**
 Eye Point **99**, **108**, **109**, **330**

F

Facial Muscles Zone **112**, **113**
 Facial Nerve Point **149**, **156**, **157**
 Fallopian Tube Point **83**, **90**, **91**
 Finger Zones **50**, **52**, **53**
 First Rib Point **296**, **297**
 Focus Indicator Points **258–263**
 Foot Zone **38**, **39**
 Forearm Zone **52**, **53**
 Forehead Point **99**
 Frontal Bone Zone **98**, **99**
 Frontal Lobe Zone **121**, **122**, **123**
 Frontal Sinus Zone **99**, **110**, **111**, **304**, **305**
 Frustration Point **221**, **232**, **233**, **269**, **270**, **302**, **303**, **304**, **305**
 FSH/LH Point **137**
 Functional Points **192–193**

G

Gall Bladder Meridian **330**, **331**
 Gall Bladder Point **57**, **67**, **76**, **77**, **330**
 Gastrointestinal System **66–77**
 Genital Zone **56**
 Germ layers **8**, **9**
 Glossopharyngeal Nerve Point **149**, **158**, **159**
 Gold Indicator Point **267**
 Gold Points **192–193**, **271**
 Gonadotropin Point **137**, **196**, **197**, **199**, **201**, **290**, **291**, **308**, **309**
 Governor Vessel (*Du Mai*) **332**, **333**
 Greater Trochanter Zone **32**

H

Head **98–116**, **118**
 Heart Meridian **324**, **325**
 Heart Point **56**, **57**, **58**, **59**, **318**, **324**
 Heel Point **38**, **39**, **40**, **41**
 Hemorrhoid Point **74**, **75**
 Hip Joint Point **29**, **32**, **33**, **45**, **310**, **311**
 Hippocampus Zone **139**
 Histamine Point **206**, **207**, **258**, **259**, **269**, **270**, **316**, **317**, **326**
 Hormone Points **194–205**, **290–291**
 Hypogastric Plexus Point **181**, **188**, **189**

Hypoglossal Nerve Point 149, **162**, 163
Hypothalamus Point 118, **132**, 133, 330

I

Indicator Points for Trace Elements **266–268**, 322, 324, 332
Indicator Points for Vitamins **266**, 267, 322, 330, 332
Inferior Cervical Ganglion Point 173, 175, **176**, 177
Inferior Mesenteric Plexus Point 181, **186**, 187
Inguinal Region Zone 32
Insulin Point **76**, 77, 93, **96**, 97, 195, **204**, 205, 269, 270, 328
Interferon Point **208**, 209, 242, 243, **248**, 249, 269, 270, **292**, 293, **304**, 305, 322
Internal Anus Point **74**, 75
Internal Organs 56–97
 see also specific organs
Intervertebral Disk Zone 13
Intestine Zone 57
 Large Intestine 67, **70**, 71, 73
 Small Intestine 67, **70**, 71
Inverted Embryo **6**, 7
Iron Indicator Point 267

J

Jejunum/Ileum Zone **70**, 71

K

Kidney Meridian **326**, 327
Kidney Point 56, 57, **82**, 83, 211, **312**, 313, 326
Knee Joint Point **34**, 35, **310**, 311, 313

L

Large Intestine Meridian **320**, 321
Large Intestine Point 67, **70**, 71, 73, 320
Lateral Pterygoid Muscle Point 100, 113
Laterality Point **236**, 237, 253, 259, **262**, 263, **292**, 293, **314**, 315
Lead Indicator Point 267

Leg Zone 32–43
Limbic System Zone **138**, 139
Lip Zones **104**, 105
Liver Meridian **330**, 331
Liver Zone 57, 67, **74**, 75
Locomotor system projection **10**, 11
Lower Jaw Zone 99, **100**, 102
Lower Leg Zone **38**, 39
Lower Limb 32–43
Lumbar Spine Zone 12, 13, **16**, 17, 25
Lumbosacral Transition Point **20**, 21
Lung Meridian **320**, 321
Lung Zone 57, **60**, 61, 63, 242, 243, **244**, 245, 320
Lymph Node Zones **80**, 81
Lymph Vessel Zones 56, **58**, 59
Lymphoid Ring 99

M

Magnesium Point 100
Masseter Muscle Zone 113
Master Omega Point **220**, 221, 231, 255, 289, 322, 332
Master Point of Oscillation **252**, 253
Master Point of Qi Flow **254**, 255
Master Point of Regulation **252**, 253
Master Point of the Upper Limb 320
Maxilla Zone 98, 99
Maxillary Sinuses Zone 99, 110, 111, 304, 305
Medulla Oblongata Zone 142, 143
Mercury Indicator Point 267
Meridians 318–323
Mesoderm **8**, 9
Metabolic Points 194–205
Metacarpal Bone Zone 50, 52
Metacarpopharyngeal Joint of Thumb Point **54**, 55
Middle Cervical Ganglion Point 173, **174**, 175, 177
Migraine Point 330
Mineral Points
 see Indicator Points for Trace Elements
Molar Point 100

N

Nervous Control Points of Endocrine Glands Zone 12, 13
 Nervous Duodenum Point 173, **178**, 179, 219
 Nervous Gall Bladder Point **178**, 179, 219, 269, 270
 Nervous Liver Point 173, **178**, 179, 218, 219, 269, 270, 271, 330
 Nervous Organ Points **178**, 179, **218**, 219
 Nervous Stomach Point **178**, 179, 219
 Neurohypophysis Zone **134**, 135
 Nicotine Analogue Point 215, **228**, 229, 247, 269, 270, 271, **302**, 303, 304, 305
 Nose Point 99, **108**, 109, 111, **304**, 305, 320

O

Occipital Bone Zone 98, 99
 Occipital Lobe Zone 121, **128**, 129
 Occipital Trigger Points **114**, 115
 Occiput Point 98
 Oculomotor Nerve Point 149, **152**, 153
 Olfactory Nerve Point 149, **150**, 151
 Omega Point I 188, 221, **222**, 223, 255, 266, 289, 332
 Omega Point II 221, **222**, 223, 255, 289, 332
 Optic Nerve Point 149, **150**, 151
 Oral Cavity Zone **104**, 105
 Organ Points **192**
 Ovary Point 83, **88**, 89, 322
 Oxytocin Point 134, **137**, 308, 309

P

Pain Memory Points **240**, 241
 Palatine Tonsil Point **80**, 81, 99, 100, 106, 107
 Palladium Indicator Point 267
 Pancreas Zone 57, 67, **76**, 77, **96**, 97, 322
 Endocrine Pancreas Point **76**, 77, 93, **96**, 97, **204**, 205, 269, 270
 Parenchymal Pancreas Point **76**, 77, 93, **96**, 97
 Paranasal Sinuses Zone **110**, 111
 Ethmoidal Sinuses 110, 111
 Frontal Sinus 99, 110, 111
 Maxillary Sinuses 99, 110, 111
 Sphenoidal Sinus 110

Parasympathetic Nervous System 164, 168–169
 Parasympathetic Nuclei of Origin Zone 165, **180**, 181
 Parathyroid Gland Point **94**, 95, 195, 271
 Endocrine Parathyroid Point 93, **94**, 95, **200**, 201, 328
 Parenchymal Parathyroid Point **94**, 95
 Paravertebral Chain of Sympathetic Ganglia Zone 12, **172**, 173
 Paravertebral Ganglions 180–189
 Paravertebral Muscles and Ligaments Zone 12, 13
 Parenchymal Organs Zone 13
 Parenchymal Pancreas Point **76**, 77, 93, **96**, 97
 Parenchymal Parathyroid Gland Point **94**, 95
 Parenchymal Thyroid Gland Point **92**, 93
 Parietal Bone Zone 98, 99
 Parietal Lobe Zone 121, **124**, 125
 Parotid Gland Point 100, **112**, 113
 Pelvis Zone **28**, 29, 45
 Penis Point 83, **90**, 91
 Pericardium Meridian **328**, 329
 Perineum Zone **30**, 31
 Peripheral Nervous System 190–191
 PGE1 Point **206**, 207, 209, **234**, 235, **242**, 243, 259, **260**, 261, 330
 PGE2 Point **208**, 209
 Pharyngeal Tonsil Point **80**, 81, 99, 106, 107
 Pharynx Zone 61, 63, **64**, 65, **106**, 107
 Phosphate Analogue Point 215, 229, 247, 269, 270, **300**, 301, 303, 304, 305
 Pineal Gland Point 215, 229, 242, 243, 247, **250**, 251, 269, 270, **302**, 303, 304, 305, 307, 309, 326
 Pituitary Gland Zone **134**, 135
 Pituitary Gland Points **136–137**, **308**, 309
 Platinum Indicator Point 267
 Point BL-62 242, 243, 247, **250**, 251
 Point C0/C1 **16**, 17, 23, 25, 27, 49, 295
 Point C7/T1 **18**, 19, 23, 25, 27, 49, 287, 297, 299, 312
 Point CN I **150**, 151
 Point CN II **150**, 151
 Point CN III **152**, 153
 Point CN IV **152**, 153
 Point CN V **154**, 155
 Point CN VI **154**, 155

Point CN VII **156**, 157
 Point CN VIII **156**, 157
 Point CN IX **158**, 159
 Point CN X **160**, 161
 Point CN XI **162**, 163
 Point CN XII **162**, 163
 Point GB-41 **242**, 243
 Point GV-4 **256**, 257
 Point KI-6 242, 243, **246**, 247
 Point L5/S1 **20**, 21, 23, 25, 27
 Point LU-7 242, 243, **244**, 245
 Point PC-6 242, 243, **246**, 247
 Point SI-3 242, 243, **248**, 249
 Point SP-4 242, 243, **248**, 249
 Point T12/L1 **18**, 19, 23, 25, 27, 49
 Point TB-5 242, 243, **244**, 245
 Point Zero 184, **256**, 257, 291, 293, 295,
 297, 299, 332
 Points Dominating Zone A **276**, 277
 Points Dominating Zone B **276**, 277
 Points Dominating Zone C **278**, 279
 Points Dominating Zone D **278**, 279
 Points Dominating Zone E **280**, 281
 Points Dominating Zone F **280**, 281
 Points Dominating Zone G **282**, 283
 Pons Zone 142, 143
 Postcentral Gyrus Zone 120, 121, 124, 125
 Posterior Masseter Muscle Point 100
 Postganglionic Sympathetic Nerves Zone
172, 173
 Precentral Gyrus Zone 120, 121, 122, 123
 Preganglionic Sympathetic Nerves Zone
170, 171
 Prevertebral Ganglia Zone 165
 Progesterone Point **196**, 197, 211, 271, 290,
 291
 Prolactin Point 134, **136**, 137, 197, 199,
200, 201, 271, 308, 309
 Prostaglandin E1 (PGE1) Point **206**, 207,
 209, **234**, 235, **242**, 243, 259, **260**, 261,
 330
 Prostaglandin E2 (PGE2) Point **208**, 209
 Prostate Point 83, **90**, 91
 Psychotherapy Point 221, **232**, 233, 269,
 270, 314, 315
 Psychotropic Points 220–233
 Pubic Bone Point **28**, 29

R

Radius Zone 45, 53
 Rectum Zone 71, **72**, 73
 Renin/Angiotensin Point **210**, 211, 271
 Respiratory System 60–65
 Reticular Formation Zone **144**, 145
 Retro-Celiac Plexus Point 242, 243, **248**,
 249
 Retro-Point Zero **248**, 249, 318, 324
 Retro-Solar Plexus Point 242, 243, **248**, 249
 Retromolar Cavity Point 100, 294
 Rib Zone 22, **24**, 25
 First Rib Point **296**, 297

S

Sacral Spine Zone **22**, 23
 Sacroiliac Joint Point 29, **30**, 31
 Sacroiliac Symphysis Zone 32
 Scapula Point 23, 45
 Sciatic Nerve Zone 32
 Sedation Point 322
Shen Men Point **238**, 239
 Shoulder Joint Point 44, 45, **46**, 47, 51, 287,
312, 313
 Silver Indicator Point 267
 Silver Points 193, **268–271**
 Small Intestine Meridian **324**, 325
 Small Intestine Zone 67, **70**, 71
 Small Vertebral Joints Zone 12, 13
 Solar Plexus Point 181, **184**, 185
 Sphenoid Bone Zone 98, 99
 Sphenoidal Sinus 110
 Spinal Cord Zone 13, 118, **146**, 147
 Spleen Meridian **322**, 323
 Spleen Zone 57, 67, **78**, 79, **292**, 293, 322
 Stannum Indicator Point 267
 Stellate Ganglion Point 175, **176**, 177, 179,
 219, 242, 243, **246**, 247, **296**, 297, 328
 Sternum Zone **24**, 25
 Stomach Meridian **322**, 323
 Stomach Zone 57, 67, **68**, 69, 322
 Sumatriptane Analogue Point 330
 Super Omega Point **254**, 255
 Superior Cervical Ganglion Point 173, **174**,
 175, 177
 Superior Mesenteric Plexus Point 181, **186**,
 187

Sympathetic Nervous System 164, 166–167
 Sympathetic Nuclei of Origin Zone 165,
170, 171

Sympathetic Trunk Zone 12, 13, 165, **172**,
 173, 218

T

Temporal Bone Zone 98, 99

Temporal Lobe Zone 121, **126**, 127

Temporomandibular (TMJ) Joint Point **100**,
 101, **294**, 295

Testis Point 83, **88**, 89

Thalamus Point 118, **130**, 131, **234**, 235,
 320

Thigh Zone **36**, 37

Thoracic Muscles/Abdominal Wall Zone 27

Thoracic Spine Zone 12, 13, **14**, 15, 25

Thorax 22–27

Throat Zone 61, 63, **64**, 65, 99, **106**, 107

Thumb Zone 45, 52, 53

Thymus Gland Point **78**, 79, 93, **94**, 95, 97,
 195, **202**, 203, 242, 243, **244**, 245, 269,
 270, **292**, 293, 328

Thyroid Gland Point **92**, 93, 195, **202**, 203,
 271

Endocrine Thyroid Gland **92**, 93, 95, **298**,
 299, 328

Parenchymal Thyroid Gland **92**, 93

Tissue Layer Control Points 264–265

TMJ Point **100**, 101, **294**, 295

Toe Points 38, 39, **42**, 43

Tongue Zone 99, **104**, 105

Tonsil Zone **80**, 81, **106**, 107, **294**, 295, 320

Palatine Tonsil Point **80**, 81, 99, 106, 107

Pharyngeal Tonsil Point **80**, 81, 99, 106,
 107

Tooth Points **102**

Trace Element Indicator Points **266–268**

Trachea Zone 61, **62**, 63

Trigeminal Nerve Point 149, **154**, 155, **294**,
 295

Triple Burner Meridian **328**, 329

Trochlear Nerve Point 149, **152**, 153

Trunk Muscles Zone **26**, 27

TSH Point **136**, 137, 197, **198**, 199, 201,
 271, 308, 309

U

Ulna Zone 45, 53

Upper Arm Zone **50**, 51

Upper Jaw Zone 98, 99, **102**, 103

Upper Limb 44–55

Ureter Point 83, **84**, 85

Urethra Point 83, **86**, 87

Urinary Bladder Point 57, 83, **84**, 85, 326

Urogenital System 82–91

Uterus Zone 83, **86**, 87, 322, 326

V

Vagina Point **88**, 89

Vagus Nerve Zone 149, **160**, 161

Valium Point 193, **214**, 215, **246**, 247, 269,
 326

Ventral Cervical Muscles Zone 27

Vertebrae Zone 13

Vertebral Column 12–22

Vestibulocochlear Nerve Point 149, **156**,
 157

Vitamin B5 Indicator Point 266, 267, 322

Vitamin C Point 259, **262**, 263, 316, 317

Vitamin Indicator Points **266**, 267

Vomiting Point **142**, 143

W

Worry Point 221, **226**, 227, 269, 270, 271,
 320

Wrist Point 44, 45, 46, 47, **50**, 51, 53, **310**,
 311, **312**, 313

Z

ZA Points **276**, 277

ZB Points **276**, 277

ZC Points **278**, 279

ZD Points **278**, 279

ZE Points **280**, 281

ZF Points **280**, 281

ZG Points **282**, 283

Zinc Indicator Point 267

Zone-Dominating Points 272–283

Zones of projection **6**, 7

Indications

A

achillodynia 357
 addictions 230, 232, 278, 405–411
 nicotine 408–411
 Adie's syndrome 152
 agnosia 124
 agraphia 126
 alexia 126
 allergies 136, 198, 204, 206, 258, 276, 280,
 319, 373–377
 asthma 258, 374–375
 eczema 376–377
 amenorrhea 136, 200
 angina pectoris 390
 ankle joint problems 356
 ankylosing spondylitis 248, 318, 347
 anorexia nervosa 407
 anosmia 150
 anxiety 138, 246, 402
 aphasia 126
 arrhythmia *see* Cardiac arrhythmia
 arterial hypertension 386
 arterial hypotension 387
 asthma 128, 206
 allergic 258, 374–375
 bronchial 212, 378
 ataxia 140
 autonomic symptoms 398–401

B

back pain *see* Disk prolapse; Intervertebral
 disk herniation; Lumbago
 balance disturbance 140
 Bechterew's disease 248, 318, 319, 347
 bed-wetting 128
 blockage
 first rib 176, 246, 341
 sacroiliac joint 346
 bronchial asthma 212, 378
 bronchitis 160, 378
 bronchopulmonary infection 378

bronchospasm 182, 212
 burning feet 359

C

calcaneal spur 358
 cardiac arrhythmia 388–389
 tachyarrhythmia 160, 180
 cardiovascular diseases 386–390
 carpal tunnel syndrome 190, 351
 catarrh 160
 cephalgia 174
 cerebral palsy 122
 cervical migraine 368
 cervical rib syndrome 176
 cervical syndrome 176
 lower 341
 middle 340
 upper 338–339
 cervicobrachial syndrome 341, 348
 childlessness *see* Fertility disorders
 chondropathy of the patella 355
 chronic polyarthritis 360
 cluster headache 369
 coccygodynia 348
 colic 160, 180, 188
 concentration, lack of 404
 congestion 244
 coxarthrosis 354
 craniocervical syndrome 338–339
 crocodile tears syndrome 156
 Crohn's disease 384

D

deafness 126, 156, 400–401
 depression 138, 224, 244, 280, 403
 diabetes mellitus 204
 diarrhea 248, 383
 disk prolapse 342–343
 dry eye 156
 duodenal ulcer 218

dysarthria 140
dysphagia 158
dystonia 162

E

eating disorders 132, 405–406
 anorexia nervosa 407
eczema, allergic 376–377
edema 276
enuresis 392
episiotomy problems 30

F

facial pain 174, 364
 see also Headache
facial paralysis 156
fertility disorders 132, 134, 136, 200, 396
fever 208, 248
fibromyalgia 361–362
first rib blockage 176, 246, 341

G

gall bladder disturbances 160
 gall bladder migraine 366–367
gastritis 160, 180, 218, 266, 322, 381–382
gastrointestinal disorders 381–385
globus hystericus 232
globus sensation 158
glossodynia 158
goiter 92
gonarthrosis 355
gouty toe 359

H

hay fever 206, 373–374
headache 176, 364–372
 cluster headache 369
 tension headache 365
 see also Migraine; Neuralgia
hemianopia 128
hemorrhoids 385
hepatitis 382
hepatopathy 382

herpes zoster 146, 280
hip joint osteoarthritis 354
hormonal disorders 394–397
hormonal migraine 368
hyperemesis gravidarum 176, 397
hypermenorrhea 134
hyperprolactinemia 136, 200
hypertension 132, 182, 210, 242, 386
hypotension 132, 387
hypothyroidism 136, 198, 202

I

impotence 393
insomnia 132, 144, 214, 246, 250, 399
intervertebral disk herniation 146, 172, 280
intestinal spasm 184
intracranial hemorrhage 122
irritable bladder 188, 214, 391–392
irritable bowel syndrome 186, 383

J

jet lag 144

K

kidney disorders 278
knee joint problems 355–356

L

lack of concentration 404
lactation 134, 136, 137, 200
lateral epicondylitis 350
laterality disorders 128
learning difficulties 128
left-handedness, suppressed 128, 236
locomotor system disorders 338–363
lower cervical syndrome 341
lumbago 342, 344–345

M

mastopathy 134
Ménière's disease 126, 156, 400–401

meniscopathy 355
menopausal symptoms 137, 194, 196, 224, 246, 395
menstrual disorders 134, 246, 394–395
mental disturbance 220, 222, 224, 278, 282, 402–404
meralgia paraesthetica 190
meteorism 186
micturition disorders 160, 188
middle cervical syndrome 340
migraine 114, 172, 174, 176, 226, 366
 cervical migraine 368
 gall bladder migraine 366–367
 hormonal migraine 368
morning sickness 142
motor aphasia 122
myogenic torticollis 162

N

neuralgia 280
 trigeminal 154, 176, 370
 zoster 176
neurodermatitis 278, 376–377
neuroma 190
nicotine addiction 408–411
nystagmus 140

O

obesity 405–406
obstipation 160, 180, 188, 384
oculomotor paralysis 152
olfactory anesthesia 150
oligomenorrhea 136, 200
oral diseases 280
oscillation 252
osteoarthritis
 ankle joint 356
 hip joint 354
overweight 132

P

pain 130, 180, 234, 236, 238, 240
 facial 174, 364
 fibromyalgia 361–362
 phantom limb pain 363

pancreatic disturbances 160
paralysis
 abducens 154
 facial 156
 oculomotor 152
 superior oblique muscle 152
 tongue 162
parotitis 380
peptic ulcer 381–382
periodontitis 276
peripheral vestibular neuronitis 156
perspiration, excessive 132, 160, 180
phantom limb pain 363
polyarthrosis of the hand 353
polyneuropathy 190
postconcussion syndrome 176
pregnancy 194, 196
premenstrual syndrome 250, 394–395

R

recurrent urinary tract infections 391–392
reflex sympathetic dystrophy 176
respiratory disorders 378–380
retrobulbar optic neuritis 150
rheumatoid diseases 206, 242, 244, 260
 rheumatoid arthritis 360
root compression 280

S

sacroiliac joint blockage 346
saliva deficiency 156
sciatica 342, 344–345
sexual problems 132
 impotence 393
shoulder problems 176, 348–349
singultus 184
sinusitis 379
Sjögren's syndrome 156
skin problems 278
sleep disorders 399
 see also Insomnia
smoking cessation 408–411
Sudeck's atrophy 176, 352
sulcus ulnaris syndrome 190
swallowing difficulties 158
sweating *see* Perspiration

T

tachyarrhythmia 160, 180
temporomandibular joint syndrome
 371–372
tenesmus 186
tennis elbow 350
tension headache 365
tinnitus 126, 156, 400–401
tongue paralysis 162
tonsillitis 379
torticollis 162, 248
trigeminal neuralgia 154, 176, 370

U

ulcer
 duodenal 218
 peptic 381–382
 ventricular 160, 180
ulcerative colitis 384

upper cervical syndrome 338–339
urinary tract infections 391–392
urogenital system disorders 391–393

V

ventricular ulcer 160, 180
vertigo 140, 156, 398
vestibular neuronitis 156

W

whiplash syndrome 176
wound healing, defective 276
wryneck 162

Z

zoster neuralgia 176